

AWCA RESOURCE CENTER

Complete Knowledge Base & Guidebook

공식 AWCA 리소스 센터 종합 가이드북 (전 12개 부문 완벽 수록)

A comprehensive, word-for-word archive of all official community resources, social welfare programs, healthcare guides, senior housing directories, Medicaid & Medicare enrollments, and legal planning guides published by the AWCA Resource Center.

12

RESOURCE SECTIONS

100

TOTAL ARTICLES

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PART I • SECTION 1

US Adult Welfare

미 성인 복지 (US Adult Welfare)

Federal adult welfare laws, the Older Americans Act (OAA), and administration policies supporting senior independence and community welfare.

This section contains 3 comprehensive guides

Older Americans Act (OAA)

The Older Americans Act (OAA), passed in 1965, is a federal law that provides home- and community-based services to help older adults (ages 60 and older) live independently. This law established the National Aging Network to coordinate services and programs at the federal (AoA), state (State Units on Aging), and local (AAA) levels.

KEY ASPECTS

Comprehensive Services for Independent Living

The OAA authorizes a wide array of services, including in-home support, meals, transportation, legal services, and other programs that promote independence and well-being for older adults.

Federal Funding Support for Aging Programs

The OAA is a major source of federal funding for programs aimed at improving the lives of older Americans, particularly those with the greatest economic and social need.

Long-Term Care Ombudsman Program Establishment

The OAA established the Long-Term Care Ombudsman Program, which provides advocates for residents of long-term care facilities to help resolve complaints and ensure their rights are protected.

Prevention of Elder Abuse, Neglect, and Exploitation

The OAA includes provisions for preventing and addressing elder abuse, neglect, and exploitation.

Reauthorization and Legislative Amendments History

Since the Older Americans Act (OAA) was first passed in 1965, the Supporting Older Americans Act of 2020 (P.L. 116-131) authorized appropriations for FY2020 through FY2024, and the official authorization period expired as of September 30, 2024. Discussions regarding subsequent reauthorization are currently underway.

OAA 7 Titles

Title	Name	Key Contents
Title I	Declaration of Objectives; Definitions	- States the goals and philosophy of the OAA - Defines key terms like "older adult" and "family caregiver" - No direct funding allocation
Title II	Administration on Aging	- Establishes the federal Administration on Aging (AoA) - Establish the basis for the creation of State Units on Aging and Area Agencies on Aging (AAA)
Title III	Grants for State and Community Programs on Aging	- Accounts for the largest share of OAA funding. - Covers meals, transportation, caregiving, in-home support - Supports family caregivers, health promotion, and vulnerable seniors
Title IV	Activities for Health, Independence, and Longevity	Alzheimer's disease research, falls prevention, chronic disease self-management programs, workforce training, and demonstration projects.
Title V	Community Service Employment for Older Americans (CSEOA) (commonly known as the Senior Community Service Employment Program, SCSEP)	- Adults aged 55 or older who are currently unemployed and have a household income at or below 125% of the Federal Poverty Guidelines. - Offers training and pathways to permanent jobs - Operated by the U.S. Department of Labor
Title VI	Grants for Services for Native Americans	- Supports American Indian, Alaska Native, and Native Hawaiian elders - Provides meals, in-home services, caregiving support

Title	Name	Key Contents
		Federal government grants directly to tribal organizations without passing through the State.
Title VII	Vulnerable Elder Rights Protection Activities	- Runs long-term care ombudsman programs - Prevents abuse, neglect, and exploitation - Includes legal assistance and rights advocacy

Resources

- [ACL.gov: OAA Overview \(Administration for Community Living\)](#).
- [Overview and funding \(Congressional Research Service\)](#).

Administration for Community Living (ACL)

The Administration for Community Living (ACL) is a federal agency under the U.S. Department of Health and Human Services (HHS). Its official mission is to maximize the independence, well-being, and health of older adults, people with disabilities across the lifespan, and their families and caregivers. To achieve this, it provides grants to states, communities, and non-profit organizations; invests in research, education, and innovation; coordinates federal programs; and engages in advocacy.

Key Aspects

Advancing Community Living

ACL supports people with disabilities and older adults so that they can live in the place and environment of their choice—rather than in institutions—and exercise their right to make their own decisions.

Supporting Independent Living

ACL funds and supports programs that help individuals with disabilities gain the skills and resources they need to live independently.

Promoting Health and Well-being

ACL provides funding for evidence-based health promotion programs, including falls prevention, chronic disease self-management, nutrition services, and brain health.

Supporting Family Caregivers

ACL operates programs such as the National Family Caregiver Support Program and the Lifespan Respite Care Program to support family caregivers.

Promoting Inclusion and Equal Opportunity

ACL is committed to ensuring that older adults and people with disabilities have access to the same opportunities as everyone else and are included in all aspects of community life.

Improving Access to Services

ACL ensures that everyone can easily connect to local services through resources such as the Eldercare Locator (eldercare.gov, 800-677-1116) for older adults and the Disability Information and Access Line (DIAL) (888-677-1199) for people with disabilities.

Enhancing Coordination

ACL strives to reduce fragmentation between federal programs and collaborates with more than 20 federal departments and agencies, as well as states, tribal and local governments, and the private sector to ensure individuals can seamlessly receive the support they need.

Resources

[ACL.gov: ACL Programs and Services Overview](#)

Administration on Aging (AOA)

The Administration on Aging (AoA) is a major agency within the U.S. Department of Health and Human Services (HHS) and a program office under the Administration for Community Living (ACL), serving as the lead federal agency responsible for implementing the provisions of the Older Americans Act (OAA).

Mission

The mission of the AoA is to develop and support a comprehensive, coordinated, and cost-effective system of home- and community-based services that helps older adults live healthy and independent lives in their homes and communities.

Aging Services Network

The AoA collaborates with the National Aging Network—including Area Agencies on Aging (AAAs), State Units on Aging (SUAs), and tribal organizations—to ensure that older adults receive the services necessary to live independently and healthily in their homes and communities. Furthermore, through the aging services network, the AoA supports home- and community-based services, advocates for the interests of older adults and their caregivers, and operates evidence-based health promotion programs such as nutrition, falls prevention, and chronic disease self-management.

Older Americans Act

The AoA was established by the OAA of 1965 and is responsible for implementing the provisions of the Act, including the administration of funding and grant programs. However, Title V (the Community Service Employment for Older Americans, CSEOA, commonly known as the Senior Community Service Employment Program, SCSEP) is separately administered by the Department of Labor (DOL).

Services

The Aging Services Network provides a wide range of services, including homemaker assistance, transportation, adult day care, caregiver support, and home-delivered meals.

State Level

Each state has a SUA, with a total of 56 units operating across the 50 states, the District of Columbia, and the territories. The SUA develops and manages multi-year state plans for older adults and their families, is responsible for the development and administration of OAA-related policies, and coordinates service delivery through AAAs.

Local Level

AAAs are public or private non-profit agencies designated by the state to address the needs and interests of all older adults within a city, a single county, or a multi-county area. They prioritize support for older adults with the greatest economic and social needs. These agencies help seniors remain in their homes through services such as home-delivered meals (commonly known as Meals on Wheels) and homemaker assistance.

Resources

[ACL.gov: AOA Overview](#)

PART I • SECTION 2

Financial Assistance

재정 지원 (Financial Assistance)

Public financial assistance programs including Social Security, SSI, SSDI, SNAP food assistance, energy bill assistance (LIHEAP, Comfort Partners, NJ SHARES, Lifeline), and Senior Farmers Market.

This section contains 9 comprehensive guides

Social Security

Social Security is a federal program that provides financial assistance to qualified individuals, including retirees, people with disabilities, and the survivors of deceased workers. The official name of this program is Old-Age, Survivors, and Disability Insurance (Old-Age, Survivors, and Disability Insurance, OASDI), and it is administered and operated by the Social Security Administration (SSA).

Social Security is funded through payroll taxes from both employees and employers, and the size of future benefits is determined based on an individual's earnings history. This program is designed to provide financial stability and support to individuals who can no longer engage in economic activity due to old age or disability. Types of benefits include Retirement Benefits, Social Security Disability Insurance (SSDI), and Survivor Benefits.

KEY ASPECTS

- **Retirement Benefits:** Provides a set monthly income to retired workers and their families.
- **Disability Benefits:** Provides financial support to workers who become unable to work due to a disability.
- **Survivor Benefits:** Supports the families of deceased workers by providing income for their livelihood.
- **Funded by Taxes:** Social Security funds are raised through payroll taxes from employees and employers.
- **Administered by SSA:** The SSA manages the entire program and oversees the full process of benefit payments.

Eligibility

- **Requirements:** You earn credits by working and paying Social Security taxes. To receive retirement benefits, you generally need 40 credits (approximately 10 years of work).
- **Employment Gaps:** Your credits never expire even if you stop working. However, because benefits are calculated using your 35 highest-earning years, significant gaps in your work history may affect your benefit amount.

Benefit Amounts

- **Average Indexed Monthly Earnings (AIME):** Calculated based on the 35 years of your highest earnings during your working life.

- **Primary Insurance Amount (PIA):** Calculated based on AIME using the following formula (for those reaching age 62 in 2026):
 - 90% of the first \$1,286 of AIME
 - 32% of AIME between \$1,286 and \$7,749
 - 15% of AIME over \$7,749

Timing and Benefit Amounts

- **Full Retirement Age (FRA):** The age at which you can receive full benefits. This varies by birth year; for those born in 1960 or later, it is 67.
- **Early Retirement (Age 62):** You can begin collecting benefits as early as age 62, but your monthly benefits will be permanently reduced. For instance, claiming early retirement at 62 instead of the FRA of 67 may result in a permanent reduction of your monthly benefits by up to 30%.
- **Delayed Retirement (Age 70):** If you wait past your FRA up to age 70, your benefit increases by approximately 8% for each year you delay. These increases stop once you reach age 70.

Social Security and Medicare

If you have been receiving Social Security benefits for at least four months before your 65th birthday, you will be automatically enrolled in Medicare Parts A and B (no separate application is required).

Retirement Planning Checklist

What to Know Before You Apply

- **Definition of "Retirement":** The SSA defines retirement as the point when you begin collecting your Social Security retirement benefits, not when you stop working entirely.
- **Work and Benefit Impact:** If you have income above a certain limit before reaching FRA, your benefits may be temporarily reduced. Once you reach FRA, you receive full benefits regardless of your income.
 - As of 2026: if you are under FRA, **\$1** is withheld for every **\$2** earned above the annual limit of **\$22,320**.
- **Taxation:** If you have other income, a portion of your Social Security benefits may be subject to federal income tax.

Special Benefit Situations

- **Spousal and Survivor Benefits:** You may receive benefits based on a spouse's work record. A spouse can receive up to **50% of the worker's PIA** (at their own FRA), provided the worker has already applied

for benefits. You will receive the higher amount between your own benefit and the spousal benefit.

- **Divorced Spouses:** You can receive benefits based on a former spouse's record if:
 - The marriage lasted at least 10 years;
 - You are currently unmarried;
 - You are age 62 or older; and
 - The former spouse is eligible for Social Security (if divorced for at least 2 years, they do not need to have applied yet for you to claim).

Additional Considerations

- **Application Timing:** You can apply up to **4 months** before the month you wish to begin receiving benefits.

Check My Benefits

- **My Eligibility:** [SSA.gov: Check Eligibility for Benefits](https://www.ssa.gov/checkeligibility/)
- **My Social Security Account:** [SSA.gov/myaccount](https://www.ssa.gov/myaccount)

How to Apply

- **Online:** [SSA.gov/apply](https://www.ssa.gov/apply)
- **Phone:** 1-800-772-1213 (TTY: 1-800-325-0778)
- **In-Person:** Visit your local SSA office ([SSA.gov/locator](https://www.ssa.gov/locator))

Contact

- AWCA provides consultations free of charge (201-862-1665 / info@awcanj.org).

Resources

- [SSA.gov: Official Website](https://www.ssa.gov)
- [SSA.gov: 2026 COLA/Social Security Fact Sheet \(PDF\)](#)
- [SSA.gov: Retirement Benefits Publication Guide](#)
- [SSA.gov: Retirement Preparation Checklist](#)
- [SSA.gov: Retirement Information for Medicare Beneficiaries](#)

Comfort Partners Program (New Jersey)

The Comfort Partners Program is a free energy-saving and education program in New Jersey that helps income-eligible residents reduce their utility bills while improving their home's safety and comfort. It is run by the New Jersey Board of Public Utilities (NJBPU) in partnership with electric and natural gas utilities.

Benefits and Services

- **Free Energy Efficiency Improvements and Education Program:** Specialists visit your home to provide personalized energy education and install energy-saving equipment—such as insulation, heating and cooling systems, and lighting—at no cost.
- **Energy Efficiency Improvements:** The program provides various energy-saving measures at no cost, including:
 - Adding insulation to the home
 - Sealing air leaks to prevent drafts
 - Installing energy-efficient lighting solutions
 - Inspecting and tuning heating and cooling systems
 - Installing water-saving devices
 - Replacing old refrigerators
 - Installing programmable thermostats
 - Installation of water heater insulation and water-saving showerheads/adapters
 - Potential waiver of utility arrears (subject to agreement on a payment plan)

Eligibility (2026)

Households with an income at or below 250% of the Federal Poverty Level and significant energy usage are eligible for the program. Additionally, beneficiaries of the following programs also qualify for services:

- Home Energy Assistance Program (HEAP / LIHEAP)
- Supplemental Security Income (SSI)
- Lifeline Credit Program
- Temporary Assistance for Needy Families (TANF)
- General Assistance (GA)
- Pharmaceutical Assistance to the Aged and Disabled (PAAD)
- Universal Service Fund (USF)

- Medicaid
- Section 8 Housing Assistance
- Supplemental Nutrition Assistance Program (SNAP)
- Ratepayer Status: You must be the ratepayer of record for gas and electricity at your primary residence.
- New Construction Exclusion: Homes built less than 5 years ago or those still under a builder's warranty are ineligible.
- Building Size: You must reside in a building with 14 units or fewer (e.g., apartments, townhouses; individual metering is required). Buildings with 15 or more units are excluded.
- For Homeowners: You must have owned the property for at least one year.

How to Apply

- **Online:** [Cleanenergy.nj.gov: Comfort Partners](https://www.cleaneconomy.nj.gov/ComfortPartners)
 - PSE&G Customers: [NJ PSEG Comfort Partners-Apply](#)
- **Phone:** 866-378-4345

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Senior Farmers' Market Nutrition Program (SFMNP)

The Senior Farmers' Market Nutrition Program (SFMNP), often referred to as the "Farmers' Market Program," is a federal and state-funded initiative that helps low-income seniors purchase fresh, locally grown fruits, vegetables, and honey through farmers' markets or community-supported agriculture programs. This program aims to improve the nutritional status of the elderly while simultaneously stimulating the local agricultural economy.

Features of SFMNP

- **Agricultural Purchase Support:** Provides vouchers or benefit cards so that seniors can purchase fresh produce.
- **Encouraging Healthy Eating Habits:** Encourages the consumption of fresh foods and promotes nutrition education.
- **Widespread Operation:** Implemented in most states.

Benefits

- **Benefit Amount:** \$50 per person annually (based on Bergen County, NJ).
- **Payment Method:** Provided in the form of an electronic QR code or a dedicated debit card.
- **Eligible Items:** Fresh fruits, vegetables, and honey produced by local farmers.
- **Note:** Processed foods (jams, canned goods), imported produce, and dairy products cannot be purchased.

Eligibility

- **Target:** Individuals aged 60 or older.
- **Income Criteria:** Household income at or below 185% of the Federal Poverty Level.
 - 2026-27 NJ: Individual \$29,526 per year / 2 People \$40,034 per year
- **Residency:** Proof of address documentation required.
- **Regional Differences:** Operation periods and availability may vary by state and local agencies.

How to Apply

- **Application:** Contact or visit your local ADRC (Aging and Disability Resource Connection).

- **Counseling Number:** 1-877-222-3737 (Automatically connects to your county's Office on Aging).
- **Bergen County Division of Senior Services**
 - **Address:** One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - **Phone:** 201-336-7400 | 1-877-222-3737
 - **Email:** seniors@bergencountynj.gov
- **Receiving Benefits:** Upon approval, receive a document with a QR code or a card via mail or in person.
- **Using the Market:** Use at designated farmers' markets displaying the "Certified Farmers Vendor" sign.
- **Balance Check:** [Homegrown Benefits Portal](#)

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [FNS.usda.gov: SFMNP Overview](https://fns.usda.gov/sfmnp/overview)
- [FNS.usda.gov: SFMNP Factsheet](https://fns.usda.gov/sfmnp/factsheet)
- [USDA.gov – 2026-27 SFMNP Income Eligibility Guidelines](https://usda.gov/2026-27-sfmnp-income-eligibility-guidelines)
- [bergenfightshunger.org – Bergen County SFMNP Guide](https://bergenfightshunger.org/bergen-county-sfmnp-guide)
- [NJ.gov/health: Farmers' Market Locator](https://nj.gov/health/farmers-market-locator)
- [NJ.gov: DoAS – New Jersey County ADRC List](https://nj.gov/doh/adrc/new-jersey-county-adrc-list)

Lifeline Utility Assistance Program (New Jersey)

The Lifeline Utility Assistance Program is a program operated by the Division of Aging Services under the New Jersey Department of Human Services (NJ DHS), providing an annual utility credit to low-income seniors and individuals with disabilities. It helps eligible residents pay for electric and natural gas costs by offering financial assistance directly applied to their utility bills.

Primary Benefits

- **Lifeline Credit Program:** A \$225 annual credit is applied directly to electric and/or gas bills for utility account holders.
- **Tenants Lifeline Assistance Program:** A \$225 annual check is issued to tenants whose utilities are included in their rent.
- **Special Utility Supplement (SSI recipients):** Lifetime benefits of up to \$18.75 per month are automatically added to your SSI check. SSI recipients are automatically enrolled and do not need to submit a separate application for Lifeline.

Eligibility

- Those who are PAAD beneficiaries or meet PAAD income limits (less than \$54,943 annually for individuals, less than \$62,390 for couples, 2026). This applies to New Jersey residents aged 65 or older, or those aged 18–64 receiving SSDI.
- Beneficiaries of Medicaid programs for seniors.

Application and Coverage

- **Automatic Enrollment:** Medicaid beneficiaries will automatically receive a Lifeline application every August.
- **Household Limit:** Only one application per household is allowed per year. The application must be completed by the person whose name is on the utility bill or the lease agreement.
- **Utility Scope:** Lifeline covers only electricity and natural gas costs.
- **Exclusions:** Lifeline does not cover heating oil, propane, wood, or coal.
- **Primary Residence:** Lifeline benefits apply only to your primary residence in New Jersey. Seasonal or temporary residences are excluded.

How to Apply

- Online: [NJSave Website](#)
- Phone: **Lifeline Utility Assistance** 1-800-792-9745

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov: DoAS – Lifeline Utility Assistance Program Information](#)
- [NJ.gov: DoAS – NJSave Guide](#)

Low-Income Home Energy Assistance Program (LIHEAP)

The Low-Income Home Energy Assistance Program (LIHEAP) is a federally funded program that helps low-income households pay for heating and cooling costs. This program provides assistance with heating and cooling costs, emergency energy crisis support, and medically necessary cooling assistance.

In New Jersey, LIHEAP is administered by the Department of Community Affairs, and residents can access these federal benefits through local partners like Greater Bergen Community Action (GBCA).

Eligibility (for Bergen County, NJ)

- Reside in Bergen County
- Income Requirement: Household gross income must be at or below 60% of the New Jersey State Median Income (SMI). (e.g., \$8,014 or less per month for a family of 4, 2026). Please visit the [NJ DCA website](#) for details.
- Utility Status: Households that pay for their own heating or cooling costs directly, or pay for them as part of their rent. (Note: Residents of public housing or those receiving rental assistance who do not pay for their own heating/cooling are generally excluded).
- Meet other specific requirements depending on the assistance program

How to Apply

Residents in Bergen County should apply through GBGA. Applications are available for download on GBGA website.

- **Offline:** Download the form from the [GBGA website](#) and submit it via email, mail, or in person.
- **Online:** [DCAid](#)
- **Email:** LIHEAP@GreaterBergen.org
- **Mail:** Greater Bergen LIHEAP
 - 505 Main Street, 3rd Floor, Suite 300 Hackensack, NJ 07601
- **In-Person(Drop Box):** 392 Main Street Hackensack, NJ 07601
- **Contact:** (201) 488-5100

Required Documentation

- Completed Application Form (Available on the [GBGA website](#))

- Applicant's Photo ID
- Proof of Income for all household members aged 18 and older: Recent pay stubs or pension statements (statements showing gross amount or Form 1099).
- Unemployment Benefit Recipients: A200 printout (issued at 60 State Street, Hackensack, NJ).
- Full-time Students (Aged 18 or older): Class schedule (showing credits) and/or Enrollment Verification Letter.
- Financial Support Verification Letter: For those receiving financial assistance from family or friends (if applicable).
- Recent Income Tax Return or a Statement of Non-Filing.
- Proof of Government Benefits (e.g., SSA, SSI, SSD).
- Social Security Cards for all household members.
- Proof of Legal Residency: Copy of Permanent Resident Card (Green Card). If you do not have a Resident Card, submit a copy of your passport along with a Statement of Non-Possession of a Social Security Number.
- Most Recent Utility Bill
- Proof of Homeownership or Lease/Rental Agreement.
- Cooling Assistance Applicants: Must submit an original Doctor's Certification Form.
- Section 8 Recipients: Must submit an additional Housing Letter.
- Homeowners with Rental Income: Must submit an additional Rental Income Form.

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov: DCA Guide on LIHEAP](#)
- [Greaterbergen.org: GBCA – LIHEAP Page](#)

New Jersey Statewide Heating Assistance and Referral for Energy Services (NJ SHARES)

New Jersey Statewide Heating Assistance and Referral for Energy Services (NJ SHARES) is a non-profit organization that provides financial assistance to New Jersey residents who are not currently eligible for other welfare programs and have a history of good-faith payments on their utility bills. Income eligibility varies by program, but NJ SHARES energy grants are available to households with incomes at or below 100% of the State Median Income (SMI).

Primary Assistance Programs

- **Energy Assistance:** Provides financial support for the payment of natural gas and electricity bills.
- **Water Assistance:** Provides dedicated funds to help pay for water utility bills.
- **Housing Assistance:** Provides support for critical housing-related expenses, including rent, mortgage payments, and property taxes.

**For the NJ SHARES Energy Assistance Grant specifically, households already receiving LIHEAP or USF benefits are not eligible to apply.*

NJ SHARES Assistance Programs

Program Name	Category	Assistance Amount	Key Notes
ACE Customer Relief Fund	Energy	Up to \$500 for electric service	For Atlantic City Electric customers with an overdue balance of \$250 or more (income eligibility applies).
NJBPU PAGE Program	Energy	Up to \$500–\$700 per utility (tiered by income)	Households at or below 60% SMI can apply for PAGE if denied LIHEAP/USF. Households between 60%–100% SMI can apply for PAGE directly.
NJ SHARES SMART Utility Assistance Program	Energy	Up to \$500 per utility	Support for households needing additional aid beyond federal/state assistance programs.
NJ SHARES Energy Assistance Grant	Energy	Up to \$700 for gas, \$700 for electric	Supports middle-income and fixed-income households facing financial crises.
NJ SHARES Municipal Customer Assistance Program (MCAP)	Electric, Sewer, Water	Up to \$200 per service	Assistance for households struggling to pay municipal utility bills .
AQUA New Jersey AQUA Aid	Water	Up to \$150 (single) / \$250 (combined)	For AQUA NJ customers facing a water or sewer bill crisis.
NJ American Water H2O Help to Others Program	Water	Up to \$500 grant and 15%–60% monthly bill discount (fixed rate + usage)	For NJ American Water customers who need help paying their water bills.
NJ SHARES Mortgage Assistance, Rent and Tax Program	Housing	Based on income/situation (Up to \$5,000 through partnerships). Income limit: 200% FPL or below.	Resolves housing instability caused by unpaid rent, mortgage, or property taxes.

How to Apply

- Online: [Sharesnation.org: Register](https://sharesnation.org/register)

Contact

- NJ SHARES | sharesnation.org
- Phone: 609-883-1626 / 866-657-4273
- Email: info@sharesnation.org
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Sharesnation.org](https://sharesnation.org): NJ SHARES Website
- [Sharesnation.org](https://sharesnation.org): Eligibility
- [NJ.gov/bpu](https://nj.gov/bpu): BPU Utility Assistance Programs

Supplemental Nutrition Assistance Program (SNAP aka Food Stamp)

The Supplemental Nutrition Assistance Program (SNAP) is a federal program in the United States that helps low-income individuals and families afford nutritious food. It was formerly known as the Food Stamp Program. SNAP benefits are distributed through an Electronic Benefits Transfer (EBT) card, which can be used at grocery stores, farmers' markets, and other authorized retailers to purchase food items.

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Eligibility

SNAP eligibility is primarily determined by household size, income levels, and financial resources. These guidelines are effective from October 1, 2025, through September 30, 2026.

Income Limits

Most households must meet both gross and net income tests to qualify for SNAP benefits. The Federal Poverty Level (FPL) is used as the baseline for these calculations.

- **Gross Monthly Income Limit (New Jersey):** Due to the application of Broad-Based Categorical Eligibility (BBCE), New Jersey extends the gross income limit to 185% of the Federal Poverty Level (FPL) for the period from October 1, 2025, to September 30, 2026. This is significantly higher than the standard federal limit of 130%.
- **Net Monthly Income Limit (100% FPL):** This is the income remaining after specific allowable deductions are subtracted. These deductions include the standard deduction, shelter costs, medical expenses for elderly/disabled members, and dependent care costs.
- **Special Rule for Elderly or Disabled Households:** Households with members aged 60 or older or individuals with disabilities are exempt from the gross income test. They only need to meet the net income limit (100% of the FPL).

Household Asset Limits

- **Target Asset:** Cash, bank account balances, stocks, bonds, and other liquid assets that can be immediately converted into cash (Countable Liquid Assets). **Primary residence, first vehicle, retirement accounts, etc., are excluded.*
- **New Jersey Standard:** Principle exemption from asset evaluation (BBCE policy applied) New Jersey completely waives the asset test for households with incomes at or below 200% of the Federal Poverty

Level (FPL). In other words, as long as the income criteria are met, the household can receive SNAP benefits regardless of the balance in their bank accounts.

- **Federal Standard** (For states where BBCE is not applied)
 - General Households: \$3,250 or less
 - Households including an elderly (60 or older) or disabled member: \$4,500 or less

Standard Deductions

Standard deductions are applied to a household's gross income to help determine the final benefit amount. For the 48 contiguous states and New Jersey:

- **Household size of 1–3 persons:** A standard deduction of \$209 is subtracted from gross income (4 persons: \$223, 5 persons: \$261, 6 or more persons: \$299).

*** Additional Allowable Deductions (Beyond Standard Deduction):*

- Earned Income Deduction: 20% of earned income.
- Dependent Care Deduction: Costs for caring for dependents (for work, education, or training purposes).
- Medical Expense Deduction for Elderly/Disabled: Out-of-pocket medical expenses exceeding \$35.
- Excess Shelter Deduction: Monthly shelter costs that exceed 50% of the household's income after other deductions. The maximum deduction for most households is \$744 (FY2026). However, for households with members aged 60+ or individuals with disabilities, there is no limit—the full excess amount can be deducted.
- Homeless Household Standard Shelter Deduction: \$198.99 per month (FY2026).

How to Apply

- **Online**
 - Eligibility Check: [NJHelps.gov](https://www.njhelps.gov) (5–10 minutes)
 - Application: [MyNJHelps.gov](https://www.myNJhelps.gov) (20–45 minutes)
 - You will be notified of your eligibility within 30 days of applying. During this period, an interview (via telephone or in-person) will be conducted.
- **In-Person**
 - Bergen County Board of Social Services
Address: 218 Route 17 North, Rochelle Park, NJ 07662
Phone: 201-368-4200
Hours: Weekdays 8:00 am thru 4:30 pm

- NYC Human Resources Administration (HRA) – Queens SNAP Center
Phone: (718) 784-6315
Address: 32-20 Northern Blvd., 2nd Fl., LIC, NY 11101
Hours: Monday – Friday 8:30am to 5:00pm

***Hours are subject to change. Check with each agency prior to visiting.*

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [FNS.usda.gov](https://www.fns.usda.gov): Official SNAP Eligibility Information
- [FNS.usda.gov:2026](https://www.fns.usda.gov/2026) Benefit and Deduction Table
- [BCBSS.com](https://www.bcbss.com): Official SNAP Information

Social Security Disability Insurance (SSDI)

Social Security Disability Insurance (SSDI) is a federal disability insurance benefit for individuals who have a qualifying work history of paying Social Security taxes (FICA) but are no longer able to work due to a disability. Administered by the Social Security Administration (SSA), SSDI requires a formal application through the SSA's Disability (SSD) determination process.

Key Features

- **Based on Work history:** Eligibility is determined by your past Social Security tax contributions. Assets and savings are **not** factored into eligibility.
- **Individually Calculated Benefits:** Benefits are calculated based on your Average Indexed Monthly Earnings (AIME). The more you've earned and the longer you've worked, the higher your benefit.
- **Integrated Medicare Coverage:** Medicare eligibility is automatically granted 24 months after your SSDI entitlement begins (the 6th month from the established disability onset date). In practice, Medicare coverage begins approximately 29 months after the onset of the disability.
- **Family Benefits:** Spouses, minor children, and other family members may also receive additional benefits based on the beneficiary's work record.

Eligibility

To qualify for SSDI you must satisfy both the SSA's strict medical definition of disability and the work credit requirements.

SSA's Definition of Disability

The SSA's disability standard is very strict. Short-term or partial disabilities do not qualify. All four of the following conditions must be met:

- **Unable to perform SGA (Substantial Gainful Activity):** Due to a medical condition, you must be unable to perform work activity that generates earnings of \$1,690 or more per month (\$2,830 for individuals who are blind). If your earnings meet or exceed this threshold, your application may be denied before your medical condition is even reviewed.
- **Lasting 12 months or more, or expected to result in death:** The disability must have lasted — or be expected to last — at least 12 consecutive months, or the condition is expected to result in death.
- **Unable to perform past work or any other work:** You must not only be unable to perform your previous work, but also be unable to transition to any other type of work, taking into account your age, education, and work experience.

- **Objective medical evidence required:** Objective medical evidence — such as diagnoses, test results, and treatment records from medical sources recognized by the SSA — must support your claim.

Work Credits

To receive SSDI, you must have work income on which Social Security taxes were paid. As of 2026, you earn **1 credit for every \$1,890** in earnings, up to a maximum of **4 credits per year**. The number of credits required depends on your age at the time your disability begins.

Age at Onset	Min. Credits	Duration of Work Test	Recent Work Test	Notes
Under 24	6 credits (1.5 yrs)	1.5 years	Within 3 yrs before onset	Shorter work periods accepted
24–31	Half the time worked since age 21	Half since age 21	Work record since age 21	E.g., disability at 27 → 12 credits (3 yrs)
31–42	20 credits (within last 10 yrs)	5 years	Within 10 yrs before onset	"At least 5 yrs worked recently"
44	22 credits	5.5 years	Within 10 yrs before onset	—
46	24 credits	6 years	Within 10 yrs before onset	—
48	26 credits	6.5 years	Within 10 yrs before onset	—
50	28 credits	7 years	Within 10 yrs before onset	—
52	30 credits	7.5 years	Within 10 yrs before onset	—
54	32 credits	8 years	Within 10 yrs before onset	—
56	34 credits	8.5 years	Within 10 yrs before onset	—
58	36 credits	9 years	Within 10 yrs before onset	—
60	38 credits	9.5 years	Within 10 yrs before onset	—
62 or older	40 credits (max)	10 years (max)	Within 10 yrs before onset	Same as retirement eligibility

** Special Rule for Blindness: Individuals who meet the definition of Statutory Blindness are only required to satisfy the Duration of Work Test without needing to meet the Recent Work Test.*

Supplemental Security Income (SSI)

SSI is a supplemental living assistance program operated separately from SSD, and it is based on financial need rather than work history. However, since the SSA reviews eligibility for both SSDI and SSI simultaneously when you apply for SSD, you may receive SSI benefits through the SSD application process in the following cases:

- **If you are receiving SSDI but the amount is low enough to meet the SSI income criteria (2026 FBR \$994/month) and your assets are below \$2,000 (individual) / \$3,000 (couple):** You can receive both benefits at the same time (Concurrent Benefits). The actual SSI payment amount will be reduced based on your SSDI benefit amount.
- **If you lack enough work credits to qualify for SSDI, but your disability is recognized and you meet the income and asset criteria:** You may be eligible for SSI alone.
- **If you are 65 or older and meet the income and asset criteria, regardless of whether you satisfy the SSDI work history requirements:** You may be eligible for SSI.
- **If you are a child under the age of 18 with a recognized disability and meet the household income and asset criteria:** You may be eligible for SSI alone, and no parental work history is required.

**For SSI eligibility criteria (income calculation methods, excluded asset items, etc.) and detailed information, please refer to the [SSI page](#).*

Check My Benefits

- **My Eligibility:** [SSA.gov – Check Disability Benefits Eligibility](https://www.ssa.gov/apply-for-disability-benefits/)
- **My Social Security Account:** [SSA.gov/myaccount](https://www.ssa.gov/myaccount)

How to Apply

- **Apply for Social Security benefits**
 - **Online:** [SSA.gov/apply](https://www.ssa.gov/apply)
 - **Phone:** 1-800-772-1213 (TTY: 1-800-325-0778)
 - **In-Person:** Visit your local SSA office ([SSA.gov/locator](https://www.ssa.gov/locator))
- **AWCA** provides counseling free of charge (201-862-1665(201-862-1665 / info@awcanj.org).

Contact

- **AWCA** provides counseling free of charge (201-862-1665(201-862-1665 / info@awcanj.org).

Resources

- [SSA.gov: Disability Benefits \(PDF\)](#)
- [SSA.gov: 2026 Red Book \(Guide for People with Disabilities\)](#)

Supplemental Security Income (SSI)

Supplemental Security Income (SSI) is a federal program in the United States that provides monthly payments to individuals who have limited income and resources and are either aged (65 or older), blind, or disabled. Unlike Social Security benefits, SSI is not based on prior work history but rather on financial need.

SSI is a federal program that provides monthly cash assistance to individuals with limited income and resources who are blind, aged 65 or older, or disabled. It's a means-tested program, meaning eligibility is based on financial need and specific criteria related to disability, blindness, or age.

Eligibility

Qualifying Status

- **Seniors:** Aged 65 or older.
- **Health:** Blind or disabled (limiting/preventing work).
- **Citizenship:** U.S. citizens or qualified non-citizens (e.g., Green Card holders).

Income & Resource Limits

- **Income:** Little to no income. (Excludes: First \$20, some wages, SNAP, and state aid).
- **Resources:** Individual (\$2,000) or Couple (\$3,000).
- **Exemptions:** Primary home, one vehicle, essential household goods, life insurance with a total face value of \$1,500 or less, burial plots, and a burial fund of \$1,500 or less, etc.

How Much Can You Receive? (2026)

- **Maximum Monthly Payment (Individual):** Approximately \$994
- **Maximum Monthly Payment (Couple):** Approximately \$1,491

** The actual amount depends on your income, resources, and whether your state provides a supplement (e.g., California, New York, etc.).*

Important Responsibilities After Receiving SSI

- You must report any changes in income or resources to the SSA.
- You must report changes in address or family status (marriage, death, etc.).
- Failure to report may lead to overpayment, which you will be required to pay back later.

Other Benefits When Receiving SSI

Medical Benefits

In most states, you are automatically eligible for Medicaid (coverage for hospital visits and prescriptions).

Food Assistance

Receiving SSI often makes the application process for SNAP (Food Stamps) easier. Recipients may also receive priority for housing assistance, transportation subsidies, and other community welfare programs.

Difference between SSI and Social Security

Supplemental Security Income (SSI) is different from Social Security.

- You can receive both at the same time, but receiving Social Security may reduce your SSI payment amount.
- If your Social Security payment is low, SSI can supplement it.

Difference between SSI and SSDI (Social Security Disability Insurance)

SSDI (Social Security Disability Insurance) is based on a work history, while SSI is based on need, regardless of work history.

SSI is a vital program for many individuals who face significant financial hardship due to age, disability, or blindness.

Check My Benefits

- **My Eligibility:** [SSA.gov: Who can get SSI](https://www.ssa.gov/who-can-get-ssi)
- **My Social Security Account:** [SSA.gov/myaccount](https://www.ssa.gov/myaccount)

How to Apply

- **Online:** [SSA.gov/ssi](https://www.ssa.gov/ssi)
- **Phone:** 1-800-772-1213 (Appointment only)
- **In-Person:** Visit your local SSA office ([SSA.gov/locator](https://www.ssa.gov/locator))

Required Documents for Application

- Social Security Number (SSN)
- Original birth certificate or proof of citizenship
- Bank account information

- Proof of income (W-2s, tax returns, etc.)
- Resource documentation (bank statements, vehicle info, real estate, etc.)

Contact

- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [SSA.gov: About SSI](#)
- [SSA.gov: SSI Benefit Amount](#)
- [SSA.gov: SSI vs Social Security](#)
- [SSA.gov: SSI Eligibility](#)
- [SSA.gov: SSI Income Calculation](#)
- [SSA.gov: SSI Resource Limits](#)
- [Medicaid.gov: 2026 SSI Guide \(PDF\)](#)

PART I • SECTION 3

Medicare

메디케어 (Medicare)

Comprehensive federal health insurance: Parts A & B (Original Medicare), Part C (Medicare Advantage), Part D (Prescription Drug Plans), Medigap supplemental coverage, Medicare Savings Programs (MSP), Buy-in, Low Income Subsidy (LIS / Extra Help), and NJ PAAD / Senior Gold.

This section contains 11 comprehensive guides

Medicare Overview

Medicare is the federal health insurance program for individuals aged 65 or older, as well as those under 65 who have been receiving SSDI benefits for 24 months, or are diagnosed with End-Stage Renal Disease (ESRD) or ALS (Lou Gehrig's disease).

The Four Parts of Medicare

- **Part A:** Covers inpatient hospital stays, care in a skilled nursing facility, hospice care, and some home health care. Most people do not pay a premium for Part A if they or their spouse paid Medicare taxes for at least 10 years (40 quarters of coverage; QCs).
- **Part B:** Covers certain doctors' services, outpatient care, medical supplies, and preventive services. Part B requires a monthly premium (\$202.90 for most in 2026).
 - Part A and B collectively are known as **Original Medicare**.
- **Part C:** Known as **Medicare Advantage**. An alternative to Original Medicare. These are all-in-one plans offered by private insurance companies approved by Medicare. They often include Part D and extra benefits like dental, vision, and hearing.
- **Part D:** Known as PDP. Helps cover the cost of prescription drugs. These plans are run by private insurance companies following rules set by Medicare.

Other Medicare Insurance Programs

- **Supplemental Insurance:** Known as **Medigap**. Helps pay out-of-pocket costs associated with Original Medicare.
- **Medicare Savings Program:** A state-offered program that pays for Medicare out-of-pocket costs.
- **Low Income Subsidy (LIS, Extra Help):** A federal program that provides financial assistance for Medicare Part D prescription drug costs.

Medicare vs Medicaid Q&A

Q1. Both are government programs, so what is the difference?

A. The biggest difference is "who" the target audience is.

- **Medicare:** Based on age. It is a federal program for anyone age 65 or older, or those with certain disabilities, regardless of wealth. Think of it as a "right" earned by paying taxes while working.

- **Medicaid:** Based on income. It is a joint federal and state-funded program, run by individual states, designed to help those with limited income and assets, regardless of their age.

Q2. How do the medical costs differ?

A. Medicare pays "part" of the cost, while Medicaid pays "almost all" of it.

- **Medicare:** Outpatient and physician services (Part B) cover 80% of the Medicare-approved amount, leaving the remaining 20% and the annual deductible as out-of-pocket expenses. Inpatient hospital services (Part A) are subject to a separate cost-sharing structure, which includes a deductible and daily copayments based on the length of the hospital stay.
- **Medicaid:** Covers almost all remaining out-of-pocket costs that Medicare doesn't pay. It also helps cover premiums and prescription drug costs.

Q3. Is the cost of a Nursing Home fully covered?

A. This is where the biggest difference lies.

- **Medicare:** Medicare covers up to **100 days** of **Skilled Nursing Care** only when a physician deems it necessary following a formal **inpatient** hospital stay of at least three days. The cost-sharing structure is as follows: **\$0** for days 1–20; **\$217 per day** for days 21–100 (as of 2026); and the full cost thereafter from day 101. Please note that **Custodial Care**, such as assistance with meals, bathing, and mobility, is not covered by Medicare.
- **Medicaid:** Supports long-term care. Medicaid covers nursing home costs or home health aide services (Home Care).

****Observation Status** is not recognized as a formal inpatient admission, which may result in a denial of Skilled Nursing Facility (SNF) benefits. When admitted, be sure to confirm with the hospital whether your status is officially **inpatient**.*

Q4. I have both. What does that mean?

A. You have "Dual Eligibility."

- If you hold both Medicare and Medicaid cards, you are considered **Dual Eligible**.
- You can enroll in a Dual Special Needs Plan (D-SNP), which coordinates the benefits of both programs into one integrated plan for more efficient care. Specific name of the D-SNP plan may vary by state.
- When you receive medical services under a D-SNP, Medicare acts as the primary payer, covering its portion first. Medicaid then serves as the secondary payer to cover the remaining costs. For more details visit our [D-SNP](#) page.

Q5. What happens if I move to another state?

A. Original Medicare stays the same; Medicaid must be reapplied for.

- **Medicare:** Since it is a federal program, you can use your Medicare plan anywhere in the United States without issue (Original Medicare only).
- **Medicaid:** Policies vary by state. If you move from New Jersey to New York, you must close your New Jersey coverage and apply for New York Medicaid to undergo a new eligibility review.

Medicare Buy-In

For individuals who do not qualify for Medicare Part A, the federal government allows them to buy-in to the program by paying a monthly premium. This typically applies to those who have not earned enough work credits through U.S. social security taxes.

Eligibility (2026)

- **Work Credit System:** Credits are earned based on annual income. One credit is earned per **\$1,890** of quarterly income(quarter of coverage), up to four credits per year.
- **The 40-Credit Rule:** Individuals generally need 40 credits (approximately 10 years of work) to receive Part A without a premium.
- **Spousal Eligibility:** Individuals aged 62 or older may qualify for premium-free Part A based on a spouse's work record. Divorced or widowed individuals may also qualify under specific Social Security guidelines (visit [NCOA.org](https://www.ncoa.org)).

Buy-In Costs (2026)

If an individual does not meet the 40-credit requirement, they may purchase Part A at the following monthly rates:

- **30–39 Credits:** \$311 per month.
- **Fewer than 30 Credits:** \$565 per month.
- **Mandatory Part B:** Typically, individuals who buy into Part A must also enroll in Medicare Part B and pay the standard monthly premium of \$202.90.

How to Buy In

- **Verify Work History:** Individuals should contact the [Social Security Administration \(SSA\)](https://www.ssa.gov) to confirm their total lifetime earned credits.
- **Check Spousal Benefits:** Individuals should determine if they can qualify through a current, former, or deceased spouse's work record to eliminate the Part A premium.
- **Apply via Social Security:** Enrollment is processed by the SSA. Applications can be submitted online at [SSA.gov](https://www.ssa.gov), by phone, or at a local Social Security office.

** If you don't qualify for premium-free Medicare Part A, make sure to review your options as soon as you turn 65. Missing your initial sign-up window could mean paying a permanent late-enrollment penalty. To learn how to protect your coverage and avoid extra costs, visit [Medicare.gov/avoid-late-penalties](https://www.medicare.gov/avoid-late-penalties).*

Resources

- [Medicare.gov: Medicare Savings Programs](https://www.medicare.gov/medicare-savings-programs)
- [SSA.gov: How to Apply for Medicare](https://www.ssa.gov/medicare)
- [CMS.gov: 2026 Medicare Part A & B Premiums and Deductibles](https://www.cms.gov/2026-Medicare-Part-A-B-Premiums-and-Deductibles)
- [NCOA.org: What Are the Medicare Buy-in Programs?](https://www.ncoa.org/what-are-the-medicare-buy-in-programs/)

Healthcare Cost Assistance

Managing the rising costs of healthcare and prescriptions is a significant challenge for many families. Whether a senior is living on a fixed income or a young adult is navigating insurance for the first time, verified programs exist to help reduce these expenses.

Comprehensive Assistance & Discounts

These services act as search engines for savings, connecting patients to public and private assistance programs.

- **NeedyMeds:** A dedicated nonprofit providing extensive resources for medication and healthcare costs. They offer immediate-use pharmacy discount cards and maintain a database of disease-specific financial aid and pharmaceutical Patient Assistance Programs (PAPs).
- **GoodRx:** A popular tool that compares prices at over 70,000 pharmacies. It allows users to compare prices between local pharmacies and use coupons to lower costs instantly.
- **RxAssist:** A comprehensive, searchable directory of pharmaceutical company PAPs. An excellent tool for finding programs specifically tailored to brand-name medications.
- **Medicine Assistance Tool (MAT):** A search engine that focuses exclusively on helping patients find manufacturer-sponsored assistance programs.
- **BenefitsCheckUp(NCOA):** A service of the National Council on Aging (NCOA) specifically designed for adults age 55 and older. It helps seniors find programs for medicine, food, utilities, and more.

Direct Financial Assistance & Grants

These foundations provide actual financial grants to help underinsured patients cover co-pays, premiums, and deductibles.

- **PAN Foundation & HealthWell Foundation:** provide direct financial grants to patients with chronic or rare conditions for out-of-pocket costs that insurance doesn't cover.
- **Patient Advocate Foundation (PAF):** Known for its "Co-Pay Relief" program, PAF provides direct financial aid and professional case management to help patients navigate complex insurance and billing issues.

Local Counseling & Support

- **SHIP (State Health Insurance Assistance Program):** A federally funded statewide program providing free counseling for Medicare beneficiaries. In New Jersey, trained volunteer SHIP counselors help with

navigating coverage options, comparing plans, and resolving complex billing problems to ensure beneficiaries can make informed healthcare decisions.

Access Resources / Contacts

Comprehensive Assistance & Discounts

- **NeedyMeds** 1-800-503-6897 | [NeedyMeds Resource Center](#)
- **GoodRx** 855-268-2822 | [GoodRx Prescription Coupons and Prices](#)
- **RxAssist** 877-537-5537 | [RxAssist Patient Assistance Program Directory](#)
- **PhRMA (MAT)** 571-350-8643 | [Medicine Assistance Tool \(MAT\)](#)
- **NCOA** 1-800-794-6559 | [BenefitsCheckUp for Seniors](#)

Direct Financial Assistance & Grants

- **PAN Foundation** 1-866-316-7263 | [Patient Access Network \(PAN\) Financial Grants](#)
- **HealthWell Foundation** 1-800-675-8416 | [HealthWell Foundation Assistance Programs](#)
- **Patient Advocate Foundation** 1-800-532-5274 | [PAF Co-Pay Relief Program](#)

Local Counseling & Support

- **NJ SHIP** 1-800-792-8820 | [NJ.gov SHIP Counseling](#)
- **Bergen County SHIP** 201-336-7413 | [SHIP Medicare Counseling](#)

Other Resources

- [CMS.gov: PAP Program Information](#)
- [CMS.gov: PAP Attestation Form](#)

Low Income Subsidy (LIS, Extra Help)

The Low Income Subsidy (LIS), also known as Extra Help, is a federal program that helps Medicare beneficiaries with limited income and resources pay for their Medicare Part D prescription drug costs. This includes help with monthly premiums, annual deductibles, and copayments.

Key Aspects (2026)

For those who qualify, the program provides substantial savings:

- **Premiums:** The monthly Part D premium is often reduced to **\$0**.
- **Annual Deductible:** The standard Part D deductible (**\$615**) is reduced to **\$0**.
- **Drug Copays:** Beneficiaries pay no more than **\$5.10** for each generic drug and **\$12.65** for each brand-name drug.
- **\$0 Cost-Sharing:** Once Part D's total out-of-pocket drug costs reach the **\$2,100** cap, the beneficiary pays **\$0** for all covered drugs for the rest of the year.
- **No Late Penalties:** Any Part D late enrollment penalties are permanently waived for those who qualify.

Eligibility

Individuals automatically qualify/do not need to apply for LIS if they already receive:

- Full Medicaid coverage (Dual Eligible).
- Supplemental Security Income (SSI).
- Help from a Medicare Savings Program (MSP) to pay Part B premiums.

Income and Asset Limits (2026)

For those who do not qualify automatically, eligibility is based on annual income and total resources/assets:

Status	Annual Income Limit*	Resource/Asset Limit**
Individual	Below \$23,475	Below \$18,090
Married Couple	Below \$31,725	Below \$36,100

**Income limits include a \$20 monthly income disregard.*

***Resource limits include \$1,500 per person set aside for burial expenses.*

Resource/Asset includes cash, bank accounts, stocks, bonds, and real estate (other than a primary residence). It does not count: a primary home, one vehicle, and personal household items.

How to Apply

- **Online:** [SSA: Online Extra Help Application](#)
- Call: 1-800-772-1213 (TTY: 1-800-325-0778).
- **Visit:** Local SSA office support available.
- **Review Notice:** After applying, the Social Security Administration will mail a letter indicating whether the individual qualifies and the level of assistance they will receive.
- **Automatic Enrollment:** If a beneficiary is found eligible but does not have a Part D plan, Medicare will often automatically enroll them in one to ensure they receive the benefit.

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [SSA: Online Extra Help Application](#)
- [Medicare.gov: Extra Help Basics](#)
- [NJ DoAS: SHIP Counseling for Extra Help Applications](#)

Medical Bills: How It Works

Medicare and secondary insurance (such as Medigap, NJ FamilyCare/Medicaid, or retiree plans) communicate with each other automatically through a system called the **Coordination of Benefits Agreement (COBA) crossover**. A manual claim option (**CMS Form 1490S**) also exists to ensure that secondary insurance benefits are applied correctly if the automatic process fails and to protect patients from being billed for costs that their insurance should cover.

The Automatic Billing System (COBA, Crossover)

The **Coordination of Benefits Agreement (COBA)** is a national system that allows Medicare to share billing data directly with secondary insurers (such as Medigap, retiree plans, or NJ FamilyCare/Medicaid).

- **The Process:** After Medicare pays its portion of a medical bill, it automatically crosses over (transfers) the claim information to the secondary payer.
- **No Action Required:** In standard scenarios, patients and providers do not need to file separate claims for the secondary insurer to pay its portion of the bill.

Filing a Manual Claim (CMS Form 1490S)

While healthcare providers are generally required to file claims, there are specific instances where a manual claim must be filed directly with Medicare to trigger payment.

Circumstances for Manual Filing

- **Provider Refusal:** A physician or medical supplier refuses to submit a claim to Medicare for a covered service.
- **Non-Participating Providers:** Services were received from a provider who is not enrolled in the Medicare program (often during emergencies or specific travel situations).
- **Technical Failures:** The automatic crossover failed, and the secondary insurer requires an official Medicare payment notice to process the remaining balance.

How to File a Manual Claim Using CMS 1490S

1. **Complete the Form:** [CMS Form 1490S](#) (Patient's Request for Medical Payment).

1. **Gather the Itemized Bill, including:**

- Date and place of service (e.g., doctor's office, hospital).

- A description of each medical service or supply provided.
- The specific charge for each individual service.
- The provider's name, address, and National Provider Identifier (NPI).

3. **Attach a Statement of Reason:**

- The doctor is not enrolled in Medicare.
- The provider refused to file the claim on the patient's behalf.
- The services were received during foreign travel or in an emergency.

4. **Mail the Package**

- **Novitas Solutions** P.O. Box 3030 Mechanicsburg, PA 17055-1834

* **Time Limit:** Claims must be submitted within **12 months** (one full calendar year) of the date the services were provided.

* **Processing Time:** It typically takes Medicare up to **60 days** to review and process a manual claim.

Resources

- [CMS.gov: COBA Overview](#)
- [Medicare.gov: How Medicare works with other insurance](#)
- [CMS Form 1490S \(PDF\)](#)
- [Medicare.gov: Filing a Claim](#)

Medicare Supplement Insurance (Medigap)

Medicare Supplement Insurance, known as Medigap, consists of private insurance policies designed to “fill the gaps” in Original Medicare (Part A and B). Because Original Medicare generally pays for about 80% of covered medical expenses, Medigap plans help beneficiaries cover the remaining out-of-pocket costs, such as deductibles, coinsurance, and copayments.

Medigap Coverage

- Out-of-pocket costs in Original Medicare (copayments, coinsurance, and deductibles)

Medigap and Original Medicare (Part A & B)

Medigap is specifically designed to work only with Original Medicare. It functions as secondary insurance to enhance a beneficiary's primary federal coverage.

- **Prerequisite:** A beneficiary must be enrolled in both Medicare Part A and Part B to purchase a Medigap policy.
- **Secondary Payer:** When a beneficiary receives covered services, Original Medicare pays its share first. The Medigap policy then pays its share of the remaining costs according to the specific plan's benefits.
- **Provider Access:** Because Medigap supplements Original Medicare, beneficiaries can see any doctor or visit any hospital in the U.S. that accepts Medicare. No referrals are required to see specialists.
- **Monthly Premiums:** Beneficiaries pay a monthly premium to a private insurance company for Medigap in addition to the standard monthly Part B premium (\$202.90 in 2026) paid to the federal government.

Important Restriction: Medigap cannot be used with Medicare Advantage (Part C). It is illegal for anyone to sell a Medigap policy to an individual enrolled in a Medicare Advantage plan unless they are switching back to Original Medicare.

How to Buy Medigap Plans

Medigap plans may be purchased using [Medicare.gov](https://www.medicare.gov)'s online tool and contacting private insurers directly.

1. **Select a Plan Online:** Go to [Medicare.gov](https://www.medicare.gov): [Find a Medigap Policy](#) and find various plans and their benefit level.
2. **Find Policies:** Find policies in local areas.
3. **Apply Directly:** Contact the private insurer online or by phone for an official quote.

Contact

- **NJ SHIP (State Health Insurance Assistance Program):** Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP website](https://www.nj.gov/ship/)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](https://www.bergencountynj.gov/SHIP-Counseling)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Medicare.gov: Medicare Basics and Parts](https://www.medicare.gov/medicare-basics-and-parts)
- [Medicare.gov: Medigap Policy Overview](https://www.medicare.gov/medigap-policy-overview)
- [NJ DoAS: Medigap 2026 Benefits Star Chart \(PDF\)](#)
- [NJ DoAS: Medigap 2026 Premium Comparison for Females \(Age 65\) \(PDF\)](#)
- [NJ DoAS: Medigap 2026 Premium Comparison for Males \(Age 65\) \(PDF\)](#)
- [NJ DoAS: Medigap 2026 Coverage for People with Disabilities \(Ages 50–64\) \(PDF\)](#)
- [NJ DoAS: Medigap 2026 Plan Information for People Under Age 50](#)

New Jersey Pharmaceutical Assistance to the Aged & Disabled (PAAD) and Senior Gold

The State of New Jersey offers two primary prescription assistance programs: Pharmaceutical Assistance to the Aged & Disabled (PAAD) and Senior Gold Discount Program (Senior Gold). These programs help eligible seniors and individuals with disabilities save significantly on the cost of prescription drugs, including insulin and other life-saving medications. Both programs are integrated with Medicare Part D to provide a seamless layer of financial protection.

Eligibility: NJ PAAD (2026)

Individuals may be eligible for PAAD program if they meet the following state requirements:

- **Residency:** Applicants must be legal residents of New Jersey.
- **Age or Disability:** Applicants must be age 65 or older, or between the ages of 18 and 64 and receiving Social Security Title II Disability benefits.
- **Medicare Part D Enrollment:** Beneficiaries must be enrolled in a Part D plan or Part C (Medicare Advantage) with prescription benefits.
- **Income Limits:** Annual income must be less than **\$54,943 for single individuals** or less than **\$62,390 for married couples**.
- **Copay:** \$5 for each covered generic drug or \$7 for each covered brand name drug, including premiums.

**Must be renewed every two years through the New Jersey state government.*

Eligibility: Senior Gold (2026)

Senior Gold program provides secondary prescription coverage for those whose income exceeds the PAAD limits:

- **Residency:** Beneficiary must be a legal resident of New Jersey.
- **Age or Disability:** Beneficiary must be 65 years of age or older, or at least 18 years of age and receiving SSDI benefits.
- **Medicare Part D Enrollment:** Beneficiaries must be enrolled in a Part D plan or Part C (Medicare Advantage) with prescription benefits.
- **Income Limits:** Annual income must be between **\$54,943 and \$64,943 if single**, or between **\$62,390 and \$72,390 if married**.

- **Copay: \$15 plus 50%** of the remaining cost of each Senior Gold covered drug.

**Must be renewed through the New Jersey state government every year.*

How to Apply

Use the NJSave portal to streamline applications for multiple senior benefits such as PAAD, Senior Gold, Lifeline, and more.

- Online Application: [NJSave](#)
- Paper Application: [NJSave \(PDF\)](#).

Contact

- **NJSave:** PAAD, Sr. Gold | 1-800-792-9745 | [NJSave Contact Us](#)
- **NJ SHIP (State Health Insurance Assistance Program):** Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP Counseling](#)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](#)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ Department of Human Services – PAAD Information](#)
- [NJ Division of Aging Services – Senior Gold Overview](#)
- [NJSave Online Application](#)
- [NJSave Paper Application \(PDF\)](#).

Medicare Part C (Medicare Advantage)

Medicare Part C, also known as Medicare Advantage, is an **all-in-one private alternative** to Original Medicare. These plans bundle Part A, Part B, and usually Part D into a single package. While provided by private insurers, they are strictly regulated and approved by Medicare.

Key Aspects

- **Bundled Coverage:** Combines hospital, medical, and often prescription drug coverage.
- **Extra Benefits:** Most plans include services not covered by Original Medicare, such as routine dental, vision, hearing, and fitness memberships.
- **Spending Limit:** Unlike Original Medicare, these plans have a mandatory **Maximum Out-of-Pocket (MOOP)** limit. Once reached, the plan pays 100% of covered medical services for the rest of the year.
- **Network-Based:** Beneficiaries typically use a specific network of doctors and hospitals (HMO or PPO).
- To enroll in Part C, an individual must already have Medicare Part A and Part B.

Common Plan Types

- **HMO (Health Maintenance Organization):** Usually requires using in-network providers and obtaining referrals for specialists.
- **PPO (Preferred Provider Organization):** Offers more flexibility to see out-of-network providers at a higher cost; referrals are typically not required.
- **SNP (Special Needs Plans):** Tailored for individuals with specific chronic conditions or those eligible for both Medicare and Medicaid.

Costs & Limits (2026)

- **Monthly Premium:** Many plans are \$0 premium. Beneficiaries must pay the standard Part B premium (\$202.90).
- **Out-of-Pocket Cap:** Federal maximum limit for in-network services: **\$9,250/month**
- **Copays:** A fixed copay is usually set for doctor visits or specialist services.

Enrollment

You can enroll in, switch, drop, or change a Medicare Advantage Plan. To join a Medicare Advantage Plan, you must be enrolled in both Medicare Part A and Part B:

Enrollment Period	Timing	Coverage Start	Key Notes
Initial Enrollment Period (IEP)	3 months before and after the 65th birthday month (7 months)	Varies by time of application (typically 1st day of birthday month)	Must have both Part A and Part B to apply. Next is the AEP.
Annual Enrollment Period (AEP)	October 15 – December 7	January 1 of the following year	Critical period for maintaining, changing, or dropping existing plans.
Open Enrollment Period (OEP)	January 1 – March 31 every year	The 1st of the month following application	Only applicable to those with a Medicare Advantage plan. May switch to a different Advantage plan or return to Original Medicare once during this period.
Special Enrollment Period (SEP)	Triggered when qualifying events occur	Varies by case; Generally within 2 months from the date of the qualifying event	Used when your existing coverage suddenly ends, if you relocate out of state, or changes in eligibility.

Automatic Rollover

Your Medicare Advantage plan will automatically renew (rollover) each year without requiring a new application.

- **Review Your ANOC:** Be sure to review the “Annual Notice of Change (ANOC)” sent by your insurance provider around September each year. Check for critical changes regarding premiums, copays, provider networks, and prescription drug formularies.
- **Service Discontinuation by Insurer:** If your current insurance carrier stops offering services in your area or terminates its contract, your existing plan will automatically end.
- If you are unsatisfied with your current plan, or if your benefits are reduced or discontinued by the insurance company, you must find a new plan during the **Annual Enrollment Period (AEP, Oct 15 – Dec 7)** or the **Medicare Advantage Open Enrollment Period (OEP, Jan 1 – Mar 31;** provided your plan is still active).

How to Buy an Medicare Advantage Plan

Medicare Advantage may be purchased directly using Medicare.gov’s online tool.

1. **Select a Plan Online:** Go to [Medicare.gov](https://www.medicare.gov) and find various plans and their benefit level.
2. **Check Service Area:** Plans are location-specific; ensure the plan is available in a specific county.

3. **Verify Providers:** Confirm that preferred doctors and pharmacies are included in the plan's network.

*** Enroll During Specific Windows:** Enrollment must occur during the Initial Enrollment Period or the annual Open Enrollment (Oct 15 – Dec 7).

Contact

- **NJ SHIP (State Health Insurance Assistance Program):** Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP Counseling](https://www.nj.gov/ship/counseling)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](https://www.bergencountynj.gov/ship/counseling)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Medicare.gov: Find and Compare 2026 Plans](https://www.medicare.gov/compare-plans)
- [Medicare.gov: Medicare Advantage Overview \(PDF\)](#)
- [NJ DoAS: 2026 NJ Medicare Advantage List](#)
- [NJ DoAS: SHIP Counseling for Plan Selection](#)

Medicare Part D (Prescription Drug Plan, PDP)

Medicare Part D, known as Medicare drug coverage or PDP, helps cover the cost of outpatient prescription drugs. Because coverage is provided by private insurance companies, plan details—including premiums, pharmacy networks, and which drugs are covered—can vary. Federal laws set standard limits and protections for all plans nationwide.

Part D Coverage: Two Options

1. Medicare Drug Plan (Part D, PDP)

- Original Medicare (Part A & B)
- Some Medicare Cost Plans
- Some Medicare Advantage Private Fee-for-Service Plans
- Medicare Medical Savings Account (MSA) Plans

2. Medicare Advantage (Part C, MAPD) or Other Medicare Plans

- Part A, Part B, and Medicare drug coverage (Part D) through a single plan

Part D Costs (2026)

- **Annual Deductible:** Enrollees pay 100% of their drug costs until the plan's deductible is reached. No plan's deductible can be higher than \$615.
- **Coverage (25% Coinsurance):** After the deductible is met, the plan begins to share costs. The enrollee pays approximately 25% for covered drugs.
- **Out-of-Pocket:** Once out-of-pocket spending reaches \$2,100, the enrollee pays \$0 for all covered drugs for the rest of the year.
- **Monthly Premium:** To keep Part D coverage active, a monthly premium is paid to the insurance provider. They do not count toward the \$2,100 out-of-pocket cap. Only the money spent at the pharmacy (deductibles and coinsurance) counts toward that limit.

How to Choose & Join a Plan

Beneficiaries are encouraged to review their coverage during the **Annual Open Enrollment (Oct 15 – Dec 7)** because plans can change.

- **Use the Official Plan Finder:** Visit [Medicare.gov/plan-compare](https://www.medicare.gov/plan-compare) to enter current medications and see which plans offer the lowest total annual cost.
- **Check Pharmacy Networks:** Confirm if a preferred pharmacy is considered In-Network or Preferred by the plan to ensure the lowest possible copays.
- **Bergen County, NJ Residents:** Bergen County residents can receive free assistance through AWCA at 201-862-1665 or via email at info@awcanj.org.

*Individuals who don't enroll when first eligible may have to pay a late enrollment penalty. For more information visit [Medicare.gov/avoid late penalties](https://www.medicare.gov/avoid-late-penalties).

Contact

- **NJ SHIP (State Health Insurance Assistance Program):** Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP Counseling](https://www.nj.gov/SHIP-Counseling)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](https://www.bergencountynj.gov/SHIP-Counseling)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Medicare.gov: Part D Basics](https://www.medicare.gov/partd-basics)
- [Medicare.gov: Find & Compare Drug Plans](https://www.medicare.gov/find-compare-drug-plans)
- [NJ DoAS.gov: 2026 NJ PDP List \(PDF\)](https://www.njdoas.gov/2026-nj-pdp-list)

Medicare Parts A & B (Medicare Original)

Medicare Original is the traditional fee-for-service health insurance program managed by the federal government. It allows beneficiaries to visit any doctor or hospital in the United States that accepts Medicare. Medicare Original consists of two primary components: Part A (Hospital Insurance) and Part B (Medical Insurance).

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Medicare Part A (Hospital Insurance)

Part A covers essential inpatient services. Most individuals qualify for premium-free Part A if they or their spouse paid Medicare taxes for at least 10 years (40 quarters). Those with shorter work histories may purchase Part A for a monthly premium.

- **Inpatient Hospital Care:** Includes semi-private rooms, meals, general nursing, and drugs provided during inpatient treatment.
- **Skilled Nursing Facility (SNF):** Covers skilled nursing or rehabilitation services following a qualifying 3-day inpatient hospital stay.
- **Hospice Care:** Specialized care for terminally ill beneficiaries focused on comfort and pain management.
- **Home Health Services:** Covers part-time or intermittent skilled nursing care and physical therapy for homebound individuals.

Part A Costs (2026)

Costs are based on benefit period:

- **Days 1-60:** Deductible of \$1,736 per benefit period.
- **Days 61-90:** Hospital Coinsurance of \$434 per day.
- **Days 91-150:** Lifetime Reserve Days of \$868 per day (up to 60 days available over lifetime).
- **Days 151~:** 100% personal contribution.
- **SNF**
 - **Days 1-20:** \$0 per day.
 - **Days 21-100:** \$217 per day.
 - **Days 101~:** 100% personal contribution.

- **Hospice Care:** \$0 copayment (small copayments may apply for outpatient prescription drugs or short-term respite care).

Medicare Part B (Medical Insurance)

Part B covers medically necessary services and preventive care. Unlike Part A, most beneficiaries must pay a monthly premium for Part B.

- **Doctor Services:** Visits to primary care doctors and specialists.
- **Outpatient Care:** Services received at a hospital without being admitted (e.g., ER visits or outpatient surgery).
- **Preventive Services:** Includes flu shots, screenings (cancer, diabetes), and an annual wellness visit.
- **Durable Medical Equipment (DME):** Covers items like walkers, wheelchairs, and oxygen equipment.

Part B Costs (2026)

- **Standard Monthly Premium:** \$202.90 (May be higher based on income—see IRMAA).
- **Annual Deductible:** \$283 per year.
- **Coinsurance:** Generally **20%** of the Medicare-approved amount after you meet your deductible.

Eligibility

- 65 or older: U.S. citizens or legal permanent residents who have resided in the U.S. for at least five continuous years.
- Under age 65 receiving Social Security Disability Insurance (SSDI) benefits or have Amyotrophic Lateral Sclerosis (ALS, 'Lou Gehrig's disease') or End-Stage Renal Disease (ESRD).

Enrollment

Medicare enrollment may be automatic or manual depending on the applicants' situation. Enrolling into Medicare during the enrollment window is critical; missing it can result in a lifelong monthly premium penalty.

Enrollment Period	Timing	Coverage Start	Key Notes
Initial Enrollment Period (IEP)	3 months before and after the 65th birthday month (7 months)	Varies by time of application (typically 1st day of birthday month)	Critical period to enroll when you become eligible for Medicare. Missing this window may lead to monthly penalties.
Annual Enrollment Period (AEP)	October 15 – December 7	January 1 of the following year	Primary period for maintaining, changing, or dropping existing plans.
Special Enrollment Period (SEP)	Within 8 months of employer-sponsored coverage ending.	The 1st day of the following month of application	For those who delayed Medicare enrollment because they were covered under active employment. Also triggered upon sudden loss of coverage or relocation.
General Enrollment Period (GEP)	January 1 – March 31	The 1st day of the following month of application	For individuals who missed their IEP.

Automatic Enrollment (No Action Needed)

Medicare enrollment is automatic if:

- **Already Receiving Benefits:** The individual has been receiving Social Security or Railroad Retirement Board (RRB) benefits for at least four months before turning 65.
- **Disability:** The individual has been receiving Social Security Disability Insurance (SSDI) for 24 months.
- **ALS/ESRD:** Individuals with Lou Gehrig's disease (ALS) or End-Stage Renal Disease (ESRD) have specific automatic or expedited enrollment rules.

Manual Enrollment (Action Required)

If an individual is not receiving Social Security benefits (for example, because they are still working or delaying retirement), they must manually sign up for Medicare.

- **Initial Enrollment Period (IEP):** A 7-month window that includes the month of the 65th birthday, the three months before, and the three months after.
- **Apply Online:** Enroll directly through [Social Security Administration \(SSA\)](https://www.ssa.gov).

*Individuals who don't enroll when first eligible may have to pay a late enrollment penalty. For more information visit [Medicare.gov/avoid late penalties](https://www.medicare.gov/avoid-late-penalties).

Contact

- **NJ SHIP (State Health Insurance Assistance Program):** Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP Counseling](https://www.nj.gov/ship/counseling)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](https://www.bergencountynj.gov/ship/counseling)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Medicare.gov: Parts of Medicare](https://www.medicare.gov/medicare-basics)
- [Medicare.gov: Fact Sheet – Medicare CostsGuide \(2026\) \(PDF\)](#).
- [CMS.gov: Medicare Part A & B Enrollment](https://www.cms.gov/medicare/medicare-eligibility)
- [SSA.gov: Sign up for Medicare](https://www.ssa.gov/sign-up)
- [SSA.gov: Medicare guide \(2026\) \(PDF\)](#).

Medicare Savings Programs (MSP)

Medicare Savings Programs (MSP) are state-administered programs that help individuals with limited income and resources pay their Medicare costs. Depending on the specific program, an MSP can cover Medicare Part B premiums and, in some cases, Part A premiums, deductibles, and coinsurance.

Program Types and Benefits

There are four primary types of Medicare Savings Programs. If a beneficiary qualifies for QMB, SLMB, or QI, they automatically qualify for Extra Help (LIS) to pay for prescription drug costs.

Qualified Medicare Beneficiary (QMB)

- **Benefits:** Pays for Part A premiums, Part B premiums, deductibles, coinsurance, and copayments.
- **Requirement:** Providers are legally prohibited from billing QMB patients for Medicare-covered services.

Specified Low-Income Medicare Beneficiary (SLMB)

- **Benefits:** Pays for the **Part B premium** (\$202.90 in 2026) only.

Qualifying Individual (QI)

- **Benefits:** Pays for the **Part B premium** only.
- **Note:** Individuals must apply every year, and benefits are granted on a first-come, first-served basis.

Qualified Disabled and Working Individual (QDWI)

- **Benefits:** Pays for the **Part A premium** only.
- **Target:** Specifically for disabled individuals who returned to work and lost their premium-free Part A.

Eligibility (New Jersey)

Program	Individual Income Limit (Monthly)	Married Couple Income Limit (Monthly)	Resource (Asset) Limit
QMB	\$1,330	\$1,803	\$9,950 (Indiv.) / \$14,910 (Couple)
SLMB	\$1,596	\$2,164	\$9,950 (Indiv.) / \$14,910 (Couple)
QI	\$1,795	\$2,434	\$9,950 (Indiv.) / \$14,910 (Couple)
QDWI	\$5,405	\$7,299	\$4,000 (Indiv.) / \$6,000 (Couple)

**Some states, like New York or Connecticut, may have higher limits or no asset test.*

How to Apply

- [Medicare.gov](https://www.medicare.gov): Official website of Medicare.
- [Medicaid.gov](https://www.medicaid.gov): Official website of Medicaid.
- [SSA.gov](https://www.ssa.gov): Apply for Extra Help for prescription drug costs (may apply with MSP at the same time).

Contact

- **NJ SHIP (State Health Insurance Assistance Program)**: Free counseling from state-trained experts
 - NJ SHIP: 1-800-792-8820 | [NJ.gov SHIP Counseling](https://www.nj.gov/ship-counseling)
 - Bergen County SHIP: 201-336-7413 | [Bergencountynj.gov SHIP Counseling](https://www.bergencountynj.gov/ship-counseling)
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

PART I • SECTION 4

Medicaid

메디케이드 (Medicaid)

State and federal healthcare assistance for low-income individuals: New Jersey FamilyCare / Medicaid, Managed Care Organizations (MCOs), ACA Medicaid expansion, Marketplace coverage, and specialized eligibility rules.

This section contains 9 comprehensive guides

Medicaid Overview

Medicaid is a health insurance program jointly operated by the federal and state governments for low-income adults, children, pregnant women, seniors, and people with disabilities. Because each state manages its own program with federal support, the specific names (e.g., NJFamilyCare in New Jersey) and eligibility requirements may vary by state; however, it fundamentally serves as a vital medical safety net for vulnerable populations.

Features and Benefits

- **Low Cost Burden:** Eligible beneficiaries pay little to no monthly premiums, and out-of-pocket costs (copays) for doctor visits or prescriptions are also free or very inexpensive.
- **Broad Range of Medical Services:** In addition to general doctor visits and hospital services, it provides extensive coverage for dental, vision, prescription drugs, and long-term care services that are often difficult to cover with private insurance.
- **Year-Round Enrollment:** Unlike general health insurance with fixed enrollment periods, beneficiaries can apply for and receive Medicaid benefits at any time of the year as long as they meet the eligibility requirements. Once eligibility is confirmed through the review process after application, coverage begins on either the date of application or the first day of the month in which the application was submitted.

Types of Medicaid

- **Pregnancy Medicaid:** Pregnancy Medicaid covers all medical expenses during pregnancy, including prenatal screenings, childbirth, and newborn care. In many states, including New Jersey, coverage is extended for up to 12 months postpartum. (Availability varies by state.)
- **Children's Health Insurance Program (CHIP):** CHIP is provided at a low cost for children (Under 19 years of age) in families whose income is higher than Medicaid limits but insufficient to afford private insurance.
- **ACA Medicaid (Medicaid Expansion):** ACA Medicaid was expanded through the ACA (Affordable Care Act), widely known as Obamacare. ACA Medicaid expanded eligibility to include all low-income adults with a household income at or below 138% of the Federal Poverty Level, regardless of parental or disability status.
- **Regular Medicaid (ABD Program):** Regular Medicaid provides essential medical services and long-term care benefits to low-income individuals who are Aged (65+), Blind, or Disabled and meet specific income and asset criteria.

- **[Long-Term Care Medicaid \(LTC\)](#)**: LTC provides financial support for low-income seniors or individuals with disabilities who have mobility issues, covering the costs of Nursing Home placement or HCBS (Home and Community-Based Services) so they can receive caregiver assistance within their own homes or community settings.

Eligibility

- **Income and Asset Screening**: Eligibility follows the Federal Poverty Level (FPL) guidelines announced by the government each year. Specifically, Long-Term Care Medicaid involves strict screening of not only income but also held assets (cash, savings, etc.).
- **Immigration Status**: Generally, the primary eligibility is for U.S. citizens or Lawful Permanent Residents (Green Card holders) who have held their status for at least five years. However, for pregnant women or children, more relaxed criteria may apply depending on state regulations.

Medicaid vs. Medicare Q&A

Q1. Both are government programs, so what is the difference?

A. The biggest difference is "who" the target audience is.

- **Medicare**: Based on age. It is a federal program for anyone age 65 or older, or those with certain disabilities, regardless of wealth. Think of it as a "right" earned by paying taxes while working.
- **Medicaid**: Based on income. It is a joint federal and state-funded program, run by individual states, designed to help those with limited income and assets, regardless of their age.

Q2. How do the medical costs differ?

A. Medicare pays "part" of the cost, while Medicaid pays "almost all" of it.

- **Medicare**: Outpatient and physician services (Part B) cover 80% of the Medicare-approved amount, leaving the remaining 20% and the annual deductible as out-of-pocket expenses. Inpatient hospital services (Part A) are subject to a separate cost-sharing structure, which includes a deductible and daily copayments based on the length of the hospital stay.
- **Medicaid**: Covers almost all remaining out-of-pocket costs that Medicare doesn't pay. It also helps cover premiums and prescription drug costs.

Q3. Is the cost of a Nursing Home fully covered?

A. This is where the biggest difference lies.

- **Medicare:** Medicare covers up to **100 days** of **Skilled Nursing Care** only when a physician deems it necessary following a formal **inpatient** hospital stay of at least three days. The cost-sharing structure is as follows: **\$0** for days 1–20; **\$217 per day** for days 21–100 (as of 2026); and the full cost thereafter from day 101. Please note that **Custodial Care**, such as assistance with meals, bathing, and mobility, is not covered by Medicare.
- **Medicaid:** Supports long-term care. Medicaid covers nursing home costs or home health aide services (Home Care).

**Observation Status is not recognized as a formal inpatient admission, which may result in a denial of Skilled Nursing Facility (SNF) benefits. When admitted, be sure to confirm with the hospital whether your status is officially inpatient.*

Q4. I have both. What does that mean?

A. You have Dual Eligibility.

- If you hold both Medicare and Medicaid cards, you are considered **Dual Eligible**.
- You can enroll in a Dual Special Needs Plan (D-SNP), which coordinates the benefits of both programs into one integrated plan for more efficient care. Specific name of the D-SNP plan may vary by state.
- When you receive medical services under a D-SNP, Medicare acts as the primary payer, covering its portion first. Medicaid then serves as the secondary payer to cover the remaining costs. For more details visit our [D-SNP](#) page.

Q5. What happens if I move to another state?

A. Original Medicare stays the same; Medicaid must be reapplied for.

- **Medicare:** Since it is a federal program, you can use your Medicare plan anywhere in the United States without issue (Original Medicare only).
- **Medicaid:** Policies vary by state. If you move from New Jersey to New York, you must close your New Jersey coverage and apply for New York Medicaid to undergo a new eligibility review.

ACA Marketplace

The ACA Marketplace (Affordable Care Act Marketplace) is a government-run insurance exchange established under the healthcare reform law. It is a system where individuals can compare and purchase private health insurance and receive government subsidies based on their income. One of its core purposes is to help those whose income is too high to qualify for Medicaid yet low enough to need assistance, enroll in health insurance at affordable premiums through government subsidies.

Key Features (2026)

- **Subsidy Benefits (Premium Tax Credits):** If your household income is between 100% and 400% of the Federal Poverty Level (FPL), you are eligible for the federal APTC (Advance Premium Tax Credit). If your income exceeds 400% of the FPL, you will not be eligible for federal subsidies, and only the New Jersey State subsidy (NJHPS) will be available.
- **Pre-existing Condition Coverage:** Insurance companies cannot deny coverage or charge higher premiums just because you have a pre-existing condition (such as cancer or diabetes).
- **10 Essential Health Benefits:** Emergency services, hospitalization, pregnancy and maternity care, mental health services, and prescription drugs must be included.
- **Dependent Coverage:** Children can remain on their parents' health insurance plan until the age of 26.

Health Plan Levels

Plans are divided into four categories based on how you and your plan share costs.

Level	Plan Pays	You Pay	Features
Bronze	60%	40%	Lowest monthly premiums, but high out-of-pocket costs when you receive care.
Silver	70%	30%	The level where subsidies are maximized (Cost-Sharing Reductions for lower income).
Gold	80%	20%	Higher monthly premiums, but lower out-of-pocket costs for medical services.
Platinum	90%	10%	Highest monthly premiums; ideal for those who require frequent medical care.

Subsidy Benefit Guide (2026)

Household Size	Federal Subsidy (APTC)	NJ State Subsidy (NJHPS)	No Subsidy
Eligibility Basis	138% – 400% FPL	400% – 600% FPL	Over 600% FPL
1 Person	\$22,025 – \$62,600	\$62,601 – \$93,900	Over \$93,900
2 People	\$29,864 – \$84,600	\$84,601 – \$128,840	Over \$128,840
3 People	\$37,702 – \$106,600	\$106,601 – \$159,900	Over \$159,900
4 People	\$45,540 – \$128,600	\$128,601 – \$192,900	Over \$192,900

- **Federal Marketplace Subsidy (APTC):** The previous year's (2025) FPL guidelines are used to calculate subsidies. (The 400% FPL ceiling is \$62,600 for a 1-person household and \$128,600 for a 4-person household.)
- **NJ State Subsidy (NJHPS):** A separate benefit provided by the state of New Jersey (NJ Health Plan Savings). By applying through **GetCoveredNJ**, applicants with incomes up to 600% FPL can receive additional financial assistance.
- **Eligibility Verification:** The figures provided in this document are for reference purposes only. Final eligibility and benefit amounts are subject to the official determination results from GetCoveredNJ or the NJ FamilyCare website. **Starting in 2026, individuals with incomes exceeding 400% FPL are no longer eligible for federal subsidies (APTC) and can only receive NJ state subsidies.*

Enrollment Periods

- **Open Enrollment Period:** Because New Jersey operates its own state-based marketplace (**GetCoveredNJ**) rather than using the federal marketplace (**HealthCare.gov**), it offers a longer enrollment deadline.
 - Dates: **November 1 – January 31**
 - Enroll by December 31 for January 1 start
 - Enroll by January 31 for February start
- **Special Enrollment Period:** If certain changes (Qualifying Life Events) occur, you can enroll within 60 days even outside the regular period:
 - Marriage or Divorce
 - Birth or Adoption of a child
 - Relocation (Moving to a different state, etc.)
 - Loss of existing employer-sponsored insurance

How To Apply

- **Documents Needed for Application**
 - Social Security Numbers (SSN) for all household members or immigration documents (Green card, etc.)
 - Proof of household income (Tax returns, pay stubs, etc.)
 - Residential address information
- **Federal Marketplace**
 - Online Application: [HealthCare.gov](https://www.healthcare.gov) – Texas, Florida, Georgia, and approx. 30 other states.
 - Phone Application: 1-800-318-2596 (24/7, Korean interpretation available)
- **State-Based Marketplaces:**
 - New Jersey: [GetCoveredNJ](https://www.getcoverednj.com)
 - New York: [NY State of Health](https://www.nystateofhealth.com)
 - California: [Covered California](https://www.coveredcalifornia.com)
 - Pennsylvania: [Pennie](https://www.pennie.com)
- **Professional Assistance:** local Navigators or certified agents can help at no cost.

Contact

- **AWCA** can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [HealthCare.gov](https://www.healthcare.gov): Official Marketplace Enrollment
- [HealthCare.gov](https://www.healthcare.gov) – Other Language Resources (Included Korean).
- [KFF.org](https://www.kff.org) – Health Insurance Marketplace Subsidy Calculator

ACA Medicaid (Medicaid Expansion)

The Affordable Care Act (ACA) is a federal law that allows each state government to expand Medicaid eligibility to low-income adults with incomes up to 138% of the Federal Poverty Level (FPL).

Officially named as NJ FamilyCare in New Jersey, this Medicaid program allows low-income adults, including those without dependent children, to receive comprehensive health insurance benefits through NJ FamilyCare with no monthly premiums.

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Eligibility

- **Target:** New Jersey residents between 19 and 64
- **Immigration Status:** Lawful Permanent Resident (LPR) status for at least 5 years.
 - **Refugee/Asylee and LPR:** Under OBBA rules, refugees or asylees who haven't adjusted to LPR status may lose eligibility starting **October 1, 2026**.
- **Work and Community Engagement Requirements (OBBA):** Proof of at least **80 hours** per month of work, education, vocational training, or community service is required. (Effective January 1, 2027)

**Work exempt groups: ① Children 18 and under, ② ABD (Aged, Blind, and Disabled) beneficiaries, ③ Parents or caregivers of children under 14 or persons with disabilities, ④ Individuals with serious physical or mental health conditions, ⑤ Pregnant women and those within one year postpartum, ⑥ Former foster youth under age 26, ⑦ Individuals who are incarcerated or recently released from incarceration. *For details refer to the [NJ DHS](#) page*

Income Guidelines (2026)

At or below 138% of the Federal Poverty Level (FPL)

Household Size	Maximum Monthly Income (138% FPL)
1 Person	\$1,836 or less
2 Persons	\$2,489 or less
3 Persons	\$3,142 or less
4 Persons	\$3,795 or less
5 Persons	\$4,449 or less
6 Persons	\$5,102 or less
7 Persons or more	Add +\$654 per additional person

- **No Cost Sharing:** Individuals within 0-138% FPL range have no monthly premium and no copayments for any covered medical services.
- **Household Size:** Includes total number of parents or caretakers, tax-dependent children under the age of 21, and any other tax dependents residing within the same household.
- **Income Verification:** Eligibility is calculated based on Modified Adjusted Gross Income (MAGI) This is typically verified electronically using federal tax reporting data to ensure a streamlined application process.
- **Annual Renewal:** Eligibility is re-evaluated on an annual basis. Once renewal documents are received, information must be updated within the specified deadline to maintain health benefits.

Comprehensive Benefits

- **Basic Medical Services:** visits to primary care physicians and specialists, routine physical check-ups, and essential immunizations.
- **Diagnostic and Specialized Care:** blood tests, X-rays, CT/MRI scans, and both inpatient and outpatient surgical procedures.
- **Dental and Vision Care:** basic dental examinations and treatments, eye exams, prescription eyeglasses.
- **Prescriptions and Mental Health:** prescription drug support, psychological counseling, and various services for substance use disorder treatments.

How to Apply

- **Online:** [NJ FamilyCare/quickstart](#) or [NJHelps.gov](#)
 - Contact: 1-800-701-0710 / TTY: 711

- **In-Person:** visit local County Board of Social Services
 - Bergen County, NJ Board of Social Services
 - 218 Route 17 North, Rochelle Park, NJ 07662
 - 201-368-4200
 - Hours: Weekdays 8:00 am thru 4:30 pm

**Enrollment is open year-round for NJ FamilyCare*

Contact

- **AWCA** can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ FamilyCare – Official Eligibility Requirements Guide](#)
- [NJ DHS – Online Application and Service Guide](#)
- [NJ.gov – County Social Service Office Location Search](#)
- [NJ FamilyCare – Q&A](#)

Children's Health Insurance Program (CHIP)

The Children's Health Insurance Program (**CHIP**) in New Jersey is a critical component of the **NJ FamilyCare** system. It is designed to provide comprehensive, affordable health coverage to children in families who earn too much to qualify for traditional Medicaid but cannot afford private health insurance.

Eligibility

- **Age:** Children under the age of 19
- **Residency:** Residents of New Jersey
- **Immigration Status:** As of 2026, under the '**Cover All Kids**' act, all children in the following household **Income Guidelines (2026)** receive the same cost benefits regardless of their immigration status.
- CHIP applications are open year-round and there is no waiting period.

Income Guidelines (2026)

Household Size	FPL 147% or below	FPL 147% – 150%	FPL 150% – 200%	FPL 200% – 355%
Premium	None	None	None	None
Copay	None	None	\$5 - 10	\$5 - \$35
1 Person	\$1,955 or less	\$1,956 ~ \$1,995	\$1,996 ~ \$2,660	\$2,661 ~ \$4,722
2 People	\$2,650 or less	\$2,651 ~ \$2,705	\$2,706 ~ \$3,606	\$3,607 ~ \$6,401
3 People	\$3,347 or less	\$3,348 ~ \$3,416	\$3,417 ~ \$4,554	\$4,555 ~ \$8,083
4 People	\$4,043 or less	\$4,044 ~ \$4,125	\$4,126 ~ \$5,500	\$5,501 ~ \$9,763
Each Add'l	+ \$696	+ \$711	+ \$947	+ \$1,682

**Children with income at or below 147% of the FPL are classified under Medicaid rather than CHIP, and all medical costs are fully covered at no charge.*

Comprehensive Benefits

- **Primary & Specialty Care:** Routine checkups, immunizations, and visits to specialists.
- **Dental Care:** Cleanings, fillings, X-rays, and essential orthodontic work.
- **Vision Care:** Eye exams and prescription eyeglasses.

- **Hospitalization:** Emergency room visits, inpatient hospital stays, and outpatient surgery.
- **Mental Health:** Counseling, behavioral health therapies, and substance abuse treatment.
- **Prescriptions:** Access to a wide network of pharmacies with low copays.

Key Changes in 2026

- **Continuous Eligibility:** Children now benefit from **multi-year continuous eligibility**, meaning they do not lose coverage due to minor fluctuations in family income during the year.
- **Telehealth Expansion:** Enhanced coverage for pediatric specialized telehealth services, including speech and occupational therapy.

Managed Care Enrollment (MCOs)

Families in New Jersey must choose an MCO to manage their child's healthcare. If families do not choose one, an MCO and a Primary Care Provider (PCP) is automatically assigned.

- Check if your preferred pediatrician is in the MCO's network.
- Review "Value-Added Services" such as free sports physicals or extra vision benefits.

* For details check the [New Jersey MCO](#) page.

How to Apply

- **Online:** [NJ FamilyCare website](#)
 - Contact: 1-800-701-0710 / TTY: 711
- **In-Person:** visit local County Board of Social Services
 - Bergen County, NJ Board of Social Services
 - 218 Route 17 North, Rochelle Park, NJ 07662
 - 201-368-4200
 - Hours: Weekdays 8:00 am thru 4:30 pm

*Enrollment is open year-round for NJ FamilyCare

Contact

AWCA can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ FamilyCare – Official CHIP Eligibility Guide](#)

- [NJ state official 'coverallkids' website](#)
- [CMS.gov – Insure All Kids Now \(CHIP\)](#)
- [CMS.gov – New Jersey CHIP \(PDF\)](#)
- [Medicaid.gov – NJ Medicaid and CHIP](#)

Hearing Aid

New Jersey operates hearing aid assistance and prescription drug subsidies in conjunction for low-income seniors and individuals with disabilities.

Reimbursement for hearing aid costs is available through the Hearing Aid Assistance to the Aged and Disabled (HAAD) program, administered by the New Jersey Division of the Deaf and Hard of Hearing (DDHH). Eligibility for this program is linked to the Pharmaceutical Assistance to the Aged and Disabled (PAAD) income standards.

Hearing Aid Assistance for the Aged and Disabled (HAAAD)

PAAD-enrolled NJ residents can receive reimbursements for hearing aids already purchased through the HAAAD program.

- **Reimbursement Amount:** \$500 per hearing aid, up to a **maximum of \$1,000 (for both ears)**. However, reimbursement is limited to once per calendar year; if you make an additional purchase in the following year, a new application must be submitted.
- **Eligibility:**
 - Must meet PAAD income limits and must be currently enrolled in PAAD or Lifeline.
 - Reimbursement is only available if the cost was not fully covered by Medicaid or other private insurance.

**If the cost of your hearing aid is fully covered by Medicaid, private health insurance, or retiree insurance, you are ineligible for the HAAAD reimbursement. If you have partial coverage, you may apply for the remaining balance. Additionally, Senior Gold beneficiaries are excluded from HAAAD eligibility.*

New Jersey Hearing Aid Project (NJHAP)

NJHAP provides free, high-quality refurbished hearing aids.

- **Age:** Individuals aged 65 or older, or those with hearing loss who have been determined disabled and are receiving SSDI (Social Security Disability Insurance).
- **Income:** Must meet NJ PAAD criteria (Single \$54,943 / Couple \$62,390).
- **Required Documents:**
 - PAAD Enrollees: [Form A](#) (Simplified process).
 - Non-PAAD Enrollees: [Form B](#) (Requires additional income verification).
 - Must submit a medical clearance or hearing exam from a physician or licensed audiologist.

Horizon NJ Health (Medicaid) Coverage

Members enrolled in Medicaid (FamilyCare/MLTSS) receive hearing aid benefits directly through their insurance without needing to apply for separate reimbursements.

- **Key Coverage:** Hearing exams, hearing aid devices, accessories, supplies, fitting, and repair services.
- **Procedures:** Coverage is typically available once every **48 months (4 years)**; a physician's exam and **Prior Authorization** from the insurer are mandatory.

**Benefit cycles may vary depending on your specific Managed Care Organization (MCO) plan; please contact your insurance provider (MCO) directly to confirm your coverage details.*

Under 21: Through the [EPSDT program](#), children and young adults receive broader access to hearing technology and related services.

How To Apply

- **HAAAD**
 - **Online Application:** [NJSave](#)
 - **Postal Submission:** Download application form from [NJ.gov HAAAD](#) and submit it by mail.
 - **Contact:** 1-800-792-9745 *PAAD/Lifeline enrollees must complete the HAAAD application and submit it with the required documentation.
- **NJHAP**
 - **Postal Submission:** Download application form from the [New Jersey Hearing Aid Project](#) and submit it by mail.
 - **Contact:** DDHH 609-588-2648 | communications2@dhs.nj.gov

**The processing period is approximately 2 weeks upon receipt of the application. Once eligibility is verified, the DDHH will mail a confirmation letter and a list of participating audiologists. However, depending on the inventory of donated hearing aids, actual delivery may take between 8 to 12 weeks.*

Contact

- **AWCA** can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ DDHH – NJ Hearing Aid Project Official Guide & Application](#)
- [NJ DHS – PAAD and HAAAD Reimbursement Procedure Guide](#)

- [Horizon NJ Health – Medicaid Hearing Aid Benefit Summary](#)
- [Horizon NJ Health – Hearing Aid Policies and Prior Authorization](#)

Long Term Care Medicaid (MLTSS)

Long Term Care Medicaid is a joint federal and state program for individuals with chronic illnesses, disabilities, or cognitive disorders who cannot care for themselves.

New Jersey's LTC Medicaid is operated by the Managed Long Term Services and Supports (MLTSS) program. Delivered as part of NJ FamilyCare, MLTSS coordinates all services through designated Managed Care Organizations (MCOs), also known as HMOs or health plans. The program's core objective is to provide a comprehensive support system that enables patients to maintain an independent life in a familiar environment for as long as possible.

Key Covered Services

- **Home Care Services:** Household assistance and personal care services provided at the patient's residence.
- **Community Residential Care:** Support for admission to Assisted Living facilities and community-based residential facilities.
- **Nursing Home Care:** Coverage for professional skilled nursing facility (Nursing Home) admission and intensive care.
- **Medical Services:** Hospital visits, specialist consultations, and prescription drug support.
- **Other Support:** Connections to customized long-term care resources based on the patient's specific condition.

LTC Management Systems

When a Medicaid beneficiary requires nursing home admission or long-term care, services are managed through one of the following two pathways:

- **NJ FamilyCare MCO:** Services are managed via selected health plans, where a care manager establishes a personalized care plan.
- **PACE (Program of All-Inclusive Care for the Elderly):** A one-stop integrated program for seniors aged 55+, providing medical and long-term care while prioritizing community living.

Eligibility

Category	Detailed Eligibility Criteria
Target	Individuals 65 or older, or those under 65 determined to have a disability by the SSA or the State of New Jersey.
Income	Monthly Gross Income Limit: \$2,982 (Individual)
Financial Requirements	Asset (Resource) Limit: \$2,000 (Individual) Spousal Impoverishment (Only one spouse applying): • MMMNA: if the spouse's monthly income falls below \$2,643.75 they are entitled to a portion of the applicant spouse's income. • CSRA: spouse living in the community may retain up to \$162,660 in assets.
Clinical Requirements	A formal determination of the need for Nursing Facility Level of Care (NFLOC) Evaluation Criteria: Comprehensive evaluation of Activities of Daily Living (ADLs)—eating, bathing, dressing, toileting, and mobility.

**For higher income, use a Qualified Income Trust (QIT) to meet the eligibility.*

**Countable Assets: Includes cash, checking/savings accounts, stocks, bonds, and secondary properties.*

**Excluded Assets: primary residence, one vehicle, and certain prepaid funeral arrangements are not counted toward the \$2,000 limit.*

**Couple both applying have different financial requirements.*

The Asset Look-back Period

The most rigorous part of the application is the Asset Look-back Period. The state verifies if assets were improperly transferred to qualify for Medicaid.

- **Cash Withdrawal Prohibition:** Unexplained high-value cash withdrawals (generally over \$500) in 5-year bank records are often flagged as 'gifts.'
- **Importance of Records:** Every spend-down expenditure must be backed by receipts and contracts to avoid being judged as an improper transfer.
- **Expert Consultation:** Medicaid rules are complex. Consulting an Elder Law Attorney is the safest way to manage high-value assets.

Application Checklist

Essential Basic Proof Documents

- **Identification:** Citizenship certificate, birth certificate, or Permanent Resident Card (I-551).
- **Residence:** Utility bills or lease agreements showing the current residence address.
- **Social Security:** Copy of the Social Security Card and current health insurance cards (Medicaid, etc.).

Income and Asset Proof Documents (5-Year Records)

- **Income:** Benefit verification letters from the SSA, pension records, and proof of all rental or interest income.
- **Financials:** Full bank statements for the last 60 months for all accounts, including closed or zero-balance accounts.
- **Property:** Property tax bills, mortgage info, life insurance policies (cash value), and vehicle titles.

Clinical and Legal Documents

- **Disability:** Official determination letters from the SSA for applicants under 65.
- **Legal:** Power of Attorney (POA) or guardianship documents, and fair market value proof for any asset movements.

Legal Asset Spend-down Strategies for Eligibility

If assets exceed the \$2,000 limit, applicants may use qualified expenditures to reduce assets to the required level legally.

Modernization of Personal and Household Assets

- **Debt Repayment:** Prioritize the payment of mortgages, credit card debts, unpaid taxes, and other legal loans.
- **Home Improvement:** Necessary repairs such as roof fixing, wheelchair ramp installation, or replacing old appliances in the primary residence.
- **Medical Equipment:** Purchasing devices not covered by insurance, such as specialized beds, wheelchairs, or hearing aids.

Irrevocable Pre-paid Funeral Contracts

Funds deposited into an Irrevocable Prepaid Funeral Trust—typically around \$15,000 to cover standard funeral expenses—are excluded from Medicaid asset calculations, provided they are supported by a detailed statement of goods and services.

- Depositing excessive sums—specifically amounts exceeding \$20,000 to \$30,000—may be flagged as an attempt to “shield” assets and may trigger a Medicaid investigation or audit.

- It is recommended to consult with a licensed funeral professional or an Elder Law Attorney before finalizing any irrevocable contracts.

Payment for Professional Services

Fees paid for legal services (estate planning), accounting, and Medicaid application assistance are considered valid spend-down expenses.

How to Apply

- **Contact your local Aging and Disability Resource Connection (ADRC)** to request an MLTSS application. Simultaneously, apply for financial eligibility (Medicaid) through the Board of Social Services. They will verify if your income and assets meet the required criteria.
- A Managed Care Organization (MCO) representative will visit your home for a clinical assessment. They will evaluate your functional needs, such as how much assistance you require with Activities of Daily Living (ADLs), including bathing, dressing, and mobility.
- Select a health insurance plan. Once you have cleared both the clinical and financial screenings, you will choose an MCO. Options include: Horizon, UnitedHealthcare, Aetna, etc. **The online application ([NJ FamilyCare/quickstart](#)) only addresses **the Financial Screening**. The ADRC consultation and the Clinical Assessment must be coordinated separately. *If you are already enrolled in NJ FamilyCare please contact your MCO's directly to request an MLTSS clinical assessment, rather than contacting your county ADRC.*

Contact

- **Long-Term Care Medicaid (MLTSS):** Contact your local Aging and Disability Resource Connection (ADRC) and the Board of Social Services.
 - Bergen County Division of Senior Services
 - Address: One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - Phone: 201-336-7400
 - Email: seniors@bergencountynj.gov
 - Bergen County Board of Social Services
 - Address: 218 Route 17 North, Rochelle Park, NJ 07662
 - Phone: 201-368-4200
 - NJ ADRC Statewide Toll-Free Number: 1-877-222-3737
- AWCA can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ DMAHS – Official Long-Term Care Medicaid Information](#)
- [National PACE Association – Understanding the PACE Model](#)

Medicaid Managed Care Organization (MCO) System of New Jersey

The Medicaid Managed Care Organization (MCO) system is a healthcare delivery framework designed to manage the cost, utilization, and quality of services provided to beneficiaries.

New Jersey began transitioning from the traditional fee-for-service model to a managed care approach in 1995. Under this system, the state Medicaid agency contracts with MCOs—health insurance companies—to provide a comprehensive package of healthcare services.

This structure aims to improve health outcomes by streamlining the delivery of care and enhancing service quality.

MCO System Framework

New Jersey's Medicaid and public health insurance services are collectively known as **NJ FamilyCare**.

Medicaid recipients in New Jersey enroll in a specific health plan managed by an MCO through the NJ FamilyCare program. The MCO is responsible for coordinating the member's overall medical care, ensuring that services are efficient and accessible.

Key Aspects

- **Structured Network:** Provides better care coordination than traditional methods through a network of verified doctors and hospitals.
- **Value-Added Services:** MCOs have the authority to offer additional benefits (such as dental, vision care, and health incentives) that go beyond the standard requirements of regular Medicaid.
- **Integrated Care:** There is an increased emphasis on the integration of behavioral health and physical health services, ensuring beneficiaries receive more comprehensive care.

List of New Jersey Managed Care Providers

Five major health plans participate in the NJ FamilyCare program. These insurers provide services in all 21 counties in the state, including Bergen, Hudson, and Essex.

**Fidelis Care does not provide services in Hunterdon County.*

- **Aetna Better Health of New Jersey**
 - Address: 3 Independence Way, Suite 400, Princeton, NJ 08540

- Member Services: 1-855-232-3596 (Available 24/7)
- TTY for Hearing Impaired: 711
- Website: aetnabetterhealth.com/newjersey
- **Fidelis Care (formerly WellCare of New Jersey)**
 - Address: 33 Washington Street, 1st Floor, Newark, NJ 07102
 - Member Services: 1-888-453-2534 (Mon-Fri, 8 AM–6 PM)
 - TTY for Hearing Impaired: 711 or 1-877-247-6272
 - Website: fideliscarenj.com
- **Horizon NJ Health**
 - Address: 1700 American Blvd., Pennington, NJ 08534
 - Member Services: 1-800-682-9090 (Mon-Fri, 8 AM–8 PM)
 - Provider Only: 1-800-682-9091
 - TTY for Hearing Impaired: 711
 - Website: horizonnjhealth.com
- **UnitedHealthcare Community Plan**
 - Address: 333 Thornall St, 9th Floor, Edison, NJ 08837
 - Member Services: 1-800-941-4647
 - Provider Only: 1-888-362-3368 (Mon-Fri, 8 AM–6 PM)
 - TTY for Hearing Impaired: 711
 - Website: uhccommunityplan.com/nj
- **Wellpoint (formerly Amerigroup New Jersey)**
 - Address: 101 Wood Avenue South, 8th Floor, Iselin, NJ 08830
 - Member Services: 1-800-600-4441 / Provider Only: 1-800-454-3730 (Mon-Fri, 8 AM–6 PM)
 - TDD: 1-800-852-7899
 - Website: wellpoint.com/nj

Resources

- [Medicaid.gov](https://www.medicaid.gov) – Official Medicaid Program Overview
- [NJ Department of Human Services – Managed Care Organization Guide](#)

Pregnancy Medicaid

Medicaid for pregnant women is administered by each individual state, and New Jersey's pregnancy Medicaid is operated as follows:

NJ FamilyCare/Medicaid provides essential health insurance to pregnant women residing in New Jersey. This program ensures that expectant mothers have access to the medical care necessary for a healthy pregnancy and delivery. Benefits cover all medical costs for the mother's health from early pregnancy through the first year after childbirth.

Eligibility

- **Residency:** current residents of New Jersey.
- **Immigration Status:** U.S. citizenship or lawful immigration status. (DACA not eligible)
- **Income Limit:** Total household income at or below 205% of the Federal Poverty Level (FPL).

Income Limit (2026)

**For pregnant women, the fetus is included in the household size (e.g., a single pregnant woman is considered a 2-person household)*

Household Size (Including Fetus)	Monthly Income Limit	Annual Income Limit
2 Persons (1 pregnant individual)	\$3,697	\$44,362
3 Persons	\$4,668	\$56,006
4 Persons	\$5,639	\$67,650
Each Add'l	+ \$971	+ \$11,644

Benefits and Extension of Care

NJ FamilyCare provides comprehensive medical services throughout all stages of pregnancy to ensure healthy childbirth and postpartum recovery.

Major Medical Coverage

Service Category	Detailed Coverage Content
Prenatal Care	Regular checkups, ultrasounds, genetic testing, prenatal vitamins, prenatal education
Delivery and Birth	Vaginal delivery, C-section, epidurals, all hospital inpatient fees, newborn care
Postpartum Care	Post-birth checkups, breastfeeding counseling, mental health (postpartum depression) support
Other Essential Benefits	Prescription drugs, dental care, vision care, specialist visits, mental health and substance abuse treatment

Postpartum Extension

Pregnancy Medicaid recipients maintain their insurance eligibility for **12 months (365 days)** after childbirth, regardless of changes in income.

- **Extension Period:** Coverage is provided for 12 months from the date of childbirth or the end of the pregnancy. (including miscarriage and stillbirth)
- **Maintenance of Benefits:** All comprehensive benefits received during pregnancy remain identical during the postpartum period.
- **Automatic Renewal:** Once approved under the pregnancy category, coverage continues for one year postpartum without a separate income redetermination.

Special Support for Older and High-Risk Pregnancies

Starting in 2026, additional monitoring and care management services for older pregnant women and high-risk pregnancy groups have been strengthened. Care management services for high-risk pregnant women are provided through MCOs (Managed Care Organizations).

OB/GYN Assignment and First Appointment

The first thing to do after Medicaid approval is to find an obstetrician/gynecologist (OB/GYN) within the network of insurance company

- **Check Insurance Network:** check the list of OB/GYNs that accept Medicaid through MCO website or customer center.
- **Use Care Management Services:** once the insurance company is informed of pregnancy, they will assign a Care Manager. Care Manager determines high-risk status and help with appointment scheduling and transportation support (Non-Emergency Medical Transportation).
- **What to Bring to First Visit:** bring NJ FamilyCare card and ID; during the special approval period for pregnancy, there are no separate copays.

Dental Care for Pregnant People

Service Item	Coverage Content
Regular Exams and Cleaning	Coverage for two basic cleanings and comprehensive exams per year
Periodontal Treatment	Support for deep cleaning (scaling) for gum disease that may worsen during pregnancy
Cavity Treatment	Fillings and crown treatments if necessary
Emergency Care	Extractions or root canals due to sudden toothaches

Additional Medicaid-Related Benefits

Each Managed Care Organization (MCO) provides incentives in addition to basic Medicaid benefits to encourage healthy childbirth (Benefits and incentive types may vary by MCO).

- **Rewards Cards:** Gift cards (\$25–\$50) for retailers like Walmart or CVS for attending prenatal or postpartum checkups.
- **Free Baby Supplies:** Providing items such as breast pumps, car seats, or diaper bags for free or via discount coupons.
- **Transportation Support:** Free taxi service or bus cards for hospital visits.

Alternative Programs

Even if you do not qualify for Pregnancy Medicaid, several assistance programs are available for pregnant residents in New Jersey to ensure healthy childbirth.

Major Alternative Options

Program Name	Target Audience and Purpose	Key Support Content
GetCoveredNJ	Individuals exceeding Medicaid income limits	Affordable private insurance through state subsidies
MEPP (Emergency)	Undocumented individuals and non-citizens	Full coverage of emergency hospital costs for labor and delivery
NJSPCP (Prenatal)	Individuals meeting NJ FamilyCare income criteria but not immigration status	Prenatal exams, ultrasounds, vitamins, and 60-day postpartum contraception
FQHC (Health Center)	Everyone regardless of income/status	Low-cost medical services based on a sliding scale
Charity Care	Low-income uninsured patients	Full or partial reduction of hospital and delivery costs

How to Apply

- **Online:** [NJ FamilyCare/quickstart](#)
 - Contact: 1-800-701-0710 / TTY: 711
- **In-Person:** visit local County Board of Social Services
 - Bergen County, NJ Board of Social Services
 - 218 Route 17 North, Rochelle Park, NJ 07662
 - 201-368-4200
 - Hours: Weekdays 8:00 am thru 4:30 pm

**Enrollment is open year-round for NJ FamilyCare*

Contact

- AWCA can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ Department of Human Services – Pregnant People Medicaid Guide](#)
- [NJ FamilyCare – Eligibility for Pregnant Women](#)

ABD Medicaid (NJ FamilyCare ABD)

The NJ FamilyCare ABD (Aged, Blind, Disabled) program, also known as Regular Medicaid, is a comprehensive healthcare initiative jointly funded by the federal and state governments. It provides essential medical coverage to New Jersey residents aged 65 or older, as well as those determined to be blind or disabled by the Social Security Administration (SSA) or the State of New Jersey.

The ABD program is not a single entity but consists of several sub-programs, including SSI-Related Medicaid, NJ Care Special Medicaid, Medicaid Only, NJ WorkAbility, and Long Term Care Medicaid (MLTSS). The specific program for which an individual qualifies depends on their income, asset levels, and personal circumstances.

Eligibility (2026)

- **Age/Status:** Must be 65+, blind, or disabled.
- **Income Limit:** \$1,330/month (Single) | \$1,804/month (Couple).
- **Asset Limit:** \$4,000 (Single) | \$6,000 (Couple).

Benefits

- **Primary and Specialist Care:** Routine physical exams, immunizations, primary care physician (PCP) visits, and specialist consultations across all medical fields.
- **Dental Services:** Routine examinations, X-rays, fillings, extractions, and other essential dental treatments.
- **Vision and Hearing:** Coverage includes regular eye exams, prescription eyewear, and hearing screening services.
- **Prescription Drug Benefits:** Access to a wide range of prescription medications through an extensive network of participating pharmacies.
- **Mental Health Services:** Comprehensive outpatient and inpatient counseling services for mental health maintenance and substance abuse recovery.

**Dual Eligibility: Individuals enrolled in Medicare may also apply for Regular Medicaid. Upon qualification, Medicaid provides assistance with Medicare premiums, copays, and deductibles.*

**Automatic Eligibility: Supplemental Security Income (SSI) recipients are automatically eligible for ABD Medicaid and are not required to submit a separate application.*

How To Apply

- **Online:** [NJ FamilyCare/quickstart](#)
 - Contact: 1-800-701-0710 / TTY: 711
- **In-Person:** visit local County Board of Social Services
 - Bergen County, NJ Board of Social Services
 - 218 Route 17 North, Rochelle Park, NJ 07662
 - 201-368-4200
 - Hours: Weekdays 8:00 am thru 4:30 pm
- **Mail:** [ABD program Application Form \(PDF\)](#)
 - Complete form and mail to local Board of Social Services

**Enrollment is open year-round for NJ FamilyCare*

Contact

- **AWCA** can provide counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ FamilyCare – Official Eligibility Requirements Guide](#)
- [NJ.gov – County Social Service Office Location Search](#)
- [NJ DHS – WorkFirst NJ \(WFNJ\) Cash Assistance Application Guide](#)
- [NJ DHS – NJ SNAP Food Stamp Application Official Guide](#)

PART I • SECTION 5

Medicaid Specials

메디케이드 특별 플랜 (Medicaid Specials)

Specialized Medicaid safety nets: Dual Eligible Special Needs Plans (D-SNP), Spousal Impoverishment protection, Charity Care hospital assistance, Emergency Medical Assistance, WFNJ/GA & TANF, Funeral Assistance, Medicaid Buy-In for disabled workers, and Medicaid Estate Recovery.

This section contains 9 comprehensive guides

Charity Care / Hospital Care Payment Assistance

The **New Jersey Hospital Care Payment Assistance Program** (also known as **Charity Care**) is a medical bill reduction program provided by state-designated acute care hospitals and university hospitals for low-income individuals who are uninsured or underinsured.

The New Jersey Hospital Care Payment Assistance Program is regulated by the **NJ Department of Health** and is funded by the state; however, the application process and eligibility screening are conducted directly by the hospital where the treatment was received.

Eligibility

- **Residency & Insurance Status:**

- Must be uninsured or have insurance that leaves the patient with excessive out-of-pocket costs.
- Individuals who are ineligible for government-subsidized insurance, such as **Medicaid**, or private health insurance.

- **Assets:**

- **Individual:** \$7,500 or less
- **Family:** \$15,000 or less

**Note: One's primary residence is excluded from asset calculations. However, cashable retirement accounts such as IRAs may be included in asset calculations (N.J.A.C. 10:52-11.10). For non-withdrawable accounts like certain 401(k)s, individual verification with the hospital is required.*

Income Levels (2026)

Income Level (% of Federal Poverty Level)	Patient Responsibility	Assistance Type
200% FPL or Below	0% (Fully Waived)	Free Care
201% – 225% FPL	20%	Partial Discount
226% – 250% FPL	40%	Partial Discount
251% – 275% FPL	60%	Partial Discount
276% – 300% FPL	80%	Partial Discount

**Patients with incomes between 201% and 300% of the FPL (eligible for partial discounts) can receive 100% additional coverage for any out-of-pocket medical expenses that exceed 30% of their annual income.*

Considerations

- Applied to medically necessary inpatient and outpatient services provided at **acute care hospitals**. Cosmetic or elective procedures are excluded.
- **Separate Physician Bills:** While hospital facility charges may be reduced, you may receive separate bills from independent specialists (e.g., anesthesiologists, radiologists) who are not hospital employees.
- **Medicaid Screening:** Hospitals will first screen for Medicaid eligibility. If you qualify for Medicaid, you must apply for it before being considered for Charity Care.

Application & Contact

- **Where to apply:** Financial Counseling or Patient Billing Department at any general hospital.
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ DOH.gov – Official Charity Care Information](#)
- [NJ DOH.gov – 2026 Hospital Care Assistance Fact Sheet](#)
- [DollarFor.org – NJ Charity Care Eligibility & FPL Calculator](#)

Dual Eligible Plan (D-SNP)

A **Dual Eligible** refers to individuals who have **Medicare**, the health insurance for seniors age 65 or older and people with disabilities, while simultaneously receiving **Medicaid**, medical assistance for low-income individuals.

Dual eligibles can receive more efficient medical services by enrolling in a **D-SNP (Dual Special Needs Plan)**, which integrates the benefits of both insurances into one. Under federal guidance, D-SNP plans are managed according to unique names and operating policies in each state.

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Three Types of D-SNPs

D-SNPs are divided into three types based on the level of integration, with FIDE SNP being the highest level of full integration.

- **CO D-SNP (Coordination-Only):** A basic plan that coordinates Medicare and Medicaid benefits. The management entities for the two insurances remain separate.
- **HIDE SNP (Highly Integrated):** A highly integrated plan where a single insurance carrier manages most Medicare and Medicaid services.
- **FIDE SNP (Fully Integrated Dual Eligible SNP):** A fully integrated plan where a single insurance carrier provides 100% integrated management of Medicare and Medicaid (including MLTSS long-term care services) benefits. In New Jersey, major carriers operate FIDE SNPs, and available plans may vary by county.

Similar Program Names by State:

- New Jersey, Massachusetts, etc.: FIDE SNP
- New York: MAP (Medicaid Advantage Plus) — A New York-specific integrated plan combining D-SNP and Medicaid Long-Term Care (MLTC)
- California: Medi-Medi Plans (Official Name)

Eligibility

- **D-SNP (CO D-SNP / HIDE SNP):** Anyone who is dually eligible for both Medicare and Medicaid can consider enrollment. Receiving **MLTSS** (long-term care) is not required.
- **FIDE SNP (New Jersey Criteria):** Must meet all four of the following conditions:

- Hold Medicare Parts A and B
- Hold full NJ Medicaid (NJ FamilyCare)
- Not enrolled in a PACE program
- Resident of New Jersey

**Receiving MLTSS (Long-Term Care) itself is not a mandatory requirement for FIDE SNP enrollment. However, those receiving MLTSS can also join a FIDE SNP to have all services managed in one place.*

D-SNP Benefits

D-SNPs coordinate Medicare and Medicaid benefits and include various additional services linked to state Medicaid.

Minimizing Medical and Drug Costs (Cost Sharing)

- **Premium:** For those receiving full Medicaid medical benefits, there is no monthly premium, and there are no out-of-pocket costs (copays) for hospital visits or tests.
- **Copay:** Generally, there are no out-of-pocket costs for **In-Network** doctor visits, specialist visits, tests, or hospitalizations. However, Medicaid beneficiaries who only receive premium assistance (QI, SLMB, etc.) from the state may pay some copays.
- **Prescription Drugs:** Through Medicare Part D **Extra Help**, most prescription drug copays are eliminated, regardless of annual limits.

Benefits Card (OTC & Grocery)

- **OTC (Over-the-Counter) Support:** Monthly or quarterly credits are provided to purchase non-prescription drugs, vitamins, and first-aid supplies. Amounts may be slightly lower than FIDE SNP.
- **Grocery & Utilities:** Provided only to eligible individuals with **chronic conditions** (diabetes, hypertension, etc.) to pay for groceries or electricity/gas bills.

Supplemental Medical Benefits

- **Dental:** Provides comprehensive dental benefits, including not only basic exams but also fillings, crowns, dentures, and implants within an annual limit.
- **Vision:** Supports annual eye exams and a set amount for prescription glasses.
- **Hearing:** Supports hearing exams along with the cost of hearing aids and fitting.

Transportation & Wellness

- **Medical Transportation:** Provides a set number of rides (Uber/Lyft or dedicated vans) for doctor appointments and pharmacy visits.
- **Fitness:** Provides free gym memberships such as **Silver&Fit** or **SilverSneakers**.

- **24-Hour Nurse Line:** Provides telephone consultation services with professional nurses 24/7 for urgent health advice.

FIDE SNP Benefits

FIDE SNP benefits from major New Jersey carriers (**Horizon, UnitedHealthcare, Aetna, Wellcare**, etc.) are as follows:

Medical Costs (Cost Sharing)

- **Copay:** Doctor visits, specialist visits, and hospitalization costs are fully waived.
- **Prescription Drugs:** Most prescription drug copays are eliminated, including Medicare Part D (Extra Help).

Integrated Card Benefits (OTC & Grocery)

- **OTC (Over-the-Counter) Support:** Monthly or quarterly amounts are loaded onto a card to purchase supplements and non-prescription drugs at CVS, Walgreens, etc.
- **Grocery & Utilities:** Most FIDE SNP plans provide an integrated card for purchasing groceries or paying utility bills (electricity, gas, water). (Approximately **\$150–\$300** per month depending on the plan).

Supplemental Benefits

- **Dental:** Covers comprehensive dental care, including implants, dentures, and scaling within an annual limit.
- **Vision:** Supports annual eye exams and the cost of purchasing glasses/contact lenses.
- **Hearing:** Provides hearing exams along with hearing aid costs and fitting services.

Transportation & Wellness

- Provides **unlimited or a set number** of rides (Uber/Lyft or dedicated vans) for hospital visits or pharmacy use.
- **Free Fitness Membership:** Access to local gyms and the YMCA, home fitness kits, and online exercise classes through **SilverSneakers** or **Silver&Fit**.
- **Healthy Rewards:** Rewards users with gift cards or cash-equivalent points for completing preventive activities such as regular checkups, flu shots, and blood sugar tests.
- **In-home Visits:** Provides the '**HouseCalls**' service where a specialist visits your home annually to check your health status and create a personalized care plan.
- **24-Hour Nurse Consultation:** Free phone consultations with professional nurses are available 24/7 for health inquiries or emergencies.
- **Wellness Digital App:** Access to real-time health tracking and wellbeing coaching (diet, exercise, etc.) via a smartphone app.

Key Differences

Category	D-SNP (CO/HIDE)	FIDE SNP
Key Features	Medicare and Medicaid are operated by separate entities, with the insurance company acting as a bridge.	A single insurance company integrates all services and finances, directly managing all decisions.
Ease of Management	Management of medical and long-term care (MLTSS) is separate, and you may need to carry separate Medicare and Medicaid cards.	Integrated Management (Access hospital, pharmacy, and caregiver services all at once with a single ID card).
Communication	You may need to contact the insurance carrier for medical inquiries and the county for social services separately.	One-Stop Service through a dedicated Care Manager who handles both medical and social service needs.
Dental/Vision	Very High (Relatively generous limits for implants and vision hardware allowances).	Moderate (Focuses on standard Medicaid coverage; additional limits may be lower).
Benefit Card	Usage is often restricted to OTC items rather than groceries. Chronic condition verification is required.	Higher limits for groceries and utilities (usually \$50–\$100 more per month). Higher combined amounts (up to approx. \$300/month).
Transportation	Primarily focused on medical purposes. Limited to a set number of trips (e.g., 24–48 times per year).	Unlimited or broader support. Available for groceries, dental/vision visits, and affiliated gyms as well as hospitals.

2026 Changes & Reminders (OBBBA Updates)

- Alignment: Federal and state governments (CMS/NJ DHS policies) actively encourage enrollment in FIDE SNPs, which unify Medicare and Medicaid carriers into one for administrative efficiency.
- Automatic Transition: Some D-SNP enrollees may be automatically transitioned to a FIDE SNP with enhanced benefits according to state guidelines; you must check enrollment notices sent by mail.
- Monthly Switching: Starting in 2025, full-benefit dual eligibles have a Monthly Integrated Care SEP (Special Enrollment Period) that allows switching between integrated plans (FIDE SNP, HIDE SNP, etc.) every month.

If you are receiving MLTSS, choose a plan based on your priorities:

- If a wider doctor network or higher dental surgery limits are important → D-SNP
- If cash-equivalent support (grocery card) and convenient administration are important → FIDE SNP

How to Apply

D-SNP

- **Online:** Medicare.gov — Find a Medicare Plan or individual insurance carrier websites.
- **Phone:** 1-800-MEDICARE (1-800-633-4227; 24/7) or individual insurance carrier phone numbers.

**Benefits generally begin on the 1st of the month following approval.*

FIDE SNP (New Jersey)

FIDE SNP is a fully integrated dual-eligible plan that manages Medicare and Medicaid benefits in one health plan. Choose the method that fits your situation:

- **To enroll in a FIDE SNP without MLTSS (Long-Term Care):** Contact 1-800-MEDICARE (1-800-633-4227) or contact the specific insurance carriers listed below directly.
- **If you also need MLTSS (Long-Term Care):** Contact your local county **ADRC** first for an **MLTSS clinical screening**. Once approved, request integration with a FIDE SNP carrier.

Bergen County Residents: Contact the Bergen County Division of Senior Services.

- Address: One Bergen County Plaza, Hackensack, NJ 07601
- Phone: 201-336-7400 | 1-877-222-3737
- Email: seniors@bergencountynj.gov

**Once MLTSS is approved, you must contact your chosen FIDE SNP insurance carrier and request to "Fully Integrate" your Medicare and Medicaid benefits with that carrier.*

Online/Phone: Individual FIDE SNP carriers (refer to the comparison table below or the official [NJ DHS page](#)).

**Once the process is complete, Medicaid management will automatically transfer from the state to your chosen FIDE SNP carrier. Keep your old cards, but use the single ID card issued by the FIDE SNP for all benefits.*

FIDE SNP Overview by Insurer (2026)

Carrier	D-SNP (Plan Name)	FIDE SNP (Plan Name)	Contact & Website	2026 Key Features
Horizon BCBS	Horizon NJ TotalCare	Horizon NJ TotalCare	1-800-543-5656 (TTY: 711) / Horizonblue.com	New Jersey's largest network. Provides up to \$3,000+ in annual benefits via the Horizon &more Card.
UnitedHealthcare	UHC Dual Complete	UHC Dual Complete	1-800-514-4911 (TTY: 711) / UHC.com	Enhanced UCard benefits. Provides approximately \$246+ in monthly credits for groceries/utilities, etc.
Aetna	Aetna Medicare D-SNP	Aetna Medicare FIDE	1-844-362-0934 (TTY: 711) / AetnaBetterHealth.com/NJ	Strong CVS pharmacy network. Provides an additional benefit card of approximately \$255 per month.
Wellcare (Fidelis)	Wellcare Dual Access	Fidelis Dual Align	1-866-892-8340 (TTY: 711) / Fideliscare.org	Expertise in Medicaid management. Excellent approval and coordination processes for MLTSS (Long-Term Care) services.
Wellpoint	Wellpoint Dual Advantage	Full Dual Advantage	1-844-765-5160 (TTY: 711) / Wellpoint.com	Community-focused service. Concentrates on simplified approval procedures and practical supplemental services.

*Carrier contact info and plan names may change annually. Check [Medicare.gov](https://www.medicare.gov) or [FIDE SNP Consumer and Provider Information](#) for the latest info.

Contact

- **NJ SHIP (State Health Insurance Assistance Program):** 1-800-792-8820 (Free counseling from state-trained experts)
- **AWCA** provides free consultations regarding these plans: (201-862-1665 / info@awcanj.org)

Resources

- [NJDHS.gov – FIDE SNP](https://www.njdhs.gov/fide-snp)
- [Medicare.gov – Medicare & You 2026 Handbook](https://www.medicare.gov/medicare-and-you/2026-handbook)
- [CMS.gov – D-SNP](https://www.cms.gov/d-snp)
- [ACL.gov – ADRC](https://acl.gov/adrc)

Emergency Medical Assistance (Emergency Medicaid)

Emergency Medicaid provides financial support through the **Medical Emergency Payment Program (MEPP)** and State Medicaid to ensure that low-income residents can receive treatment during emergencies, regardless of immigration status or insurance coverage.

Medical Emergency Payment Program (MEPP)

MEPP covers emergency room and labor/delivery costs for residents who do not qualify for regular Medicaid or adults who have held a Green Card for less than 5 years.

Eligibility

- **Immigration Status:** New Jersey residents who do not meet the status requirements for U.S. citizenship or Lawful Permanent Residency (5-year bar), but otherwise meet the **Medicaid income and asset eligibility criteria**.
- **Application Deadline:** Application must be filed within **3 months** of the date the emergency occurred.
- **Scope of Coverage:** Life-threatening emergencies, labor and delivery, and related inpatient costs.
- **Monthly Income Limits:**
 - **Adults (Ages 19-64):** Approx. **\$1,800** (Single) / **\$2,433** (Family of 2) — *Based on 138% FPL*
 - **Pregnant Women:** Approximately **\$2,647** for a single-person household (based on **199% of the FPL**).
 - **Seniors (65+) / Disabled:** Monthly income of **\$1,305** and an asset limit of **\$4,000** for a single-person household; for a two-person (couple) household, a monthly income of **\$1,763** applies, with a separate asset limit of **\$6,000**.

NJ FamilyCare & Cover All Kids

- **Target Population:** All children under 19 years of age residing in New Jersey, regardless of immigration status.
- **Income Limit:** Household income up to 355% FPL (Approx. **\$9,512/month** for a family of 4 – Deductibles may vary depending on the income bracket).
- **Benefits:** Comprehensive benefits including general checkups, dental, vision, and mental health, in addition to emergency care.

*See [CHIP](#) for more details.

NJ Supplemental Prenatal and Contraceptive Program (NJSPCP)

- **Target Population:** Pregnant women who are ineligible for **NJ FamilyCare** due to their immigration status.
- **Services Provided:** Prenatal exams, contraceptive/family planning services. (Does not cover labor, delivery, or postpartum care)
- **MEPP Linkage:** As the due date approaches, you can apply for MEPP in advance through your prenatal care provider to cover labor and delivery costs.

*See [Pregnancy Medicaid](#) for more details.

WIC Supplemental Nutrition Program

The New Jersey Supplemental Nutrition Program for Women, Infants, and Children (WIC) provides essential food and nutrition education to all low-income pregnant individuals. Managed by the State of New Jersey, this program operates independently from Medicaid and is available to those in need regardless of immigration status.

- **Eligibility:** Low-income pregnant and postpartum individuals, regardless of immigration status
- **Benefits:** Vouchers for essential nutritious foods (such as milk, eggs, and cheese) and professional nutrition education/counseling
- **Key Note:** Can be utilized alongside medical assistance programs such as NJSPCP. WIC is a supplemental initiative dedicated to supporting healthy pregnancies and infant development.

*See [NJ Official WIC Program](#) for more details.

Key Notes

- **Eligibility Review:** All enrollees must renew annually. If home address changes, the enrollee must contact **1-800-701-0710** to update records and prevent a lapse in coverage.
- **Hospital Cooperation:** Upon visiting an Emergency Room, patient or representative must clearly inform the hospital social worker or financial counselor of the **intent to apply for MEPP**.
- **Post-Emergency Application:** If an application was not completed at the hospital, the applicant must visit the local **County Board of Social Services** to submit an application **within 3 months** of the emergency.

Contact

- **General Application & Counseling:** NJ FamilyCare Hotline 1-800-701-0710
- **MEPP (Emergency Payment):** 1-800-356-1561

- **Eligibility Self-Screening:** [NJHelps.gov](https://njhelps.gov/)
- **Renewal & Status Checks:** [Stay Covered NJ](#)
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ DHS.gov – Medical Emergency Payment Program \(MEPP\)](#)
- [NJ FamilyCare.gov – Cover All Kids](#)
- [NJ DHS.gov – County Social Service Agencies](#)

Funeral & Burial Assistance

State's Department of Human Services (DHS) and county social service agencies provide financial assistance for funeral, burial, or cremation costs for residents who were receiving specific public assistance programs at the time of death, or for indigent residents without assets.

Eligibility

The deceased must meet one of the following criteria at the time of death:

- WFNJ/GA beneficiaries who had been receiving benefits continuously for at least **6 months** at the time of death.
- WFNJ recipients who passed away within the first 6 months of residency in a New Jersey public institution (excluding correctional facilities).
- New applicants whose benefits were approved within 15 days after their death.
- Recipients of community care programs (such as CCPED, AACAP, or Adult Foster Care).
- Participants in the NJ Workability Program.

**Note: Since there are various other eligibility categories, please contact your County Board of Social Services for specific inquiries.*

NJ DHS Public Assistance Burial Allowance Rates (2026)

Age Group	Funeral Allowance	Cemetery/Cremation Allowance	Total State Payment	Max Family Supplement	Max Permitted Total
Adults & Children (2+ Years)	\$3,056	\$714	\$3,770	\$1,570	\$5,340
1 Week to Under 2 Years	\$2,494	\$583(approx.)	\$3,077 (approx.)	\$1,178	\$4,255
Stillborn to 6 Days	\$1,933	\$452	\$2,385	\$785	\$3,170

**Family Supplement: According to regulations, family members may contribute up to the specified amount to supplement the cost of the funeral services without affecting the state's contribution.*

**These rates are based on official NJ DHS regulations as cited by providers such as Whigham Funeral Home.*

Coverage and Limitations

- **Covered Items:**
 - Body preparation and disposition, casket, or urn
 - Use of funeral home facilities and basic services
 - Cemetery burial plot costs or cremation fees
- **Excluded Items (Non-Covered Items):**
 - Fees for clergy, church use, or memorial hall rentals
 - Costs for publishing obituaries or death notices
 - Post-cremation expenses (such as scattering of ashes, niche interments, or other costs incurred after the cremation process)
 - Other items not explicitly listed above
- **Exclusions and Deductions:**
 - If the deceased leaves behind any **assets** (such as bank balances, life insurance, securities, real estate, or payments issued before or after death), the state will **recoup** these funds. However, this does not affect the actual allowance paid to the funeral director and the cemetery.
 - If there is a pre-established **Irrevocable Prepaid Funeral Account**, those funds must be utilized first to cover funeral expenses. Any remaining balance after the funeral costs have been paid is subject to **recoupment by the state**.

Key Notes

- **Prior Approval Required:** It is very difficult to receive reimbursement after funeral costs have already been paid in full. The responsible party **must** request the **Funeral Director** to apply for **Public Assistance Funeral** before proceeding with the services.
- **Asset Investigation:** The state will investigate the deceased's bank accounts. If the balance exceeds the support amount, the application may be denied.
- **Supplement Limit:** If family members pay an amount exceeding the **Maximum Family Supplement (\$1,570)**, the state's contribution will be reduced by the exact amount of that excess.

Example: If the family pays **\$1,670** as a supplement (**\$100 over the limit**), the state's total funeral allowance for an adult will be reduced from **\$3,770** to **\$3,670**.

How to Apply

Before or during the funeral arrangements, the Funeral Director must request eligibility verification and approval from the respective county on behalf of the family.

Contact

- **Local county social service offices**
 - New Jersey directory by county: [NJ DHS.gov – County Social Service Agencies](#)
- **NJSFDA (NJ State Funeral Directors Association):** 1-800-734-3712 | njsfda@njsfda.org
- **Applying for the Social Security Lump-Sum Death Payment (\$255):**
 - Eligibility: If the deceased had a Social Security work history, a one-time payment of \$255 may be payable to a surviving spouse or child who meets specific eligibility requirements. If there is no eligible spouse, the payment may be made to a child who meets certain criteria.
 - Application Deadline: Within 2 years from the date of death.
 - How to Apply: Call the SSA at 1-800-772-1213 or visit your local Social Security Administration (SSA) office.
 - Social Security Administration website | [SSA.gov](#)
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJSFDA \(NJ State Funeral Directors Association\) Financial Assistance Guide](#)
- [Penn Jersey Cremation – Medicaid Cremation Assistance Guide](#)
- [Woody Home for Services – Public Assistance Benefits](#)

Medicaid Buy-In (NJ WorkAbility)

NJ WorkAbility is a program that allows New Jersey residents with a disability who are currently working to receive full **Medicaid** benefits, even if their income or assets exceed the limits for standard Medicaid (NJ FamilyCare).

**SSDI recipients can also use this program to bridge the health insurance gap during the 24-month waiting period before they become eligible for Medicare.*

**While Medicare eligibility is based on a 24-month period starting from the date you begin receiving SSDI, the SSDI payments themselves do not start until after a 5-month waiting period from the established onset of the disability. Therefore, the actual total waiting period is approximately 29 months from the date the disability began.*

Eligibility

- **Age:** 16 years or older (Individuals aged **65 or older** may apply; however, the disability determination must have been established **before the age of 65**).
- **Disability:** Must have a disability determination from the **Social Security Administration (SSA)** or the **Medical Review Team** under the **DMAHS (Division of Medical Assistance and Health Services)**. If you are already receiving **SSDI**, you automatically meet the disability requirement without needing a separate determination.

**Note: If you are not currently receiving SSDI, you can contact the NJ DDS (Division of Disability Services) at 1-888-285-3036 to pre-verify your eligibility.*

- **Employment:** Must be employed part-time, full-time, or self-employed (Proof of earned income is required).
- **No Asset Limit:** Eligible regardless of the size of assets such as savings or stocks.
- **No Income Limit:** High-income earners can join (However, premiums apply based on income level).

**Income Calculation Method: Earned income and unearned income are calculated using different formulas. Since your actual premium bracket may vary based on your specific circumstances, please contact NJ DDS (1-888-285-3036) or your County Board of Social Services for an individual assessment to verify your exact eligibility and premium amounts.*

- **Spousal Income Excluded:** Eligibility and premiums are determined solely based on **the applicant's own income**.

Medicaid Buy-In Premiums (2026)

Tier Level	Income Bracket (vs. FPL)	Monthly Premium	Notes
Exempt	250% or less	None (\$0)	If you have no unearned income (such as SSDI, interest, or investment returns), you may be exempt from premiums for annual earned income up to \$80,820.
Tier 1	251% – 350%	\$203	
Tier 2	351% – 450%	\$406	
Tier 3	451% – 550%	\$609	
Tier 4	551% – 650%	\$812	
Tier 5	651% – 750%	\$1,015	
Tier 6	Over 750%	\$1,218	Maximum premium cap

** Note: The salary figures mentioned above assume zero unearned income and are examples based on the application of the earned income disregard (approximately 50%). While SSDI benefits based on your own work history are excluded from the income calculation, benefits based on others—such as a spouse's or parent's disability or survivors' benefits—are counted as unearned income. Applicants are encouraged to check eligibility as unearned income (e.g., interest, investment returns, or alimony) may change your actual premium exemption threshold.*

Key Benefits

- **Comprehensive Medicaid Coverage:** Includes hospital bills, prescription drugs, and all other standard Medicaid benefits.
- **Managed Long-Term Services and Supports (MLTSS):** It provides coverage for **Personal Care Assistant (PCA)** services, which are typically not covered by private insurance. The number of service hours provided is determined through an individual **care assessment** based on the recipient's specific needs.
- **Job Retention Support:** Even if your salary increases or you become a high-income earner, you can maintain full Medicaid coverage simply by paying the designated premium.
- **Coverage Maintenance After Job Loss:** If you lose your job through no fault of your own (e.g., layoffs, business closures, etc.), your **Medicaid coverage** will be maintained for up to **12 months** following the

loss of employment.

How to Apply

- [NJHelps.gov](https://njhelps.gov) (**Benefit Screening Portal**): You can pre-check your eligibility for various benefits, such as **SNAP**, **TANF**, and **NJ FamilyCare**, in approximately **5 to 10 minutes**. Please note that the actual application must be submitted through [MyNJHelps.gov](https://mynjhelps.gov).
- [NJFamilyCare.gov](https://njfamilycare.gov): **ABD** (Dedicated application path for **NJ WorkAbility**. You may apply online or in person at your local **County Board of Social Services**.)
- [GetCoveredNJ.gov](https://getcoverednj.gov) (If you do not qualify for Medicaid, you can enroll in a private health plan through the **ACA (Affordable Care Act) Marketplace** and receive a **Premium Tax Credit** to help lower your cost.)
- [Stay Covered NJ.gov](https://staycoverednj.gov) – [Website for current members' keeping coverage](#)

Contact

- **NJ 211:** 211 / nj211.org
- **NJ DDS:** 1-888-285-3036 / [NJ DDS.gov](https://njdds.gov)
- **NJ FamilyCare:** 1-800-701-0710 / [NJ FamilyCare.gov](https://njfamilycare.gov)
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org).

Medicaid Estate Recovery

The Medicaid Estate Recovery Program (MERP) seeks to recover the costs of Medicaid benefits paid on behalf of a recipient aged 55 or older from the deceased recipient's estate.

Target and Scope of Recovery

New Jersey utilizes an **Expanded Estate** definition, which covers a broader range of assets beyond just **Probate Assets**. This means that assets such as joint accounts, life insurance, and retirement pensions—which might otherwise pass directly to heirs without a will—can be subject to recovery.

- **Included Assets:**

- **Real Estate:** The home (including Primary Residence) and land held in the deceased's name.
- **Financial Assets:** Bank accounts, stocks, bonds, and cash.
- **Non-Probate Assets:** Life insurance proceeds (if paid to the estate), annuities, trust assets, and jointly owned property.

**Notice Regarding Life Insurance: Life insurance proceeds paid directly to a designated beneficiary are generally not subject to estate recovery. However, recovery will apply in the following cases:*

- *Estate as Beneficiary: If the estate itself is named as the beneficiary.*
- *Lapsed Beneficiary: If the designated beneficiary predeceases the recipient, causing the insurance proceeds to revert to the estate.*

- **Recoverable Amount:** All medical costs paid by Medicaid after the recipient reached age 55 (including monthly HMO capitation fees).

**Exception: As of January 1, 2010, Medicare cost-sharing expenses paid by Medicaid through Medicare Savings Programs (such as QMB) are exempt from estate recovery.*

- **Minimum Threshold:** Estate recovery may be waived if the claim amount is less than **\$500**, if the total value of the estate is less than **\$3,000**, or if the **cost of recovery** would exceed the amount to be recovered.

Conditions for Deferment or Waiver

- **Surviving Spouse:** Recovery is indefinitely deferred as long as the spouse is alive.
- **Minor Child:** Recovery is deferred if there is a surviving child under the age of 21.
- **Disabled Child:** Regardless of age, recovery is deferred if you have a child who is determined to be **blind** or **permanently and totally disabled** according to **SSA** standards.
- **Caregiver Child Exception:** The recovery of the home may be waived if the child has resided in the parent's home for at least **two consecutive years** immediately prior to the parent's admission to a nursing facility and can prove that they provided a **sufficient level of care** to allow the parent to remain at home rather than entering a facility.

**Note: The child may have outside employment or income, and care provided alongside a home health aide is still eligible for consideration. The child does not need to be the sole caregiver; the decision is based on the quality and scope of care provided. Strict documentation, such as a physician's statement, is required.*

- **Sibling Exception:** An exemption for the home may be granted if the beneficiary's **sibling** holds an **equity interest** in the property and resided in the home for at least **one year** immediately prior to the beneficiary's admission to a **nursing home**.
- **Undue Hardship Waiver:** You may apply for a waiver if the assets subject to recovery are the **sole source of income** for a surviving family member, and if the recovery would likely force that family member to rely on **public assistance** (including Medicaid). However, if assets were transferred in advance for the purpose of avoiding Medicaid recovery, a **rebuttable presumption** applies that no undue hardship exists, which may result in the waiver request being denied.

**A written request for a waiver must be submitted within 20 days of receiving the notice of recovery claim from the DMAHS. Failure to meet this deadline may result in the denial of your waiver request.*

Key Notes

- **Prior Notification:** Before distributing estate assets to heirs or other creditors, the estate executor (representative) must notify the **NJ DMAHS Estate Recovery Unit** in writing to confirm if there is an outstanding claim. **DMAHS** will initiate the initial claim process within **90 days** of receiving written notice, and the final claim or lien filing will occur within **three years** from the date of receipt of the written notice.
- **Funeral Expenses:** Before Medicaid estate recovery proceeds, the following items may be paid from the estate assets with priority:
 - **Reasonable funeral and burial expenses**

- **Reasonable costs and fees** related to the administration of the estate
- Debts owed to the **Office of the Public Guardian for Elderly Adults**
- **DMAHS recovery** (processed only after the above items have been settled)
- **Deferral of Lien Enforcement for Residing Family Members:** If a family member has continuously resided in the home since before the beneficiary's death, and the home was the beneficiary's **primary residence** and remains the primary residence of that family member, **DMAHS** may record a **lien** on the property. However, it will not enforce the lien until the family member moves out, passes away, or the property is voluntarily sold.

Contact

- **Direct Line for Inquiries: NJ DMAHS 1-609-588-2900 / 1-609-588-3016** – Direct line for all inquiries regarding Trusts, Liens, and Estate Recovery.
- **General Inquiries & Information: NJ FamilyCare 1-800-701-0710** – For general guidance
- **Mailing Address for Document Submission (Estate Unit):**
NJ DMAHS – Office of Legal and Regulatory Affairs
Attn: Estates
P.O. Box 712, Mail Code #6 Trenton, NJ 08625-0712
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org)

Resources

- [Medicaid.gov](https://www.Medicaid.gov) – Estate Recovery
- [NJ Medicaid Program and Estate Recovery – What You Should Know](#)

Spousal Impoverishment Rules

These rules are designed to protect the financial well-being of the spouse remaining at home (**Community Spouse**) while the other spouse receives Nursing Home care or Home Care through **MLTSS (Long Term Care)**, preventing the community spouse from falling into poverty.

Spouses do not need to file a separate application for Spousal Impoverishment protections; they are automatically evaluated during your Medicaid (MLTSS) application process

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

CSRA (Community Spouse Resource Allowance: Asset Protection)

The CSRA allows the community spouse to retain a certain amount of assets without affecting the applicant's Medicaid eligibility.

- **Total Assets:** Calculated by combining all countable assets (cash, savings, stocks, etc.) owned by either or both spouses.
- **Retention Limit:** In New Jersey, the community spouse can retain up to **\$162,660** in 2026.

**Spend Down: If combined assets exceed the allowed limit, the excess must be "spent down" on medical expenses, home repairs, or funeral trusts before the applicant can be approved for Medicaid.*

MMMNA (Minimum Monthly Maintenance Needs Allowance: Income Protection)

MMMNA ensures that if the community spouse has little to no income of their own, a portion of the applicant spouse's income can be transferred to them.

**New Jersey income standards; vary by state*

Minimum Allowance: If the community spouse's monthly income is below **\$2,643.75**, they can receive an allocation from the applicant's income to bridge the gap.

- **The current standard of **\$2,643.75** is effective from **July 1, 2025, through June 30, 2026**. This minimum guaranteed amount is subject to change every **July 1st** based on updates to the **Federal Poverty Level (FPL)**.*

- **Maximum Allowance:** If the community spouse has high shelter expenses (rent, mortgage, taxes, etc.), they may be allowed to keep up to **\$4,066.50** per month.
- **Benefit:** This transfer reduces the applicant's share of cost (the amount they must pay the facility), helping the household retain more total income.

Estate Recovery

While the home is generally excluded from the asset limit during the spouse's lifetime, strategic planning is required due to the **Estate Recovery** program.

- **Exemption During Residency:** The home is exempt only if the applicant or their spouse resides in it, a minor or disabled child resides in it, or if the applicant expresses an **"intent to return"** and the home equity is **\$1,130,000 or less**.
- **Estate Recovery:** After the death of a Medicaid beneficiary, the State may attempt to recover the costs of care from the deceased's estate (most commonly the home). In New Jersey, the state may file a claim for **Estate Recovery** for **Medicaid benefits received after the age of 55**.
- **Conditions for Postponement of Recovery:**
 - If a **surviving spouse** still resides in the home.
 - If there is a surviving child under age 21, or a child who is blind or permanently disabled.
 - **Caretaker Child Exception:** If a child lived in the home for at least 2 years before the parent's institutionalization and provided care that delayed the need for a nursing home, the home may potentially be transferred to that child without penalty.

**Statute of Limitations: The right to recovery is permanently forfeited once the period for filing a claim (e.g., one year) has expired.*

**\$1M Home Equity Cap: A unified federal \$1M home equity cap for Medicaid LTC eligibility with no inflation adjustments effective Jan 1, 2028. (OBBBA Update).*

Spousal Asset and Income Protection Standards (2026, New Jersey)

Category	Item	Standard Amount	Remarks
Asset Protection	CSRA (Max Asset Limit)	\$162,660	Excludes home; kept by community spouse
Income Protection	MMMNA (Monthly Income)	\$2,643.75 – \$4,066.50	Adjustable based on shelter costs
Applicant Limit	Individual Asset Limit	\$2,000	Limit for the spouse receiving care
Home Exemption	Home Equity Limit	\$1,130,000 (NJ)	Based on the home's net equity value

Contact

- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org).

Resources

- [Medicaid.gov – 2026 Federal Spousal Impoverishment Standards](#)
- [Medicaid.gov – 2026 SSI and Spousal Impoverishment Standards](#)
- [NJ DHS.gov – 2026 Medicaid Only Income and Resource Standards \(26-01\)](#)

Medicaid General Assistance (WFNJ/GA)

The Medicaid General Assistance program is a core welfare system in New Jersey that provides integrated cash assistance, job training, and emergency housing support for low-income adults and couples without children. In New Jersey, it is known as **WFNJ/GA (Work First New Jersey/General Assistance)**.

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Eligibility

- **Target:** Adults aged 18 or older residing in New Jersey (single or couples without dependent children).
- **Status:** U.S. citizens or qualified immigrants (Green Card holders for 5+ years / Refugees and Asylees are eligible immediately).
- **Income:** Eligibility is determined based on the 'Schedule I/III (Initial Maximum Allowable Income)' limits, which are significantly low—representing only about 20% to 22% of the Federal Poverty Level (FPL).
- **Assets:** Liquid assets must be **\$2,000 or less**. (Primary residence, one vehicle, and retirement assets such as 401k or IRA plans are excluded from eligibility review).
- **Employable:** Mandatory participation in **30 hours per week** of work activities, job searches, vocational training, or designated work assignments (N.J.A.C. 10:90-4.1).
- **Unemployable:** If you obtain and submit a medical certification of illness or disability (a Med-1 form) from a healthcare provider, you will receive a higher benefit amount and be fully exempt from all work requirements.

Benefit Amounts by Household Size

Category	Recipient	Maximum Benefit (Monthly)
Employable	Single Adult (1 person)	\$185
	Couple (2 people)	\$254
Unemployable (Disabled)	Disabled Adult (1 person)	\$277
	Disabled Couple (2 people)	\$382

Benefits

WFNJ/GA provides multifaceted services for self-sufficiency beyond simple cash payments.

- **Cash Assistance:** Provided for a lifetime limit of 60 months. (The clock stops if you gain employment).
- **Emergency Housing Assistance:** If you are facing a housing crisis, the program provides support for unpaid rent, emergency shelter, or utility arrears. Extensions for **Emergency Assistance (EA)** benefits are also available.

**Under NJ DHS Emergency Assistance (EA) program regulations, there is a lifetime limit of 12 months (combined for TANF and GA). Extensions are available for up to two 6-month periods for TANF and one 6-month period for GA.*

- **Employment and Education Support:**
 - Support for obtaining a High School Equivalency (GED) and Vocational Training.
 - Assistance with transportation costs for commuting and job preparation expenses.
- **Healthcare Linkage:** Enrollment in WFNJ/GA automatically grants enrollment in [ACA Medicaid \(NJ FamilyCare\)](#).

How to Apply

- **Online:** [MyNJHelps.gov](#)
- **In-Person:** Local county social service offices
 - **Bergen County Board of Social Services**
 - Address: 218 Route 17 North, Rochelle Park, NJ 07662
 - Phone: 201-368-4200
 - Hours: Weekdays (Mon–Fri) 8:00 AM – 4:30 PM

Contact

- **NJ Division of Family Development Hotline:** 1-800-792-9773
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ DHS.gov – Work First New Jersey](#)
- [NJ FamilyCare – 2026 Income Limits & Eligibility](#)
- [NJHelps.gov](#)

- [NJ S2201 – WFNJ Program Modernization Act \(2026 Reforms\)](#)
- [Bergen County Board of Social Services](#)

Temporary Assistance for Needy Families (TANF)

Temporary Assistance for Needy Families (TANF) is a core welfare program that provides cash assistance, childcare support, and job training to help low-income households with children under 18 (or pregnant women) achieve self-sufficiency. In New Jersey, it is known as **WFNJ/TANF (Work First New Jersey/Temporary Assistance for Needy Families)**. **OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details.*

Eligibility

- **Target:** Low-income households residing in New Jersey with children under 18 (or children under 19 if still in high school).
- **Income:** Monthly income of **\$5,379 or less** for a family of 3 (within 250% of the Federal Poverty Level – FPL).
- **Status:** U.S. citizens or qualified immigrants (Green Card holders for 5+ years / Refugees and Asylees are eligible immediately).
- **Assets:** Liquid assets must be **\$2,000 or less**. (Primary residence, one vehicle, and retirement assets such as 401k or IRA plans are excluded from eligibility review).
- **Employable:** Parents must participate in **job search activities, vocational training, or designated work activities** for at least **35 hours per week** (Standard WFNJ Regulations).
- **Unemployable:** If you submit a **medical certification of illness or disability (Med-1 Form)** through a healthcare provider, you will be **exempted from work requirements**.
- **Child Support Cooperation (If Applicable):** To receive cash assistance for your children, you must provide information about the other parent and cooperate in the assignment of child support rights. From the child support collected each month, an additional payment of up to **\$100 for one child**, or up to **\$200 for two or more children**, will be passed through to the recipient. Failure to cooperate without good cause may result in the **denial of benefits**.

Benefits

WFNJ/TANF provides multifaceted services for self-sufficiency beyond simple cash payments.

- **Financial Assistance**
 - **Monthly Assistance: \$559 per month** for a family of 3 – this amount is based on having **zero income**; if there is any income, the benefit amount will be **adjusted**.
 - Amounts may be adjusted based on household size and income.

- Cash assistance is provided for a **lifetime limit of 60 months**. If cash assistance stops due to employment, the remaining months are preserved and can be used in the future if help is needed again. Upon reaching **48 months** of assistance, individuals are automatically assigned to the **SAIF (Supportive Assistance to Individuals and Families) program**. After the 60-month limit is exhausted, extensions of **6 months** (up to two times) may be granted. Additionally, if specific criteria are met, you may apply for up to **24 months of additional cash assistance** through the SAIF program.
- **Post-TANF (\$200 Monthly)**: If your cash assistance ends due to employment after receiving TANF for at least 6 months, a monthly post-assistance subsidy of **\$200** is provided for up to **24 months** to support your self-sufficiency. This subsidy does **not** count toward the lifetime 60-month limit and serves as a powerful incentive for independence unique to New Jersey.

***Eligibility:** You become eligible if you voluntarily close your case after maintaining employment of at least **20 hours per week** for a minimum of **4 consecutive months**.

- **Diversions Payment (\$750 Lump Sum):**
 - **A one-time \$750 payment** provided to families to overcome temporary crises during a job search.***Consultation Required Upon Application:** The **Diversions Payment** is a one-time lump sum provided in lieu of ongoing cash assistance. Please note that receiving this payment may restrict your eligibility to apply for **TANF cash assistance** for a specified period.
- **Emergency Housing Assistance:** If you are facing a crisis or at risk of homelessness, you may receive support for back rent, emergency shelter, utility arrears, moving expenses, and security deposits. This assistance is available to **TANF, GA, and SSI** recipients, as well as certain low-income households not currently receiving ongoing benefits.
 - **Benefit Period:** Assistance is generally capped at **12 months** per case. However, **TANF** recipients may be eligible for up to two extensions (6 months each), and **GA** recipients may receive one extension (up to 6 months).
 - **Co-payment Requirement:** While receiving EA, you are required to contribute **30% of your household income** (including cash assistance) toward your housing costs.
- **Employment and Education Support:**
 - Support for obtaining a High School Equivalency (GED) and Vocational Training.
 - Assistance with transportation costs for commuting and job preparation expenses.
- **Healthcare Linkage:** Enrollment in WFNJ/TANF automatically grants enrollment in [ACA Medicaid \(NJ FamilyCare\)](#).

How to Apply

- **Online:** [MyNJHelps.gov](https://mynjhelps.gov)

- **In-Person:** Local county social service offices
 - **Bergen County Board of Social Services**
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Contact

- **NJ Division of Family Development Hotline:** 1-800-792-9773
- **AWCA** provides expert consultations free of charge (201-862-1665 / info@awcanj.org).

Resources

- [NJ DHS.gov – Work First New Jersey](https://www.njdhhs.gov/work-first-new-jersey)
- [NJHelps.gov – Benefit Screening & Application Portal](https://njhelps.gov/benefit-screening-application-portal)
- [NCCP.org – New Jersey TANF State Profile \(2026\)](https://nccp.org/new-jersey-tanf-state-profile-2026)

PART I • SECTION 6

Chronic Diseases

만성 질환 (Chronic Diseases)

Managing chronic health conditions in older adults: the aging process, heart disease, diabetes, obesity, stroke, cancer, arthritis, mental health, dementia, and mind-body integrative medicine.

This section contains 11 comprehensive guides

Chronic Diseases

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Chronic diseases are health conditions that last for one year or more, requiring ongoing medical attention or limiting activities of daily living if not properly managed. Most chronic diseases are **noncommunicable diseases**, meaning they are not directly transmitted to others. While most chronic conditions are difficult to cure, they can be effectively managed by combining professional medical treatment with proactive lifestyle improvements.

Chronic diseases represent a major public health challenge both domestically and internationally. They are the leading causes of death and disability in the United States. Beyond their impact on physical health, 90% of the nation's \$4.9 trillion in annual healthcare expenditures is used for people with chronic and mental health conditions, and these illnesses deeply affect an individual's overall quality of life. Understanding chronic diseases is the first step toward effective management and maintaining an independent life.

Types of Chronic Diseases

The following are some of the most common chronic diseases.

Lifestyle Diseases

- [Obesity](#)
- [Diabetes](#)
- [Heart Disease](#)
- [Stroke](#)
- [Cancer](#)
- [Arthritis](#)

Brain & Mental Health

- [Mental Health](#)
- [Dementia](#)
- [Mind-Body Medicine](#)

Who Gets Chronic Diseases?

Chronic diseases can affect people of all ages, races, and backgrounds. However, the risk increases with age. More than 3 out of 4 U.S. adults (approximately 76%) suffer from at least one chronic disease, and more than half (approximately 51%) have two or more conditions. Notably, over 90% of seniors aged 65 and older suffer from at least one chronic disease. Certain groups may be at higher risk due to genetics, socioeconomic factors, or access to healthcare.

Causes of Chronic Diseases

Chronic diseases are often caused by a combination of factors, including genetics, environment, lifestyle choices, and behavioral patterns. Some major risk factors include:

- Smoking and tobacco use
- Unhealthy eating habits
- Lack of physical activity
- Excessive alcohol consumption
- Genetics and family history
- Exposure to environmental hazards

Signs & Symptoms of Chronic Diseases

Symptoms vary widely depending on the specific condition. Common signs include persistent pain, fatigue, loss of function in certain body parts, or mood changes. Because these conditions are long-lasting, symptoms may fluctuate in intensity over time.

Stages of Chronic Diseases

Most chronic diseases progress through stages:

1. **Susceptibility:** Presence of risk factors.
2. **Subclinical:** Disease is present but shows no outward symptoms.
3. **Clinical:** Symptoms appear, and a diagnosis is made.
4. **Recovery, Disability, or Chronicity:** The condition is managed long-term.

Diagnosis of Chronic Diseases

Diagnosis typically involves a medical history review, physical examinations, and specific screenings such as blood tests, imaging (X-rays, MRIs), or biopsies depending on the suspected condition.

Treatment of Chronic Diseases

While many chronic diseases cannot be cured, they are managed through:

- **Medications:** To control symptoms or prevent complications.
- **Lifestyle Changes:** Diet, exercise, and stress management.
- **Therapies:** Physical, occupational, or speech therapy.
- **Surgery:** In cases like heart disease or certain cancers.

Prevention of Chronic Diseases

Many chronic diseases are preventable. Key strategies include maintaining a healthy weight, eating a balanced diet, staying physically active, avoiding tobacco, and attending regular medical checkups for early detection.

Living with Chronic Diseases

Living with a chronic condition requires proactive management. Patients often work with a healthcare team to monitor their health, adhere to treatment plans, and make adjustments to daily routines to maintain a high quality of life.

Resources

- [MedlinePlus.gov: Chronic Pain](https://pubmed.ncbi.nlm.nih.gov/19112222/)
- [MedlinePlus.gov: Encyclopedia – Chronic Illness](https://pubmed.ncbi.nlm.nih.gov/19112222/)
- [CDC.gov: About Chronic Diseases](https://www.cdc.gov/chronicdiseases/about/index.htm)
- [WHO.gov: Noncommunicable Diseases](https://www.who.int/news-room/fact-sheets/detail/noncommunicable-diseases)

Aging Process

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Aging is a natural, gradual process affecting all body systems. Beyond visible signs like gray hair and wrinkles, internal changes often impact cardiovascular, musculoskeletal, digestive, and cognitive health. Understanding these shifts is essential for maintaining a high quality of life throughout the senior years.

Aging is not a chronic disease, but the two are closely intertwined. Natural physiological changes often increase the risk of developing new health issues or accelerating the progression of existing ones. Recognizing this connection is vital for effective wellness planning and maintaining independence during the aging process.

Causes of Aging

Aging occurs as damage progressively accumulates at the molecular and cellular levels, driven by cellular senescence, oxidative stress, increased inflammatory response, decline in immune function, and telomere shortening.

Physical and Cognitive Changes with Aging

- **Cardiovascular:** Arteries stiffen (due to decreased elasticity), causing blood pressure to rise and requiring the heart to exert more effort to pump blood.
- **Musculoskeletal:** Loss of bone density (**Osteopenia**) and muscle mass (**Sarcopenia**), leading to decreased strength and balance.
- **Cognitive:** Slower processing speeds and occasional “tip-of-the-tongue” forgetfulness. Significant memory loss is **not** a normal part of aging. (see [Dementia](#))
- **Digestive & Bladder:** Slower movement through the digestive tract (constipation) and decreased bladder elasticity or weakened pelvic floor muscles (incontinence).
- **Sensory:** The lens of the eye stiffens, causing presbyopia (farsightedness), and age-related hearing loss (presbycusis)—where hearing declines due to aging and various factors such as noise exposure and genetic factors—can occur.

Stages of Aging

- **Early Adulthood (20s–30s):** Peak physical function and reproductive health.

- **Middle Adulthood (40s–60s):** Subtle declines in metabolism, skin elasticity, and the beginning of “wear and tear” on joints.
- **Late Adulthood (65+):** More pronounced physical and cognitive changes requiring proactive health management to maintain independence.

Assessing Health in Aging

- **Geriatric Assessments:** Comprehensive reviews of physical, mental, and social health.
- **Bone Density Scans (DEXA):** To monitor for osteoporosis.
- **Sensory Screenings:** Regular vision and hearing exams to prevent social isolation.
- **Cognitive Screenings:** To establish a baseline and differentiate normal aging from dementia.

How to Age Well

While aging cannot be stopped, its negative impacts can be delayed through the following methods:

- **Preventive Healthcare:** Stay current with recommended vaccinations, including Flu, Shingles, Pneumonia, RSV, and COVID-19.
 - Be sure to follow the correct dosing schedule. For example, adults 50 and older should receive two doses of the Shingles vaccine (Shingrix), spaced 2 to 6 months apart.
- **Physical Activity:** Exercise daily to maintain muscle strength and protect bone health.
 - Include **Kegel exercises** in your routine to help prevent urinary incontinence.
- **Nutrition:** Eat a balanced diet rich in fiber, calcium, vitamin D, and lean protein.
 - Staying well hydrated and limiting sodium intake are equally important for healthy aging.
- **Mental Health:** Manage stress and prioritize getting enough sleep each night
- **Social Engagement:** Keep your mind active by pursuing hobbies or joining community events.
- **Smoking and Alcohol:** Work toward quitting smoking and limiting alcohol consumption.
- **Fall Prevention:** Reduce your risk of falls through balance exercises, home safety improvements (such as grab bars and non-slip mats in the bathroom), regular vision and hearing checkups, and periodic medication reviews.

Embracing Life as an Older Adult

Aging well means adapting wisely, staying proactive, and focusing on sustaining physical, emotional, and social quality of life. Open communication with healthcare providers and partners about changes can help preserve overall well-being.

Resources

- [MedlinePlus.gov: Older Adult Health](https://medlineplus.gov/olderadulthealth)
- [NIA.nih.gov: Healthy Aging](https://nia.nih.gov/healthyaging)
- [CDC.gov: Healthy Aging Overview](https://cdc.gov/healthyaging/overview)

Arthritis

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Arthritis is a broad term for a group of conditions that cause pain and swelling in the joints. It is a leading cause of disability among older adults, often impacting mobility and quality of life.

Types of Arthritis

While there are many forms of arthritis, the following are the most common:

- **Osteoarthritis (OA):** The most frequent type, often called “wear and tear” arthritis. It occurs when the protective cartilage that cushions the ends of one’s bones wears down over time.
- **Rheumatoid Arthritis (RA):** An autoimmune and inflammatory disease where one’s immune system attacks healthy cells in one’s body by mistake, causing painful swelling in the affected joints.
- **Psoriatic Arthritis:** Affects some people who have psoriasis—a condition that features red patches of skin topped with silvery scales.
- **Gout:** High levels of uric acid in the blood lead to the formation of crystals that accumulate in the joints. It can be triggered by impaired kidney function—where the body fails to excrete uric acid efficiently—or by a diet high in meat and alcohol. Gout is typically characterized by sudden, intense pain and swelling, most commonly affecting the big toe.
- **Ankylosing Spondylitis:** An inflammatory disease that, over time, can cause some of the small bones in one’s spine to fuse.

Who Gets Arthritis?

The prevalence of arthritis increases sharply after age 65, with approximately 54% of adults aged 75 and older having been diagnosed with the condition. Key risk factors include:

- **Obesity:** Excess weight puts added stress on joints, particularly the knees and hips.
- **Previous Injuries:** Joints damaged by past injuries are more likely to develop arthritis later.
- **Genetics:** Certain genes are linked to a higher risk of RA and ankylosing spondylitis.
- **Repetitive Stress:** Jobs or sports that require repetitive joint bending can increase the risk of OA.

While the risk of stroke increases with age, younger populations face a rise in arthritis conditions. Approximately one in five U.S. adults—roughly 21% or 53 million people—live with arthritis. Notably, over half of those affected (51.7%) are within the working-age population (18 to 64 years old). Arthritis is more

prevalent in women than in men, with both osteoarthritis and rheumatoid arthritis being significantly more common among women.

Causes of Arthritis

Causes depend on the type. Osteoarthritis results from the physical breakdown of cartilage due to age or injury. Rheumatoid arthritis is caused by a dysfunctional immune response. Other forms, like Gout, are metabolic, resulting from an imbalance of chemicals in the blood.

Signs & Symptoms of Arthritis

- Persistent joint pain and stiffness, especially in the morning.
- Visible swelling, redness, or warmth around the joints.
- A “grating” sensation or sound during movement.
- Reduced range of motion that makes daily tasks difficult.

Stages of Arthritis

1. **Early Stage:** Marked by mild inflammation and the onset of cartilage wear. Patients typically experience morning stiffness and occasional aches that subside with rest, having little impact on daily routines.
2. **Intermediate Stage:** As tissue damage progresses, pain and swelling become more frequent and persistent. Everyday tasks like climbing stairs become difficult, and discomfort may begin to interfere with sleep and energy levels.
3. **Late Stage:** Severe cartilage loss leads to joint deformity and constant chronic pain. With range of motion significantly restricted, daily independence declines, often requiring assistive devices or joint replacement surgery.

Diagnosis of Arthritis

- **Physical Exams:** Checking joints for fluid, redness, and range of motion.
- **Imaging:** X-rays to see bone changes and cartilage loss; MRI for detailed views of soft tissue.
- **Laboratory Tests:** Blood tests to look for inflammation markers (like C-reactive protein) or specific antibodies (like Rheumatoid Factor).
- **Surgery:** Joint replacement surgery (Arthroplasty) for severely damaged hips or knees.

Treatment of Arthritis

- **Medications:** Including nonsteroidal anti-inflammatory drugs (NSAIDs) or disease-modifying antirheumatic drugs (DMARDs) for RA.

- **Physical Therapy:** Exercises to strengthen muscles around the joints and increase flexibility.
- **Weight Management:** Reducing body weight significantly lowers pressure on weight-bearing joints.
- **Surgery:** Joint replacement surgery (arthroplasty) for severely damaged hips or knees.

Prevention of Arthritis

- **Maintaining a Healthy Weight:** Engaging in regular physical activity—such as 150 minutes of moderate-intensity exercise per week (e.g., swimming, cycling, or walking)—is the most effective way to reduce joint pain, improve function, and slow the progression of the disease.
- **Joint Protection:** Using proper form when lifting and wearing protective gear during sports.
- **Low-Impact Exercise:** Activities like swimming, cycling, or walking keep joints flexible without excessive strain.

Living with Arthritis

Living with arthritis involves pacing activities to avoid overexertion. The use of heat therapy (to relax muscles) and cold therapy (to reduce inflammation), along with assistive devices like jar openers or canes, can help reduce joint strain and preserve independence.

Support Groups for Arthritis

- [Arthritis Foundation – Your Connect Group](#)

Resources

- [MedlinePlus.gov: Arthritis](#)
- [NIH.niams.gov: Arthritis and Rheumatic Diseases](#)

Cancer

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Cancer encompasses over 100 diseases where abnormal cells grow uncontrollably. While cancer remains the second leading cause of death in the United States (accounting for roughly 20% of all deaths) and the primary cause of death for those under 65,, early detection and support services improve outcomes.

Types of Cancer

Cancer is categorized by the type of cell or part of the body where it starts. Common categories include:

- **Carcinomas:** The most common type, originating in the skin or tissues that line internal organs.
- **Sarcomas:** Originate in the bone, cartilage, fat, or muscle.
- **Leukemias:** Originate in the blood-forming tissue of the bone marrow.
- **Myelomas:** Originate in the plasma cells of the bone marrow.
- **Lymphomas:** Originate in the cells of the immune system.

**Beyond these, there are mixed-type cancers and other rare variations, each differing in their primary site and clinical characteristics.*

Most Common Cancers in the United States

- **Breast Cancer:** The most common cancer diagnosed in women.
- **Prostate Cancer:** The most common cancer diagnosed in men.
- **Lung Cancer:** The leading cause of cancer death for both men and women.
- **Colorectal Cancer:** Affects the colon or rectum.
- **Stomach (Gastric) Cancer:** Cancer that starts in the lining of the stomach. While less common in the general U.S. population, it represents a significant health disparity for Asian Americans.

Who Gets Cancer?

The average age at cancer diagnosis in the U.S. is 67, with the risk of developing the disease increasing significantly as people age. Risk increases with age. Genetics and ethnicity also play a role: for example, African Americans have higher death rates than all other racial groups for many cancer types. Lifestyle factors, especially tobacco use, remain the largest preventable risk factor.

Stomach cancer represents a critical health disparity for Asian Americans, particularly those of Korean descent. Korean Americans are roughly five times more likely to develop stomach cancer compared to non-Hispanic whites. Stomach cancer remains a leading oncological concern within the Korean American community. Clinical data indicates it is the most frequently diagnosed cancer among Korean men and the second most common among Korean women.

Causes of Cancer

Cancer is caused by mutations to the DNA within cells, which may be inherited or acquired. Common triggers include:

- **Carcinogens:** Exposure to chemicals (like tobacco smoke) and radiation (UV rays).
- **Lifestyle:** Poor diet, obesity, and lack of exercise.
- **Biological Factors:** Chronic inflammation or viral infections (such as HPV or Hepatitis B/C).

Signs & Symptoms of Cancer

- Unexplained weight loss or persistent fatigue.
- Persistent cough or trouble breathing.
- Changes in skin or unusual lumps.
- Changes in bowel or bladder habits.

Stages of Cancer

Most cancers are staged from 0 to 4:

- **Stage 0:** Abnormal cells are present but have not spread.
- **Stages I–3:** Cancer is present; higher numbers indicate larger tumors or deeper spread.
- **Stage 4:** Cancer has spread to distant parts of the body (Metastasis).

Diagnosis of Cancer

- **Physical Exams & Lab Tests:** Checking for lumps and analyzing blood, urine, or tissue samples to identify tumor markers. These are proteins or specific substances produced in higher amounts by cancer cells—or by the body in response to cancer—and are measured to confirm a diagnosis.
- **Imaging:** CT scans, MRI, PET scans, or X-rays to see inside the body.
- **Biopsy:** Taking a small tissue sample to look at cells under a microscope. This is the only way to be certain of a cancer diagnosis.

Treatment of Cancer

- **Surgery:** To remove the tumor.
- **Chemotherapy:** Using drugs to kill fast-growing cancer cells.
- **Radiation Therapy:** Using high-energy beams to destroy cancer cells.
- **Immunotherapy:** Helping your own immune system fight cancer.
- **Targeted Therapy:** Blocks the growth of cancer by specifically targeting the unique genes or proteins found within cancer cells.

Prevention of Cancer

- Avoiding tobacco and limiting alcohol consumption.
- Protecting skin from UV rays (sunscreen and shade).
- Maintaining a healthy weight and staying active.
- **Getting regular screenings:** Routine mammograms, colonoscopies, and Pap tests can find cancer early when it is easiest to treat.
- **Vaccinations:** The HPV vaccine prevents cervical cancer and other HPV-related malignancies, while the Hepatitis B (HBV) vaccine helps reduce the risk of developing liver cancer.

Living with Cancer

Living with cancer involves more than just medical treatment. It includes managing physical symptoms (pain, nausea) and emotional health through integrative care, nutrition, and psychological counseling for both the patient and their family.

Support Groups for Cancer

- [National Cancer Institute – Support Service Finder](#)
- [American Cancer Society – Patient Programs](#)
- [Cancer Care – NJ Support Groups](#)
- [Holy Name Support Group for Cancer](#)

Resources

- [National Library of Medicine – Cancer Overview](#)
- [National Cancer Institute Official Portal](#)
- [CDC – Types of Cancer](#)

Dementia

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Dementia refers to a group of conditions characterized by cognitive decline severe enough to interfere with daily life. Alzheimer's is the most common type, but others like Lewy body dementia and vascular dementia also affect many adults.

Dementia or Alzheimer's?

Dementia is an umbrella term, not a single disease. It describes a group of symptoms like memory loss and confusion. **Alzheimer's** is a specific disease and the most common cause of dementia.

Other types of dementia include **Vascular Dementia** (caused by strokes) and **Lewy Body Dementia**. While they all affect the brain, they can start with different symptoms.

Because many types of dementia share similar symptoms, it is very difficult to distinguish between them. A definitive diagnosis can only be made by a healthcare professional.

Who Gets Dementia?

Dementia primarily affects older adults, but it is not a normal part of aging. Risk increases significantly after age 65. Factors such as family history, race (older African Americans and Hispanics are at higher risk), and cardiovascular health play a role.

Causes of Dementia

Dementia is caused by damage to brain cells. This damage interferes with the ability of brain cells to communicate with each other. When cells cannot communicate normally, thinking, behavior, and feelings can be affected. Different types of dementia are associated with specific types of brain cell damage in particular regions of the brain.

Signs & Symptoms of Dementia

- **Cognitive Changes:** Memory loss; difficulty communicating or finding appropriate words; trouble performing complex tasks, planning, reasoning, or problem-solving; confusion regarding time or place (disorientation); and difficulty understanding visual images and spatial relationships.

- **Psychological Changes:** Noticeable personality shifts; symptoms of depression or anxiety; inappropriate social behavior; and the presence of paranoia or hallucinations.
- **Physical Changes:** Poor coordination, loss of balance, and a general reduction in motor functions.

Stages of Dementia

1. **No Impairment:** No visible symptoms, but brain changes may be starting.
2. **Early-stage (Mild Decline):** Forgetfulness, losing items, or difficulty finding words.
3. **Middle-stage (Moderate Decline):** Difficulty with complex tasks (like managing finances) and increased confusion.
4. **Late-stage (Severe Decline):** Requires significant help with daily activities; eventually, the individual may lose the ability to speak or perform basic motor skills.

Diagnosis of Dementia

Medical professionals perform cognitive and neuropsychological tests, neurological evaluations, brain scans, and lab tests to determine the specific type of dementia.

Treatment of Dementia

While most types of dementia are irreversible, symptoms can often be managed through:

- **Medications:** Cholinesterase inhibitors and memantine to help manage cognitive symptoms.
- **Occupational Therapy:** To make the home safer and teach coping behaviors.
- **Environmental Modifications:** Reducing clutter and noise to help the person focus and reduce confusion.

Prevention of Dementia

While there is no guaranteed prevention, a **heart-healthy** lifestyle may reduce risk:

- Staying physically active and maintaining a healthy weight.
- Not smoking and limiting alcohol.
- Managing heart health factors like blood pressure and cholesterol.
- Keeping the brain active through lifelong learning and social interaction.

Living with Dementia

Living with Dementia requires a strong support system. Safety modifications in the home (like removing trip hazards), routine-based daily living, and caregiver support are vital as the disease progresses.

Support Groups for Dementia

- [Alz.org: Support Groups](https://www.alz.org/support-groups)
- [Alzfdn.org: Caregiving Resources](https://www.alzfdn.org/caregiving-resources)
- [Actnowfoundation.org: Dementia Programs](https://www.actnowfoundation.org/dementia-programs)
- [Dfamerica.org:- Memory Cafe Directory](https://www.dfamerica.org/memory-cafe-directory)

Resources

- [National Library of Medicine – Dementia Overview](https://www.nlm.nih.gov/medlineplus/dementia/)
- [National Library of Medicine – Lewy Body Dementia](https://www.nlm.nih.gov/medlineplus/lewybodydementia/)
- [Dementia Society of America](https://www.dementiasociety.org/)

Diabetes

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Diabetes is a chronic health condition that affects how the body turns food into energy. Most of the food eaten is broken down into sugar (glucose) and released into the bloodstream. When blood sugar goes up, it signals the pancreas to release insulin, which acts like a key to let blood sugar into the body's cells for use as energy. In individuals with diabetes, the body either doesn't make enough insulin or can't use it as well as it should.

Approximately 40.1 million people—about 1 in 8—have diabetes, and more than 1 in 4 of them do not know they have the disease in the United States.

Types of Diabetes

- **Type 1 Diabetes:** An autoimmune condition where the immune system attacks and destroys insulin-producing cells in the pancreas. People with Type 1 diabetes must take insulin every day to survive. It is usually diagnosed in children and young adults.
- **Type 2 Diabetes:** The **most common form** of diabetes. The body does not use insulin properly (insulin resistance) and cannot keep blood sugar at normal levels. It is often linked to weight, genetics, and lifestyle factors.
- **Prediabetes:** A condition where blood sugar levels are higher than normal, but not yet high enough to be classified as Type 2 diabetes. This is a critical stage where lifestyle changes can prevent the progression to full diabetes. Around 115.2 million adults have prediabetes, and about 8 out of 10 of them are unaware that they have the condition in the United States.
- **Gestational Diabetes:** Develops in pregnant women who have never had diabetes. It usually goes away after the baby is born but increases the mother's risk for Type 2 diabetes later in life.

Who Gets Diabetes?

Type 1 is often diagnosed in youth. Type 2 is common in adults, especially those who are overweight or have a family history. Especially, 52% of adults aged 65 and older (about 31.3 million people) have prediabetes. Regular screening is recommended for those who are older, overweight, or have a family history of the condition.

Signs & Symptoms of Diabetes

- Frequent urination, often at night.
- Extreme thirst and unexplained weight loss.
- Extreme hunger (feeling hungry even while eating).
- Very dry skin or slow-healing sores.
- Numbness or tingling in the hands or feet (neuropathy).
- Blurry vision and increased fatigue.

Stages of Diabetes (for Type 2)

The condition begins with insulin resistance (a decline in the body's ability to utilize insulin), progresses through prediabetes, and eventually leads to full-blown type 2 diabetes. If not properly managed, the final stage is accompanied by chronic complications affecting the heart, kidneys, eyes, nerves, and feet.

Diagnosis of Diabetes

- **A1C Test:** Measures average blood sugar over the past 3 months.
- **Fasting Plasma Glucose Test:** Measures blood sugar after an overnight fast.
- **Glucose Tolerance Test:** Known as Oral Glucose Tolerance Test (OGTT). Measures the body's response to a sugary drink.

Treatment of Diabetes

- **Type 1:** Requires daily insulin injections or an insulin pump.
- **Type 2:** Managed with healthy eating, active living, and oral medications (like Metformin) or insulin if necessary.
- **Monitoring:** Frequent blood sugar checks using a glucose meter or continuous glucose monitor (CGM).

Prevention of Diabetes

There is currently no known way to prevent Type 1 diabetes. However, **Type 2 diabetes can often be prevented or delayed** through lifestyle modifications:

- Losing a modest amount of weight (5% to 7% of body weight).
- Engaging in regular physical activity (at least 150 minutes per week).
 - Combining a 5–7% weight loss with at least 150 minutes of physical activity per week can reduce the risk of developing type 2 diabetes by up to 58%.

Living with Diabetes

Living with diabetes requires a proactive approach to prevent complications. This includes regular foot exams to check for sores, annual eye exams for retinopathy, and kidney function tests. Consistent medical follow-ups are vital to ensure blood pressure and cholesterol remain within target ranges.

Support Groups for Diabetes

- [Diabetes.org: Get Connected](https://diabetes.org/get-connected)

Resources

- [MedlinePlus.gov: Diabetes](https://medlineplus.gov/diabetes)
- [MedlinePlus.gov: Diabetes Encyclopedia](https://medlineplus.gov/diabetes-encyclopedia)
- [NIDDK.nih.gov: What Is Diabetes?](https://niddk.nih.gov/what-is-diabetes)
- [CDC.gov: Diabetes Basics](https://cdc.gov/diabetes-basics)
- [CDC.gov: National Diabetes Prevention Program](https://cdc.gov/national-diabetes-prevention-program)

Heart Disease

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Heart disease refers to several types of heart conditions, the most common being Coronary Artery Disease (CAD), which can lead to a heart attack. A heart attack occurs when blood flow to a part of the heart muscle is severely reduced or blocked. Heart disease is the leading cause of death in the United States.

Heart Attack vs. Cardiac Arrest vs. Heart Failure

Condition	Primary Issue	Description and Impact
Heart Attack	Plumbing	A blocked artery prevents blood flow, causing heart muscle tissue to die if not treated immediately.
Cardiac Arrest	Electrical	The heart suddenly stops beating due to an arrhythmia (abnormal heart rhythm) such as ventricular fibrillation. Without immediate CPR and the use of an Automated External Defibrillator (AED), death can occur within minutes.
Heart Failure	Pumping	A chronic condition where the heart is too weak or stiff to pump enough blood to meet the body's demands.

Who Gets Heart Disease?

Individuals with high blood pressure, high cholesterol, or diabetes are at the highest risk. Lifestyle factors like smoking and physical inactivity also contribute. Nearly half of all Americans have at least one of these risk factors.

While the risk increases with age, heart disease does not only affect older adults. In 2023, approximately 1 in 6 deaths from cardiovascular disease occurred in individuals under the age of 65.

Causes of Heart Disease

Heart disease is often caused by **Atherosclerosis**, a condition where plaque (a buildup of fat and cholesterol) accumulates in the walls of the arteries. This narrows the vessels over time and restricts the flow of oxygen-rich blood to the heart.

Signs & Symptoms of Heart Disease

- **Chest pain (Angina):** Pressure, squeezing, or discomfort.
- **Shortness of breath:** Especially during physical activity or while lying down.
- **Referred Pain:** Pain or numbness in areas outside the heart, such as the shoulders, arms, neck, jaw, back, or upper abdomen. These atypical symptoms are especially common in women.
- **Extreme Fatigue:** Feeling unusually tired for no clear reason.

Stages of Heart Disease

1. **Stage A (At Risk):** No structural heart damage but presence of risk factors like hypertension or diabetes.
2. **Stage B (Pre-Heart Failure):** Structural damage is present (like a low ejection fraction) but no outward symptoms.
3. **Stage C (Symptomatic):** Structural damage with current or prior symptoms like fatigue or breathlessness.
4. **Stage D (Advanced):** Severe symptoms that do not improve with standard treatment.

Diagnosis of Heart Disease

- **ECG/EKG:** Records the electrical activity of the heart.
- **Echocardiogram:** An ultrasound used to examine the structure of the heart (atria, ventricles, valves, etc.) and evaluate the pumping function of the ventricles (including ejection fraction, etc.).
- **Stress Testing:** Monitoring the heart during physical exercise.
- **Cardiac Catheterization:** A procedure to check for blockages using a catheter and special dye.

Treatment of Heart Disease

- **Lifestyle Changes:** Adopting a heart-healthy diet (low sodium/trans fats) and regular exercise.
- **Medications:** Beta-blockers, statins (for cholesterol), and ACE inhibitors to manage blood pressure.
- **Procedures:** Angioplasty (using a balloon to open arteries), stents, or coronary artery bypass surgery.

Prevention of Heart Disease

- Managing stress through healthy outlets.
- Quitting smoking and avoiding secondhand smoke.
- Maintaining blood pressure, blood sugar, and cholesterol levels within target ranges through regular screenings.

- Adopting a healthy diet (recommended: low-sodium, low-saturated fat, the DASH diet, etc.).
- Engaging in regular physical activity.
- Consuming alcohol in moderation.

Living with Heart Disease

Living with heart disease requires a proactive approach. This includes daily monitoring of vitals (blood pressure and weight), strictly adhering to medication schedules, and participating in cardiac rehabilitation—a medically supervised program designed to improve cardiovascular health after a heart event.

Sudden Cardiac Arrest - CPR/AED Instructions

Emergency Response Guidelines:

- **Call 911** immediately.
- **Start CPR.**
- **Use a nearby AED** (Automated External Defibrillator).

**Rapid response saves lives.*

Support Groups for Heart Disease

- [Heart.org – Support Network](https://www.heart.org)
- [AACVPR.org – Cardiac Program Directory](https://www.aacvpr.org)

Resources

- [MedlinePlus.gov: Heart Disease](https://medlineplus.gov)
- [CDC.gov: About Heart Attack](https://www.cdc.gov)
- [NHLBI.nih.gov: What Is Coronary Heart Disease?](https://www.nhlbi.nih.gov)

Mental Health

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Mental health covers emotional, psychological, and social well-being. It affects how people think, feel, and act, and helps determine how they handle stress, relate to others, and make choices. Accessible treatment and early intervention are essential for recovery and well-being.

Common Types of Mental Health Disorders

- **Anxiety Disorders:** Characterized by persistent, excessive fear or worry. This includes **Panic Disorders** and **Phobias**.
- **Obsessive-Compulsive and Related Disorders (OCD and Related Disorders):** Characterized by uncontrollable, recurring thoughts (obsessions) and behaviors (compulsions).
- **Mood Disorders:** Affect a person's emotional state.
 - **Depression:** Persistent sadness and loss of interest.
 - **Bipolar Disorder:** Cycles of extreme highs (mania) and lows (depression).
 - **Seasonal Affective Disorder (SAD):** Depression triggered by changing seasons.
- **Trauma-Related Disorders:** Such as **Post-Traumatic Stress Disorder (PTSD)**, which develops after experiencing or witnessing a terrifying event.
- **Psychotic Disorders:** Conditions like Schizophrenia that involve a loss of contact with reality, such as hallucinations or delusions.
- **Eating Disorders:** Severe disturbances in eating behaviors and related thoughts and emotions.
- **Personality Disorders:** Long-term patterns of thoughts and behaviors that are unhealthy and inflexible.
- **Substance Use Disorders:** Occur when the recurrent use of alcohol and/or drugs causes significant impairment.

Who Gets Mental Health Challenges?

Mental health issues are very common. Approximately 23% of U.S. adults (more than 1 in 5) live with a mental health condition. This affects all age groups, regardless of income, geography, or education level. While certain issues—such as depression resulting from cognitive decline or isolation—may increase in later life, mental health problems are also rising significantly among younger populations. Approximately 1 in 5 adolescents aged 12–17 in the U.S. currently has a diagnosed mental or behavioral health disorder.

In particular, a significant number of those aged 65 and older experience mental health issues such as depression and anxiety disorders, which often tend to go undiagnosed or untreated. It is important to seek early help from a doctor or mental health professional if symptoms appear.

Causes of Mental Health Challenges

There is no single cause for mental health conditions. Development is typically influenced by:

- **Biology:** Brain chemistry (neurotransmitters like serotonin) and genetic predispositions.
- **Life Experiences:** Exposure to trauma, abuse, or significant chronic stressors.
- **Medical Conditions:** Chronic physical illnesses (such as cancer or heart disease) can increase the risk of developing mental health struggles.

Signs & Symptoms of Mental Health Challenges

Signs vary but often include:

- Persistent sadness, “empty” mood, or irritability.
- Excessive fear, worry, or anxiety.
- Extreme mood swings or difficulty controlling emotions.
- Withdrawal from social activities and loved ones.
- Significant changes in sleeping (insomnia) or eating habits.
- Delirium: Sudden confusion or disorientation, often seen in seniors.
- Suicidal Ideation: Thoughts of self-harm or suicide
 - If you have thoughts of self-harm or suicide, call or text **988** (Suicide & Crisis Lifeline, free 24/7) immediately. In a life-threatening emergency, call **911**.

Stages of Mental Health Challenges

Various mental health conditions manifest in different ways and can range from mild states to crisis situations.

1. **Mild Disruption:** Difficulty sleeping or temporary distress.
2. **Moderate/Severe Illness:** Symptoms interfere with work, relationships, or daily function.
3. **Crisis:** High risk of harm or inability to care for oneself.

Diagnosis of Mental Health Challenges

A mental health professional (Psychiatrist, Psychologist, or Licensed Counselor) uses clinical interviews and the DSM-5 manual to evaluate symptoms. They may also use physical exams to rule out other medical

causes.

Treatment of Mental Health Challenges

- **Therapy:** Cognitive Behavioral Therapy (CBT) helps identify and change negative thought patterns.
- **Medication:** Antidepressants, mood stabilizers, or anti-anxiety medications.
- **Mind-Body Practices:** Meditation, yoga, and mindfulness to support emotional regulation.
- **Faith-Based Care:** Complementary support and recovery through spirituality and pastoral care.

Prevention of Mental Health Challenges

While not all conditions can be prevented, health can be supported by:

- Maintaining strong social connections.
- Prioritizing consistent sleep and physical activity.
- Practicing stress-management techniques.
- Seeking professional help early before symptoms escalate.

Living with Mental Health Challenges

Living with mental health challenges or recovery involves professional treatment, a strong support network, and healthy lifestyle choices. Many individuals with mental health conditions live full, productive lives by adhering to their treatment plans and utilizing community resources.

Support Groups for Mental Health

Organization	Key Services	Contact/Link
Bergen New Bridge Medical Center	Inpatient Care: Provides 24/7 inpatient services and intensive treatment for mental health and substance use disorders.	NewBridgeHealth.org (201) 967-4000
SAMHSA (Substance Abuse & Mental Health Services Admin)	Federal agency under the US HHS that provides local resources and support for substance use disorders and mental health.	SAMHSA.gov - Find Help FindTreatment.gov
NAMI (National Alliance on Mental Illness)	Peer-led support groups and educational programs for individuals with mental illness.	NAMI.org - Support Groups
Mental Health America	Mental health screening tools and connections to specialized professionals.	mhanational.org

24/7 Crisis Assistance

SAMHSA's free, confidential national hotline, available in English/Spanish.

- **Substance abuse/mental health helpline:** treatment referral and information service.
 - **1-800-662-HELP (4357)**
- **Suicide & Crisis Lifeline:** Immediate support via call, text, or chat.
 - **988**

Care Plus NJ

New Jersey-based nonprofit specializing in comprehensive mental health counseling and care for children, adults, and families.

Headquarters

- **Address:** 610 Valley Health Plaza, Paramus, NJ 07652
- **Primary Phone:** (201) 986-5000
- **Website:** CarePlusNJ.org

Regional Outpatient OfficesBergen County Office (Rochelle Park)

- **Address:** 365 W. Passaic Street, Suite 115, Rochelle Park, NJ 07662
- **Hours:** Monday – Thursday | 9:00 AM – 3:00 PM

Essex County Office (Bloomfield)

- **Address:** 650 Bloomfield Avenue, Suite 106, Bloomfield, NJ 07003
- **Hours:** Monday – Thursday | 9:00 AM – 1:00 PM

Morris County Office (Morris Plains)

- **Address:** 520 Speedwell Avenue, Ste 216, Morris Plains, NJ 07950
- **Hours:** Wednesday & Thursday | 9:00 AM – 1:00 PM

Resources

- [MedlinePlus.gov: Mental Health Topics](https://pubmed.ncbi.nlm.nih.gov/)
- [NIMH.nih.gov: Caring for Your Mental Health](https://www.nimh.nih.gov/health/topics/mental-illnesses-and-conditions/)
- [Christianhealth.org : Faith-Based Behavioral Health](https://www.christianhealth.org/)
- [NAMI: Benefits of Religion & Spirituality](https://www.nami.org/About-NAMI/Benefits-of-Religion-and-Spirituality/)
- [SAMHSA.gov: Substance Use Disorder](https://www.samhsa.gov/substance-use-disorder/)
- [SAMHSA.gov: Mental Health](https://www.samhsa.gov/mental-health/)

Mind-Body Medicine

Mind-body practices include a variety of techniques such as meditation, yoga, and mindfulness, which are designed to enhance the mind's positive impact on physical and emotional health. These practices are increasingly integrated into mainstream medical settings as **complementary and integrative therapies**, working alongside conventional treatments to improve overall patient outcomes.

Core Mind-Body Practices

Several techniques form the foundation of mind-body medicine, each offering unique benefits for the nervous system and physiological health:

- **Meditation:** A practice that involves focusing attention or suspending the stream of thoughts to achieve mental clarity and emotional stability.
- **Mindfulness:** The practice of maintaining a non-judgmental state of heightened or complete awareness of one's thoughts, emotions, or experiences on a moment-to-moment basis.
- **Yoga:** A system of physical postures, breathing techniques, and meditation used to improve flexibility, strength, and mental focus.
- **Tai Chi and Qi Gong:** Mind-body exercises that combine slow, deliberate movements with meditation and breathing.
- **Deep Breathing and Progressive Relaxation:** Techniques used to physically lower the body's stress response and reduce muscle tension.

Benefits and Effectiveness

Mind-body medicine is utilized to manage symptoms and improve the quality of life for various conditions:

- **Pain Management:** Practices such as yoga and meditation can help reduce chronic pain, including back pain and headaches.
- **Mental Health:** Mindfulness practices can be effective in alleviating symptoms of anxiety, depression, and stress.
- **Cardiovascular Health:** Research findings suggest that regular meditation and mind-body practices can help lower blood pressure.
- **Cancer Care:** Mind-body practices can be used to manage side effects experienced by cancer patients during treatment, such as nausea, fatigue, and emotional distress.

Mind-Body Medicine and Standard Medical Care

Mind-body practices are not a replacement for standard medical treatment, but are **complementary and integrative** health therapies used alongside it. Certain physical postures (such as yoga) may require the guidance of a trained instructor for individuals with existing injuries or chronic conditions. In particular, seniors aged 65 and older, and those with osteoporosis, joint diseases, or heart disease, should consult with a healthcare provider before beginning.

Local Mind-Body Medicine Providers

- **Mind-Body Clinics**
 - Acupuncture
 - Oriental Medicine
 - Massage Therapy
 - Meditation/Qi Gong
 - Yoga, Pilates
 - Chiropractic
 - Psychotherapy

- **Major Mind-Body Medicine Providers in New Jersey**
 - [Englewood Health – Integrative Medicine](#)
 - [Hackensack Meridian Health – Integrative Health & Medicine](#)
 - [Valley Health – Integrative Medicine](#)
 - [RWJBarnabas Hamilton – Holistic Integrative Medicine](#)

- **Mind-Body Medicine Academic and Specialty Centers**
 - [Columbia University – Integrative Health](#)
 - [Mount Sinai Hospital – Integrative Medicine](#)
 - [Memorial Sloan Kettering Cancer Center – Integrative Medicine](#)

**When choosing a facility, verify their certifications and professional training. Please refer to the NCCIH guide on selecting a complementary health practitioner.*

Resources

- [NCCIH.nih.gov: Mind and Body Practices](https://nccih.nih.gov/health/mind-body-practices)

- [NCCIH.nih.gov: Meditation and Mindfulness](https://nccih.nih.gov/meditation)
- [Cancer.gov: Complementary and Alternative Medicine](https://cancer.gov/complementary)

Obesity

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Obesity is a complex, chronic disease characterized by excessive body fat that presents a risk to health. It is not merely a cosmetic concern but a medical condition that increases the risk of other diseases and health problems,

Who Gets Obesity?

Obesity affects individuals of all ages, races, and socioeconomic groups. While the risk of obesity increases with age—partly due to metabolic changes and decreased muscle mass—younger populations face a significant rise in obesity rates. 2 out of 5 adults in the United States (40.3%) are diagnosed with obesity, and approximately one in five U.S. children and adolescents ages 2 to 19 (about 19–20%) are obese, which stands as a factor that threatens lifelong health.

Causes of Obesity

Obesity generally results from a long-term energy imbalance where more calories are consumed than are used by the body. Contributing factors include:

- **Lifestyle Choices:** Sedentary behaviors, high screen time, and diets high in processed, calorie-dense foods.
- **Environment:** Limited access to affordable healthy foods or safe places for physical activity.
- **Genetics and Metabolism:** Genetic traits can affect how the body processes food into energy and how it stores fat.
- **Health Conditions and Medications:** Certain diseases (like hypothyroidism) or medications (like steroids) can contribute to weight gain.

Signs & Symptoms of Obesity

The primary indicator of obesity in adults is a **Body Mass Index (BMI) of 30 or higher**. Associated symptoms and physical signs include:

- Increased body fat, particularly around the waist.
- Breathlessness during minor physical exertion.
- Increased sweating and snoring (often linked to sleep apnea).

- Persistent joint and back pain due to excess strain on the musculoskeletal system.

Diagnosis of Obesity

- **BMI Calculation:** A screening tool that compares height and weight.
 - CDC Official BMI Classifications:
 - Stage 1: BMI 30–34.9
 - Potential risk factors (such as borderline hypertension) or mild physical symptoms.
 - Stage 2: BMI 35–39.9
 - Chronic diseases related to obesity (such as type 2 diabetes, sleep apnea, etc.).
 - Stage 3: BMI 40 or higher (Severe)
 - Serious disabilities occurring due to obesity-related chronic diseases, such as End-Stage Renal Disease (ESRD), severe heart disease, and cirrhosis of the liver.
- **Waist Circumference:** Measuring fat stored around the middle, which is a key indicator of health risk. Waist circumference of 35 inches (88 cm) or more for women and 40 inches (102 cm) or more for men may lead to increased health risks.
- **Blood Tests:** Checking cholesterol, glucose levels, and liver function to assess metabolic health.

Treatment of Obesity

Treatment focuses on sustainable health improvements rather than quick weight loss:

- **Dietary Changes:** Shifting toward nutrient-dense foods and reducing calorie intake.
- **Physical Activity:** Aiming for at least **150 minutes of moderate-intensity aerobic activity** per week, along with muscle-strengthening exercises at least twice a week.
- **Behavioral Counseling:** Identifying triggers for overeating and developing healthier coping mechanisms.
- **Medical Interventions:** In some cases, healthcare providers may prescribe weight-loss medications or recommend bariatric surgery.

Prevention of Obesity

- Limiting the consumption of sugary beverages and highly processed snacks.
- Eat healthy foods and engage in safe physical activity.
- Reducing sedentary “screen time” and increasing daily movement.
- Prioritizing consistent, quality sleep, as sleep deprivation can disrupt hormones that control hunger.

Living with Obesity

Living with obesity involves long-term lifestyle management. Success is often measured by improvements in cardiovascular fitness, blood pressure, and blood sugar levels, rather than just the number on a scale.

Support Groups for Obesity

- [Obesity Action Coalition – Community Support](#)

Resources

- [MedlinePlus.gov: Obesity](#)
- [CDC.gov: About Obesity](#)
- [NHLBI.nih.gov: BMI Calculator: Calculate Your BMI](#)
- [NIDDK.nih.gov: Weight Management in Older Adults \(Research\)](#)
- [NCOA – Obesity in Older Adults](#)

Stroke

Disclaimer: This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

A stroke occurs when the brain's blood supply is interrupted or reduced, preventing brain tissue from getting oxygen and nutrients. Within minutes, brain cells begin to die. A stroke is a critical medical emergency that requires immediate intervention to minimize brain damage and potential complications.

Who Gets Stroke?

Stroke risk increases significantly with age. High-risk individuals include those with:

- **High blood pressure (Hypertension):** The leading cause of stroke, which is increasingly found in younger populations.
- **Atrial Fibrillation (AFib):** An irregular heart rhythm that can cause blood clots.
- **High Cholesterol and Diabetes:** Both conditions damage blood vessels over time.

While the risk of stroke increases with age, younger populations face a notable rise in stroke cases. 10 to 15% of all strokes occur in individuals between the ages of 18 and 50. In particular, the prevalence of stroke among adults aged 18 to 64 has been steadily increasing in recent years.

Causes of Stroke

- **Ischemic Stroke (Most Common):** Occurs when a blood clot or fatty deposits (plaque) block an artery leading to the brain. Approximately 87% of all strokes are ischemic.
- **Hemorrhagic Stroke:** Occurs when a weakened blood vessel in the brain bursts and bleeds into the surrounding tissue.
- **Transient Ischemic Attack (TIA):** Often called a "mini-stroke," this is a temporary blockage that serves as a warning sign of a future full-scale stroke. TIA is also a medical emergency that requires immediate attention. If left untreated, the risk of a major stroke significantly increases following a TIA. Even if symptoms disappear within minutes, call 911 or go to the emergency room immediately.

Signs & Symptoms of Stroke

Health professionals emphasize the **F.A.S.T.** acronym to recognize the sudden onset of symptoms:

- **F – Face:** One side of the face droops or is numb.

- **A – Arm:** One arm is weak or numb; when both arms are raised, one drifts downward.
- **S – Speech:** Speech is slurred or the person is unable to speak or understand.
- **T – Time:** Call 911 immediately if any of these symptoms are present.

*A sudden, severe headache with no known cause, may be a major warning sign of a hemorrhagic stroke. Call **911** immediately.*

What to Do During a Stroke

The period immediately following a stroke is known as the **Golden Time**. It is critical to begin receiving treatment within **3 hours** of the onset of symptoms. Since some patients may still be eligible for treatment up to 4.5 hours, do not hesitate to call **911** immediately even if the three-hour window has passed. The success of stroke treatment depends entirely on **‘how quickly the patient arrives at the hospital.’** Faster intervention minimizes brain damage, and taking swift action within the Golden Time is the most decisive factor in a patient’s recovery.

1. **Call 911 Immediately:** Do not attempt to drive the person to the hospital yourself. Paramedics can begin life-saving treatment in the ambulance and will alert the hospital’s “stroke team” so they are ready the moment the patient arrives.
 2. **Note the Time:** Try to identify the exact time the first symptom appeared. This is the most critical piece of information for the medical team, as it determines whether the patient can safely receive “clot-busting” medications (tPA).
 3. **Perform the F.A.S.T. Check.**
 4.
 - **Do not give them food or drink:** Strokes can affect the ability to swallow, leading to choking or aspiration.
 - **Do not give them aspirin:** While aspirin helps with some heart attacks, it can be fatal if the stroke is caused by a brain bleed (hemorrhagic) rather than a clot.
- Keep the Person Calm and Safe:**
- **Positioning:** If the patient is conscious, lay them down in a safe location and take care to ensure an open airway. For specific instructions regarding positioning, please follow the guidance of the emergency medical technicians (EMTs) upon their arrival.

Stages of Stroke Recovery

- **Acute Phase:** The immediate hours following the event, focused on stopping the stroke and stabilizing the patient.
- **Subacute Phase:** The days and weeks following stabilization where medical teams assess damage and prevent secondary complications.
- **Rehabilitation Phase:** The long-term process of regaining lost skills through various therapies.

Diagnosis of Stroke

- **CT Scan or MRI:** To identify the location of the stroke and determine if it is caused by a clot or a bleed.
- **Carotid Ultrasound:** To check for fatty deposits that may have caused an ischemic stroke.
- **Echocardiogram:** To see if a blood clot from the heart traveled to the brain. For large blood clots, a procedure called a mechanical thrombectomy may be performed to physically remove the clot.

Treatment of Stroke

- **For Ischemic Stroke:** Clot-busting drugs like **tPA (tissue plasminogen activator)** can dissolve the clot if administered within a specific timeframe (usually 3 to 4.5 hours).
- **For Hemorrhagic Stroke:** Surgery may be required to repair the vessel or relieve pressure on the brain. For large blood clots, a procedure called a mechanical thrombectomy may be performed to physically remove the clot.
- **Rehabilitation:** Speech, physical, and occupational therapy are used to help survivors relearn motor skills and communication.

Prevention of Stroke

Managing high blood pressure is the most important step in stroke prevention. Other preventive measures include:

- Treating heart disease (especially AFib).
- Maintaining a healthy weight and diet.
- Quitting tobacco use and limiting alcohol.

Living with Stroke

Recovery from stroke continues long after leaving the hospital. Support involves home modifications (like grab bars), the use of assistive devices (like walkers), and emotional support for both survivors and families to manage the psychological impact of the event.

Support Groups for Stroke

- [American Stroke Association – Support Group Finder](#)

Resources

- [MedlinePlus.gov: Stroke](#)
- [NIA.nih.gov: Stroke: Signs, Cause](#)

PART I • SECTION 7

Long Term Care Program

장기 요양 프로그램 (Long Term Care Program)

Long-term healthcare infrastructure: Long-Term Care (LTC) overview, rehabilitation & skilled nursing facilities, nursing home care, hospice palliative care, adult day care, and the Long-Term Care Ombudsman.

This section contains 6 comprehensive guides

Long Term Care Overview

Long term care goes beyond simple medical treatment. It is a comprehensive support system designed to assist individuals who have difficulty performing Activities of Daily Living (ADLs) such as bathing, dressing, eating, etc. due to physical or cognitive limitations.

Types of Long-Term Care Services

Residential Care

- [Assisted Living](#)

A residential setting for those who cannot live independently but do not require the intensive medical care of a nursing home. It provides help with meals, medication management, and personal hygiene. Costs are primarily paid out-of-pocket, though Medicaid benefits may apply in limited cases.

- [Nursing Home / Skilled Nursing Facility](#)

Facilities for individuals requiring 24-hour skilled nursing care and continuous medical supervision. Medicare covers part of the cost for short-term stays, while long-term custodial care are covered by Medicaid or private funds.

[Rehabilitation and Skilled Nursing Care](#)

- Provided directly by medical staff when complex treatments (wound care, IV therapy, etc.) or intensive rehabilitation (physical/occupational/speech therapy) are required.

Home and Community-Based Services (HCBS)

- [Home Health Care](#)

Medical services provided by professionals such as nurses, home health aides, and physical/occupational therapists who visit the home per doctor's prescription.

- [Adult Day Care](#)

Provides social activities and health monitoring in a safe facility during the day, offering respite to family caregivers.

Hospice Care

- A special form of care for terminally ill patients diagnosed with a life expectancy of six months or less, and for their families. It focuses on pain relief (Palliative Care) and physical, emotional, and spiritual support rather than curative treatment. Care can continue beyond six months if recertified by a doctor.

Find a Long-Term Care Facility

- **National Facility Search:** [Medicare.gov Care Compare](https://www.medicare.gov/care-compare)
- **New Jersey:** [NJ.gov/health/ltc/find-facility](https://www.nj.gov/health/ltc/find-facility)

Considerations for Selection

- **Check the 5-Star Quality Rating:** Review the 5-star ratings for each facility on [Medicare.gov Care Compare](https://www.medicare.gov/care-compare). The rating consists of three domains:
 - Health Inspections
 - Staffing
 - Quality Measures
- **Review Survey Reports:** Request the most recent state inspection report kept at the facility, or view it online via [Medicare.gov Care Compare](https://www.medicare.gov/care-compare) and state government websites.
- **Consult the [Long-Term Care Ombudsman](#):** The Long-Term Care Ombudsman is an independent advocacy agency appointed by the Governor to protect the rights of residents in long-term care facilities and investigate reports of abuse, neglect, or complaints. The Ombudsman is the direct, confidential line for intervention and protection for residents, family members, or concerned parties.

References

- [NIA.gov: Residential Care Guide \(Assisted Living, Nursing Homes, etc.\)](https://www.nia.gov/health-topics/assisted-living)
- [NIA.gov: What Is Long-Term Care?](https://www.nia.gov/health-topics/long-term-care)
- [ACL.gov: Long-Term Care Overview and Local Service Connections](https://www.acl.gov/long-term-care-overview-and-local-service-connections)
- [Medicare.gov Care Compare: Compare Facilities and Check Coverage](https://www.medicare.gov/care-compare)
- [New Jersey Long-Term Care Ombudsman \(NJLTCO\)](https://www.njltpco.org/).

Adult Day Care

Adult Day Care provides a safe and supportive environment for seniors during the day. It is an essential community-based service that allows family caregivers to work or take necessary respite while ensuring their loved ones receive proper care.

Key Service Models

Adult Day Care typically operates under two primary models, with memory care options available in both.

- **Adult Social Day Care:** Focuses on social interaction, recreation, meals, and basic personal care.
- **Adult Day Health Care (Medical Model):** Provides professional medical services, including medication management and rehabilitation therapies, with on-site nurses or physical therapists. This model is suitable for those with complex medical needs or those at risk of nursing home placement.
- **Alzheimer's/Dementia Adult Day Services:** Offers a secure environment and cognitive stimulation programs designed for individuals with dementia or cognitive decline. This can be integrated into either social or medical day care centers, and levels of specialization vary by facility.

Cost and Coverage

Medicare

Original Medicare (Parts A and B) does not cover the cost of adult day care services. However, some Medicare Advantage (Part C) plans may provide limited coverage for adult day care.

**Conditions vary by plan, so prior verification is required.*

Private Pay

Costs vary significantly based on location, type of facility, and services provided. Additional fees may apply for transportation, specialized diets, or extended hours.

**For exact pricing, contact facilities directly or use the [Eldercare Locator](#).*

New Jersey MLTSS (Managed Long Term Services and Supports)

In New Jersey, Adult Day Care costs may be covered through the Medicaid MLTSS program. This managed care system involves NJ FamilyCare MCOs (Managed Care Organizations) paying providers directly for services.

**For details, check [LTC Medicaid \(MLTSS\)](#).*

NJ Alzheimer's Adult Day Services Program (AADSP)

Administered by the NJ DHS Division of Aging Services, this program specifically supports dementia care. It subsidizes 20% to 100% of Adult Day Care costs for up to 5 days per week.

- **Eligibility:** Diagnosis of Alzheimer's or a related dementia
- Income requirements: Individual ≤ \$50,256; Couple ≤ \$58,632 (2025 limits)
- Current non-enrollees in government assistance programs such as MLTSS Medicaid or Jersey Assistance for Community Caregiving (JACC).

Contact: NJ DHS Division of Aging Services, 609-438-4733 / www.nj.gov/humanservices/doas/services/a-k/aads/

JACC (Jersey Assistance for Community Caregiving)

Supports seniors aged 60+ who require a nursing facility level of care but choose to live in the community. Services include social adult day care, personal care assistance, and transportation. Cost-sharing may be required based on income.

Contact: County AAA/ADRC (Aging and Disability Resource Connection), 1-877-222-3737

**For details, check [Jersey Assistance for Community Caregiving \(JACC\) program](#).*

VA (Veterans Affairs) Benefits

Registered veterans may be eligible for Adult Day Care support through the VA healthcare system.

Other Payment Methods

Long-Term Care Insurance: Private policies may cover Adult Day Care.

**Reimbursement varies by plan; check your own plan.*

Questions to Ask When Choosing a Facility

- **Staff-to-Participant Ratio:** Verify whether the staff-to-participant ratio is low. While the New Jersey Department of Health (NJDOH) licensing standards establish the minimum requirements, staffing levels may vary by facility.
- **Staff Professionalism:** Verify the availability of on-site nursing staff and, in the case of specialized dementia day care, the presence of dedicated professionals with relevant expertise.

- **Transportation:** Determine whether safe transportation services are available between the residence and the facility.
- **Licensing and Inspection Records:** NJ facilities must be licensed by the NJDOH.

**Verify the facility's licensure status and review the findings of recent inspections:*

Healthapps.nj.gov/facilities

Korean Adult Day Care Centers in Bergen County

Facility Name	Address	Contact
Bergen Adult Day Care Center	139 E Central Blvd, Palisades Park, NJ 07650	201-907-4680
Bergen New Bridge Medical Center	230 E Ridgewood Ave, Paramus, NJ 07652	201-967-4000
Broadway Adult Day Care	24-20 Broadway, Fair Lawn, NJ 07410	201-797-1177
Golden Adult Day Care	53-57 West Fort Lee Road, Bogota, NJ 07603	201-457-1007
Magnolia Adult Day Care	2300 3rd Street, Fort Lee, NJ 07024	201-482-4500
My Home Adult Day Care Center Inc	37 Brook Avenue, Maywood, NJ 07607	201-291-8811
New Jersey Day Care Center-Serenity Gardens	105 Challenger Road, 6th Fl, Ridgefield Park, NJ 07660	201-773-9090
New Life Adult Medical Day Care	340 Evelyn Street, Paramus, NJ 07652	201-265-0606
Our Home Adult Day Care	475 Grand Avenue, Englewood, NJ 07631	201-731-3636
Peaceful Adult Medical Day Care Center	101 Industrial Avenue, Suite C & B, Little Ferry, NJ 07643	201-870-6265
Sunshine Adult Day Health Care Center	16 N. Washington Avenue, Bergenfield, NJ 07621	201-387-8500

References

Government Agencies (Official Sources)

- [Eldercare Locator \(ACL\) Find Local Adult Day Care Facilities](#)
- [NIA.gov — Guide to Home Services and Adult Day Care](#)
- [NJ DHS — NJ MLTSS Program Overview](#)
- [Bergen County Key Service Guide \(PDF\)](#)

Hospice Care

Hospice care is a specialized medical service designed for terminally ill patients for whom curative treatment is no longer the primary goal. The focus shifts from curing the underlying disease to ensuring the patient spends their remaining time with dignity, comfort, and peace. This palliative medical service (comfort care) model prioritizes expert symptom management while providing comprehensive psychological and spiritual support for both the patient and their families.

Service Types and Settings

Hospice is a philosophy of care rather than a specific physical location; it is designed to meet wherever they reside.

- **Settings:** Patient's home (most common), assisted living facilities, nursing homes, dedicated hospice facilities, or hospitals. Hospice service also may be provided in short-term inpatient hospital.
- **Team-Based Care:** A multidisciplinary team consisting of doctors, nurses, social workers, counselors, and volunteers works together to address the physical, emotional, social, and spiritual needs of the patient and their family.

Cost and Coverage

Medicare (Part A)

Eligibility Requirements (Must meet all conditions below):

- **Terminal Diagnosis:** The hospice medical director and the patient's attending physician (if applicable) must certify that the patient has a life expectancy of six months or less, assuming the illness runs its normal course. If the patient does not have a primary physician, the hospice medical director can provide the certification alone.
- **Transition of Care:** The patient must sign an Election Statement choosing hospice care for their terminal illness instead of other Medicare-covered curative treatments.
- **Certified Agencies:** Services must be obtained through a hospice provider officially certified by Medicare (CMS).

**Benefit Periods: Medicare hospice benefits are available for two 90-day periods, followed by an unlimited number of 60-day periods. Even after six months, care can continue in 60-day increments provided the physician recertifies the patient.*

**Medicare Hospice Out-of-Pocket Costs:*

- *General Covered Services: \$0 (No deductible)*
- *Prescription Drugs for Pain Relief (Outpatient): Copay of \$5 or less per prescription*
- *Inpatient Respite Care: 5% coinsurance of the Medicare-approved amount for short-term stays*

LTC Medicaid (MLTSS)

- **Benefit Scope:** Provides comprehensive coverage (with zero or minimal cost-sharing) for professional nursing, physician services, medications for symptom management, and medical equipment.
- **Room and Board:** For residents in nursing homes, room and board costs may be covered if the individual separately meets Medicaid eligibility requirements. Since regulations vary by state, verification is required.

**For details, check [LTC Medicaid \(MLTSS\)](#).*

Hospice Care within Nursing Facilities

In instances where a resident receives hospice care while residing in a nursing home, a distinct cost-sharing structure applies:

- **Hospice Medical Expenses:** Medicare provides full coverage for all clinical hospice services.
- **Nursing Home Room and Board:** Medicare does **not** cover room and board costs. These expenses must be paid out-of-pocket, unless the individual qualifies for **Long-Term Care Medicaid**, which may provide financial assistance for those meeting specific eligibility criteria.

Veterans Affairs (VA) Benefits

U.S. military veterans can receive hospice care through the VA healthcare system.

- **Eligibility:** All veterans enrolled in the VA healthcare system are eligible for hospice care if clinically determined to need it. This benefit is provided regardless of service-connected disability ratings or income levels. (Procedures for enrolling in the VA healthcare system can be found at [VA.gov](#)).
- **Cost Sharing:** There is no copayment (Copay) for services provided by the VA or VA-contracted agencies.
- **Locations:** Care can be received at home, in VA outpatient clinics, VA inpatient facilities (including Community Living Centers), or through VA-contracted community hospice providers.

**For instructions on how to enroll in the VA health care system, please visit [VA.gov](#).*

Private Health Insurance and Long-Term Care (LTC) Insurance

- **Private Plans:** Most private insurers mirror the Medicare hospice model. However, patients may still be responsible for deductibles or copayments.
- **LTC Insurance:** Depending on the specific policy, this may cover hospice costs in a nursing home or at home within a set daily limit.

Charity Care / Uninsured

This serves as the final safety net for individuals without insurance who face financial hardship.

- Many non-profit hospice agencies offer care at no cost or at a discounted rate through community donations or foundation funds.
- **Sliding Scale:** Costs are adjusted based on the patient's income and asset levels.

If hospice care is required while uninsured, please inquire about financial assistance options through non-profit hospice agencies or a designated social worker. (For inquiries regarding **NJ Charity Care, contact 1-866-588-5696.)*

Prescription Drug Benefit (Medicare Part D) — Separate Medicare Benefit

As of 2026, the annual out-of-pocket cap for prescription drugs covered under Medicare Part D is limited to \$2,100. After reaching this limit, the enrollee pays nothing for covered drugs for the remainder of the year.

**Note for Hospice Patients: Medications related to the terminal illness are already covered under the Medicare Hospice benefit with a maximum copay of \$5. The Part D \$2,100 cap primarily applies to medications unrelated to the terminal illness. Additionally, plan premiums and drugs covered under Part B (such as certain injectables) are not included in the calculation toward this cap.*

Prescription Drug Benefits (Medicare Parts A and D) — For Medicare Beneficiaries

- **Medications Related to Terminal Illness:** These are covered under **Medicare Part A**. Beneficiaries are responsible only for a copayment of up to \$5 per prescription.
- **Other Prescription Drugs:** Medications unrelated to the terminal illness are covered under **Medicare Part D**. A deductible of \$2,100 per year (as of 2026) applies.
- **Medicare Advantage (Part C) Enrollees:** You may maintain your existing prescription drug coverage through your current plan without the need to enroll in a separate Medicare Part D plan.
- **Extra Help (Low-Income Subsidy):** Beneficiaries also enrolled in **Medicaid** may qualify for the **Extra Help** program, which significantly reduces or eliminates the cost of medications for conditions unrelated to hospice care.

Find a Hospice Care Facility

- **National Facility Search:** [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- **New Jersey:** [NJ.gov/health/hospice/find-facility](https://www.nj.gov/health/hospice/find-facility)

**Hospice care is voluntary. You can stop hospice services at any time to return to curative treatment and may re-elect hospice benefits again in the future if needed.*

Korean-Speaking Hospice Care Facilities in Bergen County, NJ

Facility Name	Location	Contact
Family of Caring Healthcare at Park Ridge, LLC	120 Noyes Drive, Park Ridge, NJ 07656	201-505-1777
Holy Name Medical Center	718 Teaneck Road, Teaneck, NJ 07666	201-833-3756

Considerations for Selection

- **Check the 5-Star Quality Rating:** Review the 5-star ratings for each facility on [Medicare.gov Care Compare](https://www.medicare.gov/care-compare). The rating consists of three domains:
 - Health Inspections
 - Staffing
 - Quality Measures
- **Review Survey Reports:** Request the most recent state inspection report kept at the facility, or view it online via [Medicare.gov Care Compare](https://www.medicare.gov/care-compare) and state government websites.
- **Consult the [Long-Term Care Ombudsman](#):** The Long-Term Care Ombudsman is an independent advocacy agency appointed by the Governor to protect the rights of residents in long-term care facilities and investigate reports of abuse, neglect, or complaints. The Ombudsman is the direct, confidential line for intervention and protection for residents, family members, or concerned parties.

Resources

- [Medlineplus.gov – Hospice Care](https://medlineplus.gov/hospice-care)
- [Medicare.gov – Hospice Care](https://www.medicare.gov/hospice-care)
- [Medicaid.gov – Hospice Care](https://www.medicicaid.gov/hospice-care)
- [NIA.gov – Palliative Care and Hospice Care](https://www.nia.gov/palliative-care-and-hospice-care)
- [VA.gov – Hospice card for VA](https://www.va.gov/hospice-card-for-veterans)

Long Term Care Ombudsman

The New Jersey Office of the Long-Term Care Ombudsman (NJLTCO) is an independent agency appointed directly by the Governor, operating without external oversight or control pursuant to the Older Americans Act. The Ombudsman ensures that residents of long-term care facilities, including seniors aged 60 and older, receive high-quality care and are protected from abuse, neglect, and exploitation.

**All services provided by the Ombudsman are free of charge.*

Legal Authority and Role

- **Confidential Complaint Intake:** Anyone can file a complaint via phone, fax, mail, email, or online form. All consultations are held in strict confidence, and anonymous reporting is available.
- **Independent Investigation and Referral:** The office investigates received complaints to resolve them directly or, when necessary, refers cases to jurisdictional regulatory agencies such as the New Jersey Department of Health (NJDOH) or the Department of Community Affairs (NJDOH). Suspected Medicaid fraud or criminal activity may also be referred to local police, County Prosecutors, or the NJ Office of the Attorney General.
- **Unlimited Access Rights:** The Ombudsman has the right to meet with elderly residents of long-term care facilities at any time and possesses full authority to review a resident's medical and administrative records.
- **Power to Hold Hearings and Subpoena Records:** When necessary, the office can hold public or private hearings and legally subpoena relevant documents or testimony.
- **Support for Self-Advocacy:** The office cooperates with Resident Councils and Family Councils to empower long-term care residents to advocate for their own rights and interests.

Problem-Solving Procedures

1. **Complaint Intake:** A resident or their representative files a complaint (anonymous reporting is possible). Complaints must be filed within three months of the incident.
2. **Assignment:** The complaint is reviewed by a supervisor and assigned to a Field Investigator/Advocate.
**The Ombudsman is not an emergency response agency. In an emergency involving immediate danger, call 911 immediately.*
3. **Field Investigation:** Investigators visit the facility unannounced and begin the investigation after obtaining the resident's consent.

4. **Fact-Finding:** They review relevant records and conduct interviews with facility staff and witnesses to identify objective facts.
5. **Resolution and Action:** The primary goal of the investigation is to resolve issues in a manner satisfactory to the resident while ensuring their safety, rights, and dignity are upheld. In cases where abuse or neglect is identified or suspected, the matter is referred to the appropriate authorities.

**Important: The Ombudsman only assists those currently residing in a facility (or those expected to return after hospitalization). If a resident has already been discharged, contact the NJ Department of Health directly at 1-800-792-9770.*

Major Areas of Investigation

- **Financial Exploitation:** Misappropriation of assets or financial loss.
- **Involuntary Transfer and Discharge:** Forced discharge orders that bypass proper legal procedures.
- **Inappropriate Use of Restraints:** Misuse of chemical or physical restraints.
- **Abuse and Neglect:** Physical, verbal, or mental abuse; refusal to provide services necessary to maintain physical or mental health; unreasonable confinement.
- **Low Quality of Care:** Neglect of personal hygiene, delayed response to call bells or requests for help, etc.
- **Violations of Rights and Dignity:** Acts that undermine a resident's basic human rights or dignity.

Major Facilities Covered by the Ombudsman

- Nursing Homes
- Skilled Nursing & Rehab Facilities
- Assisted Living Facilities
- Adult Day Care Centers
- Residential Health Care Facilities
- Continuing Care Retirement Community
- Long-Term Acute Care Hospitals
- Veterans' Hospitals

**For issues in facilities not listed above, contact the following agencies directly:*

- **NJDOH** (Nursing Homes/Assisted Living): 1-800-792-9770
- **NJDCA** (Boarding Homes, etc.): 609-984-1704

Contact

NJ Long-Term Care Ombudsman

Phone: 1-877-582-6995

Fax: 1-609-943-3479

Email: ombudsman@ltco.nj.gov

Online Complaint: nj.gov/ooie/contact/contact-complaint.shtml

Mail: NJ Long-Term Care Ombudsman, P.O. Box 852, Trenton, NJ 08625-0852

**In case of emergency, call 911 immediately.*

References

- [NJ.gov: Long-Term Care Ombudsman](#)
- [NJ.gov Guide on How to File a Complaint to the Ombudsman](#)
- [NJ Department of Health — Long-Term Care Facility Search](#)
- [ACL.gov — National Long-Term Care Ombudsman Program Information](#)

Nursing Home Care

A nursing home provides 24-hour clinical oversight and professional nursing care for individuals who require medical treatment alongside assistance with daily activities. The government applies strict standards to ensure the safety and quality of care for residents.

Key Services

- **Medical and Nursing Care:** Professional medication management, wound care, monitoring of blood pressure and diabetes, etc.
- **Rehabilitative Therapy:** Physical therapy, occupational therapy, and speech therapy to restore physical functions.
- **Personal Support:** Assistance with Activities of Daily Living (ADLs) that are difficult to perform independently, including eating, bathing, dressing, and mobility.
- **Dietary and Management:** Customized diets designed by nutritionists and 24-hour safety monitoring.

Cost and Coverage

Short-Term Rehabilitation

Contrary to the common perception of nursing homes as permanent residences, many nursing homes also function as specialized **Short-Term Rehabilitation Centers**. They provide intensive professional care designed to help patients regain functional strength after surgery or hospitalization, ensuring a safe and successful transition back to their homes.

**For details regarding costs and coverage for short-term stays, check [Rehabilitation & Skilled Nursing](#).*

Long-Term Care: MLTSS

If continued residency in a nursing facility is required after the 100-day Medicare benefit period concludes, Medicaid-enrolled patients receive long-term coverage through **Managed Long-Term Services and Supports (MLTSS)**.

- **Coverage:** Unlike Medicare, MLTSS covers long-term **Room and Board** and assistance with activities of daily living without a specific time limit.
- **Financial Eligibility (2026)**
 - **Income and Assets:**

- **Income: \$2,982/Month**
- **Assets: \$2,000** for individuals / **\$3,000** for couples
- **Excluded Assets:** Primary residence (valued up to **\$1,130,000** in New Jersey), one vehicle, and personal effects.
- **5-Year Look-back Period:** At the time of application, the state examines all financial records from the previous 60 months to identify any assets gifted to family members or sold below fair market value.

- **Clinical Eligibility**

- A state-appointed clinician conducts an in-person assessment **NJ Choice**, a comprehensive assessment tool used to determine clinical eligibility for Nursing Facility Level of Care (NFLOC).
- The applicant must require significant assistance with at least **three Activities of Daily Living (ADLs)**, such as eating, bathing, or mobility.

**Spousal Protections: Under the [Community Spouse](#) Resource Allowance (CSRA), a spouse living at home may protect assets up to a maximum of \$162,660 (as of 2026) to prevent spousal impoverishment.*

- *Cost Structure: Beneficiaries must contribute nearly all of their monthly income toward the facility's costs. They are permitted to keep a \$50/month Personal Needs Allowance (PNA). MLTSS covers the remaining balance of the facility's fees.*

Important Considerations

- **Application Timing:** Begin preparations **2 to 3 months** before short-term rehabilitation benefits end. The approval process typically takes **3 to 9 months**, though coverage is applied retroactively to the date of application.
- **Distinction from Standard Medicaid:** Holding a NJ FamilyCare Medicaid does not automatically grant access to long-term care. You must undergo a specific clinical and financial review to be authorized for the MLTSS program.
- **Professional Consultation Advised:** Due to the state's rigorous audit of all financial records from the 60 months (5 years) preceding your application, it is strongly advised to consult with a professional social worker or an elder law attorney before submitting your application.

**For details, check [LTC Medicaid \(MLTSS\)](#).*

Prescription Drug Benefits (Medicare Part D) — For Medicare Beneficiaries

For individuals receiving long-term care in a nursing home, prescription medications are covered under **Medicare Part D**.

- **Out-of-Pocket Costs (2026):** The annual out-of-pocket cap is **\$2,100**. Once you reach this limit, you will have **\$0 copays** for all covered drugs for the remainder of the calendar year.

- **Medicare Advantage (Part C) Enrollees:** If you are enrolled in a Medicare Advantage plan that includes drug coverage (MAPD), you maintain your benefits through that plan. You do not need to enroll in a separate Part D plan.
- **Patients Without Part D:** Individuals without a Part D plan are responsible for the full cost of their medications out-of-pocket.
 - **Special Enrollment Period (SEP):** Nursing home residents have a **Special Enrollment Period**, allowing you to join, switch, or drop a Part D plan or Medicare Advantage plan that better suits their needs at any time during their facility stay.

Find a Nursing Home Facility

- **National Facility Search:** [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- **New Jersey:** [NJ.gov/health/ltc/find-facility](https://www.nj.gov/health/ltc/find-facility)

Korean-Speaking Nursing Home Facilities in Bergen County, NJ

Facility Name	Location	Contact
<u>ExcelCare at Norwood</u>	100 McClellan Street, Norwood, NJ 07648	201-768-6222
<u>Family of Caring Healthcare at Park Ridge, LLC</u>	120 Noyes Drive, Park Ridge, NJ 07656	201-505-1777
<u>Holy Name Medical Center</u>	718 Teaneck Road, Teaneck, NJ 07666	201-833-3756

Considerations for Selection

- **Check the 5-Star Quality Rating:** Review the 5-star ratings for each facility on [Medicare.gov Care Compare](https://www.medicare.gov/care-compare). The rating consists of three domains:
 - Health Inspections
 - Staffing
 - Quality Measures
- **Review Survey Reports:** Request the most recent state inspection report kept at the facility, or view it online via [Medicare.gov Care Compare](https://www.medicare.gov/care-compare) and state government websites.
- **Consult the [Long-Term Care Ombudsman](#):** The Long-Term Care Ombudsman is an independent advocacy agency appointed by the Governor to protect the rights of residents in long-term care

facilities and investigate reports of abuse, neglect, or complaints. The Ombudsman is the direct, confidential line for intervention and protection for residents, family members, or concerned parties.

Rights Residents and Families Must Know

- **Participation in Care Plan:** The right for the individual or family to participate directly in establishing the direction of treatment and care plans.
- **Protection of Assets:** The right to manage one's own funds or to receive transparent reports when entrusting fund management to the facility.
- **Defense Against Improper Discharge:** The right to not be arbitrarily evicted. Must receive written notice and the right to object to and appeal unilateral discharge actions by the facility.
- **Prevention of Abuse:** The right to be protected from physical and mental abuse or neglect. Facilities cited for abuse are identified with a warning icon on [Medicare.gov Care Compare](#).

**For details check [Long-Term Care Ombudsman](#).*

Resources

- [NIA.gov: Long-Term Care Facilities](#)
- [Medicare.gov Care Compare](#)
- [NJ DOH.gov: New Jersey Long-Term Care Facility Search](#)
- [New Jersey Long Term Care Ombudsman \(NJLTCO\)](#)

Rehabilitation & Skilled Nursing

Rehabilitation and skilled nursing services are specialized medical services intended to maintain, restore, improve, or prevent the deterioration of daily living functions and skills that have been lost or impaired due to illness, injury, or disability.

These services are designed for seniors aged 65 and older, individuals with disabilities eligible for Medicare, and anyone recovering from surgery, illness, or injury with the goal of returning to daily life and achieving independent living.

Key Services

- **Skilled Nursing:** Procedures requiring the skills of a nurse or specialized therapist, such as intravenous (IV) therapy, wound care, feeding tube management, and monitoring of diabetes or complex conditions.
- **Physical Therapy:** Improving mobility through gait training, muscle strengthening, and restoration of joint range of motion.
- **Occupational Therapy:** Functional rehabilitation for independent living, including eating, dressing, and grooming.
- **Speech Therapy:** Restoration of communication skills and treatment for swallowing disorders (dysphagia).

Cost and Coverage

Insurance coverage depends on the specific level of care you receive. Medicare classifies rehabilitation into two distinct settings based on the intensity of care needed:

- **Skilled Nursing Facilities (SNF):** Short-term skilled nursing and therapy care provided at a nursing home.
- **[Inpatient Rehabilitation Facilities \(IRF\)](#):** An inpatient rehab hospital or unit for those requiring intensive therapy.

Medicare (Part A) — Short-term Rehabilitation

Coverage Conditions (Must meet all 4):

1. **3-Day Inpatient Stay (SNF Only):** A minimum of 3 consecutive days of formal hospital **Inpatient Admission** is required before entering SNF (the day of discharge does not count). Time spent in the Emergency Room or under Observation Status is excluded from the count. This requirement does not

apply to IRF.

**Confirm your inpatient status with the hospital.*

2. **Admission within 30 Days:** Patients must enter SNF within generally 30 days of hospital discharge.
3. **Provider Certification:** A physician or healthcare provider must certify that you require daily skilled nursing or intensive rehabilitative therapy provided by or under the supervision of professional staff.
4. **Medicare-Certified Facility:** You must receive care at a Medicare-certified facility.

Out-of-Pocket Costs (Based on Benefit Period)

- **Skilled Nursing Facilities (SNF)**
 - 1~20 days: **\$0/day** (given \$1,736 deductible already spent at inpatient care).
 - 21~100 days: **\$217/day** coinsurance
- **Inpatient Rehabilitation Facilities (IRF):** An inpatient rehab hospital or unit for those requiring intensive therapy.
 - Covered under Part A with the same cost structure as a hospital stay
 - 1–60 days: **\$0/day** after paying \$1,736 deductible
 - 61–90 days: **\$434/day**
 - 91 days+: **\$868/day**; Lifetime Reserve Days is used when beyond 91 days at IRF.

**Benefit Period Note: A Benefit Period ends when you have not received SNF care for 60 consecutive days. A new period requires a new \$1,736 deductible.*

**Doctor services while at SNF/IRF are covered under Medicare Part B.*

**Out-of-pocket costs change annually; check [Medicare.gov](https://www.medicare.gov) for the latest amounts.*

**For details, check [Medicare Part A/B](#).*

Medicare Advantage (Part C) & Private Insurance

- **Prior Authorization:** Most **Medicare Advantage** and private insurance plans require **Prior Authorization** prior to admission.
- For beneficiaries enrolled in **Medicare Advantage**, the **3-day inpatient hospital stay requirement** may be waived.
- Benefits typically apply only when using in-network facilities.

**Prior authorization process and inpatient stay requirements vary by plan; please check your own plan for specific requirements.*For details, check [Medicare Part C](#).*

Medicare Supplement Insurance (Medigap)

Depending on the specific plan, Medigap can cover the \$217 daily coinsurance for days 21–100 in full or in part. Since coverage varies by plan type, it is essential to verify these benefits before enrolling.

*Medigap does not cover non-medical long-term residency costs (custodial care, room and board) at SNF.

*For details, check [Medigap](#).

Medicaid (NJ FamilyCare)

Unlike Medicare, Medicaid (NJ FamilyCare) beneficiaries incur no out-of-pocket costs for services at Skilled Nursing and Rehabilitation Facilities. However, if care transitions from short-term rehab to long-term residency, coverage shifts to the **Managed Long-Term Services and Supports (MLTSS)** program.

- For **NJ FamilyCare** beneficiaries aged 64 and under, if rehabilitation becomes prolonged and exceeds the 100-day threshold, a **Resource Test (Asset Review)** will be applied. While NJ FamilyCare waived asset limits, MLTSS has strict asset limitations (\$2,000 for individuals, \$3,000 for married couples). Proactive financial planning is encouraged to ensure eligibility and coverage.

*For details, check [ACA Medicaid](#).

Private Pay

If insurance benefits have ended or an individual does not meet eligibility requirements for assistance, they must pay for care directly. This includes lodging, meals, and care costs, which vary significantly by region and facility.

Find a Rehab & Skilled Nursing Facility

- **National Facility Search:** [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- **New Jersey:** [NJ.gov/health/ltc/find-facility](https://www.nj.gov/health/ltc/find-facility)

Major Rehab & Skilled Nursing Facilities in Bergen County

Facility Name	Location	Contact
Bergen New Bridge Medical Center	Paramus	551-383-6019
Jewish Home at Rockleigh	Rockleigh	201-784-1414
Christian Health	Wyckoff	201-729-3711
The Actors' Fund Home	Englewood	201-871-8882
Emerson Health & Rehab Center	Emerson	201-265-3700
Maple Glen Center	Fair Lawn	201-797-9522
Oakland Rehabilitation & Healthcare Center	Oakland	201-337-3300

**For details check directly on [Medicare.gov Care Compare](#)*

References

- [Medicare.gov: SNF Care](#)
- [Medicare.gov: IRF Care](#)
- [Medicare.gov: SNF Care Costs and Coverage \(PDF\)](#)
- [Medicare.gov Care Compare: Find and Compare Nearby Rehab Facilities](#)
- [CMS.gov — Skilled Nursing Facility Services](#)
- [Medicare.gov: 2026 Medicare Costs Summary Fact Sheet \(PDF\)](#)
- [Bergen County Key Services Guide \(Includes List of Rehab Facilities\) \(PDF\)](#)
- [Medicare.gov: Compare Medigap Plan](#)

PART I • SECTION 8

Hospital Care

병원 케어 (Hospital Care)

Navigating acute hospital and medical services: ambulance services, emergency room care, observation care status vs. inpatient admission, inpatient rehabilitation, medical home care, community healthcare, and bereavement support.

This section contains 9 comprehensive guides

Ambulance Services

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Ambulance services provide emergency transportation to a hospital or skilled nursing facility when a patient's health condition is in serious danger and safe transportation by other means is not possible. This service ensures that individuals receive immediate medical attention and stabilization while en route to a healthcare facility.

Emergency Medical Condition Definition

An **Emergency Medical Condition** is defined as "An illness, injury, symptom or condition so serious that a reasonable person would seek care right away to avoid severe harm." (healthcare.gov). This is determined based on the **Prudent Layperson Standard**, meaning a situation that any person with average health knowledge would reasonably consider an emergency.

Specifically, **ambulance services** are reserved for life-threatening or severe conditions—such as chest pain or difficulty breathing—where the patient's state makes it unsafe to transport them by a standard vehicle.

Coverage & Benefits

Medicare Part B

- **Coverage:** Covers ambulance costs when the service is medically necessary.
- **Limited Coverage for Non-Emergency Transport:** Medicare may provide limited coverage for non-emergency ambulance transportation if a doctor provides a written order stating it is medically necessary (e.g., for End-Stage Renal Disease patients requiring dialysis transport).
 - If non-emergency transport occurs multiple times within 30 days, prior authorization from Medicare may be required starting with the 4th trip.
- **Facility Limits:** Medicare only covers transportation to the nearest appropriate facility capable of providing required care. If a patient requests transportation to a more distant facility, they must pay the additional costs.
- **Air Transportation:** Emergency transportation by airplane or helicopter is covered only if the location is inaccessible by ground or if medical obstacles, such as long distances or heavy traffic, prevent rapid access to care.

- **Patient Cost:** After meeting the deductible, the patient is responsible for 20% of the Medicare-approved amount. The Part B coinsurance may be covered through a [Medigap](#) plan.

Medicare Advantage (Part C)

- **Coverage:** Like Original Medicare, Medicare Advantage covers ground and air ambulance services when medically necessary in an emergency.
- **Non-Emergency Transport:** Coverage requires a physician's order and prior authorization from the insurer. Some plans may also include a Non-Emergency Medical Transportation (NEMT) benefit for routine medical appointments.
- **Facility Limits:** Coverage is limited to the nearest appropriate facility. Using an out-of-network provider or a more distant facility may result in higher out-of-pocket costs.
- **Air Transportation:** Covered only when ground transport is not feasible or when a delay would put the patient's life at risk. Higher cost-sharing may apply compared to ground ambulance.
- **Patient Cost:** Plans often charge a fixed copay instead of the 20% coinsurance, and costs count toward the plan's annual out-of-pocket maximum.

Medicaid (NJ FamilyCare)

- **Coverage:** Ground and air ambulance services are covered when immediate medical help is required. Prior authorization is not required for emergency transport.
- **Non-Emergency Medical Transportation (NEMT):** NEMT is available for individuals who lack a working vehicle, do not have a driver's license, or have a physical or mental disability preventing them from traveling alone. In New Jersey, NEMT is coordinated through Modivcare (phone: 1-866-527-9933; tripcare.modivcare.com). Rides for medical appointments must generally be scheduled at least two to three days in advance.

Private Insurance

- **Coverage:** Services are covered when immediate medical help is required.
- **In-Network vs. Out-of-Network:** Patients are responsible for paying the deductibles and co-payments as defined by their specific insurance plan. In New Jersey, Patient Cost for ambulance services in an emergency are adjusted to in-network rates even when the service is provided by an out-of-network provider. Note that in-network protections for ambulance services may vary by state.
- **Prior Authorization:** While not required for emergencies, prior authorization from the insurer may be necessary for non-emergency inter-facility transfers (e.g., moving from a hospital to a specialized rehabilitation center).

** **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance*

plan.

* **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.

Cost Sharing per Coverage Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part B	\$283	20% of the Medicare-approved amount
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C), Private Insurance	Varies by plan	Varies by plan; Out-of-network providers billed at In-network rates for emergencies

Hospitals and Providers Search

- National Directory: [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- New Jersey Directory: [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Ambulance Service Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital Name	Address	Phone Number	Website
Bergen New Bridge Health Emergency Medical Services	230 East Ridgewood Avenue, Paramus, NJ 07652	201-967-4000	newbridgehealth.org/emergency-care
Englewood Health Emergency Medical Services	350 Engle Street, Englewood, NJ 07631	201- 894-3000	englewoodhealth.org/emergency-services
Hackensack Meridian Health-Pascack Valley Emergency Medical Services	250 Old Hook Road, Westwood, NJ 07675	201- 383-1035	pascackmedicalcenter.com/emergency-care
Hackensack University Medical Center Emergency Medical Services	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org/emergency-care
Holy Name Medical Center BLS Ambulance	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org/emergencycare
The Valley Hospital EMS Fleet	4 Valley Health Plaza, Paramus, NJ 07652	201-447-8000	valleyhealth.com/emergency-services

Resources

- [Medicare.gov](http://www.Medicare.gov) – Ambulance Services (Medicare Part B)
- [NJ FamilyCare.org](http://NJFamilyCare.org)
- [NJ DoHS.gov](http://NJDoHS.gov) – Modivcare Medical Transportation

Bereavement Care

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Bereavement care is the process of supporting individuals through the emotional and physical grief that follows a loss. Through professional grief counseling and therapy, bereavement care helps people navigate the pain of loss in a healthy way. While everyone's grief journey is different, recovery and a return to daily life are supported by family, community, and access to appropriate mental health resources.

Coverage & Benefits

Medicare Part A (Hospice Benefit)

- **Eligibility:** Bereavement care is included as part of the Medicare hospice benefit for those with a terminal illness (life expectancy of 6 months or less).
- **Support for Family:** Social workers, chaplains, or bereavement coordinators provide emotional and spiritual support to the family. Counseling services are available for up to **13 months** after the patient's death.
- **Patient Costs:** There are typically **\$0** out-of-pocket costs for bereavement counseling provided by a Medicare-approved hospice program.

Medicare Advantage (Part C)

- **Coverage:** Medicare Advantage plans are required to cover all services provided by Original Medicare (Parts A and B), including bereavement care under the hospice benefit.
- **Supplemental Benefits:** Some Part C plans may offer additional grief counseling or expanded family support programs through supplemental benefits.
- **Claims Processing:** Even for Part C members, hospice-related bereavement services are generally processed through Original Medicare (Part A), while the Part C plan continues to manage other supplemental services and prescription drug coverage (Part D).
- **Patient Costs:** Standard bereavement services provided through the hospice benefit generally have **\$0 out-of-pocket costs**. Costs for using a private counseling network outside of the hospice benefit will vary by plan.

Medicaid (NJ FamilyCare)

- **Hospice Care:** NJ FamilyCare covers hospice services, including bereavement support for the family, when medical necessity criteria are met.

- **Patient Costs:** For eligible recipients in New Jersey, there are generally **\$0 copayments** for hospice-related services, including bereavement counseling provided through the hospice team.
- **Palliative Care:** The Community-Based Palliative Care (CBPC) benefit provides additional support for NJ FamilyCare members with serious illnesses — a terminal diagnosis is not required. Services include symptom management, caregiver support, and psychosocial counseling. This benefit operates separately from hospice and can be used while continuing current treatment.

Private Insurance

- **Hospice Benefits:** Most private insurance plans follow Medicare guidelines for hospice care, which include bereavement services for surviving family members.
- **Mental Health Parity:** If counseling is sought outside of a hospice program, coverage is typically governed by the plan's outpatient mental health benefits. These services must be covered at a level comparable to medical and surgical benefits.
- **Continuity of Care:** After the death of a policyholder, dependents may qualify for a **60-day Special Enrollment Period** to secure new coverage (for Marketplace plans). Those on employer plans should inquire about COBRA or other available options.

Medicare Part A (Hospice Benefit)

- **Eligibility:** Bereavement care is included as part of the Medicare hospice benefit for those with a terminal illness (life expectancy of 6 months or less).
- **Support for Family:** Social workers, chaplains, or bereavement coordinators provide emotional and spiritual support to the family. Counseling services are available for up to **13 months** after the patient's death.
- **Patient Costs:** There are typically **\$0** out-of-pocket costs for bereavement counseling provided by a Medicare-approved hospice program.

Cost Sharing by Insurance Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part A (Hospice)	\$0 for Hospice	\$0 for Bereavement Counseling
Medicaid (NJ FamilyCare)	\$0	\$0 for covered hospice/bereavement services
Medicaid Advantage (Part C), Private Insurance	Varies by Plan	Varies by plan; Hospice services often have \$0 or low cost-sharing

Hospice and Bereavement Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital/Provider Name	Address	Phone Number	Website
Family of Caring Hospice (Korean-speaking staff)	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org
Valley Hospice	15 Essex Road, Paramus, NJ 07652	201-447-8000	valleyhealth.com
Hackensack Meridian Health Hospice	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org
Ennoble Care (In-Home)	Serving Bergen County	800-548-3249	ennoblecare.com

Contact

- 988lifeline.org 988 (call/text)
- [AARP Grief and Loss Program](#): 800-687-2277
- [Vantage Health – Survivors After Suicide](#) : 201-385-4400 / info@vantagenj.org

Resources

- Medicare.gov: Hospice Care Coverage
- NJFamilyCare.org
- [SAMHSA: Grieving Resources](#)
- NJ 211.org

Community Clinics & Free Healthcare

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

For residents without insurance or those with limited financial means, community clinics provide high-quality medical and dental care. These facilities offer services on a sliding fee scale or at no cost to ensure that every community member has access to essential healthcare.

Free and Charitable Clinics

Free and Charitable Clinics are non-profit organizations that provide medical, dental, vision, and mental health services at no cost or low cost to vulnerable and uninsured residents.

- [Bergen Volunteer Medical Initiative \(BVMI\)](#): A volunteer-based nonprofit community clinic providing free primary and preventive care for **working uninsured residents of Bergen County**. Services include chronic disease management, women's health, and mental health.
 - **Eligibility (all criteria must be met):**
 - Bergen County residents 19 and above
 - At least one working member in the household
 - Household income at or below 250% of the Federal Poverty Level (FPL)
 - No health insurance coverage (including Medicare or Medicaid)

**Eligibility requirements and services vary by clinic.*

Mobile Health Units

Mobile health units are medical vehicles equipped with professional equipment that bring care directly to the community. Mobile health units are especially helpful for residents who have difficulty visiting a physical office due to medical or geographical barriers.

- [Zufall Health Mobile Medical](#): Providing screenings, primary care, and dental services across Morris, Warren, Sussex, Hunterdon, Essex, Somerset, and Middlesex Counties.

Federally Qualified Health Centers (FQHC)

Federally Qualified Health Centers (FQHCs) are community-based healthcare organizations certified and funded by the Health Resources and Services Administration (HRSA) to provide comprehensive care to local communities.

- **[North Hudson Community Action Corporation \(NHCAC\)](#)**: Operates multiple sites in Bergen County, including Hackensack, Englewood, and Garfield. Services include adult medicine, pediatrics, dentistry, and behavioral health.
- **[Zufall Health Center](#)**: Provides affordable healthcare across multiple counties in Northern/Central NJ, specializing in primary care, dental services, and chronic condition management.
 - Eligibility (largely the same across FQHCs)
 - Open to all regardless of status
 - Uninsured individuals with incomes at or below 200% FPL are charged based on income and family size
 - Low-income patients may pay little to nothing
 - Medicare, Medicaid, and private insurance are accepted.

Hospital-Based Services & Support

Hospitals often provide outpatient community clinics, mobile health unit services, and financial assistance programs for financially disadvantaged members of the community.

Hospital-Run Community Clinics

- **Valley Health Family Care Center (Ridgewood)**: An outpatient clinic providing high-quality care to all ages regardless of insurance status. Services include primary care, pediatrics, OB/GYN, and chronic disease management. Multilingual staff available.
- **Hackensack MH Internal Medicine Academic Practice (Hackensack)**: A residency-based clinic where resident doctors provide care under specialist supervision. Offers internal medicine and preventive care with flexible fees for low-income and uninsured patients.
- **Hackensack MH Adult Health Center (Glen Ridge)**: A community clinic located at the Mountainside Medical Center campus providing primary care and chronic disease management for uninsured adults.

[Charity Care \(Hospital Financial Assistance\)](#)

Charity Care is a medical bill reduction program provided by state-designated acute care hospitals and university hospitals for low-income individuals who are uninsured or underinsured. Please note that Charity Care is not health insurance.

Community Clinics & Free Healthcare Search

- National Charitable Clinics Search: NAFCclinics.org/find-clinic
- FQHC Search: HRSA.gov/Find a Health Center
- NJ Free/Low Cost Services: NJ.gov/Free or Low Cost Medical Treatment
- NJ Community Health Search: <https://healthapps.nj.gov/fhs>

Community Clinics in Northern New Jersey

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Clinic	Clinic Type	Address	Phone	Website
Bergen Volunteer Medical Initiative (BVMI)	Free and Charitable Clinic	75 Essex Street, Suite 100 Hackensack, NJ 07601	201-342- 2478	bvmi.org
NHCAC Health Center - Englewood	FQHC	197 South Van Brunt Street Englewood, NJ 07631	201-210- 0200 (Main Number)	nhcac.org
NHCAC Health Center - Hackensack	FQHC	25 East Salem Street Hackensack, NJ 07055	201-210- 0200 (Main Number)	nhcac.org
NHCAC Health Center - Garfield	FQHC	535 Midland Ave Garfield, NJ 07026	201-210- 0200 (Main Number)	nhcac.org
NHCAC Health Center - Passaic	FQHC	220 Passaic Street Passaic, NJ 07055	201-210- 0200 (Main Number)	ennoblecare.com
NHCAC Health Center - West New York (Main Site)	FQHC	5301 Broadway West New York, NJ 07093	201-210- 0200 (Main Number)	ennoblecare.com
Zufall Health Center - Morristown	FQHC	4 Atno Ave Morristown, NJ 07960	973-267- 0002	zufallhealth.org
Zufall Health Center - Newton	FQHC	238 Spring Street Newton, NJ 07860	973-862- 6650	zufallhealth.org
Zufall Health Center - Plainsboro	FQHC	One Plainsboro Road Plainsboro, NJ 08536	609-853- 7600	zufallhealth.org
Zufall Health Center - West Orange	FQHC	49 Mount Pleasant Ave West Orange, NJ 07052	973-325- 2266	zufallhealth.org
Zufall Health Center - Mobile Medical	FQHC Mobile Health Unit	Middlesex, Morris, Sussex, Warren, Hunterdon,	973-328- 3344	zufallhealth.org/about-us/locations/zufall-health-mobile-medical/

Clinic	Clinic Type	Address	Phone	Website
		Somerset, and Essex Counties		
Valley Health Family Care Center	Hospital-Run Community Clinic	223 N. Van Dien Ave Ridgewood, NJ 07450	201-689-3900	valleyhealth.com/services/family-care-center
Hackensack MH Internal Medicine Academic Practice	Hospital-Run Community Clinic	385 Prospect Ave Suite 204 Hackensack, NJ 07061	551-996-9150	getcare.hackensackmeridianhealth.org/en/locations/hmh-internal-medicine-academic-practice-hackensack
Hackensack MH Mountainside Medical Center - Adult Health Center	Hospital-Run Community Clinic	123 Highland Ave. Suite 201 Glen Ridge, NJ 07028	862-333-4700	mountainsidemedicalcenter.com/services/adult-health-center

Resources

- HRSA.gov
- nafcclinics.org
- NJ.gov – [Charity Care](#)

Emergency Room (ER) Care

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Emergency Department services provide immediate evaluation and treatment for sudden illnesses or injuries. These services are essential when a person's health or life is at risk and requires rapid medical intervention to stabilize their condition.

Coverage & Benefits

Medicare Part B

- **Coverage:** Emergency room services are generally covered under Medicare Part B as **outpatient services**. This applies even if the patient stays in the ER overnight for observation.
- **Observation Services:** Patients may be kept in a "Clinical Decision Unit" or under "Observation Status" to determine if they need to be admitted as an inpatient. Typically, this stay does not exceed two nights.
- **Admission to Inpatient Care:** If a patient is formally admitted to the same hospital as an inpatient for the same condition within 3 days of the ER visit, the ER services are generally covered under **Medicare Part A** instead of Part B.
- **Patient Cost:** After meeting the Part B **deductible**, the patient is responsible for 20% of the Medicare-approved amount for the doctor's services(**coinsurance**) and a **copayment** for each hospital outpatient department visit.

Medicaid (NJ FamilyCare)

- **Coverage:** Medicaid covers emergency room visits when a medical need is immediate. For eligible recipients, emergency services are a mandatory benefit.
- **Cost Sharing:** In New Jersey, Medicaid recipients typically have **\$0** copayments for emergency room services that meet the medical necessity criteria.
- **Prior Authorization:** No prior authorization is required for emergency department services.

Private Insurance

- **Coverage:** Most private plans cover ER visits based on the prudent layperson standard.
- **In-Network vs. Out-of-Network:** Under the federal No Surprise Act and New Jersey's Consumer Protection Act, emergency services and subsequent observation care costs for patients are In-Network rates even if the facility or provider is out-of-network.

- **Admission Notice:** While the initial ER visit does not require pre-approval, the hospital must notify the insurer if the patient is subsequently admitted to the hospital.

* **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance plan.

* **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.

Cost Sharing by Insurance Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part B	\$283	20% of the Medicare-approved amount + Hospital Copayment
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C), Private Insurance	Varies by plan	Varies; In-network rates apply to emergencies and observation cares

Hospitals and Providers Search

- National Directory: [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- New Jersey Directory: [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Emergency Care Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital Name	Address	Phone Number	Website
Bergen New Bridge Health Emergency Medical Services	230 East Ridgewood Avenue, Paramus, NJ 07652	201-967-4000	newbridgehealth.org/emergency-care
Englewood Health Emergency Medical Services	350 Engle Street, Englewood, NJ 07631	201- 894-3000	englewoodhealth.org/emergency-services
Hackensack Meridian Health-Pascack Valley Emergency Medical Services	250 Old Hook Road, Westwood, NJ 07675	201- 383-1035	pascackmedicalcenter.com/emergency-care
Hackensack University Medical Center Emergency Medical Services	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org/emergency-care
Holy Name Medical Center Emergency Care Center	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org/emergencycare
The Valley Hospital Clinical Decision Unit	4 Valley Health Plaza, Paramus, NJ 07652	201-447-8000	valleyhealth.com/emergency-services

Resources

- [Medicare.gov: Emergency Department Services](https://www.medicare.gov/emergency-department-services)
- [Medicare.gov – Outpatient hospital services](https://www.medicare.gov/outpatient-hospital-services)
- [NJ FamilyCare.org](http://NJFamilyCare.org)

Inpatient Admission

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Inpatient Care refers to health care services provided to a patient who has been formally admitted to a hospital based on a doctor's order. This level of care is generally required for serious medical conditions, surgeries, or treatments that require continuous monitoring by a medical team.

Hospital Status and Admission

A person is considered an inpatient starting the moment they are formally admitted to the hospital with a physician's order. It is important to note that staying in a hospital bed overnight—even in the Emergency Room—does not automatically make someone an "inpatient."

If a patient is placed under Observation Status, they are classified as an outpatient. This distinction matters because it affects how much you pay and whether Medicare will cover follow-up care at a skilled nursing or rehabilitation facility. Time spent under observation does not count toward Medicare's required 3-consecutive-day inpatient stay for skilled nursing facility (SNF) coverage.

Coverage and Benefits

Medicare Part A (Hospital Facility Costs)

- **Coverage:** Covers semi-private rooms, meals, general nursing, drugs provided as part of inpatient treatment, and other hospital services and supplies. Private rooms and private-duty nurses are not covered.
- **Patient Cost:** Patient pays the inpatient deductible (\$1,736 in 2026) first. Part A covers the rest.
- **Benefit Period:** Benefit period begins the day you are formally admitted as an inpatient. If you are readmitted within 60 days of discharge, it counts as the same benefit period—and the same deductible applies. If readmitted after 60 days, a new benefit period begins and a new \$1,736 deductible applies. There is no limit on the number of benefit periods.
- **Lifetime Reserve Days:** If a hospital stay exceeds 90 days in a single benefit period, Medicare provides an additional 60 days of coverage over your lifetime. Once these reserve days are used up, you are responsible for the full cost of any remaining hospital days.

Medicare Part B (Physician Services)

- **Coverage:** Covers physician and surgical services received while you are an inpatient.

- **Patient Cost:** Patient pays the annual deductible (\$283 in 2026). After that, Part B generally covers 80% of physician costs, and the patient pays the remaining 20%.

**Even during an inpatient stay, physician services are billed separately under Part B and not Part A.*

Medicare Advantage (Part C)

- **Coverage:** Covers semi-private rooms, meals, general nursing, drugs provided as part of inpatient treatment, and other hospital services and supplies. Private rooms and private-duty nurses are not covered.
- **Prior Authorization:** Most Advantage plans require prior authorization (PA) from the insurer before a non-emergency inpatient admission. Skipping this step may result in reduced or denied coverage.
- **Patient Cost:** Many plans charge a set daily copayment for the first few days of a hospital stay, after which coverage is typically \$0. All costs count toward your plan's annual Maximum Out-of-Pocket limit.

**Coverage and cost vary by plan—always review your plan details.*

Medicaid (NJ FamilyCare)

- **Coverage:** Medicaid covers all medically necessary inpatient hospital services for eligible residents.
- **Patient Cost:** Medicaid recipients typically have little to no copayment for inpatient hospital stays.
- **Prior Authorization:** Some elective (non-emergency) inpatient admissions may require prior authorization from the plan or the state to confirm medical necessity.

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details. Refer to the [official website](#) for the most current information.*

Private Insurance

- **Coverage:** Most private plans cover inpatient admissions, but often require prior authorization unless the admission follows an emergency.
- **In-Network vs. Out-of-Network:** Costs are significantly lower when using in-network hospitals. Out-of-network admissions may result in much higher out-of-pocket expenses or no coverage at all for elective procedures.
- **Patient Cost:** Some plans charge a per-day copayment for the first several days, while others apply a flat deductible per admission. Cost structures vary by plan—always review your plan documents.

** **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance plan.*

** **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.*

Cost Sharing by Insurance Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part A (Hospital Facility Costs)	\$1,736	Days 1–60: \$0 Days 61–90: \$434/day Days 91+: \$868/day (per reserve day). After reserve days: full cost
Medicare Part B (Physician Services)	\$283	20% of Medicare-approved amount
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C) Private Insurance	Varies by plan	Varies by plan ; Prior authorization may be required

Hospitals and Providers Search

- National Directory: [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- New Jersey Directory: [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Inpatient Care Hospitals in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital Name	Address	Phone Number	Website
Bergen New Bridge Medical Center	230 East Ridgewood Avenue, Paramus, NJ 07652	201-967-4000	newbridgehealth.org
Englewood Health	350 Engle Street, Englewood, NJ 07631	201- 894-3000	englewoodhealth.org
Hackensack Meridian Health-Pascack Valley	250 Old Hook Road, Westwood, NJ 07675	201- 383-1035	pascackmedicalcenter.com
Hackensack University Medical Center	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org
Holy Name Medical Center	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org
The Valley Hospital	4 Valley Health Plaza, Paramus, NJ 07652	201-447-8000	valleyhealth.com

Resources

- Medicare.gov: Inpatient Hospital Care Coverage
- [NJ FamilyCare.org](http://NJFamilyCare.org): Benefit Overview

Inpatient Rehabilitation Care

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Inpatient rehabilitation services provide intensive therapy for patients recovering from serious surgery, illness, or injury. This level of care is designed for individuals who require a coordinated team of doctors and therapists, including at least three hours of intensive therapy per day (such as physical, occupational, or speech therapy).

Coverage and Benefits

Medicare Part A

- **Qualification:** Inpatient rehabilitation is covered under Medicare Part A (Hospital Insurance) when a physician certifies that the patient requires intensive rehabilitation, ongoing medical supervision, and coordinated interdisciplinary care. Services must be provided in a specialized Rehabilitation Hospital or a dedicated inpatient rehabilitation unit of a general hospital.
- **Qualifying Stay:** For most facilities, transfer to a rehabilitation facility following a qualifying inpatient hospital stay is a standard requirement. Certain inpatient rehabilitation facilities (IRFs) may apply specific clinical criteria for admission based on the intensity of therapy needed.
- **Coverage:** Covers semi-private rooms, meals, general nursing, drugs provided as part of inpatient treatment, and other hospital services and supplies. Private rooms and private-duty nurses are not covered.
- **Duration of Care:** Medicare covers up to 100 days of care per benefit period. A benefit period begins on the day of admission. If a patient is readmitted within 60 days of discharge, it counts as the same benefit period. If readmitted after 60 days, a new benefit period begins—resetting the day count and triggering a new deductible.
- **Patient Cost:** After meeting the Part A deductible, there is \$0 coinsurance for days 1–20. Daily coinsurance applies for days 21 through 100.

Medicare Part B (Physician Services)

- **Coverage:** Covers physician and specialist services received while you are an inpatient.
- **Patient Cost:** After meeting the annual Part B deductible (\$283 in 2026), Medicare covers 80% of the approved cost for physician services, and the patient pays the remaining 20%.

**Even during an inpatient stay, physician services are billed separately under Part B and not Part A.*

Medicaid (NJ FamilyCare)

- **Coverage:** NJ FamilyCare covers inpatient rehabilitation services when they are medically necessary and prescribed by a physician. Coverage typically includes room and board, nursing care, and rehabilitative therapies.
- **Cost Sharing:** For most NJ FamilyCare beneficiaries, there is a \$0 copayment for inpatient services
- **Prior Authorization:** Most Medicaid Managed Care Organizations (MCOs) in New Jersey require prior authorization from the insurance provider before admission to a rehabilitation facility.

Private Insurance

- **Coverage:** Most private health plans cover inpatient rehabilitation, but the duration and facility types depend on the specific policy.
- **In-Network vs. Out-of-Network:** Patients are protected from balance billing for certain services. However, for non-emergency inpatient rehab, using in-network facilities is essential to minimize out-of-pocket costs.
- **Clinical Review:** Private insurers often require concurrent review—periodically checking the patient's progress to determine whether continued inpatient care remains medically necessary.

Cost Sharing by Insurance Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part A (Hospital Facility Costs)	\$1,736	Days 1–60: \$0 Days 61–90: \$434/day Days 91+: \$868/day (per reserve day). After reserve days: full cost
Medicare Part B (Physician Services)	\$283	20% of Medicare-approved amount
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C) Private Insurance	Varies by Plan	Varies by plan ; Prior authorization may be required

Rehabilitation Facilities Search

- **National Directory:** [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- **New Jersey Directory:** [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Inpatient Rehabilitation Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital Name	Address	Phone Number	Website
Allendale Rehabilitation & Healthcare Center	85 Harreton Rd, Allendale, NJ 07401	201-825-0660	allendalehc.com
Christian Health (Bolger Short-Term Rehab)	301 Sicomac Ave, Wyckoff, NJ 07481	201-848-5200	christianhealthnj.org
Encompass Health Rehabilitation Hospital	250 Old Hook Road, Westwood, NJ 07675	201-358-5800	encompasshealth.com
Hackensack University Medical Center - Rehab Unit	30 Prospect Avenue, Hackensack, NJ 07601	551-996-5555	hackensackmeridianhealth.org
Holy Name Medical Center - Rehab Unit	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org
Kessler Institute for Rehabilitation	300 Market St, Saddle Brook, NJ 07663	201-368-6000	kessler-rehab.com

Resources

- [Medicare.gov: Inpatient Rehabilitation Care](https://www.medicare.gov/inpatient-rehabilitation-care)
- [NJ FamilyCare: Member Benefits Overview](https://www.njfamilycare.org/member-benefits-overview)
- [Medicare.gov: Official Medicare Booklet: Medicare & You 2026](https://www.medicare.gov/publications/official-medicare-booklet-medicare-you-2026)

Medical Home Care

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Medical home care brings professional medical services directly to patients in the comfort of their own home without the need for a hospital stay or nursing facility. It allows people recovering from illness or injury to maintain their independence and daily routines while still receiving skilled care from nurses and therapists.

Medical home care is provided through Medicare and/or Medicaid-certified Home Health Agencies (HHAs), under a physician's orders. Services may include hospital-based home care programs, visiting nurse services, physical, occupational, and speech therapy, and medical social services.

For day-to-day assistance such as bathing, meal preparation, or light housekeeping, please visit our [Home Care](#) page.

Coverage and Benefits

Original Medicare (Part A & B)

- **Eligibility:** Original Medicare covers home health care for patients who are homebound. A doctor or nurse practitioner must conduct a face-to-face evaluation to certify the need for home care. This evaluation must occur within 90 days before or 30 days after care begins. Brief, infrequent absences, such as attending a religious service or a medical appointment, are permitted.
- **Coverage:** Costs may be covered under Medicare Part A (Hospital Insurance) or Part B (Physician Insurance).
- **Prior Hospitalization:** Both Part A and Part B can cover home health care. A prior hospital stay is not required if the homebound requirement and physician's order are met, Part B can provide coverage. Part A may apply following a hospitalization or a Skilled Nursing Facility (SNF) stay.
- **Coverage Scope and Limitations:**
 - **Skilled Nursing & Rehab Services:** Covered for as long as care is medically necessary and the patient remain homebound (physician re-certification required every 60 days; no visit limit).
 - **Home Health Care:** Covered without a time limit, as long as it is medically necessary (physician re-certification required every 60 days). Typically limited to up to 8 hours per day and 28 hours per week; may be temporarily expanded to up to 35 hours per week (under 8 hours/day) when medically needed.

- **Not Covered:** 24-hour around-the-clock care, meal delivery, housekeeping (cleaning, laundry), shopping, and other non-medical custodial services.
- **Patient Costs:** There is no copay for approved home health services. If durable medical equipment (DME, wheelchair or walker) is included, a 20% coinsurance applies under Part B.

Medicare Advantage (Part C)

- **Coverage:** Includes all standard Original Medicare benefits, and some plans may offer additional support beyond what Original Medicare covers.
- **Extra Benefits:** Some plans provide additional services not covered by Original Medicare, such as transportation to medical appointments or limited in-home assistance.
- **In-Network vs. Out-of-Network:** Benefits apply when using home care providers that are in-network with your plan. Using out-of-network providers may result in significantly higher out-of-pocket costs, or no coverage at all, depending on the plan.

** **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance plan.*

** **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.*

**Most Medicare Advantage plans require prior authorization before home care services begin. Contact your plan ahead.*

Long-Term Care Medicaid (MLTSS)

- **Eligibility:** MLTSS is available to Medicaid recipients who require nursing home-level care (assistance with 3 or more Activities of Daily Living) and meet income and asset requirements. For those who have both Medicare and Medicaid (**dual eligibles**), Medicare generally covers skilled home health care, while MLTSS covers long-term personal care services.
- **Patient Costs:** For those who qualify, services are typically provided at no cost to the patient.

**For details visit our [MLTSS](#) Page.*

Private Insurance

- **Coverage:** Coverage scope and duration vary by plan. Review your policy or contact your insurer before receiving services.
- **Prior Authorization:** Most private insurers require prior authorization before services begin.

Cost Sharing by Insurance Type (2026)

Insurance Type	Deductible	Coinsurance / Copayment
Original Medicare (Part A & B)	\$0 for approved services)	\$0; 20% coinsurance for DME after Part B deductible of \$283 is met
Long Term Care Medicaid (MLTSS)	\$0	\$0
Private Insurance	Varies by Plan	Varies by plan; Prior authorization may be required

Medical Home Care Agency Search

- [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- [NJ.gov Official NJ Home Care License Lookup](https://www.nj.gov/office-of-the-attorney-general/divisions/health-care-licensing/)
- [NJ Home Care Accreditation Council Registry \(cahcusa\)](https://www.cahcusa.org/)
- [National Alliance of Care at Home Provider Directory](https://www.nationalallianceofcareathome.org/provider-directory/)

Skilled Nursing Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Agency	Phone Number	Website
Adoration Home Health	973-365-5200	adorationhealth.com/location/adoration-home-health-clifton-new-jersey
Amedisys Home Health and Hospice Services	201-342-6311	locations.amedisys.com/nj
Ascend Home Health	201-358-2666	ascendhealth.com
Holy Name Home Care	201-833-3740	holyname.org/forseniors/home-health-care
VNA of Englewood	201-894-3333	allianceforcareathome.org/nahc_provider/vna-of-englewood-home-health-hospice
Hackensack MH Home Health Care/Visiting Health Services NJ	1-844-777-0711	hackensackmeridianhealth.org/en/services/home-health-care-service
Valley Home Care	201-291-6000	valleyhealth.com/homecare

Resources

- Medicare.gov: Home Health Services
- Medicare.gov: What's not covered by Part A & Part B?
- NJ.gov MLTSS

Observation Care

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Observation care is a hospital-based outpatient service that includes short-term monitoring, assessment, and treatment, provided to determine whether a patient requires formal inpatient admission or can be safely discharged.

Observation Status Definition

- **Duration:** Observation care is a short-term service. The decision on whether formal inpatient admission is needed is typically made within 24 to 48 hours.
- **Transition to Inpatient:** If the medical condition worsens or requires long-term care, the status may convert to formal inpatient admission.
- **Impact on Follow-up:** Time spent in observation status does not count toward the 3-consecutive-day inpatient stay requirement for Medicare coverage of skilled nursing facility (SNF) or rehabilitation facility care.

Coverage & Benefits

Medicare Part B

- **Coverage:** Observation care is classified as an outpatient service and is covered under Medicare Part B.
 - **Medicare Outpatient Observation Notice:** Hospitals are required to provide a formal notice to Medicare beneficiaries who have been in observation status for more than 24 hours, informing them that they are being treated as an outpatient.
- **Patient Cost:** After meeting the Part B deductible (\$283 in 2026), patients are responsible for 20% coinsurance for each of the following: the ER visit copayment, ancillary emergency services (such as X-rays, CT scans, and EKGs), and physician services based on the Medicare-approved amount. Part B coinsurance may be covered through a Medigap plan.
- **Prescription Drug Coverage:** Prescription drugs taken during an ER visit or observation stay — including your regular medications — are not covered under Medicare Part B. They are covered under your **Part D plan**. If the hospital pharmacy is out-of-network for your Part D plan, you may need to pay the full cost upfront and then submit a claim for reimbursement.

** Copayments for ancillary emergency services will not exceed the Part A inpatient deductible (\$1,736*

in 2026) per service. However, receiving multiple services simultaneously may lead to the combined copayment amounts exceeding this limit.

** Original Medicare (Part A and Part B) generally does not cover emergency services received outside the United States, with limited exceptions.*

Medicare Advantage (Part C)

- **Coverage:** Observation care is classified as an outpatient service under Medicare Advantage as well.
 - **Medicare Outpatient Observation Notice:** Hospitals are required to provide a formal notice to Medicare Advantage beneficiaries in observation status.
- **Skilled Nursing Facility Eligibility:** Time spent in observation status does not count toward the 3-day inpatient stay requirement for SNF benefits. However, many Medicare Advantage plans waive this 3-day requirement. Check your healthcare plan for details.
- **Patient Cost:** Outpatient copayments apply instead of an inpatient deductible. Separate copayments may apply for individual services such as tests or medications, but the copayment for any single service typically does not exceed the inpatient deductible.

**Coverage and cost vary by plan—always review your plan details.*

Medicaid (NJ FamilyCare)

- **Coverage:** NJ FamilyCare covers observation care when it is determined to be medically necessary to evaluate a patient's condition.
- **Patient Cost:** For most eligible NJ FamilyCare members, there is little to no copayment for observation care provided at a participating hospital.
- **Prior Authorization:** Emergency observation typically does not require upfront approval, but the facility must follow standard notification protocols.

Private Insurance

- **Coverage:** Most private insurance plans cover observation care as an outpatient service.
- **In-Network vs. Out-of-Network:** Under the federal No Surprise Act and New Jersey's Consumer Protection Act, patients in New Jersey receive in-network rates for observation care even if the facility is out-of-network.

** **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance plan.*

** **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.*

Cost Sharing by Insurance Type (2026)

Insurance Type	Annual Deductible	Coinsurance / Copayment
Medicare Part B	\$283	Observation care copay + 20% coinsurance for physician services and each individual service
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C), Private Insurance	Varies by plan	Varies by plan ; In-network rates apply to emergencies and observation cares

Hospitals and Providers Search

- National Directory: [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- New Jersey Directory: [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Observation Care (Emergency Room) Providers in Bergen County

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Hospital Name	Address	Phone Number	Website
Bergen New Bridge Health Emergency Medical Services	230 East Ridgewood Avenue, Paramus, NJ 07652	201-967-4000	newbridgehealth.org/emergency-care
Englewood Health Emergency Medical Services	350 Engle Street, Englewood, NJ 07631	201- 894-3000	englewoodhealth.org/emergency-services
Hackensack Meridian Health-Pascack Valley Emergency Medical Services	250 Old Hook Road, Westwood, NJ 07675	201- 383-1035	pascackmedicalcenter.com/emergency-care
Hackensack University Medical Center Emergency Medical Services	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org/emergency-care
Holy Name Medical Center Emergency Care Center	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org/emergencycare
The Valley Hospital Clinical Decision Unit	4 Valley Health Plaza, Paramus, NJ 07652	201-447-8000	valleyhealth.com/emergency-services

Resources

- [Medicare.gov: Inpatient vs. Outpatient Status and Costs](https://www.medicare.gov/inpatient-vs-outpatient-status-and-costs)
- [Medicare.gov – Outpatient hospital services](https://www.medicare.gov/outpatient-hospital-services)
- [NJFamilyCare.org: Benefits and Covered Services](https://www.njfamilycare.org/benefits-and-covered-services)

Treatment

This content is for informational purposes only. For medical advice or diagnosis, consult a healthcare professional. In case of an emergency, immediately call 911.

Treatment, usually chronic pain management, involves medical care and services designed to manage persistent or recurring pain lasting longer than 3 months. These services aim to improve a patient's quality of life through a coordinated care plan that includes assessment, treatment planning, and ongoing management.

Coverage and Benefits

Medicare Part A (Hospital Facility Costs)

- **Coverage:** Covers semi-private rooms, meals, general nursing, drugs provided as part of inpatient treatment, and other hospital services and supplies. Private rooms and private-duty nurses are not covered.
- **Patient Cost:** Patient pays the inpatient deductible (\$1,736 in 2026) first. Part A covers the rest.
- **Benefit Period:** Benefit period begins the day you are formally admitted as an inpatient. If you are readmitted within 60 days of discharge, it counts as the same benefit period—and the same deductible applies. If readmitted after 60 days, a new benefit period begins and a new \$1,736 deductible applies. There is no limit on the number of benefit periods.
- **Lifetime Reserve Days:** If a hospital stay exceeds 90 days in a single benefit period, Medicare provides an additional 60 days of coverage over your lifetime. Once these reserve days are used up, you are responsible for the full cost of any remaining hospital days.

Medicare Part B (Physician Services)

- **Coverage:** Outpatient chronic pain management (CPM) services are covered under Medicare Part B. This includes diagnosis, pain rating scale administration, medication management, and care coordination.
- **Integrated Services:** Coverage extends to physical therapy, occupational therapy, and acupuncture (specifically for chronic low back pain) when performed by Medicare-enrolled providers.
- **Patient Cost:** After meeting the annual Part B deductible, patients are responsible for 20% of the Medicare-approved amount. If services are received in a hospital outpatient clinic, an additional copayment to the hospital may apply.

Medicare Advantage (Part C)

- **Coverage:** Plans cover integrated pain management services that are medically necessary for patients with chronic pain lasting 3 months or longer.
- **Covered Services:** Includes pain assessment, medication management (both opioid and non-opioid), physical therapy, behavioral health counseling, and individualized care planning.
- **Prior Authorization & Limits:** Procedures such as epidural injections or nerve blocks, as well as opioid prescriptions, may require prior authorization. Physical therapy and certain other services may have annual visit limits.
- **Integrated Care Approach:** Plans are encouraged to go beyond prescriptions alone — specialists and therapists work together under a coordinated care plan to help restore function and improve quality of life.
- **Patient Cost:** Copays vary by service type. All costs count toward the plan's annual maximum out-of-pocket limit.

**Coverage and cost vary by plan—always review your plan details.*

Medicaid (NJ FamilyCare)

- **Coverage:** NJ FamilyCare covers all medically necessary treatment, including specialist visits, surgery, physical therapy, occupational therapy, and pain management services.
- **Patient Cost:** Most eligible recipients pay \$0 for covered services. Adults at or below 138% of the Federal Poverty Level (FPL) have no out-of-pocket costs. Only CHIP enrollees may have a small copay.
- **Prior Authorization:** Some specialized treatments or long-term therapy may require prior authorization from the managed care organization (MMO).

**OBBA (One Big Beautiful Bill Act) implementations may affect eligibility and program details. Refer to the [official website](#) for the most current information.*

Private Insurance

- **Coverage:** Most private plans cover chronic pain treatments including injections, physical therapy, and behavioral health services when they are medically necessary.
- **Medical Necessity:** Insurers typically require detailed documentation from a physician showing that conservative treatments such as medication or exercise were tried first.
- **New Jersey Regulations:** Under New Jersey law, providers must enter into a formal **pain management agreement** with the patient when prescribing certain medications for chronic pain, and the treatment plan must be reviewed every 3 months.
- **In-Network vs. Out-of-Network:** Costs are significantly lower when using in-network hospitals. Out-of-network admissions may result in much higher out-of-pocket expenses or no coverage at all for elective procedures.

* **In-Network** refers to hospitals, physicians, and providers (including doctors, nurse practitioners, and physical therapists) who have a contracted agreement with your insurance plan.

* **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.

Cost Sharing by Insurance Type (2026)

The following are non-emergency contacts. In case of an emergency, immediately call 911.

Insurance Type	Deductible	Coinsurance / Copayment
Medicare Part A (Hospital Facility Costs)	\$1,736	Days 1–60: \$0 Days 61–90: \$434/day Days 91+: \$868/day (per reserve day). After reserve days: full cost
Medicare Part B (Physician Services)	\$283	20% of Medicare-approved amount
Medicaid (NJ FamilyCare)	\$0	Typically \$0 for eligible services
Medicare Advantage (Part C) Private Insurance	Varies by Plan	Varies by plan ; Prior authorization may be required

* **In-Network**: Healthcare facilities and providers (physicians, nurse practitioners, physical therapists, etc.) that have a contract with your insurance company.

* **Out-of-Network** providers do not have a contract with your insurer, which can result in significantly higher out-of-pocket costs.

Hospitals and Providers Search

- National Directory: [Medicare.gov/care-compare](https://www.medicare.gov/care-compare)
- New Jersey Directory: [Healthapps.nj.gov/facilities](https://healthapps.nj.gov/facilities)

Major Hospitals in Bergen County

Hospital Name	Address	Phone Number	Website
Bergen New Bridge Medical Center	230 East Ridgewood Avenue, Paramus, NJ 07652	201-967-4000	newbridgehealth.org
Englewood Health	350 Engle Street, Englewood, NJ 07631	201- 894-3000	englewoodhealth.org
Hackensack Meridian Health-Pascack Valley	250 Old Hook Road, Westwood, NJ 07675	201- 383-1035	pascackmedicalcenter.com
Hackensack University Medical Center	30 Prospect Avenue, Hackensack, NJ 07601	551-996-2000	hackensackmeridianhealth.org
Holy Name Medical Center	718 Teaneck Road, Teaneck, NJ 07666	201-833-3000	holyname.org
The Valley Hospital	4 Valley Health Plaza, Paramus, NJ 07652	201-447-8000	valleyhealth.com

Resources

- [Medicare.gov: Chronic Pain Management & Treatment Services](https://www.medicare.gov/chronic-pain-management)
- [Medicare.gov: Pain Management Coverage](https://www.medicare.gov/pain-management-coverage)
- [NJ FamilyCare.org](http://NJFamilyCare.org)

PART I • SECTION 9

Housing

주거·주택 (Housing)

Senior and affordable housing options: Senior Apartments (Section 202/HUD), Section 8 Housing Choice Vouchers, Affordable Housing (LIHTC), Retirement Communities, Continuing Care Retirement Communities (CCRC), Residential Health Care Facilities, Assisted Living, Reverse Mortgages, and NJ Property Tax Relief (Senior Freeze, ANCHOR, Stay NJ).

This section contains 12 comprehensive guides

Affordable Housing (LIHTC)

The Low-Income Housing Tax Credit (LIHTC) program is a program in which the federal government provides tax credits to private developers, who then sell these credits to investors to construct and operate affordable rental housing. Unlike general public housing, it operates under a fixed rent system tailored to the "local income level."

Eligibility

- **Income Limit:** Most LIHTC developments use a criteria of 50% or 60% of the AMI or less, though some developments may allow up to 80% by applying the Average Income Test (AIT).
- **Asset Screening:** Earnings from assets such as bank balances, stocks, and real estate (interest, dividends, etc.) are included in annual income and can affect the income eligibility review.
- **Direct Application:** Applications must be submitted individually through each apartment management office or the development's Administrative Agent, and checking the Waiting List is essential.

Rent Determination Principle

Rent for affordable housing is predetermined based on the Area Median Income (AMI) of the region, rather than individual income.

- **Fixed Amount:** Rent is set at a level that cannot exceed 30% of the income corresponding to that specific AMI percentage.
- **Minimum Income Requirement:** Since government subsidies are not paid monthly to the landlord, tenants must prove they have a minimum income sufficient to pay the fixed rent.
- **Rent Adjustment:** Rents may increase based on changes to HUD MTSP (Multifamily Tax Subsidy Projects) limits. However, this may vary depending on the policies of each individual housing development.

New Jersey (Region 1) Income Limits (2025)

Applicable Areas: Bergen, Hudson, Passaic, and Sussex Counties.

To qualify for affordable housing, the annual Gross Income based on household size must be below the following amounts:

Household Size	30% AMI (Very Low)	50% AMI (Low)	60% AMI (Typical LIHTC)	80% AMI (Moderate)
1 Person	\$26,730	\$44,550	\$53,460	\$71,280
2 Persons	\$30,540	\$50,900	\$61,080	\$81,440
3 Persons	\$34,350	\$57,250	\$68,700	\$91,600
4 Persons	\$38,160	\$63,600	\$76,320	\$101,760
5 Persons	\$41,220	\$68,700	\$82,440	\$109,920

Source: HUD FY2025 MTSP Income Limits (huduser.gov/portal/datasets/mtsp.html)

Minimum Income

In addition to the maximum limits set by the government (table above), leasing offices usually require a monthly income of approximately 2.5 times the rent to verify the ability to pay.

New Jersey (Region 1) Estimated Rent

Actual rent may vary depending on whether utilities are included, but the general guidelines are as follows:

- 1 Bedroom (at 50% AMI): Approx. \$1,100 – \$1,200
- 2 Bedroom (at 50% AMI): Approx. \$1,350 – \$1,500
- 3 Bedroom (at 50% AMI): Approx. \$1,550 – \$1,750

Recertification

Even after moving in, you must report changes in household income and composition to the management office annually.

- **140% Rule (Tenant Protection):** Even if your income has increased since the time of move-in, you can continue to reside in the unit as long as it does not exceed 140% of the currently applicable income limit (based on AMI) during recertification.
- **Exceeding the Limit:** Even if your income exceeds 140%, you will not be evicted immediately. However, if your income exceeds 140%, the next available unit in the building will be assigned to a qualifying low-income household, and the rent for your current unit will remain the same.

Household Composition Regulations

- **Adding Members:** The income of any new adult entering the household will be combined for a new eligibility screening. Living together without approval is a lease violation and grounds for eviction.

- **Reducing Members:** If family members move out and the number of occupants becomes too small for the unit size, you may be requested to transfer according to the building's policy (this varies by development).
- **Student Household Restriction:** Eligibility may be restricted if the entire household consists only of "full-time students." (Exceptions exist for single parents, welfare recipients, etc.).

Application and Search

- [NJHRC](#) (NJ Housing Resource Center)
- [Affordable Homes New Jersey \(CGP&H\)](#)
- [Piazza & Associates](#)
- AWCA provides counseling free of charge. (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov – New Jersey Department of Community Affairs \(NJDC\)](#)
- [NJ.gov – New Jersey Housing and Mortgage Finance Agency \(NJHMFA\)](#)
- [Fair Share Housing Center: Non-Profit Organizations for Tenant Rights Protection](#)

ANCHOR(Affordable New Jersey Communities for Homeowners and Renters)

ANCHOR is a property tax relief program implemented annually to alleviate the property tax burden on New Jersey residents. Benefits are determined based on residency status, income, and age for the corresponding year.

Eligibility

The ANCHOR program is available to those who owned or rented a home in New Jersey as of a specific date (usually October 1st) set by the State.

Homeowners

- **Qualification:** Residents who owned and occupied a home in New Jersey as of the base date and paid property taxes.
- **Benefit Amount:**
 - Annual income of \$150,000 or less: \$1,500 (Age 65+ receives \$1,750)
 - Annual income between \$150,001 and \$250,000: \$1,000 (Age 65+ receives \$1,250)

Renters

- **Qualification:** Residents who resided in and paid rent for a rental property in New Jersey (subject to local property taxes) as of the base date. However, tenants of government-owned, tax-exempt housing, or tax-exempt buildings owned by non-profit organizations are excluded.
- **Benefit:**
 - Annual income of \$150,000 or less: \$450 (Age 65+ receives \$700)
 - Renters with an income exceeding \$150,000 are generally excluded from benefits.

*Even if you do not have a Social Security Number (SSN), you may apply if you are a New Jersey resident and possess an Individual Taxpayer Identification Number (ITIN).

How to Apply

- **General Applicants (Under age 65)**

- For most eligible applicants (under age 65 and non-disabled), the state government will automatically process the application.
 - If there are no changes to the information on the confirmation letter, the application is filed automatically without any further action required. You should only apply manually, either online or by mail, if you need to correct information or if you did not receive a confirmation letter.
- **Seniors and Disability Benefit Recipients** (Individuals aged 65 or older, or recipients of Social Security Disability Insurance (SSDI) or Railroad Retirement Disability benefits)
 - The government does not automatically submit the application for you. You must apply manually each year, either online or by mail, using the [PAS-1 integrated application](#).
 - Through the integrated application, you can have your eligibility for ANCHOR, Senior Freeze (PTR), and Stay NJ benefits reviewed all at once, ensuring you receive all applicable benefits without omission.
- **Deadline:** Generally, the application closes between late October and early November each year.
- **Documents to Prepare**
 - Social Security Number (SSN) or ITIN.
 - New Jersey Gross Income information for the designated base year.
 - For homeowners: Block and Lot numbers from your property tax bill.

Key Considerations

- Social Security benefits are not included in New Jersey Gross Income; please ensure your income excluding Social Security is within the limits.
- If a property is jointly owned or if multiple people are registered as renters (e.g., spouses), the combined income must not exceed the limit.
- Choosing Direct Deposit when applying online allows you to receive your refund faster than a paper check.

Contact

- ANCHOR Dedicated Hotline: 1-888-238-1233/609-826-4282
- Text Telephone (for hearing impaired): 1-800-285-3036
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ Division of Taxation – ANCHOR Portal](#)
- [PAS-1 integrated application](#)

Assisted Living

Assisted Living is a form of housing for those who need a certain level of help with daily activities but generally require routine care rather than 24-hour intensive medical nursing. The core concept is to receive support for a safe and dignified life through 24-hour on-site staff while respecting individual independence to the fullest extent.

Services and Features

- **Personalized Care (ADLs):** Provides support for Activities of Daily Living, such as bathing, dressing, mobility, and personal hygiene, tailored to individual needs.
- **24-Hour Safety Management:** Every unit is equipped with an emergency call system, and professional staff are on-site 24/7 to respond immediately to emergencies.
- **Health and Medication Management:** A system is in place where nurses (RN/LPN) are reachable 24 hours a day, and depending on the facility, full-time nurses may be stationed on-site. They provide medication management and basic health monitoring.
- **Quality Dining:** Provides balanced meals designed by nutritionists in a dining hall, with customized menus available for those requiring special diets.
- **Enriching Social Programs:** Offers various programs such as fitness, arts, outdoor excursions, and educational lectures to promote cognitive function and emotional vitality.

Eligibility

- **Age:** Under NJ state statutes, there is no specific age requirement, and each facility sets its own admission criteria.
- **Health Status:**
 - Suitable for those who are ambulatory or can move with the use of assistive devices, though individual consultation with the facility is required depending on their condition.
 - It may not be appropriate for those requiring 24-hour intensive nursing supervision or those who are medically unstable; in such cases, a transition to a Long-term Care Facility may be necessary.
- **Cognitive State:** A level of cognitive health that does not pose a threat to others and allows for communal living is required. (For those with progressing dementia, admission to an on-site 'Memory Care' section is available.)
- **Financial Support:** In New Jersey, Long Term Care Medicaid (MLTSS) provides partial coverage for Assisted Living care costs for eligible low-income seniors. However, it is important to note that Room & Board costs are generally not included and must be paid by the resident.

*Eligibility requirements must be verified directly with the facility prior to moving in.

Considerations When Choosing

- **Care Levels (Tiered Costs):** It is important to verify how additional costs are applied based on the level of service required beyond the base fee.
- **Staff-to-Resident Ratio:** Check if the number of staff members relative to residents is sufficient to ensure meticulous care.
- **Continuum of Care:** It is recommended to choose a facility that allows for a transition to Skilled Nursing services within the same community if health conditions decline.

Assisted Living Facilities in New Jersey

Town	Community Name	Features	Contact
Englewood	The Bristol at Englewood	A luxury Assisted Living facility offering gourmet dining and a diverse range of activity programs. Famous for its sophisticated programs for active seniors and exceptional dining.	201-331-9204
Wyckoff	Courtyard at Longview (Christian Health	Operated by a non-profit foundation with a history of over 100 years. It focuses on the quality of care rather than profit, and offers excellent medical connectivity as specialized medical facilities and skilled nursing facilities are operated together within the complex.	201-528-1295
Tenaflly	Brightview Tenaflly	Provides access to the high-level community infrastructure of Tenaflly. Assisted Living with a rich variety of educational and arts programs.	201-228-3135
Old Tappan	Sunrise of Old Tappan	Prioritizes personalized care and emotional bonding over large-scale facility feel. Located in a quiet, prestigious residential area.	848-317-8670
Montville	The Chelsea at Montville	A facility consistently ranked at the top of New Jersey evaluations. Renowned for its modern safety systems and systematic medication management.	973-791-9649
West Caldwell	Crane's Mill	Beyond simple Assisted Living, it provides a CCRC (Continuing Care Retirement Community) model, offering everything from independent living to Skilled Nursing within a single campus. This ensures a stable transition as health conditions change.	973-276-6700

** Facility information and service details are subject to change; please verify directly with each facility prior to moving in.*

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov DOH – Residential Health Care Facility Search](#)

- [NJ MLTSS](#)
- [NJ.gov – Assisted Living Facility Search](#)
- [NJ 211.org](#)

Continuing Care Housing

Continuing Care Housing, commonly known as CCRC (Continuing Care Retirement Community) or Life Plan Community, is a comprehensive retirement complex designed to provide a continuous spectrum of housing and care services as a resident's health status changes over time.

Residents start with Independent Living and, as they require more assistance over time, are guaranteed continuous care by transitioning within the same community through the stages of Assisted Living and Skilled Nursing Facility.

Eligibility

Continuing Care Housing typically has more stringent entry requirements than standard senior housing.

- **Age:** The standard age is generally 55 years or older, though this may vary by community.
- **Health Status**
 - Residents must be healthy enough to sustain Independent Living at the time of entry.
 - An entrance physical exam and cognitive testing are required; if a resident already requires intensive care, admission to an independent living unit may be denied.
- **Finances**
 - Proof of assets and income is required to cover the high entrance fees and monthly service fees.
 - The community conducts a strict financial review to ensure the resident can afford the costs for the duration of their life.
- **Insurance:** Many communities require enrollment in Medicare Parts A & B as well as Medigap (supplemental insurance).

**Verify directly with each individual CCRC prior to moving in.*

Financial Structure

Continuing Care Housing involves significantly high initial costs.

- **Entrance Fee:** Varies from \$50,000 to over \$1,000,000, depending on the region and unit size. The total amount depends on whether the fee is Refundable or not.
- **Monthly Service Fee:** Includes meals, housekeeping, facility access, and security; typically costs several thousand dollars per month and varies significantly depending on the community and level of care (direct verification with the community is required).

Contract Types

Contracts are generally divided into three types based on cost and the scope of services provided.

Contract Type	Name	Features
Type A	Life Care	Most expensive, but monthly fees remain largely stable even if you move to a nursing facility. (Minimizes financial risk for medical care)
Type B	Modified	Lower entrance fee. When nursing services are needed, they are provided free or at a discount for a limited period; additional costs apply thereafter.
Type C	Fee-for-Service	Lowest initial cost. However, if care services are needed, the resident must pay the full Market Rate at that time.

Pros

- Spouses can remain close within the same community even if only one person requires medical care.
- Reduces the financial and physical burden of caregiving on children.
- Predictable future healthcare costs (specifically with a Type A contract).

Cons

- Very high initial entry costs.
- Risk that the entrance fee may not be easily refundable if the community's finances are unstable.
- Requires a quick decision while still healthy, as early entry is mandatory.

Continuing Care Retirement Communities in New Jersey

Town	Community Name	Key Features	Contact
Wyckoff	The Vista	Operated by Christian Health, which is based on Christian principles. It features the most modern facilities and customized wellness systems within Bergen County.	201-689-5513
Pompton Plains	Cedar Crest	A large-scale community situated on a 130-acre site. It features its own on-site medical center and a system of dedicated medical staff.	973-834-8513
Monroe	Monroe Village	History of winning Newsweek's America's Best CCRCs. It features robust infrastructure in the Monroe area and high levels of community satisfaction.	732-521-6400
Maplewood	Winchester Gardens	Awarded U.S. News "Best Facility" for 2026. It features historic architecture and an elegant environment.	973-762-5050
East Windsor	Meadow Lakes	2025 Newsweek award winner, this community is chosen by seniors who prefer a quiet and intellectual living environment.	609-448-4100
Basking Ridge	Fellowship Village	Operates wellness and health management programs. It combines a pleasant 72-acre environment with high-quality professional nursing services.	908-498-7302

Before deciding to move in, you must review the NJ DCA's CCRC Disclosure Statement and consult with financial and legal professionals.

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov – Department of Community Affairs \(DCA\) Housing Laws](#)
- [NJ DCA CCRC Consumer Guidebook\(PDF\)](#)
- [NJ 211.org](#)

New Jersey Property Tax Relief Programs

The State of New Jersey has integrated its property tax relief application processes to reduce the tax burden on homeowners and renters. In particular, the PAS-1 Integrated Application, introduced for seniors aged 65 or older and disability benefit recipients, is the most efficient way to be evaluated for three major benefits through a single application.

Integrated Application (PAS-1) Programs

Program Name	Primary Eligibility	Benefit
Senior Freeze	Individuals aged 65 or older, or recipients of Social Security Disability or Railroad Retirement Disability benefits.	Property Tax Increase Reimbursement (Reimbursement only for the increase compared to the base year)
ANCHOR	Homeowners and Renters	Based on 2025 Tax Year • Homeowners income ≤\$150K: \$1,500 (Age 65+: \$1,750) • Homeowners income \$150K–\$250K: \$1,000 (Age 65+: \$1,250) • Renters income ≤\$150K: \$450 (Age 65+: \$700)
Stay NJ	Homeowners aged 65 or older (Disability status alone does not qualify)	50% of property taxes — paid after subtracting ANCHOR and Senior Freeze benefits. Maximum net payment limit of \$6,500 for 2025.

Key Considerations

Please note the difference in application methods. Automatic filing depends on your specific situation.

- General Homeowners/Renters under age 65 (ANCHOR only):
 - For previous beneficiaries, the state government will process the application on your behalf and send a 'Confirmation Letter.' If there are any changes to your address, bank account, or information, you must apply directly.

- Seniors aged 65+ or disability benefit recipients (SSDI/SSI, etc.):
 - The application must be renewed annually
 - Caution: Even if you only qualify for the ANCHOR benefit, if you are a senior, you must directly submit the PAS-1 integrated application online or by mail.
- Payments may be delayed if income or homeownership information does not match your NJ-1040; please verify all data against your tax return before submission.

Eligibility

- **Age Criteria:** Must be at least 65 years old by December 31 of the tax year for which you are applying.
- **Disability Recipient Criteria:** Even if you are under the age of 65, if you are receiving Social Security Disability (SSDI) from the Social Security Administration (SSA) or Railroad Retirement Disability benefits, you are eligible to apply under the same criteria as seniors.
- **Residency Requirement:** You must own or rent your primary residence in New Jersey. (For Senior Freeze, you must have resided in NJ and owned the same home for at least 3 consecutive years, including the application tax year.)

How to Apply

- **Online Filing (Recommended):** Visit propertytaxreliefapp.nj.gov.
 - **ID.me Verification:** For security, identity verification via driver's license, state-issued ID, or passport is required.
 - ANCHOR application(under 65 years of age): [ANCHOR](#)
- **Documents to Prepare:** * Social Security Number (SSN) or ITIN.
 - **ANCHOR:** NJ-1040 Line 29 Gross Income / Senior Freeze · Stay NJ: Separate income calculation form (includes non-taxable income; refer to PAS-1 instructions)
 - **PAS-1 Applicants (Age 65+):** NJ-1040 for the past two years and property tax bills (County Code, Block/Lot/Qualifier, tax amount) are required.

Resources

- [NJ Division of Taxation](#)
- [Property Tax Relief Filing](#)
- [NJ Division of Taxation – FAQ](#)

Property Tax Reimbursement (Senior Freeze)

The New Jersey Property Tax Reimbursement (PTR) program, also known as Senior Freeze, is designed for eligible seniors and individuals with disabilities to reimburse them for any property tax increases. The core of this program is to “freeze” your property taxes at the amount paid during your first year of eligibility (Base Year Amount) by refunding any taxes paid above that level.

Eligibility

- **Age and Disability:**
 - Must be 65 or older as of December 31 of the tax year.
 - Must be receiving Social Security Disability benefit payments or Railroad Retirement Disability benefit payments from the Social Security Administration (SSA) on your own behalf; cases where you receive benefits on behalf of someone else are excluded.
- **Residency Requirement:** Must have owned and lived in the same home as your principal residence in New Jersey for at least 3 years, including the application year.
- **Property Tax Payment:** Property taxes for each year of application must be paid in full.
- Immigration status is not a requirement as long as you have an ITIN for tax purposes.

Income Limits

Filing Year	Source Income Year	Income Limit (Gross Income)
Filing in 2024	2023 Income	\$163,050 or less
Filing in 2025	2024 Income	\$168,268 or less
Filing in 2026	2025 Income	\$172,475 or less

***Definition of Income:** Based on Gross Income, which includes Social Security, pensions, IRA withdrawals, and all other sources of income.

How to Apply

Seniors can apply easily through the PAS-1 Integrated Application.

- **New Applicants (PTR-1):** First-time applicants must submit income and tax information for the past two years to establish their “Base Year.”

- **Previous Recipients (PTR-2):** The State will mail a pre-printed application containing your information.
- **Integrated Portal:** The government does not automatically submit your application. You must apply manually every year—either online or by mail—using the [PAS-1 integrated application form](#) to ensure you are reviewed for Senior Freeze as well as other senior benefits (Stay NJ, ANCHOR).
- **Deadline:** Generally, the application closes between late October and early November each year.
- **Payment Timeline:** Generally, payments are issued sequentially starting from July 15, depending on the timing of the application.

Key Considerations

- **Interaction with Stay NJ:** Stay NJ is applied after the Senior Freeze and ANCHOR benefits are first calculated. The Stay NJ amount is adjusted to ensure that the combined total of all three benefits does not exceed 50% of the property tax (up to a \$6,500 limit).
- **Moving:** If you move within New Jersey, you must meet the 3-year ownership and residency requirements at your new home before you can apply again. A new “Base Year” will be established.
- **Multi-Unit Properties:** Your property must not exceed 4 units, and you must occupy one of the units yourself. Properties with more than one commercial unit are excluded.

Contact

- **NJ Division of Taxation Official Hotline:** 1-800-882-6597
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ Division of Taxation -Senior Freeze\(PTR\)](#)
- [Online Integrated Application for PTR](#)

Residential Health Care Facilities (RHCF)

Residential Health Care Facilities (RHCF) are residential settings designed for individuals who can maintain a certain level of independence but require some medical supervision and support with daily living due to chronic illness or aging. It is commonly understood as an intermediate stage between living at home and Assisted Living, and its purpose is to provide residents with a safe living environment along with basic health care services.

Services and Support

RHCF provides supervised health care services that go beyond simple housing, including the following:

- **24-Hour Supervision:** Professional staff are on-site to manage and respond to emergencies.
- **Healthcare Services:** Basic medical monitoring, such as blood pressure checks and Medication Management, is provided.
- **Daily Living Assistance:** Includes meal services, laundry, housekeeping, and personal hygiene support.
- **Social Activities:** Various recreational and social programs are operated for the emotional well-being of residents.
- **Dietary Management:** Provides a balanced diet considering nutrition (some facilities operate under the guidance of a nutritionist).

Eligibility

- **Age:** Mainly for seniors aged 60 and older
- **Health Status:**
 - **Medical Necessity:** The individual must be in a condition where hospital-level care is not required, but living alone at home poses safety concerns or requires regular help, such as medication management.
 - **Mobility:** Most RHCFs prefer individuals who can move to some extent independently or with the help of assistive devices (canes, walkers).
 - **Mental Health:** Must have a cognitive state capable of communal living without posing a risk to others. (In cases of severe dementia or serious mental illness, specialized Memory Care or mental health facilities may be more appropriate.)
- **Financial Proof:** Evaluation of the economic ability to pay monthly rent and service fees. Generally, these facilities are not covered by Medicaid. Approximately 40% of NJ RHCF residents utilize SSI, 50%

pay out-of-pocket, and the remainder use Veterans Benefits or other sources. Please verify SSI eligibility in advance.

Residential Healthcare Facilities in New Jersey

Town	Community Name	Features	Contact
Allendale	Allendale Senior Living	Located in Bergen County, it provides basic care services.	201-825-0660
Montclair	Horizon Manor South	Provides basic care (RHCF) and Memory Care, focusing on housing, meals, and basic health monitoring. It is suitable for those who prefer a small, family-like community atmosphere.	973-746-9047
Bloomfield	Job Haines Home	A non-profit facility with over 120 years of history.	973-743-0792

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ.gov DOH Search for Residential Health Care Facilities](#)
- [NJ.gov – Residential Health Care Facility Search](#)
- [NJ 211.org](#)

Adult Retirement Communities

Adult Retirement Communities, commonly referred to as “Active Adult Communities” or “55+ Communities,” are managed residential complexes designed for seniors capable of healthy and independent living. Unlike Assisted Living or nursing homes, these communities do not provide medical services or assistance with daily living activities. Instead, they focus on fostering an environment where residents can lead socially vibrant lives alongside peers in a similar life stage. The primary appeal of these communities lies in a quiet living environment secured by age restrictions, resort-style amenities, and the elimination of burdensome home maintenance tasks such as landscaping and snow removal. These communities vary widely in form, ranging from large-scale planned cities with thousands of households to small-scale gated communities.

Eligibility

Age

- Under the federal law, at least 80% of the occupied units must be occupied by at least one person who is 55 years of age or older.
- Communities for those 62 years of age or older are subject to separate HOPA(Housing for Older Persons Act) exemption provisions, in which case 100% of the residents must be 62 or older without exception, rather than the 80% rule).
- Co-residents must generally be at least 18 years of age, though this may vary depending on the specific regulations of each complex.

Restrictions on Minor Residency

- Most 55+ communities do not allow the permanent residency of minor children. However, as this may vary depending on the specific community bylaws, it must be verified prior to moving in.
- The permitted length of stay for visitors varies from 2 weeks up to 90 days per year according to the community’s covenants, conditions, and restrictions (CC&Rs), and must be verified prior to moving in.

Independent Living Ability

- These communities are intended for independent residents capable of managing their own daily routines. Medical services or caregiving are not provided.

Financial and Identification Verification

- Along with documents to prove age (such as a passport or ID), a screening process is required: income and credit checks for rentals, and financial capability assessments for ownership. Specific criteria vary by community.

Pet Policies

- Even if permitted, there are restrictions on weight and the number of pets, so verification by the community is required.

Housing Options and Ownership Structures

Adult Retirement Communities offer various housing styles to suit residents' budgets and preferences. Single-family detached homes are popular for those seeking privacy, while townhouses, villas, and condominiums are preferred by those looking for easier-to-manage homes. In urban areas, they may operate as luxury high-rise apartments. Ownership structures also vary as follows:

- **Fee Simple:** The resident directly owns both the home and the land; commonly found in traditional suburban developments.
- **Condominium/Co-op:** A form where you either hold direct ownership of the unit or own corporate shares that grant the right to reside in a specific unit. In addition, there are shared common areas, and the HOA manages the exterior and landscaping.
- **Lease/Rental:** There is a growing trend toward premium rental models that offer luxury amenities at a fixed monthly rent.

New Jersey Adult Retirement Communities (55+)

Town	Community Name	Housing Type	Features	Contact
Florham Park	Del Webb at Florham Park	Single-family homes + Townhouses (attached) + Condominiums (Total of 3 housing types)	Representative 55+ brands. These feature indoor and outdoor swimming pools, smart home systems, and an on-site dedicated lifestyle director.	973-969-3495
Wayne	Four Seasons at Wayne	1–2 story condominiums (elevator accessible)	A 55+ gated community consisting of 230 units constructed	503-746-4122
Wanaque	Del Webb's Wanaque Reserve	Mid-rise Condo	A large-scale luxury 55+ community	1-800-870-0010
Livingston	The Pointe at Livingston	Condo (Gated)	Located in Essex County. Offers a quiet, formal atmosphere with highly active community engagement among residents.	973-810-8208
Livingston	The Enclave at Livingston	Rental apartments (gated)	Provides a private and quiet living environment for seniors who wish to enjoy Livingston's excellent local infrastructure.	973-422-9300
Tinton Falls	Enclave at Tinton Falls	Single-family Home	A 55+ detached housing community where the association manages all maintenance, including landscaping and snow removal.	732-800-2005

***Items to Check During Visit:** Be sure to verify if the Homeowners Association (HOA) has healthy Reserve Funds, if the current resident age group matches your preferences, and if there are high-quality general hospitals nearby.

**Community policies (age requirements, HOA regulations, etc.) are subject to change, so you must verify them directly with each community's management office prior to moving in.*

Contact

- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ 211.org](http://NJ211.org)
- [NJ Housing Resource Center \(NJHRC\)](#)

Reverse Mortgage

Reverse Mortgage is a loan designed for seniors aged 62 and older, allowing them to receive cash by using their home equity as collateral while continuing to live in their home for life. The most representative product is the HECM (Home Equity Conversion Mortgage), which is insured by the Federal Housing Administration (FHA).

Eligibility

- **Age:** 62 or older / Spouses under the age of 62 can be registered as a Non-Borrowing Spouse (NBS). If HUD eligibility requirements are met, the spouse's right to reside in the home can be protected even after the borrower's death. (Subject to separate verification of eligibility requirements).
- **Residency:** The property must be your Principal Residence, and you must live there for more than 6 months out of the year.
- **Home Equity:** You must own the home outright or have a very low existing mortgage balance that can be fully paid off with the Reverse Mortgage proceeds.
- **Financial Assessment:** Prior to loan origination, the borrower's credit history, income and cash flow, and ability to pay property taxes and insurance are comprehensively evaluated. Additionally, the loan may be restricted if there is any delinquent federal debt.

Loan Limits (2026)

- **Maximum Claim Amount:** \$1,249,125 is the maximum home value limit recognized by the FHA for loan calculation purposes; the actual amount received is determined based on factors such as age, interest rates, and home value.
- **Payment Options:** You can choose from a lump sum, monthly annuity-style payments, or a Line of Credit. You can also combine multiple payment methods.

Mandatory Procedures

For Reverse Mortgage, counseling with a Department of Housing and Urban Development (HUD) approved counselor is a legal requirement before applying.

- **Counseling Content:** Includes a review of loan costs, tax implications, inheritance issues, and alternatives (such as downsizing).
- **Waiting Period:** Under federal HECM regulations, a loan application can only be submitted at least 3 business days after the completion of the counseling session.

- **Validity Period:** The HECM counseling certificate is valid for 180 calendar days from the date of issuance.

Key Considerations

Reverse Mortgage is not free money. Rather, it is a loan that must be repaid eventually.

- **Repayment Timing:** The full loan amount must be repaid when the last borrower passes away, sells the home, or vacates the residence for more than 12 consecutive months (e.g., admission to a nursing home).
- **Default Risk:** Failure to pay property taxes, maintain insurance, or neglect home maintenance can lead to foreclosure and eviction.
- **Non-Recourse Protection:** Even if the loan balance exceeds the home's value, the borrower or heirs are not responsible for paying the excess amount.
 - *For example, if the loan balance is \$500,000 but the home value has dropped to \$400,000 at the time of the borrower's death, the heirs do not need to pay the \$100,000 deficit.*

Options for Heirs

Within 30 days of the borrower's date of death, the servicer must be notified of the intent regarding the home. However, it is crucial that the heirs inform the servicer of the death as soon as possible.

- **Option 1:** Sell the home and keep the remaining equity
 - If the home value exceeds the loan balance: The remaining equity (the difference) belongs to the heirs.
 - If the loan balance exceeds the home value: In cases where the debt is greater than the home's worth, the home can be sold for at least 95% of its current appraised value. The remaining balance is then covered by FHA insurance, ensuring that the heirs are not held personally liable for the deficiency.
- **Option 2:** Pay off the loan and keep the home
 - If the heirs wish to keep the home, the lender (not HUD) may approve a 90-day extension upon submission of documentation, and a total processing time of approximately 6 months may be granted.
 - To keep the home, you must pay the lesser of the full loan balance or 95% of the home's appraised value.
- **Option 3:** Surrender the deed to the lender (Deed in Lieu of Foreclosure)

- If the debt is higher than the home's value or if the selling process is too burdensome, heirs can voluntarily transfer the deed to the lender.

Contact

- HUD (U.S. Dept. of Housing and Urban Development): 1-800-569-4287
- Bergen County Reverse Mortgage Housing Counseling: 201-336-7431
- NJ Housing and Mortgage Finance Agency (NJHMFA): 1-800-654-6873 -Housing financial program management for seniors
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [HUD.gov – Find a HUD-Approved Housing Counseling Agency](#)
- [HUD.gov – Official Reverse Mortgage Regulations and Loan Limits](#)
- [CFPB.gov – Consumer Protection Guide and Risk Factors](#)

Section 8

Section 8 is a program where the U.S. Department of Housing and Urban Development (HUD) subsidizes a portion of rent so that low-income households can enjoy a safe and pleasant living environment in the private market. Its official name is the Housing Choice Voucher (HCV) program.

The most significant feature of Section 8 is that the amount paid by the tenant is linked to their income.

- Tenants generally pay 30% of their adjusted monthly income, and if the gross rent exceeds the payment standard at initial occupancy, they may pay up to a maximum of 40%.
- Public Housing Authority pays the landlord directly in the form of Housing Assistance Payment.

Two Types of Section 8

Category	Housing Choice Voucher (Tenant-Based)	Project-Based Voucher (Project-Based)
Target	Voucher is issued to the tenant (individual)	Subsidy is assigned to a specific apartment unit
Mobility	The voucher stays with the tenant when moving	When moving, the voucher remains with the specific unit. However, for PBV (Project-Based Vouchers) only, after residing there for 12 months, you may apply for a tenant-based voucher to relocate.
Housing Choice	Any private housing that accepts the voucher	Must move into the designated apartment to receive benefits
Remarks	Commonly known as the "Section 8 Voucher"	Specific units designated by the PHA, such as senior apartments, housing for individuals with disabilities, and redeveloped housing.

Eligibility

- **Income Standards**
 - Annual Gross Income of the household must be 50% or less of the Area Median Income (AMI).
 - 75% of vouchers are allocated to 'Extremely Low-Income' households, whose income is at or below 30% of the Area Median Income.

- **Household Composition:** Seniors aged 62 or older, individuals with disabilities, households with children, and veterans may be considered for priority.
- **Status:** U.S. citizens or those with eligible immigration status (Permanent Residents, etc.).
- **Exclusions:** Eligibility may be restricted for individuals with records of drug-related crimes or sex offenses. Detailed criteria vary by PHA. Additionally, eligibility may be restricted if there is a record of prior eviction from public housing.
- **Asset Limit:** Eligibility may be restricted if Net Family Assets exceed \$105,574 (2026) or if you own habitable real property. (Essential assets such as vehicles and medical equipment may be excluded.)
- **Income Deductions:** Income deduction standards (such as medical expenses) applied to senior and disabled households are being adjusted according to HOTMA (Housing Opportunity Through Modernization Act).

**Policies may vary by PHA; please check with each individual agency.*

Application and Leasing Process

1. **Application:** Submit an application to the [Public Housing Authority \(PHA\)](#) of your desired area.
 - At [HABCNJ](#), you can check and apply for all housing program waiting lists operated by the HABC (Section 8 vouchers, public housing, senior apartments, etc.) in one place.
 - However, the opening period for the Section 8 (HCV) voucher waiting list is irregular and may remain closed for several years. You must check the official HABC website frequently.
2. **Waitlist Registration:** Since demand is high, waitlists are often closed; you must register immediately when a list opens. (Seniors/Disabled individuals may receive priority).
3. **Eligibility Interview:** When your turn comes, the PHA will contact you. At this stage, you will submit supporting documents for income, assets, and family composition and undergo an interview.
4. **Voucher Issuance:** Once approved, the voucher is issued. You must find housing within a minimum of 60 days to a maximum of 120 days after the voucher is issued. (Extensions may be granted at the PHA's discretion.)
5. **Housing Inspection:** Once you find a home, you must submit a Request for Tenancy Approval (RFTA) together with the landlord, and the unit must pass a housing safety standards inspection (HQS or NSPIRE — standards may vary by PHA) conducted by the Public Housing Authority.
6. **Contract Execution:** A Housing Assistance Payments (HAP) contract is signed between the PHA and the landlord, and a lease agreement is signed between the tenant and the landlord simultaneously.

Contact

- [HUD.gov PHA Contact Information](#)

- AWCA provides counseling free of charge. (201-862-1665 / info@awcanj.org)

Resources

- [HUD.gov – Housing Choice Vouchers Fact Sheet](#)
- [NJ.gov – New Jersey Housing Authority Contact List](#)
- Bergen County Section 8 Official Links
 - Official Website: habcnj.org
 - Application Portal (HCV Applicant Portal): habcnj.org/apply_for_housing
 - Waitlist Status Check: waitlistcheck.com

Senior Apartments

Senior apartments are housing facilities designed for independent seniors aged 62 and older. They are independent rental apartments that feature senior-friendly designs and social services without providing medical care or nursing assistance.

Income-Linked Type Housing

HUD (U.S. Department of Housing and Urban Development) Section 202 (non-profit operated, HUD subsidized) and Public Housing (PHA operated) are the methods adopted by most low-income senior apartments supported by the federal government.

- **Rent:** Residents pay approximately **30% of their Adjusted Monthly Income (AMI)**.
- **Utilities:** Public utilities such as electricity and gas are often included. If not included, a certain amount (Utility Allowance) may be deducted from the rent.

Eligibility

- At least one household member must be a U.S. citizen or an eligible non-citizen (including Lawful Permanent Residents) aged 62 or older.
- Eligibility may vary by property; generally, a history of HUD housing evictions and specific criminal records are subject to screening.

Income Limits

Household Size	30% AMI (Extremely Low)	50% AMI (Very Low)
1 Person	Approx. \$28,850 or less	Approx. \$48,000 or less
2 Persons	Approx. \$32,950 or less	Approx. \$54,850 or less

These figures are the income standards commonly used by HUD for all federal housing assistance programs. The eligibility ceiling for Senior Apartments is 50% AMI (Very Low Income), while 30% AMI is the threshold used for granting admission priority.

Asset Limits

- **Asset Limit:** Asset limit (**\$105,574 as of 2026**) may apply. Since implementation may vary by building, you must verify with the property manager.

- **Home Ownership:** Eligibility may be restricted if you own habitable real property, although exceptions related to disabilities and other specific circumstances may be recognized.

Fixed Rent Type (LIHTC / Affordable Housing)

The LIHTC (Low-Income Housing Tax Credit) is a program that encourages private developers to provide affordable rental housing through federal tax incentives.

- Instead of paying 30% of your income, you pay a predetermined fixed amount based on the Area Median Income (AMI).
- While much cheaper than market prices, you must pay the designated minimum rent even if your income is very low. (Usually set at 50%–60% of the AMI).
- Rent may be adjusted according to changes in the regional AMI announced annually by HUD.

How to Apply

Information Gathering

In New Jersey, you must apply online or visit the specific apartment in person. Since waitlist opening periods vary, you should inquire by phone in advance. Check the following sites frequently:

- **NJHRC (New Jersey Housing Resource Center):** njhrc.gov – New Jersey's official housing search site.
- **Affordable Homes New Jersey:** affordablehomesnewjersey.com – Operated by a NJ DCA-approved private administrative agent (CGP&H) (Applications for some low-income housing waitlists are available).
- **Local Housing Authority:** Visit the website of the Housing Authority in your county to check for "Senior Housing" announcements. HUD.gov [PHA Contact Information](#)

Waitlist Registration

Wait times can be long in most popular areas. You may apply to multiple senior apartment locations.

Contact

- HUD.gov [PHA Contact Information](#)
- AWCA provides counseling free of charge. (201-862-1665 / info@awcanj.org)

Resources

- NJ.gov – [Housing Resource Center \(NJHRC\)](#)
- HACBNJ.org – [Bergen County Housing Authority](#)

- [Income Limit](#)
- [HUD Income Limits by Region\(PDF\)](#)

Stay NJ Program

Stay NJ is a groundbreaking property tax relief program introduced to help New Jersey seniors remain in their cherished homes despite high tax burdens. Stay NJ operates as a separate benefit program alongside ANCHOR and Senior Freeze, and it is designed so that the combined benefits of all three programs reach 50% of property taxes (up to a maximum of \$6,500).

Benefit Calculation

- A benefit equivalent to 50% of the property taxes paid will be provided.
- Based on a maximum annual property tax of \$13,000, the refund is capped at \$6,500 (even if the property tax exceeds \$13,000, the maximum limit remains \$6,500).
- **Calculation Method:** Stay NJ benefits are not issued in isolation. Instead, the total amounts from ANCHOR and Senior Freeze are calculated first, and Stay NJ covers the remaining balance to reach the 50% threshold.

**Example: Property tax \$10,000 → Target benefit \$5,000. If you receive \$1,750 from ANCHOR and \$500 from Senior Freeze (totaling \$2,250), Stay NJ will provide an additional \$2,750.*

Eligibility

- **Age:** Individuals who are 65 years of age or older as of the application year (Social Security Disability Insurance (SSDI) recipients are not eligible for Stay NJ. Eligibility for Senior Freeze and ANCHOR may be reviewed separately).
- **Income:** Annual Gross Income of \$500,000 or less.
- **Homeownership:**
 - You must have resided in your own home in New Jersey for the entire period from January 1 to December 31 of the applicable tax year. However, if you moved within New Jersey during that same year, you are required to complete a separate Schedule.
 - Mobile home owners are not eligible for Stay NJ. However, Senior Freeze includes mobile homes, and ANCHOR treats mobile home owners as “tenants,” allowing for a separate application.
- **Immigration Status:** If you have an Individual Taxpayer Identification Number (ITIN) for tax filing, **your immigration status does not matter.**

How to Apply

- **First-Time Applicants:** Submit information regarding your income and property taxes for the current and previous years through the [PAS-1 integrated application](#). (Income data from two years is used for the Senior Freeze eligibility review).
- **Returning Applicants:** You must apply for Stay NJ manually each year through the [PAS-1 integrated application](#). There is no automatic application or automatic mailing of benefits. You must file online or by mail every year to have your eligibility reviewed for Stay NJ along with other senior benefits (ANCHOR and Senior Freeze).

Disbursement Methods

- Stay NJ benefits are distributed in equal quarterly installments (February, May, August, and November).
- Online applicants may choose Direct Deposit for certain benefits (primarily ANCHOR). Those who submit paper applications will receive all benefits via check.
- Quarterly schedules are announced annually based on the State budget and the time of application.

Contact

- Stay NJ & Integrated Application Hotline: 1-888-238-1233
- NJ Division of Taxation Main Line: 1-609-292-6400
- AWCA provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NJ Division of Taxation – Stay NJ Portal](#)
- [PAS-1 integrated application](#)

PART I • SECTION 10

In-Home Care

재택 케어 (In-Home Care)

Services supporting aging in place at home: Personal Preference Program (PPP), Jersey Assistance for Community Caregiving (JACC), Respite Care for family caregivers, Home Health Care, Meals on Wheels, and Congregate Meal sites.

This section contains 6 comprehensive guides

Congregate Meals

Congregate Meals is a nutritious meal service provided in group settings such as senior centers, churches, schools, and community centers. This program is designed to provide healthy food to the elderly while promoting social interaction among seniors to alleviate feelings of isolation. Beyond simply providing a meal, it plays a vital community role as a place where seniors can communicate with neighbors and gain emotional support.

Key Features

Nutritious Diet: Provide balanced nutritional meals that meet the Dietary Guidelines for Americans (DGA) and Dietary Reference Intakes (DRI) in accordance with Older Americans Act (OAA) standards.

- **Social Engagement:** Reduces feelings of alienation in old age and improves overall well-being through interaction with others.
- **High Accessibility:** Various locations, including senior centers, religious facilities, community centers, and select partner facilities.
- **Additional Services:** Provides beneficial programs alongside meals, such as wellness checks, chronic disease management education, and fall prevention training.

Eligibility

- Seniors aged 60 or older, or the spouse of a beneficiary (regardless of age).
- Individuals with disabilities who reside with a beneficiary aged 60 or older (regardless of age).
- Program volunteers who provide services during meal hours

**Specific eligibility requirements may vary by local program, so please contact your local ADRC for inquiries.*

Cost

Meals are provided free of charge. To support the program, a voluntary donation of \$1.25 per meal is encouraged; services are provided regardless of whether a donation is made. All donation records are kept confidential.

Application and Contact

- **Advance Reservation Required:** As the number of meals per location is limited, reservations must typically be made 24 hours in advance.
- Contact your local Aging and Disability Resource Connection (ADRC).
 - **Bergen County Division of Senior Services**
 - Address: One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - Phone: 201-336-7400
 - Email: seniors@bergencountynj.gov
 - **NJ ADRC Toll-Free Hotline:** 1-877-222-3737
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [ACL.gov: Nutrition Services Official Guide](#)
- [Bergen County Nutrition and Wellness Services Information](#)
- [Bergen County Senior Lunch Brochure \(English\) \(PDF\)](#)

Home Health Care

Home Health Care is a program that dispatches professional caregivers to residents with mobility issues, allowing them to live safely and independently in their own homes instead of a nursing home.

Features

A Certified Home Health Aide (CHHA) from an accredited home care agency will visit your home. The agency is regulated by the New Jersey Department of Health, and the individual caregiver (CHHA) is managed by the New Jersey Board of Nursing.

- **Personal Hygiene Support:** Assistance with bathing, hair washing, dressing, oral care, and toileting.
- **Mobility Assistance:** Help with transferring from bed to wheelchair and walking indoors or outdoors.
- **Nutritional Management:** Meal preparation, feeding assistance, and special diet management.
- **Household Support:** Light cleaning of the patient's living space, laundry, grocery shopping, and medication reminders (Note: Caregivers cannot directly administer medication).
- **Professionalism:** All services are performed according to a "Plan of Care" established under the supervision of a Registered Nurse (RN).

Eligibility (2026)

To receive home health care services, both financial and medical requirements must be met.

Financial Requirements

- Home health care services require eligibility for Long-Term Care Medicaid (MLTSS), rather than just simple general Medicaid (NJ FamilyCare). Even if you already possess general Medicaid, additional enrollment is required through an MLTSS clinical assessment.
- **Income Limit:** Monthly income of \$2,982 or less.
 - If income exceeds this limit, eligibility can be secured through a "Qualified Income Trust (QIT)."
- **Asset Limit:** Total assets of \$2,000 or less.
 - One primary residence and one automobile are excluded from the asset calculation.

Medical Requirements

- **Nursing Facility Level of Care:** Must receive a clinical determination that independent living is impossible and that assistance from another person is required for at least three Activities of Daily Living (ADLs).

- **Doctor's Prescription:** A physician must issue a medical opinion stating that "home health care services are necessary."

Application Process

1. **Consultation and Pre-screening:** Contact your local county Aging and Disability Resource Connection (ADRC) for an initial telephone screening.
2. **Financial Eligibility Review:** Financial qualification for Medicaid is conducted through the County Board of Social Services.
3. **Clinical Assessment:** A nurse from a Managed Care Organization (MCO) visits the home to assess the patient's condition (ADLs) and determine the number of hours available per week.
4. **Agency Assignment:** Select a home care agency to provide services for the approved hours.
Commencement of Service: A caregiver is dispatched to begin care according to the fixed schedule.

**The processing time for home health care services varies by case. If services are needed urgently, please contact the ADRC or your 담당 CSSA (Care Management agency) regarding Fast-Track processing.*

Contact

- Long-Term Care Medicaid (MLTSS):
Contact your local Aging and Disability Resource Connection (ADRC) and the Board of Social Services.
 - **Bergen County Division of Senior Services**
 - Address: One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - Phone: 201-336-7400
 - Email: seniors@bergencountynj.gov
 - **Bergen County Board of Social Services**
 - Address: 218 Route 17 North, Rochelle Park, NJ 07662
 - Phone: 201-368-4200
 - **NJ ADRC Statewide Toll-Free Number:** 1-877-222-3737
- **AWCA** can provide counseling free of charge (201-862-1665 / info@awcanj.org)
- **AWCA Homecare Agency**
 - **Korean:** 201-862-9111
 - **Chinese:** 201-862-0006

Resources

[NJ Division of Disability Services \(DDS\) PCA Guide](#)

[NJ FamilyCare \(Application\)](#)

[Long-Term Care Medicaid \(MLTSS\) Detailed Guide](#)

[cahcosa.org: Home Care Agency Search](#)

[NJ.gov: Official New Jersey License Verification](#)

[NJ.gov: Certified Home Health Aide \(CHHA\) Overview](#)

Jersey Assistance for Community Caregiving (JACC)

The Jersey Assistance for Community Caregiving (JACC) is a program designed to prevent or delay nursing home placement for New Jersey seniors who require physical assistance, supporting them in maintaining an independent lifestyle at home. This program includes a variety of customized services to help seniors live safely and independently in a familiar environment.

Benefits

- **Care Management:** A dedicated care manager develops a personalized care plan for each individual.
Personal Care Assistance /
- **Homecare Services:** Assistance with Activities of Daily Living (ADLs), such as bathing and eating
- **Respite Care:** Provides temporary relief and substitute care for family caregivers.
- **Chore Services:** Assistance with daily household tasks such as cleaning, laundry, and errands.
- **Home Modifications:** Residential modifications to improve safety and accessibility (e.g., wheelchair ramp installation, doorway widening, bathtub replacement).
- **Specialized Medical Equipment and Supplies:** Provides essential equipment and items necessary for home care.
- **Personal Emergency Response System (PERS):** Installation of a system that allows for 24-hour emergency assistance requests.
- **Home-Delivered Meals:** Support for nutritious meal delivery services for seniors.
Adult Day Care: Access to daytime programs providing social interaction and medical support.
- **Transportation Assistance:** Provision of transportation services needed for medical appointments and other essential visits.
- New Jersey pays the service provider on behalf of each participant up to a maximum of \$1,090 per month (care management costs excluded).

Eligibility

- **Age/Status:** New Jersey residents aged 60 or older who are U.S. citizens or have qualified alien status (including permanent residents; however, eligibility may vary depending on the type of visa).
- **Living Environment:** Must reside at home (excludes nursing homes or other facilities).
- **Health Status:** Individuals who are clinically assessed as requiring assistance with at least three Activities of Daily Living (ADLs), such as bathing, toileting, dressing, and mobility.
Must not be eligible for Medicaid or Managed Long Term Services and Supports (MLTSS).

- **Assets:** Liquid assets must be \$40,000 or less for an individual, or \$60,000 or less for a couple.
- **Income(2026):** Countable income must be at or below 365% of the Federal Poverty Level (\$4,854/month for a single, \$6,582/month for a couple).
- **No Duplicate Benefits:** If you are already receiving benefits from any of the following—the Alzheimer’s Adult Day Services Program (AADSP), the Statewide Respite Care Program (SRCP), or the Personal Assistance Service Program (PASP)—you must either terminate your current program before applying for JACC or continue with your existing program instead of JACC.
- If eligibility requirements are met, a self-directed care option is provided, allowing you to hire family members or friends as your direct care providers.

Copay

JACC is not a free program. Copays are determined on a sliding scale based on the income of the participant (and their spouse).

Application and Contact

- Contact your local Aging and Disability Resource Connection (ADRC)
 - **Bergen County Division of Senior Services**
 - **Address:** One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - **Phone:** 201-336-7400
 - **Email:** seniors@bergencountynj.gov
 - **NJ ADRC Toll-Free Hotline:** 1-877-222-3737
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

[NJ.gov](https://www.nj.gov) – Official New Jersey JACC Program

Meals on Wheels

Meals on Wheels helps seniors live independently, improves their health, and promotes social connection by delivering meals directly to their homes.

Beyond just providing food, it is a welfare service that delivers the warmth of community care, supporting many seniors so they can stay healthy in their own homes for as long as possible instead of moving into a nursing home.

Key Features

- **Home-Delivered Meals:** Meals are delivered during lunchtime from Monday to Friday (5 days a week). Each meal consists of a nutritionist-planned, balanced diet, including a main dish, vegetables, bread, fruit, milk, and dessert.
- **Social Interaction:** Volunteers provide companionship and check on the recipient's well-being while delivering meals.
- **Safety and Wellbeing Support:** Helps prevent malnutrition among seniors, maintains independence, and ensures they can live safely.
- **Community-Based Operation:** Operated through the participation of various local non-profit organizations and volunteers.
- **Cost:** The service is free, but a voluntary donation of \$1.25 per meal is encouraged. Services are provided regardless of whether a donation is made.
- After registering for the service, a representative will visit your home every six months to conduct an assessment for continued eligibility.

Eligibility

- Seniors aged 60 or older, or their spouses and caregivers.
- Individuals with disabilities (ages 18–59): Those living alone who have difficulty preparing meals due to physical or mental disabilities (permanent or temporary). Eligibility can be determined through a doctor's note (Medical Necessity) or other documentation proving disability status
- Homebound status (unable to leave home without assistance), inability to purchase groceries or prepare nutritious meals independently, having no family or friends to provide help, and not receiving home care services for more than 4 hours per day (Eligibility is granted only to those receiving 4 hours or less).
- Non-recipients of Long-Term Care Medicaid (MLTSS).

**Specific eligibility requirements may vary by region and operating agency.*

Meals on Wheels Services in Northern New Jersey

Agency	Phone	Hours	Service Area
Bergen County Division of Senior Services	201-336-6000	Mon ~ Fri 8am ~ 4pm	33 towns including Bergenfield, Cliffside Park, E. Rutherford, Edgewater, Englewood, Englewood Cliffs, Fort Lee, Leonia, Palisades Park, Ridgefield, etc. *Check link for full list
Meals on Wheels North Jersey	201-358-0050	24/7 Inquiry available	32 towns including Alpine, Closter, Cresskill, Demarest, Dumont, etc. *Check link for full list
Community Meals, Inc.	201-447-8295	Mon ~ Fri 10am ~ 2pm	Allendale, Glen Rock, Ho-Ho-Kus, Midland Park, Ridgewood, Waldwick, and parts of Saddle River
Essex County Office of Senior Services	973-395-8375	Mon ~ Fri 10am ~ 2pm	All of Essex County
Jersey City Meals On Wheels	201-547-5750	Mon ~ Fri 8am ~ 4pm	All of Jersey City
Middlesex County Nutrition Services	732-745-3295	Mon ~ Fri 8:30am ~ 2:30pm	All of Middlesex County
Passaic County Nutrition Services	973-569-4099	Mon ~ Fri 8:30am ~ 4:30pm	All of Passaic County
Rockland County Meals on Wheels	845-624-6325	Mon ~ Fri 9am ~ 5pm	All of Rockland County
KCS NJ Home-Delivered Meals Program *Korean Meals Provided	201-212-1054	Mon, Wed, Fri	Fort Lee, Leonia, Palisades Park. [Check link for details]

Application and Contact

- [Meals on Wheels Application](#)
- Contact your local Aging and Disability Resource Connection (ADRC).
 - **Meals on Wheels Hotline:** 201-336-7420 (월-금 8:00 AM – 4:00 PM)
 - **Bergen County Division of Senior Services**
 - Address: One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - Phone: 201-336-7400
 - Email: seniors@bergencountynj.gov
 - **NJ ADRC Toll-Free Hotline:** 1-877-222-3737
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [Meals on Wheels America: Find a Local Program](#)
- [Bergencountynj.gov – Bergen County Meals on Wheels Overview](#)
- [Bergencountynj.gov – Meals on Wheels Online Application Form](#)

Personal Preference Program (PPP)

The Personal Preference Program (PPP) is a New Jersey Medicaid (NJ FamilyCare) program that allows qualified individuals to directly manage their own personal care services instead of using a traditional home health care agency. This program grants patients greater control, flexibility, and choice over their own care.

Key Features

- **Direct Hiring:** You can directly hire a spouse, family member, relative, friend, or neighbor who is at least 18 years old. However, a legal guardian serving as an Authorized Representative cannot simultaneously be the caregiver (worker). Caregivers must have legal authorization to work in the United States.
- **Self-Directed Operation:** You can set the caregiver's work schedule and wages within the budget range approved by the MCO (Managed Care Organization).
- **Monthly Budget Support:** The monthly budget is calculated by converting the MCO-approved PCA hours into a dollar amount based on the current reimbursement rate, minus administrative fees.
- **Flexible Funding Use:** Funds can be used to purchase approved goods and services that support independent living. Examples: Laundry services, errand services, and equipment for independent living (e.g., microwave, washing machine, etc.). However, medical services such as Private Duty Nursing are excluded from coverage.
- **Non-Duplication:** Using both the PPP and traditional Agency PCA services simultaneously is considered a duplication of benefits and is not permitted.

Eligibility

- The patient must be enrolled in NJ FamilyCare.
- There must be a physician's prescription stating that PCA services are required for at least 6 months.
- The individual must practically require assistance with ADLs (Activities of Daily Living) such as bathing, eating, and mobility. Minors may also apply if they meet the requirements (with a guardian acting as a representative).
- The individual must have the ability to manage services directly or be able to manage them through an Authorized Representative.
- Residents of private homes, apartments, or group homes are eligible. However, those residing in assisted living facilities, adult foster care, or facilities where PCA services are already included in the service package are excluded.

Application and Contact

- **Contact Your MCO:** Contact your MCO and the state that you are “interested in the PPP” or request a “PCA Assessment.” An MCO Care Manager will then provide options counseling and guide you through the application process.
- **Assessment & PPP Selection:** During the nurse’s home visit assessment, clearly state your preference to use the PPP (Self-Directed) option instead of an agency-provided service.
- After MCO approval, a Financial Consultant from the assigned Fiscal Intermediary will assist with developing the budget plan and completing the enrollment process.

Managed Care Organizations (MCO) in New Jersey

Managed Care Organization (MCO)	Inquiry & Application Phone Number
Aetna Better Health	1-855-232-3596
Fidelis Care (formerly WellCare)	1-855-642-6185 (Select option 3, then option 2)
Horizon NJ Health	1-855-465-4777
UnitedHealthcare Community Plan	1-800-645-9409 (Select option 3)
Wellpoint (formerly Amerigroup)	1-855-661-1996 (Select option 1)

- **NJ PPP Direct Inquiry:** 609-631-2481 / MAHS.PPP@dhs.nj.gov
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [Official New Jersey PPP Website](#)
- [New Jersey PPP Application Phone Number List](#)
- [New Jersey PPP Program \(Fact Sheet\)](#)

Respite Care

Respite Care is a program designed for caregivers. While the services are provided to the care recipient (the patient), the primary purpose is to provide the caregiver with a much-needed break. This service ensures that the patient receives professional care and supervision while the caregiver takes time to rest.

Types of Respite Care

Respite care can be provided in various ways depending on the patient's condition and the situation of the primary caregiver (usually a family caregiver):

- **In-Home Respite:** A professional caregiver visits the home to care for the patient and provide light housekeeping support.
- **Adult Day Care:** A structured daytime program that provides social interaction and medical support.
- **Residential Respite Care:** This is a service where the patient stays away from home in a professional facility for a short period—ranging from a few days to several weeks—to receive 24-hour care.

Benefits

- **Caregiver Stress Relief:** Prevents mental and physical burnout caused by continuous caregiving.
- **Social Interaction:** Provides the patient with opportunities for new social contacts and activities.
- **Guaranteed Continuity of Care:** Ensures that medical and personal needs are met even when the caregiver is absent.
- **Securing Personal Time:** Helps caregivers attend their own medical appointments, travel, or get sufficient rest.

Eligibility

- **Age/Target:** New Jersey residents aged 18 or older.
Living Environment: Must reside at home (excludes nursing homes or other facilities).
- **Health Status:** Those who have a functional impairment and have received confirmation from their attending physician that they require assistance from others.
- **Caregiving Condition:** Those who receive daily basic care and/or supervision from an unpaid caregiver aged 18 or older, such as a spouse, family member, friend, or neighbor.
- **Assets:** Liquid assets must be \$40,000 or less for an individual, or \$60,000 or less for a couple.
- **Income:** Monthly income of \$2,982 or less for a single person, or \$5,964 or less for a couple.

- **Duplicate Program Participation:** Those who do not participate in Long-Term Care (MLTSS), Jersey Assistance for Community Caregiving (JACC), Alzheimer's Adult Day Services Program (AADSP), Congregate Housing Services Program (CHSP), or other NJ FamilyCare programs. Transitioning to Respite Care from JACC or AADSP is possible.

Copay

Respite Care is not a free program. Depending on the recipient's income, a copayment ranging from 0% to a maximum of 25% of the service cost may be required.

Application and Contact

- Contact your local Aging and Disability Resource Connection (ADRC)
 - **Bergen County Division of Senior Services**
 - **Address:** One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601
 - **Phone:** 201-336-7400
 - **Email:** seniors@bergencountynj.gov
 - **NJ ADRC Toll-Free Hotline:** 1-877-222-3737
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org)

Resources

- [NIA.gov National Institute on Aging \(NIA\): What Is Respite Care?](#)
- [NJ.gov Statewide Respite Care Program \(SRCP\) Official Guide](#)
- [ARCH.net National Respite Locator Service](#)

PART I • SECTION 11

Senior Protection

시니어 권익 보호 (Senior Protection)

Protecting older adults from harm and abuse: Adult Protective Services (APS), Senior Scam and Cyber Security prevention, Ombudsman advocacy, and legal services for seniors.

This section contains 4 comprehensive guides

Adult Protective Services (APS)

This content is for informational purposes only. In case of an emergency, call 911 immediately.

Adult Protective Services (APS) is a state-mandated safety net designed to support vulnerable adults aged 18 and older who are unable to protect themselves due to physical or mental limitations. County-designated APS agencies receive, investigate, and respond to reports to ensure the safety of those at risk and connect them with the services they need.

Focus Areas

- **Physical & Emotional Abuse:** Acts of violence, threats, verbal abuse, or any conduct that inflicts psychological distress.
- **Neglect & Self-Neglect:** Failure to provide essential food, medication, or hygiene care, including instances where a senior can no longer care for themselves.
- **Financial Exploitation:** The unauthorized or improper use of a senior's funds, property, or assets for someone else's benefit.

** APS reporting is open to the public, and the identity of the reporter is protected by law. Healthcare professionals, law enforcement officers, firefighters, and EMTs are designated as mandatory reporters.*

Investigation & Support Procedures

- **Rapid Field Investigation:** Within 72 business hours of receiving a report, an APS caseworker will conduct a home visit to thoroughly assess the situation.
- **Tailored Service Connection:** To eliminate immediate risks, APS connects individuals with essential services, including:
 - **Medical Support:** Access to emergency medical treatment and ongoing healthcare services.
 - **Housing Safety:** Assistance with relocating to a safe shelter or alternative housing if necessary.
 - **Financial & Legal Aid:** Connection to financial management services and legal counseling to protect personal rights.

Reporter Protection Policy

- **Strict Confidentiality:** The identity of anyone reporting in good faith is strictly protected by law.

- **Immunity from Liability:** As long as a report is made with reasonable cause, the reporter is immune from any civil or criminal liability, regardless of the investigation's ultimate outcome.

Major APS Contacts in Northern New Jersey

County	APS Agency	Address	Phone
Bergen	Bergen County Board of Social Services	218 Route 17 North Rochelle Park, NJ 07662	Daytime: 201-368-4300 After Hours: 1-800-624-0275
Essex	FOCUS Hispanic Center for Human Dev.	441-443 Broad St. Newark, NJ 07102	Daytime: 866-903-6287 After Hours: 911
Hudson	Hudson County Adult Protective Services	6100 Adams Street West New York, NJ 07093	Daytime: 201-537-5631 After Hours: 911
Middlesex	Family and Children's Service	191 Bath Avenue Long Branch, NJ 07740	Daytime: 732-745-3635 After Hours: 911
Passaic	Passaic County Board of Social Services	80 Hamilton Street Paterson, NJ 07505	Daytime: 973-592-1954 After Hours: 1-800-315-4561

Resources

- [NJ Adult Protective Services \(APS\)](#).

Legal Services for Seniors

This content is for informational purposes only. In case of an emergency, call 911 immediately.

Legal Services for Seniors connects low-income older adults with free or low-cost legal assistance for civil legal matters, ensuring their rights and dignity are protected.

Key Areas of Support

- **Abuse Response:** Legal support and counseling to protect seniors from physical, emotional, or verbal abuse, isolation, and threats, including obtaining restraining orders and other protective measures.
- **Advance Planning:** Assistance with Power of Attorney (POA), Healthcare Proxy Directives, and Living Wills to ensure your future wishes are legally documented.
- **Housing Rights:** Legal counsel for eviction defense, landlord-tenant disputes, and maintaining housing stability.
- **Financial Protection:** Legal response to consumer fraud, debt management, and safeguarding personal assets.
- **Public Benefits:** Support for issues regarding Medicare, Medicaid, Social Security, and other essential public assistance programs, including eligibility reviews and appeals for denied benefits.

Legal Support Network

- **Adult Protective Services (APS):** Serves vulnerable or at-risk adults 18 and older, especially those who may have difficulty protecting themselves due to disability or aging, to quickly ensure their safety and connect them with needed services.
 - For details check the [Adult Protective Services \(APS\)](#) page.
- **LSNJ (Legal Services of New Jersey):** Operates a statewide legal hotline specifically for low-income seniors, offering expert legal advice from experienced attorneys.
- **Local Legal Representation:** Coordination with county-level legal service offices for case handling and courtroom representation.
- **Advocacy & Education:** Regular educational resources to help seniors understand their legal rights and prevent legal complications before they arise.

How to Access Services

1. **Application:** Request an initial consultation via the statewide hotline or online portal.
2. **Eligibility Review:** Assessment of household income, age, residency, and the nature of the legal issue.

3. **Expert Assignment:** Matching the case with a qualified attorney or legal advocate for counsel, document preparation, or representation.

Contact

- **LSNJ Hotline:** 1-888-576-5529 | lsnjlawhotline.org/intake | lsnj.org
- **Bergen County NJ APS:** 201-368-4300 | 24-Hour Hotline 1-800-624-0275
- **National Domestic Violence Hotline:** 1-800-799-7233 | thehotline.org/
- [NJ 211.org: Find Legal Services for Seniors](https://nj211.org/)
- [NNJLS.org – Free Legal Services for Bergen, Hudson, and Passaic](https://nnjls.org/)
- [CHLP.org – New Jersey advocacy for seniors and people with disabilities](https://chlp.org/)

Ombudsman

This content is for informational purposes only. In case of an emergency, call 911 immediately.

An Ombudsman is an independent advocate who receives, investigates, and resolves complaints regarding the operations of government agencies, long-term care facilities, or specific organizations. Acting as a neutral third party, they provide fair and transparent mediation for citizens facing complex administrative procedures or unfair treatment.

**All Ombudsman services are free and confidential.*

Core Service Areas

- **Rights Advocacy & Investigation:** Conducting independent inquiries into government errors, delays, lack of response, or unfair decisions.
- **Long-Term Care Oversight:** Ensuring the safety, quality of care, and dignity of residents in Nursing Homes and Assisted Living facilities.
- **Conflict Mediation:** Facilitating communication between complainants and agencies to seek informal, efficient resolutions before legal litigation.
- **Policy Recommendations:** Analyzing recurring complaints to recommend systemic improvements to government departments or legislative bodies.
- **Information & Education:** Providing consultations and resources to help citizens understand their rights and comply with relevant regulations.

Operational System

The Ombudsman operates independently across federal, state, and local departments.

- **Long-Term Care Ombudsman (LTCO):** Specializes in issues of abuse, neglect, or service dissatisfaction within long-term care facilities across New Jersey.
 - For details check the [Long-Term Care Ombudsman](#) page.
- **Agency-Specific Ombudsmen:** Offices within departments such as the State Department or the Social Security Administration (SSA) to handle grievances related to specific administrative services.
- **Independence & Confidentiality:** Ombudsmen maintain an independent status free from agency interference. Consultation details and complainant identities are strictly protected by law.

Ombudsman Process

1. **Intake:** Submit grievance details via phone, email, or official website forms.
2. **Initial Review:** Verification of whether the complaint falls within the Ombudsman's jurisdiction and collection of preliminary data.
3. **Investigation & Mediation:** Conducting neutral inquiries through interviews and document reviews to recommend solutions.
4. **Resolution Notice:** Formal delivery of investigation results and subsequent action plans to the complainant.

Resources

- [U.S. Department of State \(Civil Rights & Ombudsman\)](#)
- [NJ Long-Term Care Ombudsman \(NJ LTCO\)](#) | 1-877-582-6995
- [NJ Dept. of Human Services \(DHS\) Ombudsman](#) | 1-800-357-2260
- [Administrative Conference \(ACUS\) Resources](#)
- [NJ 211.org](#)

Senior Scam & Cyber Security

This content is for informational purposes only. In case of an emergency, call 911 immediately. For fraud recovery and reporting, contact the National Elder Fraud Hotline: 1-833-372-8311 (Weekdays 6am–11pm ET).

Scams targeting seniors are growing more sophisticated every day—from government impersonators to AI-generated voice fraud to social media deception. Staying informed about how these crimes work is one of the most powerful ways to protect yourself and the people you love.

Common Scam Types & Warnings

- **Government Impersonation:** Scammers pose as Social Security Administration (SSA), IRS, or Medicare representatives to steal personal information or pressure you into sending money under threat of legal action. Real government agencies will never demand immediate payment or sensitive data by phone or text.
- **Financial Exploitation:** Trusted individuals—family members, caregivers, or legal representatives—exploit a senior’s physical or cognitive vulnerabilities to gain unauthorized access to their finances or assets.
- **Tech Support Scams:** A fake pop-up warns that your device has been compromised and tells you to click a link or call a number for help. The “technician” then asks to remotely access your device and uses that access to steal financial information and drain accounts.
- **Fake Ads & Social Media Scams:** Fraudulent ads exploit celebrity names or promise free prizes to get you to enter personal information, sign up for fake investments, or unknowingly authorize charges for paid services.
- **Grandparent / Family Emergency Scams:** A scammer poses as a grandchild or other family member in crisis—claiming to be arrested, in an accident, or in danger—and demands an immediate wire transfer before you have time to think it through.
- **Romance Scams:** After building trust and affection through social media or messaging apps, the scammer fabricates emergencies—medical bills, travel costs, legal fees—and repeatedly requests money.
- **Phishing & Malicious Links:** Text messages pretending to be from government agencies or delivery services contain links designed to hack your device and steal personal and financial information.

Prevention & Response

- **Hang Up First — Then Verify**
 - Government agencies like the SSA, IRS, and local police will never call demanding immediate payment, threatening arrest, or asking for a card number. If something feels off, hang up.
 - If someone claims your grandchild was in an accident and needs money right away, hang up and call your family member directly at the number you already know.
- **Never Click Unknown Links or Install Unknown Apps**
 - Do not tap links in texts about package deliveries, fines, or account alerts from numbers you don't recognize.
 - Never install an app someone sends you or that appears in a pop-up ad, especially one claiming to be a security tool.
- **Gift Cards and Wire Transfers Are Always a Scam**
 - No legitimate government agency or business will ever ask you to pay a fine, fee, or penalty using gift cards, Bitcoin, or a cash wire transfer.
 - Never allow someone you don't know to remotely access your computer or phone, even if a warning screen tells you to call for help.
- **Never Share Personal Information**
 - Keep your bank passwords and full Social Security Number private, even from family members.
 - If a stranger on social media reaches out, becomes friendly quickly, and then asks to borrow money or recommends an investment, block them immediately.
- **Ask for Help the Moment You Suspect Fraud**
 - Being scammed is never your fault. It takes courage to speak up. Ask for help right away.
 - If you sent money or shared account information, call your bank immediately and ask them to place a hold on your card or cancel it.

Scam Types	Government Contacts
Government Impersonation SSA, IRS, or Medicare reps demanding personal info or payment	National Elder Fraud Hotline 1-833-372-8311 FTC: Report Fraud reportfraud.ftc.gov
Financial Exploitation Trusted individuals misusing a senior's finances or assets	NJ Division of Consumer Affairs 1-800-242-5846 APS (Bergen County) 1-800-624-0275 201-368-4300
Tech Support Scams Fake virus warnings leading to remote device access and theft	FTC: Report Fraud reportfraud.ftc.gov NJ Division of Consumer Affairs 1-800-242-5846
Fake Ads & Social Media Fraudulent ads using celebrities or prizes to steal info or charge cards	FTC: Report Fraud reportfraud.ftc.gov NJ Division of Consumer Affairs 1-800-242-5846
Family Emergency Scams Impersonating a grandchild or family member to demand urgent money	Local Police (911) National Elder Fraud Hotline 1-833-372-8311
Romance Scams Building online trust, then repeatedly requesting money for fake situations	FTC: Report Fraud reportfraud.ftc.gov
Phishing & Malicious Links Fake agency or delivery texts with links that hack your device	FTC: Report Fraud reportfraud.ftc.gov

**In case of an emergency, call 911 immediately. For fraud recovery and reporting, contact the National Elder Fraud Hotline: 1-833-372-8311 (Weekdays 6am–11pm ET).*

Contacts

- **National Elder Fraud Hotline:** 1-833-372-8311
- **FTC Reporting:** reportfraud.ftc.gov
- **NJ Division of Consumer Affairs:** 1-800-242-5846

- **NJ Office of Attorney General:** 609-984-5828

Resources

- [FTC.gov – Identity Theft](#) | [FTC.gov – Report Fraud](#) | [FTC.gov – Do Not Call](#)
- [OVC.gov – Helping Victims Find Their Justice](#)
- [NJ Consumer Affairs.org](#)
- [NJ OAG.gov – Office of Attorney General](#)

PART I • SECTION 12

Retirement Planning

은퇴·노후 준비 (Retirement Planning)

Essential legal and lifestyle planning for retirement: Durable Power of Attorney (POA), Advance Healthcare Directives, Living Wills, Last Wills & Testaments, Revocable Living Trusts, Medicaid Asset Protection Trusts, AARP benefits, Age-Friendly Communities, Senior Transportation Services, Leisure & Recreation, and reverse immigration considerations.

This section contains 11 comprehensive guides

AARP (American Association of Retired Persons)

AARP is the nation's largest nonprofit, **nonpartisan** organization dedicated to empowering people aged 50 and older to choose how they live as they age. While its core focus is on those 50+, membership is open to anyone aged 18 or older. AARP provides an extensive array of resources and benefits covering health, financial security, leisure, and overall quality of life.

Areas of Support

- **Finance and Fraud Prevention:** Provides retirement planning guides and manages the **Fraud Watch Network**, which offers education on preventing phone scams (voice phishing) and financial fraud, along with victim support services.
- **Health and Medical Resources:** Offers Medicare comparison tools, dementia and caregiving resources, and access to discounts and information regarding vision and hearing health.
- **Cost-of-Living Discounts:** Members gain access to exclusive savings across various sectors, including travel (hotels, flights), dining, retail, and auto/home insurance.
- **Employment and Career Support:** Provides job-search tools tailored for workers over 50, skills retraining programs, and a job board featuring employers who have signed the **AARP Employer Pledge**.
- **Driver Safety Education:** Operates the **AARP Smart Driver™ Course** in both online and in-person formats, helping seniors improve road safety skills and potentially qualify for auto insurance discounts.

Support Systems

- **AARP Foundation:** The organization's charitable affiliate, which manages programs for low-income seniors, including housing stability, food security, and the **Tax-Aide** program.
 - **Tax-Aide:** Provides free tax preparation assistance with a focus on low-to-moderate-income taxpayers aged 50 and older. However, this service is open to everyone regardless of age, income, or AARP membership status.
- **Fraud Watch Network:** Operates a dedicated fraud helpline to share information on the latest scams and provide real-time guidance to victims.
- **State Offices (AARP New Jersey):** Local state chapters, including New Jersey, conduct community-based advocacy work and host social events for residents.
- **AARP Publications:** Delivers the latest trends in health, finance, and technology through **AARP The Magazine** (published bimonthly) and the **AARP Bulletin**.

Contact

- [AARP](#) General Line: 1-888-OUR-AARP (1-888-687-2277)
- [AARP New Jersey State Office](#)
- [AARP Foundation Job Board / Career Center](#)

Advance Directive

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

An **Advance Directive** is a legal document that allows you to specify the scope of medical treatment you wish to receive and designate a representative in advance, should you become unable to make or communicate your own decisions due to future illness or accident. This is an essential planning process to preserve your dignity and alleviate the burden of decision-making for your family.

In New Jersey, an Advance Directive only takes effect when your attending physician determines that you lack the capacity to make your own healthcare decisions. Any individual aged 18 or older may execute an Advance Directive.

Key Components

Advance Directives typically include the following two elements, which can be prepared together or separately:

Category	Primary Content	Role
Instruction Directive (Commonly known as a Living Will)	Specifies the wish to withhold or continue life-sustaining treatment in particular circumstances.	Communicate your treatment guidelines.
Proxy Directive (Durable Power of Attorney for Healthcare)	Designates a representative to make medical decisions on your behalf.	Ensure the legal representative's decision-making authority.

Five Wishes: A Comprehensive Advance Directive

Created by the non-profit organization Aging with Dignity, Five Wishes covers not only medical decisions but also your emotional and personal desires. It is legally recognized in most U.S. states, including New Jersey, and is available in 31 languages, including English and Korean, making it highly accessible for Korean-American seniors.

It is legally valid in all 50 states and Washington D.C., though four states (Kansas, New Hampshire, Ohio, and Texas) may require additional procedures such as attaching state-specific forms.

**Can be obtained for free through healthcare providers and hospice partners.*

Elements of Five Wishes

1. **The Person I Want to Make Care Decisions for Me When I Can't:** Legally designates a person who will communicate with medical staff and make final decisions when you cannot. This can be a family member or a friend.
2. **The Kind of Medical Treatment Desired or Refused:** Specifies the extent of life-sustaining treatment you wish to receive, such as CPR, ventilator use, or artificial nutrition.
3. **How Comfortable I Want to Be:** Details specific requests for pain management and creating a peaceful environment (e.g., music, bathing, bedding).
4. **How I Want People to Treat Me:** Requests specific emotional considerations, such as how staff and family should interact with you, having family present, physical touch (holding hands), or religious support.
5. **What I Want My Loved Ones to Know:** Includes final messages to loved ones, funeral arrangements, and the philosophical legacy of how you wish to be remembered.

Execution Procedures (New Jersey)

- **Decision-Making and Consultation:** Reflect on your values and consult thoroughly with your primary care physician and family.
- **Document Preparation:** Complete the New Jersey standard form or the Five Wishes form.
 - Five Wishes is available for purchase (\$5) on their [official site](#) or for free through healthcare/hospice partners. **Korean version available.*
- **Legal Requirements:** Requires signatures from two adult witnesses (18 and above).
 - The designated Health Care Agent cannot serve as a witness.
 - To prevent disputes, it is recommended to choose a third party who is not a beneficiary of your estate.
 - **Notarization:** In lieu of witness signatures, you may sign before a Notary Public or an attorney-at-law.
- **Storage and Distribution:** Keep the original in a safe but accessible place (avoid locked safes). Provide copies to your family, primary physician, Health Care Agent (including alternates), and any hospital you expect to be admitted to.

Contact

Agency	Services Provided	Contact Information
NJ ADRC (Aging & Disability Resource Connection)	Service connection and counseling for seniors, individuals with disabilities (18 and above), and family caregivers by county.	1-877-222-3737 (Connects automatically to your local county office)
Bergen County Division of Senior Services	Information, assistance, and referrals for legal services.	One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601 201-336-7400 seniors@bergencountynj.gov
NNJLS (Northeast New Jersey Legal Services)	Free legal aid for low-income residents, seniors, and the disabled (subject to eligibility).	190 Moore Street, Suite 100, Hackensack, NJ 07601 201-487-2166 NNJLS.ORG
LSNJ (Legal Services of New Jersey Hotline)	Statewide legal hotline providing free legal advice and information.	1-888-576-5529 lsnjlawhotline.org

Resources

- [Medicare.gov: Advance Care Planning coverage for beneficiaries](#)
- [Official NJ Advance Directive Page \(including Downloadable Forms\)](#)
- [Five Wishes Forms and Information](#)
- NJ211.org — Legal Assistance Available to Seniors

Age-Friendly Communities

An **Age-Friendly Community** is a global initiative dedicated to fostering physical and social environments where individuals can age safely, healthily, and actively within their own neighborhoods (**Aging in Place**). The criteria for age-friendly communities are applied in accordance with the standard guidelines established by the World Health Organization (WHO).

Key Areas of Support

- **Mobility and Transportation (Bergen Busing):** Complimentary **door-to-door** transportation services for Bergen County residents aged 60 and older, individuals with disabilities, and veterans.
 - This includes the operation of specialized **Senior Van** services.
- **Housing Modification and Stability:** Assistance with **home modifications** (such as installing grab bars for fall prevention), expanding access to low-income senior housing, and providing resources for property tax relief.
- **Social and Human Connection:** Socialization programs offered through over 20 senior centers in Bergen County, along with **reassurance call services** for socially isolated seniors.
- **Bridging the Digital Divide:** Education on smartphone and tablet usage to empower seniors to apply for government benefits online and stay connected with their families.
- **Safe Pedestrian Environments:** Improvements to the physical infrastructure, including extended crosswalk signal times, lowered curb heights, and the installation of adequate seating in public parks.

Support Systems

- **AARP Network of Age-Friendly States and Communities:** Provides technical guidelines for cities across the U.S. to improve the **8 Domains of Livability**.
 - **The 8 Domains of Livability include:**
 1. Outdoor Spaces and Buildings
 2. Transportation
 3. Housing
 4. Social Participation
 5. Respect and Social Inclusion
 6. Civic Participation and Employment
 7. Communication and Information

8. Community Support and Health Services

- **Age-Friendly North Jersey (AFNJ):** A statewide network of over 10 municipalities in Northern New Jersey, including Bergen County, focused on sharing best practices for aging-friendly environments.
- **Bergen County Division of Senior Services:** Oversees county-level age-friendly policies and provides comprehensive resources through the **Key Services Guide**.
 - In May 2022, Bergen County was officially designated as an Age-Friendly Community by AARP and the WHO.

How to Access Services

- **Verify Residency:** Confirm if your specific municipality is currently operating age-friendly initiatives.
- **Request Services:** Contact the county or local municipal senior center via telephone to book transportation or request housing assistance.
- **Community Engagement:** Participate in local age-friendly surveys or town hall meetings to suggest necessary improvements for your community.

Korean Community Support Agencies in Bergen County

Agency	Services Provided	Contact Information
AWCA	Senior Activity Center (social programs, congregate meals, health screenings). Home Care Services (Korean-speaking Home Health Aides) and a Welfare Consultation Hotline.	9 Genesee Ave., Teaneck, NJ 07666 201-862-1665 awcanj.org
Family Touch	Non-profit providing family counseling and mental health education. Offers senior depression programs with 50% fee subsidies (sliding scale based on income).	218 Main St, Little Ferry, NJ 07643 201-242-4422 familytouchusa.org
Give Chances	After-school education and art programs for low-income children and the disabled. Seniors can participate as mentors or volunteers.	500 Broad Ave, Palisades Park, NJ 07650 201-266-0640 givechances.org
KAANJ (Korean American Association of New Jersey)	Established in 1975 as a non-profit 501(c)(3). Focused on community advocacy, settlement assistance for new residents, local networking, and cultural event programming.	21 Grand Ave. #216-B, Palisades Park, NJ 07650 201-945-9456 kaanj.org
Korean American Senior Citizens Assoc. of NJ (KASCA NJ)	Integrated support for health, finance, and well-being. Operates a social welfare counseling center, medical info center, and beauty/barber volunteer services.	133 Fort Lee Rd. #3FL, Leonia, NJ 07605 201-945-2400 kascanj.org
KACE (Korean American Civic Empowerment)	Established in 1996 as a non-profit, nonpartisan organization. Conducts voter education, civic advocacy, youth leadership development, and grassroots community organizing.	263 Broad Ave., 2A Palisades Park, NJ 07650 201-488-4201 kace.org
KCS New Jersey (Korean Community Services of NJ)	Provides integrated programs for Korean and Asian immigrants, including mental health counseling, immigration services, workforce development, and public health programs. *Income and eligibility requirements apply.	100 Grove St., Tenafly, NJ 07670 201-541-1200 kcsus.org
MinKwon Center (MinKwon Center for Community Action)	Established in 1984. Provides social services for Korean, Asian, and immigrant communities, including immigration legal aid, citizenship application assistance, housing rights	316 Broad Ave., 2nd Fl., Palisades Park, NJ 07650

Agency	Services Provided	Contact Information
	advocacy, and public benefit enrollment (SNAP, HEAP). Also conducts civic engagement and voter education.	201-546-4657 minkwon.org

Contact

- **Bergen County Division of Senior Services:** 201-336-7400 | co.bergen.nj.us/division-of-senior-services
- **Age-Friendly North Jersey:** agefriendlynj.org
- **AARP Livable Communities:** aarp.org/livable
- **Bergen County Transportation (ADAPT):** 201-336-7400 (Senior shuttle reservations)
- **NJ 211:** Dial 211 or visit nj211.org

Resources

- [Bergen County Social Services Directory\(PDF\)](#)
- [Bergen County Key Services Guide \(PDF\)](#)
- [WHO Age-Friendly World](#)

Leisure Activities & Senior Centers

Structured physical activity and social engagement are critical factors in preserving cognitive function and extending longevity for older adults. Local **Senior Centers** provide a diverse array of programs designed to meet these needs.

Core Activities

- **Social Engagement:** Playing games (Chess, Cards) with peers, video calling family, and joining hobby-based clubs.
- **Learning Opportunities:** Classes in culinary arts, fine arts, and foreign languages; participating in book or film clubs; and learning musical instruments.
- **Community Involvement:** Volunteering at schools, libraries, or hospitals; participating in community gardening; and joining choral groups.
- **Physical Activity:** Gardening and yard work, senior-specific fitness classes, walking with friends, and swimming.
- **Stress Management:** Reading, practicing mindfulness, Tai Chi, or Yoga.

Senior Centers

Municipal and county-run senior centers throughout Bergen County, New Jersey, operate specialized programs. We encourage you to participate based on your personal interests and health status.

• Health & Wellness

- **Congregate Meals:** Balanced, nutritionist-designed lunches provided at an affordable rate (typically via a suggested donation). For those with limited mobility, these centers coordinate with [Meals on Wheels](#) delivery services.
- **Specialized Fitness Programs:**
 - **SilverSneakers:** A fitness program offered at no cost to many **Medicare Advantage** and **Supplement** plan members. Please verify eligibility with your insurance provider.
 - **Fall Prevention Education:** Balance training designed to prevent falls, the leading cause of accidental injury among seniors.
 - **Zumba Gold & Chair Yoga:** Low-impact dance and yoga sessions designed to be gentle on the joints.

- **Health Screenings:** Routine blood pressure monitoring, glucose checks, and guidance for seasonal Flu and COVID-19 vaccinations.

- **Lifelong Learning & Hobbies**

- **Digital Literacy:** Training on smartphone usage, video conferencing (**Zoom/FaceTime**), online banking, and **fraud/scam prevention** education.
- **Arts & Crafts:** Activities such as watercolor painting, calligraphy, knitting, and origami to strengthen cognitive function through fine motor skills.
- **Language Classes:** English as a Second Language (**ESL**) classes and multicultural language exchange programs unique to Bergen County's diverse community.

- **Transportation Services**

- **Shuttle Services:** Dedicated senior shuttles for center visits, local grocery shopping, and medical appointments. (Advance reservations are mandatory).

Senior Center Directory in Bergen County

Center Name	Address	Phone Number
AWCA Senior Activity Center	9 Genesee Ave., Teaneck, NJ 07666	201-862-1665
Bergenfield Senior Center	293 Murray Hill Ter., Bergenfield, NJ 07621	201-387-7212
East Rutherford Senior Center	147 Hackensack St., East Rutherford, NJ 07073	201-935-8920
Elmwood Park Senior Center	387 Market St., Elmwood Park, NJ 07407	201-796-3342
Garfield Senior Center	480 Vreeland Ave., Garfield, NJ 07026	973-478-0502
KCS New Jersey Social Adult Daycare (Private)	100 Grove St., Tenafly, NJ 07670	201-541-1200 x 111
North Arlington Senior Center	11 Bayard Ave., North Arlington, NJ 07031	201-998-5636
Northwest Senior Center	46-50 Center St., Midland Park, NJ 07432	201-445-5690
Ridgefield Park Senior Center	159 Park St., Ridgefield Park, NJ 07660	201-641-7170
Wallington Senior Center	24 Union Blvd., Wallington, NJ 07057	973-777-5815

Contact

- **Bergen County Division of Senior Services:** 201-336-7400
- [AARP Official Website](#)
- **NJ 211:** Statewide helpline for community resource connection. Call 211.
- [NJ Division of Parks & Forestry:](#) Information on the **Senior Pass** (Free admission to state parks for NJ residents 62+).
 - Call 1-800-843-6420
- **AWCA** provides counseling free of charge (201-862-1665 / info@awcanj.org).

Resources

- [List of County-Operated Senior Centers in Bergen County](#)
- [Bergen County Key Services Guide \(PDF\)](#)

Living Trust

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

A **Living Trust** is a legal arrangement in which the asset owner (**Grantor/Settlor**) transfers the authority to manage their assets to a **Trustee** during their lifetime. It serves as a vital supplement to a Will by specifying how assets will be managed and distributed after the Grantor's passing.

Types of Living Trusts

- **Revocable Living Trust:** The most common form of trust. The Grantor retains full control over the assets and can modify or dissolve the trust at any time during their lifetime.
- **Irrevocable Living Trust:** Once established, this trust is extremely difficult to amend or revoke. It is primarily used for **asset protection**, estate tax reduction, or qualifying for government benefits such as **Medicaid (Long-term Care)**.

Key Advantages of a Living Trust

Avoidance of Probate

While a Last Will and Testament must go through the **Surrogate's Court** for **Probate**, a Living Trust allows assets to pass directly to beneficiaries without court intervention. Although New Jersey's probate process is relatively streamlined compared to other states, a trust ensures an even more immediate transfer of assets.

Privacy Protection

A Living Trust remains a private document; therefore, the size of the estate and the details of the distribution are not disclosed to the public. In contrast, a Will becomes a **Public Record** once filed with the court, making it accessible to anyone.

Incapacity Planning

If the Grantor becomes unable to manage their affairs due to illness (such as dementia) or accident, a pre-designated **Successor Trustee** can immediately step in to manage the assets. This bypasses the need for a costly and time-consuming court-ordered **Conservatorship** or **Guardianship** proceeding.

Protection of Assets for Minor Children

A Living Trust allows you to set specific conditions—such as milestone ages—for when children receive their inheritance, which promotes long-term financial stability.

Considerations

- **Setup and Maintenance Costs:** Establishing and maintaining a trust requires significant initial legal fees and administrative effort. For smaller estates, these costs may potentially exceed the eventual cost of probate.
- **Requirement of a "Pour-Over Will":** Even with a trust, a **Pour-Over Will** is essential. This ensures that any assets accidentally left out of the trust's name are legally "poured" into the trust upon the owner's death.
- **Funding the Trust:** Simply signing the trust documents is insufficient. For the trust to be effective, you must **"fund"** it by changing the titles of your assets—such as real estate (**Deeds**) and bank accounts—into the name of the trust (e.g., *The [Name] Living Trust*).
- **Alternative Options:** Simpler and more cost-effective methods for asset transfer exist, such as **Payable on Death (POD)** or **Transfer on Death (TOD)** designations on accounts.
- **Professional Selection:** Seek advice from a certified **Estate Planning Attorney**. If you meet certain income requirements, you may be eligible for free legal assistance through organizations such as the **Northeast New Jersey Legal Services (NNJLS)**.

Contact

Agency	Primary Services	Contact Information
NJ ADRC (Aging & Disability Resource Connection)	Service connection and counseling for seniors, individuals with disabilities (18 and above), and family caregivers by county.	1-877-222-3737 (Calls are automatically routed to your local county office)
Bergen County Division of Senior Services	Information, assistance, and referrals for legal services.	One Bergen County Plaza 2nd Floor Hackensack, NJ 07601 201-336-7400 seniors@bergencountynj.gov
NNJLS (Northeast New Jersey Legal Services)	Free legal aid for low-income residents, seniors, and the disabled (subject to eligibility).	190 Moore Street, Suite 100, Hackensack, NJ 07601 201-487-2166 NNJLS.ORG
LSNJ (Legal Services of New Jersey Hotline)	Statewide legal hotline providing free legal advice and information.	1-888-576-5529 lsnjlawhotline.org

Resources

- [New Jersey Will Registry Information](#)
- [NIH.gov–Getting Your Affairs in Order Checklist](#)
- [NCOA \(National Council on Aging–Estate Planning Checklist & Guide](#)

Instruction Directive (Living Will)

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

An Instruction Directive, commonly referred to as a Living Will, is a component of an Advance Directive. It is a legal document in which you explicitly state the medical treatments you wish to receive or refuse under specific circumstances. This document becomes effective only after an attending physician formally determines that you have lost the capacity to make your own healthcare decisions. It serves as a crucial guide for family members and medical providers to honor your values and make informed decisions on your behalf.

Key Characteristics

- **Life-Sustaining Treatment:** Specific instructions regarding the refusal or acceptance of interventions intended to prolong life, such as Cardiopulmonary Resuscitation (CPR) or mechanical ventilation.
- **Artificial Nutrition and Hydration:** Decisions on whether to continue artificial nutrition (feeding tubes) or intravenous (IV) hydration when you are no longer able to take food or water by mouth.
- **Renal Dialysis:** Instructions on whether to undergo blood filtration via a dialysis machine in the event of kidney failure.
- **Palliative and Hospice Care:** Choices regarding comfort-oriented care focused on pain management and a dignified end-of-life experience rather than life-prolonging interventions.

The Role of a Living Will

- **Secures Self-Determination:** Legally protects your right to make medical choices aligned with your personal values and religious beliefs.
- **Alleviates Family Burden:** Prevents potential guilt or conflict among family members who might otherwise struggle with treatment decisions during a crisis.
- **Provides Clear Clinical Guidance:** Communicates your wishes directly to medical staff, preventing unnecessary or unwanted aggressive interventions.

Drafting and Execution Procedures

Applications or inquiries can be made by individuals, legal representatives, government agencies, or hospitals. (You may draft this document yourself without the involvement of an attorney or physician.)

1. **Consultation:** Carefully consider your preferred scope of treatment and discuss these values with your family and primary care physician.
2. **Document Preparation:** New Jersey does not mandate a specific format; the NJ Department of Health standard form, Five Wishes, or a self-drafted document are all valid. (Often drafted alongside a Proxy Directive/Healthcare POA.)
3. **Legal Requirements:** Must be signed by two adult witnesses (excluding the designated Health Care Agent) OR signed before a Notary Public or attorney.
 - *Notarization is not mandatory if two witnesses are present.
4. **Storage and Sharing:** Provide copies to your primary physician, family, and your designated Health Care Agent.
 - *Keep the original in an accessible location. **Do not** store it in a locked safe or a Safe Deposit Box where it cannot be reached in an emergency.*

POLST

POLST (Practitioner Orders for Life-Sustaining Treatment) is an actual **Medical Order** used in conjunction with an Advance Directive along with Living Will. While an Advance Directive outlines your “wishes,” a POLST is an official order signed by a physician (or APN/PA) based on those wishes. It is a binding document that first responders, ER staff, and nursing home personnel must follow immediately in an emergency.

****A POLST does not replace an Advanced Directive and Living Will. The Advance Directive sets the general direction, while the POLST provides specific, actionable instructions for **emergency scenes**.***

When is a POLST Necessary?

A POLST is not for everyone. Consultation with a physician is recommended for individuals who:

- Have a serious, chronic, or progressive illness, or a life expectancy of less than 5 years.
- Are planning to enter a nursing home or long-term care facility.
- Are receiving in-home hospice services.
- Have an increased risk of emergency due to advanced age.

Medical Treatment Level Codes

- **Full Code**
 - Perform all possible resuscitative measures (CPR, ventilator, defibrillation) in the event of cardiac or respiratory arrest.
 - This is the hospital default unless otherwise specified.

- **DNR (Do Not Resuscitate)**

- Instructs medical staff **not** to perform CPR if your heart or breathing stops.
- Other treatments (oxygen, medication, pain relief) will continue.

- **DNI (Do Not Intubate)**

- Refuses the insertion of a breathing tube (intubation) for mechanical ventilation.
- This is distinct from DNR; if only DNI is specified, CPR may still be performed.

- **DNH (Do Not Hospitalize)**

- Directs that you receive **Comfort Care** at your current residence (home, nursing home) rather than being transferred to a hospital during an emergency.
- Typically applied to hospice or long-term care patients.

Keep your completed POLST (green paper) in a prominent location (refrigerator door, bedside, etc.) and share copies with your doctor and agent.

Contact

Agency	Services Provided	Contact Information
NJ ADRC (Aging & Disability Resource Connection)	Service connection and counseling for seniors, individuals with disabilities (18 and above), and family caregivers by county.	1-877-222-3737 (Connects automatically to your local county office)
Bergen County Division of Senior Services	Information, assistance, and referrals for legal services.	One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601 201-336-7400 seniors@bergencountynj.gov
NNJLS (Northeast New Jersey Legal Services)	Free legal aid for low-income residents, seniors, and the disabled (subject to eligibility).	190 Moore Street, Suite 100, Hackensack, NJ 07601 201-487-2166 NNJLS.ORG
LSNJ (Legal Services of New Jersey Hotline)	Statewide legal hotline providing free legal advice and information.	1-888-576-5529 lsnjlawhotline.org

Resources

- Medlineplus.gov – Advance Care Directive
- NJ.gov – Advance Directive
- NJ.gov – Living Will (Instruction Directive) Form (PDF)
- [NJ 211.org](http://NJ211.org)–Legal Assistance for Seniors
- [Five Wishes](#)
- [Five Wishes – Sample \(PDF\)](#)
- NJ.gov – POLST

Medicaid Trusts

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

A **Medicaid Trust** is a legal instrument designed to help seniors or individuals with disabilities protect and manage their assets while maintaining eligibility for government benefits. It is essential to select the appropriate trust type based on your specific objectives and circumstances.

Primary Types

- **Revocable Trusts:** Offers high flexibility as the grantor can modify or terminate the trust at any time. While useful for **probate avoidance**, they are generally not suitable for Medicaid asset protection because the assets are still considered countable resources in eligibility assessments.
- **Irrevocable Trusts:** Once established, these trusts generally cannot be modified or revoked. While any portion that could potentially be paid to the grantor may still be counted as an asset, a properly structured Irrevocable Trust is highly effective for **Medicaid planning and asset protection**.
- **Pooled Trusts:** Managed by non-profit organizations, these trusts are available to individuals with disabilities of all ages. Assets from multiple beneficiaries are pooled for investment purposes to increase returns, though individual sub-accounts are maintained. *In New Jersey, if your income exceeds the Medicaid limit, a **Qualified Income Trust (QIT/Miller Trust)** is required. Pooled trusts are primarily used to safeguard a disabled person's assets to enhance their quality of life without disqualifying them from Medicaid.*
 - Individuals 65 or older should consult a professional before transferring assets to a pooled trust, as a **Look-back Penalty** may apply.
- **Qualified Income Trust (QIT / Miller Trust):** In New Jersey, if an individual's monthly income exceeds the institutional Medicaid limit (\$2,982 as of 2026), the excess income must be deposited into a QIT to maintain eligibility.

** Limits are subject to annual update; check the latest [NJ DHS](#) standards.*

Benefits

- **Asset Protection:** Prevents the depletion of your estate by high medical or long-term care costs, establishing a foundation for passing assets to the next generation.
 - Medicaid may seek to recover costs for services provided to individuals 55 and older from their estate after death.

- Recovery is deferred if there is a surviving spouse, a child under 21, or a disabled child. A properly established trust helps shield assets from this recovery process.
- **Maintenance of Eligibility:** Assists in receiving necessary welfare benefits while complying with Medicaid's strict asset and income limitations.
 - 2026: *Individual Asset Limit \$2,000 / Income Limit \$2,982 per month. (Subject to annual [NJ DHS updates](#)).*
- **Professional Management:** Especially in the case of Pooled Trusts, professional non-profits manage the assets, ensuring transparency and stability.
- **Long-Term Care Planning:** The **Look-back Period** involves a 60-month (5-year) review of asset transfers prior to a long-term care Medicaid (MLTSS) application. Transfers made for less than **Fair Market Value** during this window will trigger a **Penalty Period** (delay in benefits). Advance planning is essential.

Implementation Procedures

1. **Professional Consultation:** Work with a specialized attorney to analyze your financial situation and Medicaid eligibility.
2. **Determination of Trust Type:** Select the optimal trust model based on asset size, health status, and family needs.
3. **Drafting and Notarization:** Legal requirements vary by trust type. Notarization is recommended for standard trusts; a QIT requires DHS-approved templates and the opening of a dedicated bank account.
4. **Funding the Trust:** Complete the process by retitling assets (e.g., real estate, accounts) into the name of the trust.

Contact

Agency	Services Provided	Contact Information
NJ ADRC (Aging & Disability Resource Connection)	Service connection and counseling for seniors, individuals with disabilities (18 and above), and family caregivers by county.	1-877-222-3737 (Connects automatically to your local county office)
Bergen County Division of Senior Services	Information, assistance, and referrals for legal services.	One Bergen County Plaza 2nd Floor Hackensack, NJ 07601 201-336-7400 seniors@bergencountynj.gov
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LSNJ (Legal Services of New Jersey Hotline)	Statewide legal hotline providing free legal advice and information.	1-888-576-5529 lsnjlawhotline.org

References

- [NJ.gov – Qualified Income Trusts](#)
- [NJ.gov – Estates and Trusts \(PDF\)](#)

Power of Attorney (POA)

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

A **Power of Attorney (POA)** is a legal instrument that authorizes a trusted third party (referred to as the “Attorney-in-Fact” or “Agent”) to act on your behalf in legal, financial, and medical matters. This is typically established to ensure your affairs are managed if you become unable to make decisions due to physical or mental incapacity, or absence. It serves as a vital safeguard to honor your wishes and protect your interests in unforeseen circumstances.

Under New Jersey law, an agent must be **at least 18 years of age and must possess the mental capacity to act.*

Key Characteristics

- **Flexible Configuration:** POA documents can be tailored to specific needs, such as a “Limited POA” for a set duration or a “Springing POA” that only becomes effective upon a specific condition.
- **Revocability and Amendment:** As long as the principal (the person granting the power) remains mentally competent, they may modify or revoke the POA at any time in writing. Such changes must be communicated to the agent and relevant institutions.
- **Legal Continuity:** In New Jersey and New York, a standard POA is typically terminated if the principal loses mental capacity. (varies by state)
 - To ensure continued authority after the principal becomes incapacitated, a “**Durable Power of Attorney**” clause must be explicitly included in the document.

Primary Authorities of the Agent

- **Financial and Legal Matters:** Managing bank accounts, buying/selling assets, filing tax returns, and executing legal contracts.
- **Healthcare and Medical Decisions (Medical POA):** Selecting treatment options, consenting to surgical procedures, and managing hospital admissions/discharges.
 - *In New Jersey, a Medical POA is formally known as a **Proxy Directive** (Durable Power of Attorney for Health Care) and is a component of an **Advance Directive**.*
- **Personal Care and Welfare:** Making decisions regarding daily residence and selecting personal care services.

- **Educational Decisions:** Acting on behalf of minor children or the principal regarding academic services and educational matters.
- **Travel and Logistics:** Planning travel, managing passports, and handling administrative tasks related to relocation.

Considerations for Drafting

- **Selection of Agent:** Since the agent is legally bound to act in your best interest, you must choose a highly trustworthy individual. The principal must be at least 18 years old and mentally competent at the time of signing.
- **Notarization:** In New Jersey, a **Financial POA** must be signed before a Notary Public to be legally enforceable. A **Proxy Directive** requires either two adult witnesses or notarization.
- **Expiration:** A POA expires immediately upon the death of the principal. Following death, asset management is governed by a **Last Will and Testament** or through the **Estate Probate Process**.

**New Jersey does not provide a standard official form for a Financial POA; professional legal consultation is recommended.*

Contact

Agency	Services Provided	Contact Information
NJ ADRC (Aging & Disability Resource Connection)	Service connection and counseling for seniors, individuals with disabilities (18 and above), and family caregivers by county.	1-877-222-3737 (Connects automatically to your local county office)
Bergen County Division of Senior Services	Information, assistance, and referrals for legal services.	One Bergen County Plaza, 2nd Floor, Hackensack, NJ 07601 201-336-7400 seniors@bergencountynj.gov
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LSNJ (Legal Services of New Jersey Hotline)	Statewide legal hotline providing free legal advice and information.	1-888-576-5529 lsnjlawhotline.org

Resources

- [NJ Department of Health — Official Proxy Directive \(Medical POA\) Form \(PDF\)](#)
- [NJ211.org — Legal Assistance Available to Seniors](#)

Reverse Immigration to Korea

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

Reverse Immigration refers to the process where overseas Koreans relocate their primary residence back to South Korea. Legally, this involves “Reporting Permanent Return” and “Restoration of Nationality.” It is crucial to understand the resulting changes in taxation, health insurance, and welfare benefits beforehand.

Strategic Considerations: Which Path is Best for You?

Scenario A: Maintaining Dual Citizenship

- **U.S. Pension Recipients:** Those who wish to receive stable Social Security (SSA) benefits while accessing Korea’s affordable, high-quality healthcare.
- **Parents with Children in the U.S.:** Those who may need to stay in the U.S. long-term at any time to visit family.
- **Asset Holders in the U.S.:** Those who need to manage or bequeath U.S. real estate or investment assets.

Scenario B: Formal Renunciation of U.S. Status

- **High-Net-Worth Individuals:** Those wishing to eliminate complex U.S. tax reporting obligations and the burden of double taxation (requires an **Expatriation Tax** assessment).
- **Individuals with No Intent to Return:** Those who, due to health or personal reasons, have no future plans to return to the U.S.
- **Seekers of Exclusive Domestic Benefits:** Specific situations where foreign citizenship may be a hurdle, such as entering public office or high-level national security roles.

Procedures

- **Report of Permanent Return:** A procedure for overseas Koreans returning with the intent to reside permanently in Korea, filed at the local immigration office.
- **Nationality Restoration (Ages 65+):** U.S. citizens aged 65 or older may enter Korea, apply for nationality restoration, and maintain dual citizenship by signing a “**Vow Not to Exercise Foreign Nationality**” within Korea.

- **Resident Card (F-4/Residence Report):** Used as a substitute for a Resident Registration Card for financial and medical transactions prior to nationality restoration or for long-term stays as a citizen of another country.

Areas of Review

Considerations

The following sections regarding the **Expatriation Tax, Final Tax Filing, Formal Renunciation Procedures**, and **South Korean Nationality Verification** apply specifically to **<Scenario 2: Renunciation of U.S. Status>**. For **<Scenario 1: Maintaining U.S. Citizenship>** and **<Scenario 3: Maintaining U.S. Permanent Residency>**, the sections on **Foreign Account Reporting (FBAR/FATCA)** and **Double Taxation Prevention** apply globally.

1. Taxation and Asset Management

- **Expatriation Tax (Exit Tax):** Citizens exceeding specific asset or income thresholds must evaluate their tax obligations before renouncing citizenship. You are considered a **"Covered Expatriate"** if you meet any of the following:
 - **Net Worth Test:** Global net worth of **\$2,000,000** or more at the time of expatriation.
 - **Tax Liability Test:** Your average annual net income tax for the 5 years prior exceeds the statutory threshold (**\$206,000 for 2025 / \$211,000 for 2026** — adjusted annually).
 - **Compliance Test:** Failure to certify that you have met all U.S. federal tax obligations for the 5 years preceding expatriation.
 - *Even if your assets are minimal, you must verify that you have filed for the past 5 years to avoid this status.*
- **Foreign Account Reporting (FBAR/FATCA):** If you retain U.S. citizenship or a Green Card after relocating to Korea, you must report your Korean financial accounts to the IRS annually.
 - **Pre-Renunciation:** Ensure all tax filings are up to date through the year of expatriation.
 - **Reporting Compliance:** *Continue FBAR and FATCA reporting through the official date of expatriation.*
- **Prevention of Double Taxation:** Professional consultation is essential to determine which country has taxing rights over your pension or rental income based on the **U.S.-Korea Tax Treaty**.

2. Final Tax Filing (The Year Following Renunciation)

- You must submit **Form 8854 (Initial and Annual Expatriation Statement)** to the IRS. Your tax relationship with the U.S. is not officially terminated until this form is filed.
 - *Failure to file Form 8854 may result in an **annual penalty of \$10,000** and ongoing U.S. tax obligations.*

3. Formal Renunciation Procedures

- **For U.S. Citizens:**

- Schedule an appointment with the **U.S. Embassy in Seoul** (or a consulate in the U.S.) for a renunciation interview. *Wait times may span several months.*
- The consular officer will verify that you fully comprehend the legal implications and consequences of renouncing.
- Perform the official **Oath of Renunciation** and receive your **Certificate of Loss of Nationality (CLN)**.
- **Fee: \$450** (Effective April 13, 2026 — *reduced from the previous \$2,350*).

- **For Green Card Holders (LPRs):**

- Complete **Form I-407 (Record of Abandonment of Lawful Permanent Resident Status)** and submit it to USCIS or the U.S. Embassy.
- This is processed via documentation without an interview; you must surrender your **Green Card** at this time.

4. Verification or Restoration of South Korean Nationality

- **If you already hold South Korean Nationality:** File for the reinstatement of your **Resident Registration** without additional procedures.
- **If South Korean Nationality was lost:** Apply for **Nationality Restoration** through the Korea Immigration Service or the **Overseas Koreans Agency (OKA)**. *Processing takes approximately 6 months to 1 year.*
- *Once completed, you can receive your **Resident ID Card** at a local community center and begin applying for health insurance and other welfare benefits.*

5. Health Insurance and Welfare Benefits

- **National Health Insurance (NHI):** You may become eligible after residing in Korea for a specific period (typically 6 months) or immediately upon reporting your **Permanent Return**.
- **Basic Pension and Senior Welfare:** After Resident Registration is complete, you may access senior benefits, such as the **Basic Pension** (subject to income tests) and free subway transit for seniors.

6. Receipt of Social Security Benefits

- Eligible individuals can continue to receive **U.S. Social Security** payments while residing in Korea.
 - However, since eligibility conditions may vary between citizens and Green Card holders, direct verification with the **Social Security Administration (SSA)** is required.

7. Relocation and Housing

- **Duty-Free Moving Goods:** If you have resided abroad for more than one year and import your household goods within 6 months of your return, you may be eligible for a **Customs Duty Exemption**. *Contact the **Korea Customs Service** (1544-1285 / customs.go.kr) for specific requirements.*

- **Document Authentication:** U.S.-issued documents (Birth/Marriage Certificates, FBI Background Checks) **must** be authenticated with an **Apostille** to be legally valid in Korea.
- **Green Card Maintenance:** If a Green Card holder stays in Korea long-term without a **Re-entry Permit**, they may be deemed to have abandoned their residency. Decide whether to apply for this permit before departure.
- **Administrative Gap:** Account for the “administrative gap” between surrendering your U.S. passport and receiving South Korean identification. Consolidate financial transactions and banking matters beforehand.

Comparison of Reverse Immigration Scenarios

Scenario 1. Maintaining U.S. Citizenship & Restoring South Korean Nationality (Dual Citizenship)

This applies to individuals who lost their South Korean nationality upon acquiring U.S. citizenship but now wish to restore their Korean nationality while retaining their U.S. status.

Key Procedures

1. Pre-Departure Preparation (3–6 months prior to relocation)

- **FBI Criminal Background Check:** Obtain your **Identity History Summary** from the FBI and secure an **Apostille** from the U.S. Department of State.
 - This document is generally valid for only 6 months.
- **Document Gathering:** Organize originals and copies of essential records, including your **Certificate of Naturalization** and Marriage Certificate.
- **Tax Review:** Consult with a CPA to understand that your **U.S. Tax Filing (IRS)** and **Foreign Account Reporting (FBAR/FATCA)** obligations continue even while residing in Korea.
- **U.S. Asset Management:** Decide whether to maintain U.S. bank accounts, dispose of real estate, and set up a mail forwarding service.
- **SSA International Payment Setup:** Research and plan for a Korean bank account (e.g., a foreign currency account) to receive your Social Security benefits.

2. Entry into Korea & Report of Nationality Loss

- **Entry on U.S. Passport:** You **must** use your U.S. passport to enter South Korea. (A visa-free entry typically allows for a 90-day stay).
- **Report of Nationality Loss:** This is an administrative step to formally record the loss of your Korean nationality that occurred when you naturalized in the U.S. Visit the local Immigration Office or the **Overseas Koreans Agency (OKA)** Service Center.
 - *The nationality restoration process cannot begin until your Family Relationship Register is updated to reflect this loss.*

3. F-4 Visa Issuance & Nationality Restoration Application (Ages 65+)

- **F-4 (Overseas Korean) Visa & Residence Report:** Apply for a **Domestic Residence Card** first. This card serves as your legal ID for financial and medical services during your long-term stay.
- **Application for Nationality Restoration:** Once the Residence Card is issued, submit your application for nationality restoration to the Immigration Office.
 - **Processing Time:** Typically 6 months to 1 year (varies by Ministry of Justice workload).
- **Notification of Approval:** Upon receiving the official approval notice from the Ministry of Justice, visit your local community service center.

4. Vow Not to Exercise Foreign Nationality & Resident Registration

- **The Non-Exercise Vow:** Within one year of approval, you must visit the Immigration Office to sign a formal declaration stating you will **not exercise your rights as a U.S. citizen** while within South Korean territory.
 - *Failure to do this within the one-year window may result in the cancellation of your nationality restoration.*
- **Resident ID Issuance:** Take your Vow Confirmation to a local community center to receive your **Resident Registration Card**. At this stage, you legally hold both ROK and U.S. passports.
- **Surrender of Residence Card:** Your old Residence Card must be returned. From this point forward, you must use your **Korean passport** for entry and exit in South Korea.

5. Post-Settlement Procedures

- **Health Insurance Enrollment:** Immediately after Resident Registration, visit the **National Health Insurance Service (NHIS)** to verify your eligibility.
- **Basic Pension Application:** If you are 65+ and meet the income criteria, apply at your local community center.
 - **2026 Criteria:** Monthly recognized income of 2.47 million KRW or less for single-person households. Standards are updated annually by the Ministry of Health and Welfare (check bokjiro.go.kr).
 - **Note:** U.S. Social Security benefits are included in income calculations.
- **Update SSA Records:** Update the **Social Security Administration (SSA)** with your new Korean address and banking information.

Pros	Cons
Dual Welfare Benefits: Concurrent access to Korea's National Health Insurance and senior perks (e.g., free subway transit) alongside U.S. Social Security (SSA) benefits.	Dual Tax Reporting: You remain obligated to file annual income tax returns with the U.S. IRS on your worldwide income while living in Korea.
Freedom of Residence: Ability to travel freely between the U.S. and Korea without visa concerns and the right to reside indefinitely in either country.	Financial Transaction Constraints: Due to FATCA (Foreign Account Tax Compliance Act) , you may face complex administrative hurdles when opening certain bank accounts or managing assets in Korea.
Full Property Rights: Full legal protection for acquiring real estate, managing inheritances, or gifting assets in both nations.	Non-Exercise Vow: You are legally prohibited from asserting your rights or protections as a U.S. citizen while in South Korea.

Scenario 2. Renouncing U.S. Status & Maintaining/Restoring South Korean Nationality

This applies to individuals who choose to fully relinquish their U.S. Citizenship or Legal Permanent Residency to settle exclusively under South Korean nationality.

Key Procedures

1. Tax Preparation (6 months to 1 year prior to departure)

- Consult with a CPA or tax professional to determine if you meet any of the three criteria for **"Covered Expatriate"** status. Meeting even one of these may trigger the **Expatriation Tax (Exit Tax)**:
 - **Net Worth Test:** Global net worth of **\$2,000,000** or more at the time of expatriation.
 - **Tax Liability Test:** Your average annual net income tax for the 5 years prior exceeds the statutory threshold (**\$206,000 for 2025 / \$211,000 for 2026** — *adjusted annually*).
 - **Compliance Test:** Failure to certify tax compliance for the 5 years preceding expatriation.
 - *This applies regardless of asset size. You must verify that you have fulfilled all filing obligations for the past 5 years.*

2. Complete Final U.S. Tax Filings (Prior to Renunciation)

- **FBAR/FATCA Reporting:** Maintain up-to-date filings for the year of expatriation. Ensure all **Foreign Bank Account Reports (FBAR)** and **FATCA** obligations are fulfilled up to the date of status relinquishment.

3. Formal Renunciation Process

- **For U.S. Citizens:**
 - Schedule an appointment for a renunciation interview at the **U.S. Embassy in Seoul** (or a consulate in the U.S.). *Wait times may span several months.*

- Participate in a consular interview to confirm you fully understand the legal implications and consequences of renouncing.
 - Perform the official **Oath of Renunciation** and receive your **Certificate of Loss of Nationality (CLN)**.
 - **Fee: \$450** (Effective April 13, 2026 — *reduced from \$2,350*).
- **For Green Card Holders (LPRs):**
 - Complete and submit **Form I-407 (Record of Abandonment of Lawful Permanent Resident Status)** to USCIS or the U.S. Embassy.
 - This process is handled via documentation without an interview; you must surrender your **Green Card** upon submission.

4. Final Tax Filing (The year following renunciation)

- Submit **Form 8854 (Initial and Annual Expatriation Statement)** to the IRS. This document is required to officially terminate your tax relationship with the United States.
 - *Failure to file Form 8854 may result in an **annual penalty of \$10,000** and the continuation of your U.S. tax obligations.*

5. Verification or Restoration of South Korean Nationality

- **If you already hold South Korean Nationality:** Apply for the reinstatement of your **Resident Registration** at a local community center.
- **If South Korean Nationality was previously lost:** Apply for **Nationality Restoration** through the Immigration Office or the **Overseas Koreans Agency (OKA)**.
 - *Approximately 6 months to 1 year (subject to Ministry of Justice workload).*
- Obtain your Resident Registration Card at your local community center and begin applications for National Health Insurance, the Basic Pension, and other senior welfare benefits.

Pros	Cons
Termination of Tax Obligations: You are no longer required to file income tax returns or FBAR/FATCA reports with the U.S. IRS.	U.S. Re-entry Restrictions: Future visits to the U.S. will require a visa-free waiver (ESTA) or a visitor visa, and your duration of stay will be limited.
Administrative Simplification: Financial and real estate transactions are streamlined as you only need to manage your identity as a South Korean citizen.	Expatriation Tax (Exit Tax): For high-net-worth individuals, the IRS may treat assets as if they were sold on the day of expatriation, resulting in significant tax liability.
Emotional Stability: Provides a sense of complete integration and belonging as a full member of South Korean society.	Irreversibility: Once U.S. citizenship is formally renounced, it is extremely difficult to regain in the future.

Scenario 3. Maintaining U.S. Permanent Residency & South Korean Nationality

This applies to U.S. Green Card holders who relocate to South Korea but wish to preserve their Lawful Permanent Resident (LPR) status.

Key Procedures

1. Application for a Re-entry Permit (Mandatory Before Departure)

- File **Form I-131 (Application for Travel Document)** with **USCIS** while you are still physically present in the United States.
- **Timing is Critical:** *You must apply **before** leaving the U.S.* It is highly recommended to remain in the country until your **Biometrics (fingerprinting)** appointment is completed, as the application cannot be initiated from abroad.
- **Validity:** A Re-entry Permit is typically valid for **2 years**. You must return to the U.S. within this period to maintain your Green Card status.
 - *Staying outside the U.S. for more than one year without a Re-entry Permit may lead to the automatic loss of your Permanent Resident status.*

2. Entry into Korea & Report of Permanent Return

- Enter South Korea with your valid Re-entry Permit and Green Card.
- File a **Report of Permanent Return** at the local Immigration Office or the **Overseas Koreans Agency (OKA)** Service Center.
- Upon filing, your administrative status changes from "Overseas Citizen" to "Resident," and your **Resident Registration** is restored.
 - **Identification:** *As a Green Card holder, you will be issued a **Resident Registration Card for Overseas Koreans** rather than a standard Resident ID. Please note that there may be slight variations in how certain administrative services are accessed with this card.*

3. Enrollment in Health Insurance and Welfare Benefits

- You are eligible to enroll in the **National Health Insurance (NHI)** program immediately upon the restoration of your Resident Registration.
- If you are aged 65 or older and meet the income threshold, you may apply for the **Basic Pension** at a local community center.
 - *U.S. Social Security (SSA) benefits are included in your total income calculation, so verify your eligibility for the Basic Pension in advance.*

4. Ongoing U.S. Tax Compliance

- As a Green Card holder, you maintain **U.S. Tax Resident** status even while living in Korea.
- You are required to report all worldwide income—including wages, rental income, and pensions earned in Korea—to the **IRS**.
- Foreign Account Reporting (FBAR/FATCA):

- **FBAR:** You must file **FinCEN Form 114** if the aggregate value of your Korean financial accounts exceeds **\$10,000** at any time during the year.
- **FATCA:** You must file **Form 8938** if you meet the specific asset thresholds.
 - *FATCA Standards for Residents in Korea (Single Filers):* Filing is required if assets exceed **\$200,000** at year-end (December 31st) or **\$300,000** at any point during the year. (These limits differ significantly from the \$50,000/\$75,000 thresholds for U.S. residents; consult a CPA for specific guidance based on your filing status).

5. Renewal of Re-entry Permit and Long-Term Strategy

- You must return to the United States to confirm your intent to maintain residency before your 2-year permit expires.
- If necessary, you may re-apply for a new **I-131 Re-entry Permit** while in the U.S.
 - Repeated renewals may lead USCIS to determine that you lack a **“bona fide intent to reside permanently”** in the U.S., which could result in the revocation of your Green Card. Long-term, it is more practical to either apply for **U.S. Naturalization** or formally abandon your **Green Card (Scenario 2)**.

Pros	Cons
Preservation of U.S. Residency: Serves as a reliable " Backup Plan " should you decide to return to the United States in the future.	Maintenance Hardships: You must renew your Re-entry Permit every two years, and frequent renewals carry the risk of status revocation.
South Korean Health Insurance: You gain immediate access to the South Korean National Health Insurance system through the Permanent Return report.	Ambiguous Administrative Status: In South Korea, you are classified as an "Overseas Citizen," requiring the use of a specialized Resident ID card rather than a standard one.
	Dual Tax Obligations: You remain burdened with reporting all income and foreign accounts (FBAR/FATCA) to the IRS while residing in Korea.

Key Agencies and Contacts

	Agency	Primary Services	Contact Information
South Korean Government Agencies	Overseas Koreans Agency (OKA)	Comprehensive guidance on permanent return, nationality restoration, and general services for overseas Koreans (Established 2023).	oka.go.kr +82-2-6747-0404
	Hi Korea (Immigration Service)	Ministry of Justice portal for domestic residence reporting, immigration control, visa information, and online applications.	hikorea.go.kr 1345 (Direct line within Korea)
	Consulate General of the Republic of Korea in New York	Reporting loss or restoration of nationality, Apostille authentication, and passport services.	overseas.mofa.go.kr/us-newyork-ko/index.do 646-674-6000
	National Health Insurance Service (NHIS)	Inquiries regarding health insurance eligibility and enrollment for overseas Koreans; verification of immediate coverage upon return.	nhis.or.kr +82-1577-1000
U.S. Federal Agencies	Social Security Administration (SSA)	Information on benefit eligibility for residents abroad and applications for international direct deposit to Korean accounts.	ssa.gov/foreign 800-772-1213
NJ Korean Community Resources	Korean American Association of New Jersey (KAANJ)	Established in 1975 as a 501(c)(3) nonprofit. Provides networking for the reverse immigration community, resettlement support, and local resource guidance.	21 Grand Ave. #216-B, Palisades Park, NJ 07650 201-945-9456 kaanj.org/

Senior Transportation Services

Senior transportation support provides safe and convenient mobility options for older adults at little to no cost. These services are designed to help seniors maintain independent living, prevent social isolation, and facilitate easier access to medical facilities.

Key Features

- **Door-to-Door Service**
 - **Customized Pickup:** Safe and convenient transit from your doorstep to your final destination.
 - **Personalized Assistance:** Drivers can provide physical support during boarding or operate wheelchair lifts upon request.
- **Cost-Effective Benefits**
 - **Complimentary Access:** Most county and municipal shuttles are free of charge or operate on a "Suggested Donation" basis.
 - **Affordable Rates:** Private non-profit transportation organizations provide specialized services at reduced costs.
- **Reservation-Based Systems**
 - **Scheduled Bookings:** Vehicles arrive and depart at pre-arranged times, requiring advance reservations.
 - **Monitoring Systems:** Select services offer real-time vehicle tracking, allowing family members or caregivers to monitor the transit process.

Major Transportation Services in Bergen County

Service Category	Bergen County Community Transportation	EZ Ride (Non-profit TMA)	NJ Transit (Public Transit)	Municipal Senior Shuttles
Eligibility	Seniors (60+), Adults with Disabilities (18+), and Veterans.	Adults 18 and older.	Reduced Fare: Seniors (62+). Access Link: Individuals with disabilities unable to use fixed-route transit.	Seniors residing in the specific municipality.
Service Description	Transportation to senior centers, medical facilities, supermarkets, etc.	Provision of Uber/Lyft rides and dedicated shuttle vehicles.	Statewide access via bus, rail, and light rail. Access Link provides paratransit services. *MicroLink service available (pilot test)	Local circulation within the town, including senior centers, hospitals, and shopping malls.
Cost	Free of charge.	Fee-based (at discounted rates).	Reduced Fare: 50% discount on bus/rail. Access Link: Fee-based (standard bus fare equivalent; Reduced Fare cards not accepted).	Free of charge (or suggested donation).
How to Access	Pre-registration and advance booking with the County Division of Transportation.	Pre-registration required. 201-939-4242 ext.4 *on-demand request available. *no reservation needed	Reduced Fare: Present Senior ID or Medicare Card Access Link: Mandatory advance reservation (973-491-4224)	Contact your local Town Hall for specific routes and schedules.
Contact	201-368-5955	201-939-4242	Reduced Fare: 973-275-5555	Fort Lee: 201-592-3500 ext.

Service Category	Bergen County Community Transportation	EZ Ride (Non-profit TMA)	NJ Transit (Public Transit)	Municipal Senior Shuttles
			Access Link: 973-491-4224	1518 Palisades Park: 201-585-4114 Teaneck: 201-837-7130 ext. 7040

***MicroLink**(from home to nearest bus stop) from NJ Transit is currently in a pilot service phase. Please refer to the [official page](#) for more details.

Considerations

- **Advance Reservations:** Most county and municipal services require booking at least a few days to two weeks in advance.
- **Service Boundaries:** County and local shuttles typically operate only within designated regional limits.
- **Companion Policies:** Please specify if you use a wheelchair or require a caregiver companion at the time of booking.

Resources

- [NJ DOT — Transportation Management Association \(TMA\) Directory](#)
- [NJ Transit — Reduced Fare Program for Seniors & Persons with Disabilities](#)
- [NJ.gov — Transportation Application Guide for Seniors & Disabled Individuals](#)
- [EZ Ride.org Bergen County Mobility Guide \(PDF\)](#)
- [NJ Transit Access Link — ADA Paratransit Service](#)
- [Bergen County Community Transportation Guide \(2025 PDF\)](#)
- [NJ 211 — Comprehensive Guide to NJ Transportation Services](#)

Will

This content is provided for informational purposes only. For specific legal advice, please consult with a qualified professional or an official government agency.

A **Will** is a legal document that formally outlines an individual's final wishes regarding the distribution of assets, guardianship of minor children, and the appointment of an executor after their passing. A Will helps prevent confusion and disputes among surviving family members, ensuring that the estate is managed according to the decedent's wishes rather than by court-mandated procedures.

Any adult who is at least 18 years of age and mentally competent (possessing testamentary capacity) may execute a Will.

Key Characteristics

- **Clarity of Asset Distribution:** You designate specific beneficiaries for all assets, including real estate, bank accounts, and personal belongings.
 - *New Jersey does not impose an Estate Tax, but an **Inheritance Tax** may apply. Immediate family members (Class A beneficiaries, such as spouses, children, parents, and grandchildren) are exempt.*
 - *However, Class C or D beneficiaries (siblings, friends, etc.) may be subject to taxation.*
 - * For specific legal advice consult a tax professional.*
- **Protection of Minors:** Protect your children's welfare by pre-designating a legal Guardian to care for them in the event of your passing.
- **Appointment of an Executor:** Designate a trusted individual or institution to faithfully carry out the terms and conditions of the Will.
- **Simplified Legal Procedures:** Without a Will, assets are distributed according to **Intestacy Laws**, and the court will oversee the estate administration at its discretion. With a Will, assets are distributed according to your wishes through the **Probate** process, overseen by the **Surrogate's Court**.

Primary Components

1. **Appointment of an Executor:** Designate a representative to manage the entire process, including funeral arrangements, tax payments, and the distribution of assets.
2. **Specification of Beneficiaries:** Clearly identify individuals or charitable organizations intended to receive your assets.
3. **Guardianship Provisions:** If you have children under 18, designate a guardian to provide care in your absence.

4. **Settlement of Debts and Taxes:** Include instructions on how to settle outstanding debts and pay any applicable inheritance taxes.
5. **Residuary Clause:** Determine the distribution of any remaining property not specifically mentioned elsewhere in the document.

Considerations

- **Holographic Wills:** A Will that is entirely handwritten and signed by the testator (Holographic Will) may be valid without witnesses, but it is generally discouraged due to the risk of legal errors.
- **Testamentary Capacity:** The testator must be of sound mind and fully understand the implications of the document at the time of signing.
- **Witnesses and Notarization:** Per New Jersey law, a Will must be signed by at least two adult witnesses.
 - *While notarization is not mandatory, executing a “**Self-Proving Will**” before a Notary Public simplifies the probate process by exempting witnesses from having to appear in court later.*
 - Beneficiaries may serve as witnesses, but it is highly recommended to use disinterested third parties to prevent potential disputes.
- **Continuous Updates:** You should revise your Will (via a **Codicil**) or draft a new one following major life events such as marriage, divorce, birth, or significant changes in assets.
- **Secure Storage:** Keep the original in a safe location and share its location with your executor. **The New Jersey Will Registry** offers a voluntary service to record the existence and location of your Will to prevent loss or disputes.
 - *Registration/Search Fee: \$10 each.*

Contact

Agency	Primary Role and Content	Contact Information
New Jersey Will Registry (Secretary of State)	Voluntary registration of the existence and storage location of a Will. *Registration/Search Fee: \$10 each	Mail: P.O. Box 300, Trenton, NJ 08625-0300 (Include \$10 check/money order) 609-777-2581 nj.gov/state/archives/catstate.html
Bergen County Surrogate's Court	Oversees legal procedures including Probate, estate administration, and guardianship appointments. *Guide materials available in Korean.	Two Bergen County Plaza, Suite 5000, Hackensack, NJ 07601 201-336-6700
NNJLS (Northeast New Jersey Legal Services)	Free legal aid for low-income residents, seniors, and the disabled (subject to eligibility).	190 Moore Street, Suite 100, Hackensack, NJ 07601 201-487-2166 NNJLS.ORG
LSNJ (Legal Services of New Jersey Hotline)	Free legal consultation and information provision.	1-888-576-5529 lsnjlawhotline.org

Resources

- [New Jersey Will Registry Information](#)
- [NIH.gov–Getting Your Affairs in Order Checklist](#)
- [NCOA \(National Council on Aging–Estate Planning Checklist & Guide](#)